

MEETING ABSTRACTS

Open Access



Proceedings of the 18th Annual Conference on the Science of Dissemination and Implementation in Health

Oxon Hill, MD, USA. 14-17 December 2025

Published: 20 May 2026

I1 Introduction

Gila Neta¹, David A. Chambers¹, Aaron Carroll²

¹National Cancer Institute, Rockville, MD, USA; ²AcademyHealth, Washington, DC, USA

Implementation Science 2026, 21(2):1

In the 18th year of the Annual Conference on the Science of Dissemination and Implementation in Health, co-hosted by the National Institutes of Health (NIH) and AcademyHealth, we embraced this milestone year of entering “adulthood” through the conference theme Realizing the Benefits of Dissemination & Implementation Science. This year’s theme addressed our existing and future efforts to maximize the benefits of D&I science through plenary sessions highlighting stories of impact and strategies to manage ongoing change.

This year’s conference had the largest turnout in the event’s history, with nearly 1600 registrants. In collaboration with our co-sponsor, the Patient Centered Outcomes Research Institute (PCORI), the conference convened researchers, practitioners, and other partners from across the United States and globally in National Harbor, MD on December 14–17, 2025.

Plenary sessions included two memorable keynote speakers from outside of the D&I research community. John Green, author of *Everything is Tuberculosis* and *The Fault in Our Stars*, opened the conference with a powerful story of impact in addressing the tuberculosis epidemic. Vic Strecher, Professor of Health Behavior and Health Equity at the University of Michigan School of Public Health, closed the second day with a thoughtful reflection on the value of purpose to guide dissemination and implementation strategies. New this year, we created an open call for submissions on impact of D&I Science to be included in a plenary session. The resulting panel, Realizing the Benefits of D&I Science: Stories of Impact, featured eight short stories describing how D&I science led to measurable and sustained health and service delivery improvements, economic or payment reforms, or policy change. Stories spanned a range of health interventions, settings, and populations, and explored different types of benefits that can be derived from D&I studies:

- Increasing the reach of effective addiction health services via an evidence-based implementation strategy (Becker)

- From Cath Labs to Rural Hospitals: Implementing the Relational Playbook to Advance Workforce Resilience in the VA (Gilmartin)
- From research to action: Harnessing implementation science to move the needle in reducing firearm injury and mortality in the United States (Hoskins)
- Integrating NCD and HIV Care in Nigeria: The MAP-IT Study as a Scalable Policy-Aligned Implementation Model (Ogedegbe)
- Dynamic Sustainment of Evidence-Based Health Promotion in Behavioral Health Settings (Aschbrenner)
- In church, people witness to save souls, in the Witness Project we witness to save lives! Impacting lives for 36 years (Erwin)
- Spreading Healthcare Access, Activities, Research and Knowledge (SHAARK): A Decade of Partnership between Implementation Scientists and Operational Leaders to Support the National VA Learning Health System (Jackson)
- Population Health Impact of Incorporating Lung Cancer Screening Education in State Tobacco Quitlines (Lowenstein)

Finally, the plenary panel, Safeguarding the Benefits of D&I Science: Strategies to Manage Ongoing Change, wrapped up the conference with a discussion exploring strategies to buffer from or address derailment of D&I studies. Speakers addressed ways to maximize design flexibility, capture context, and better anticipate and respond to unexpected events during the conduct of D&I studies.

Conference participants included researchers, practitioners, and other partners worldwide, including 11 low- and middle-income countries (LMICs) in sub-Saharan Africa, Latin America, and Asia. We were pleased to welcome 156 trainees, five patient scholarship recipients, and 13 presenters from LMICs, whose broad range of perspectives added greatly to conference discourse. We were able to award 11 travel scholarships to various participants globally and domestically included in the conference program.

In addition to plenary sessions, the conference included concurrent podium and poster sessions, discussion forums, and multiple networking events. The call for abstracts generated 1111 submissions, including individual paper presentations, posters, and panel presentations spread across nine thematic tracks. Those tracks provide an organizational structure for this supplement, which includes 166 abstracts from the concurrent paper and panel sessions, representing a variety of dissemination and implementation research funded by our conference



© The Author(s) 2026. **Open Access** This article is licensed under a Creative Commons Attribution-NonCommercial-NoDerivatives 4.0 International License, which permits any non-commercial use, sharing, distribution and reproduction in any medium or format, as long as you give appropriate credit to the original author(s) and the source, provide a link to the Creative Commons licence, and indicate if you modified the licensed material. You do not have permission under this licence to share adapted material derived from this article or parts of it. The images or other third party material in this article are included in the article's Creative Commons licence, unless indicated otherwise in a credit line to the material. If material is not included in the article's Creative Commons licence and your intended use is not permitted by statutory regulation or exceeds the permitted use, you will need to obtain permission directly from the copyright holder. To view a copy of this licence, visit <http://creativecommons.org/licenses/by-nc-nd/4.0/>.

sponsors and other agencies, organizations, and systems. The additional 739 abstracts from the poster sessions are not included here but can be viewed at <https://academyhealth.confex.com/academyhealth/2025di/meetingapp.cgi/ModulePosterSessions/0>.

Six pre-conference workshops, with 392 total registrants, featured topics including methods for implementation strategies optimization, artificial intelligence, implementation mapping, the use of CFIR, and planning for sustainment. NIH staff also hosted a pre-conference workshop on navigating D&I grant submissions. The conference also featured a pre-recorded orientation to D&I with live Q&A for those new to the field, an evening musical gathering based on attendee song requests, a “first timers” session, fun run, poster walks, meet the editors, ancillary meetings, and daily morning coffee chats with D&I experts facilitating open discussions about key priorities for the field. The networking sessions were hugely popular and well attended, allowing attendees to connect with the leaders in the field.

We look forward to welcoming attendees to the 19th Annual D&I Science conference on December 13-16, 2026, at the Gaylord Convention Center in National Harbor, MD.

Behavioral health

S1

Testing multi-level implementation strategies for two evidence-based autism interventions in children’s mental health services and schools: outcomes and mechanisms of a statewide hybrid type 3 trial

Lauren Brookman-Frazee¹, Aubyn Stahmer², Scott Roesch³, Anna Lau⁴, Elizabeth Rangel⁵

¹Implementation Science and Team Effectiveness in Practice (IN STEP) Children’s Mental Health Research Center, San Diego, USA; ²University of California Davis, Sacramento, CA, USA; ³San Diego State University, San Diego, CA, USA; ⁴University of California, Los Angeles, Los Angeles, CA, USA; ⁵SDSU/UC San Diego Joint Doctoral Program in Clinical Psychology, San Diego, CA, USA

Correspondence: Lauren Brookman-Frazee (lbrookman@health.ucsd.edu)
Implementation Science 2026, **21**(2):S1

Background: Children with autism are served in multiple public service systems. Evidence-based interventions (EBIs) have been developed, but are not routinely delivered. Identifying effective implementation strategies is critical to ensuring that EBIs reach families and children. The TEAMS project was a cluster randomized hybrid type 3 implementation-effectiveness trial testing the effects of two implementation strategies when paired with AIM HI (An Individualized Mental Health Intervention for Autism) in mental health programs (Study 1) and CPRT (Classroom Pivotal Response Teaching) in classrooms (Study 2). The TEAMS Leadership Institute (TLI) implementation strategy targeted implementation leadership and climate, and TEAMS Individualized Provider Strategy (TIPS) targeted provider motivation and engagement. This presentation reports the effects of the implementation strategies on implementation and child outcomes and on implementation mechanisms.

Methods: Fifty programs/districts across four training cohorts were enrolled. Organizations were randomized to receive a leader-level strategy, provider strategy, both strategies, or neither strategy (EBI provider training only). Leader and provider participants were recruited from enrolled programs/districts and child participants were recruited from providers’ caseloads or classrooms. Data from a total of 387 providers and 385 children were analyzed. Outcomes were assessed over 6 months. Provider outcomes included provider EBI certification and observed EBI fidelity. Clinical outcomes were standardized caregiver report measures (ECBI, PDD-BI). Mechanisms included the Implementation Leadership Scale, Implementation Climate Scale, and the Implementation Support Strategies scale, the Evidence-Based Practice Attitudes Scale, and Provider Motivation for Training Scale.

Findings: TLI was associated with significantly higher EBI fidelity compared to non-TLI ($B = 0.37, p = 0.04$). Moreover, a statistically significant TLIXTime interaction was found for child outcome T-scores ($B = -10.47, p = 0.03$), with a significant reduction in T-scores across time only for those in the TLI condition. TLI engaged implementation climate, but not implementation leadership. Additionally, TLI was associated with significantly greater use of implementation support strategies used by programs and school districts. TIPS did not engage the hypothesized mechanism of provider motivation for training, actual training engagement, or general EBP attitudes.

Implications for D&I Research: Findings support the effectiveness of leader focused strategies to promote implementation and clinical outcomes of autism EBIs in multiple public service systems and for multiple EBIs.

Primary Funding Source: National Institutes of Health

S2

Implementation under pressure: how motivation and context shape EBP delivery in community settings during a multisite learning collaborative

Sean Snyder, Michaela Harris, Vanessa Liang, Jessica Fitts, Robert Franks
The Baker Center for Children and Families, Boston, MA, USA

Correspondence: Sean Snyder (seanesnyder@gmail.com)

Implementation Science 2026, **21**(2):S2

Background

Implementation readiness and capacity are critical but often under-examined determinants of successful uptake of evidence-based practices (EBPs) in behavioral health settings. This study presents findings from a mixed-methods evaluation of a 16-month learning collaborative supporting five community mental health agencies implementing the Modular Approach to Therapy for Children with Anxiety, Depression, Trauma, or Conduct Problems (MATCH). The study aimed to examine changes in agency-level readiness and capacity over time and identify contextual factors shaping those trajectories.

Methods

A repeated-measures design was used to assess readiness and capacity at four time points using the Readiness Thinking Tool. These data were triangulated with field-based qualitative observations coded using the Consolidated Framework for Implementation Research (CFIR) observation tool, offering a dynamic view of contextual factors influencing implementation. We also analyzed site-specific implementation outcomes, including number of clinicians trained, EBP sessions delivered, and clients reached.

Findings

Across all five sites, general capacity declined over the implementation period. However, four sites showed increased motivation, suggesting sustained commitment despite resource challenges. End scores for capacity and motivation were mixed: three sites ended with below-average capacity, while two maintained higher levels; motivation was high at endpoint in three sites and low in two. The number of clinicians trained ranged from 8 to 19 per site, with average MATCH sessions delivered per clinician ranging from 39 to 130. Client exposure to the intervention ranged from 7 to 13 sessions on average. Even among lower-capacity sites, MATCH was implemented, though volume and consistency varied.

Qualitative findings revealed that motivation may buffer the negative impact of declining capacity. Sites with strong internal processes and team cohesion were more likely to sustain training and service delivery. Conversely, external instability and staff turnover disrupted implementation.

Implications for D&I Research: These findings underscore the need to tailor implementation support based on each site’s evolving readiness, internal dynamics, and external pressures. Motivation

emerged as a potential protective factor in sustaining implementation, even in the context of declining infrastructure. Monitoring both agency capacity and motivation over time, alongside contextual observations, may help inform run-time implementation strategies and improve implementation outcomes in real-world settings.

Primary Funding Source: National Institutes of Health

S3

Individualized quality improvement interventions catalyze improvements in women's behavioral healthcare delivery across primary care settings with varying behavioral healthcare infrastructure

Busra Ceviren¹, Melissa Leonard¹, Seuli Bose Brill², Rachel Gordon², Lisa Christian², Bethany Panchal², Colleen Waickman², Alicia Rooney¹, Abigail Ezzeldin¹, Wendy Groznik¹

¹The Ohio Colleges of Medicine Government Resource Center, Columbus, OH, USA; ²The Ohio State University Wexner Medical Center, Columbus, OH, USA

Correspondence: Busra Ceviren (busra.ceviren@osumc.edu)

Implementation Science 2026, **21**(2):S3

Background

Focus on ME: Mental Health is Essential Health aims to improve care related to anxiety and depression among women of reproductive age (WRA) in primary care (PC) settings across Ohio through a statewide quality improvement (QI) learning collaborative. The collaborative evaluated the differential effect of behavioral health (BH) evidence-based quality improvement (EBQI) between statewide PC practices with and without integrated BH.

Methods

The Focus on ME collaborative, using Institute for Healthcare Improvement methodology, engaged multidisciplinary PC teams in structured QI activities to enhance anxiety and depression care for women aged 18–44. Twenty PC sites participated in phase 2, receiving an evidence-based toolkit developed by experts with a workflow for screening and recommended interventions based on symptom severity. QI activities included site-level QI coaching informed by an interactive data dashboard to guide intervention testing and monthly webinars with peer learning.

Findings

Of the twenty participating sites, 9 had integrated BH services and 11 did not. The project included 17,641 unique patients across baseline and implementation periods. Of these, 43.8% were Medicaid beneficiaries, 23.4% non-Hispanic Black, 47.3% non-Hispanic White, and 16.5% Hispanic/Latina.

During baseline, sites without integrated BH care had significantly lower anxiety and depression screening and appropriate care rates compared to those with integrated BH care ($p < 0.001$). In the implementation period, screening rates improved at both site types, with greater gains at sites without integrated care (24.2% for anxiety, 7.8% for depression) than integrated care sites (13.5% and 2.4%). The initiative helped close the gap by raising screening and care rates at non-integrated sites to levels comparable with integrated care sites.

Implications for D&I Research

Integrated BH in primary care remains an evidence-based model, but most U.S. practices have not adopted it due to policy, payment, and workforce barriers. Findings suggest that context-tailored QI methodology can improve BH screening and treatment across sites with varied capacity. For certain measures, this approach can help practices with limited integration approximate metrics achieved by fully integrated environments. Personalizing QI interventions to local context can catalyze change across diverse practices.

Primary Funding Source: Ohio Department of Medicaid and Ohio Department of Children and Youth

S4

Comparing the impacts of a base versus enhanced implementation bundle on fidelity to a complex evidence-based practice

Sonya Gabrielian^{1,2}, Kristina Cordasco^{2,3}, Camilla Cummings², Gracielle Tan⁴, Maharshi Rawal², Lisa Edwards⁵, Edwin Jacobo⁶, Erin Finley^{5,7}

¹University of California, Los Angeles David Geffen School of Medicine, Los Angeles, CA, USA; ²Veterans Health Administration, Los Angeles, CA, USA; ³David Geffen School of Medicine, University of California, Los Angeles, CA, USA; ⁴Central Occupational Medicine Providers, Los Angeles, CA, USA; ⁵VA Greater Los Angeles Healthcare System, Center for the Study of Healthcare Innovation, Implementation and Policy (CSHIIP), Los Angeles, CA, USA; ⁶UCLA/VA Center of Excellence on Veteran Resilience and Recovery, Los Angeles, CA, USA; ⁷Long School of Medicine, University of Texas Health Science Center, San Antonio, TX, USA

Correspondence: Sonya Gabrielian (sonya.gabrielian@va.gov)

Implementation Science 2026, **21**(2):S4

Background

Implementation of Critical Time Intervention (CTI)—a complex, evidence-based, and time-limited case management practice for persons with behavioral health disorders—can substantively improve housing stability among homelessness-experienced adults. We conducted a hybrid type 3 implementation-effectiveness trial to compare two strategies for supporting CTI implementation in 32 community-based agencies that serve homeless-experienced Veterans. As fidelity to CTI influences its effectiveness, we examined the impacts of these strategies on CTI fidelity.

Methods

We used the Replicating Effective Programs (REP) implementation bundle to support all 32 agencies to achieve fidelity to CTI, while accommodating agency-level adaptations to fit local contexts. Half ($n = 16$) the agencies also received 9 months of external facilitation (“enhanced REP [E-REP]”), i.e., tailored supports to adopt CTI in routine care processes. To compare the impacts of REP versus E-REP on CTI fidelity, we measured agency-level CTI fidelity 12 months after REP initiation. Specifically, we assessed adherence to CTI's core components over the prior 6 months by integrating data from: a CTI implementation self-assessment completed by implementing providers; 90-min videoconferences with providers to review clinical documentation of two “exemplar cases”; and field notes from provider narratives during these videoconferences. Each agency was classified as having adequate versus inadequate CTI fidelity (adherent to ≥ 5 of 9 core CTI components).

Findings

Over half (59.4%, 19/32) of agencies agreed to participate in fidelity assessments. Of these agencies, 10 (52.6%) received REP and 9 (47.3%) received E-REP. We were unable to assess fidelity for 1 of the 9 E-REP agencies due to recent provider turnover and inability to review exemplar cases. Agencies that received REP alone were less likely than agencies that received E-REP (50.0%, 5/10; 87.5%, 7/8, respectively) to have adequate CTI fidelity.

Implications for D&I Research

Strategies that initiate deliberate and interactive problem-solving to support evidence-based practice implementation (e.g., external facilitation) may improve practice fidelity and ultimately patient outcomes. In this study, though E-REP was a higher intensity approach to support CTI implementation, external facilitation may have improved practice fidelity by catalyzing provider and organizational changes. Tailored and flexible supports may be useful to address site-level challenges in implementing complex behavioral health interventions with high fidelity.

Primary Funding Source: Department of Veterans Affairs

S5

Implementing Written Exposure Therapy (WET) for PTSD in underserved primary care settings: a matrixed multiple case studyAriella Korn¹, Chloe Gomez², Andrea Cassells³, Tameir Holder³, T.J. Lin³, Mishael Lopez³, Jonathan Tobin^{3,4}, Stephanie Williamson², Eunice Wong², Brian Marx^{5,6}, Denise Sloan^{5,6}, Lisa Meredith²¹RAND, Boston, MA, USA; ²RAND, Santa Monica, CA, USA; ³Clinical Directors Network, Inc. (CDN), New York, NY, USA; ⁴The Rockefeller University Center for Clinical and Translational Science, New York, NY, USA; ⁵Veterans Health Administration, Boston, MA, USA; ⁶Boston University School of Medicine, Boston, MA, USA**Correspondence:** Ariella Korn (akorn@rand.org)*Implementation Science* 2026, **21**(2):S5

Background: Access to evidence-based treatment for PTSD in underserved primary care settings needs improvement. Written Exposure Therapy (WET) is a promising evidence-based treatment because it is brief (six weekly sessions), easy for therapists to learn and deliver with high fidelity, and is well-tolerated by patients. However, understanding WET implementation in primary care is limited. We examined variation in the implementation of WET for PTSD in Federally Qualified Health Centers (FQHCs) within the context of collaborative care, a team-based and measurement-based health care delivery model.

Methods: We used a multiple case study approach with data from the Building Experience for Treating Trauma and Enhancing Resilience (BETTER) study: a hybrid type 1 cluster RCT that randomized 12 FQHCs to collaborative care alone or collaborative care plus access to WET. Patients ($n=722$) were enrolled in three waves between 2022–2025. Guided by RE-AIM and CFIR, we analyzed data from the study's care management registry, surveys, and interviews to identify patterns in implementation success across the six WET sites.

Findings: Patient WET completion rates were low-to-moderate across sites (range 19–42%), with no differences by delivery mode (telehealth, in-person, hybrid). The site with greatest treatment completion had highest scores for health center leadership engagement. The FQHC with lowest treatment completion had low scores for learning climate, prioritization of patient needs, and available resources, relative to other sites. Median treatment duration was similar across five of six sites (range 6.3–7.1 weeks), reflecting high fidelity to WET protocol timeframes. The sixth FQHC, with longer treatment duration (median 8 weeks), had the fewest therapists and mostly delivered WET in person. Across sites, patients who completed WET had clinically significant reductions in PTSD symptom severity from baseline to after the last session, as assessed during treatment with the PTSD Checklist for DSM-5 (PCL-5). Staff reported implementation barriers at the patient-level (e.g., unmet social needs), provider-level (e.g., discomfort delivering exposure-based therapy), and systems-level (e.g., limited resources, staffing capacity).

Implications for D&I Research: Findings suggest that WET implementation success within FQHC collaborative care is contingent on patient engagement throughout the course of treatment. Effective strategies are needed to mitigate multi-level barriers to PTSD treatment among the primary care population.

Primary Funding Source: National Institutes of Health

S6

SAFER: development and initial implementation of a patient decision aid for firearms lethal means safety planningJeffrey Smith¹, Michelle Mengeling², Brian Cook³, James Torner³, Mark Vander Weg³, Alison Hamilton⁴, Jonathan Platt³, Amanda Heeren⁵, Lindsey Fuhrmeister⁵, Anne Sadler⁵¹Veterans Health Administration, Little Rock, AR, USA; ²CADRE (152), Suite 42–260, Veterans Health Administration, Iowa City, IA, USA; ³University of Iowa, Iowa City, IA, USA; ⁴Veterans Health Administration, Los Angeles, CA, USA; ⁵Veterans Health Administration, Iowa City, IA, USA**Correspondence:** Jeffrey Smith (Jeffrey.Smith6@va.gov)*Implementation Science* 2026, **21**(2):S6

Background: Firearms are the leading method of suicide death in women Veterans, and the firearm suicide rate among women Veterans is nearly 1.5 times higher than that of non-Veteran women. Development and implementation of interventions tailored to reduce firearm suicide risk among women Veterans are under-studied. This presentation describes development and initial implementation of a patient decision aid (PtDA) to support firearms lethal means safety planning (an evidence-based suicide prevention strategy) for women Veterans.

Methods: Utilizing a 3-phase, iterative co-design process involving 60 women Veterans and 26 providers, we developed a PtDA and complementary resources (toolkit, implementation playbooks) to promote safe firearm storage and suicide prevention. The tools were designed to provide Veterans with information tailored to women to support informed safety choices and to facilitate Shared Decision-Making (SDM) between Veterans and providers. Mixed-methods survey data on the PtDA's acceptability and utility were collected from Veterans and providers. Women Veterans living in rural areas were over-sampled as they represent a high-risk population for firearm suicide.

Findings: Women Veterans reported high satisfaction with the PtDA's information, tone, and preventive approach. Most (70%) indicated that SDM with a provider made them more likely to identify and act on a safety plan than if they had received the PtDA alone. Nearly all (95%) agreed that providers should routinely use SDM when discussing firearm safety and suicide risk. All would recommend SDM to other Veterans, and 70% indicated their safety plan made them feel safer. Among providers, all reported that the PtDA improves women Veterans' knowledge about firearm suicide risk, and three-fourths reported the PtDA decreases barriers for providers in discussing firearms and suicide risk with women Veterans.

Implications for D&I Research: D&I science emphasizes the importance of end-user engagement. This work produced a women Veteran-tailored PtDA for firearms safety planning that Veterans and providers find usable and beneficial, demonstrating feasibility for framing/structuring firearm suicide risk discussions and safety planning in ways that are acceptable to both Veterans and providers. Further work is needed to scale-up use of the PtDA to assess its potential for broader impact in promoting firearms lethal means safety planning.

Primary Funding Source: Department of Veterans Affairs

S7

Evaluating the adoption, sustainment, and clinical effectiveness of a brief counseling intervention for patients experiencing intimate partner violence in VHAKatherine Iverson¹, Kelly Stolzmann¹, Tara Brunswick¹, Erin Mangan¹, Abigail Krout², LeAnn Bruce², Julianne Brady¹¹Veterans Health Administration, Boston, MA, USA; ²Veterans Health Administration, Washington, DC, USA**Correspondence:** Katherine Iverson (katherine.iverson@va.gov)*Implementation Science* 2026, **21**(2):S7

Background: Routine screening for intimate partner violence (IPV) is required for reproductive-aged women and is recommended for other patient groups in the Veterans Health Administration (VHA). VHA facilities are also required to offer trauma-informed services for patients experiencing IPV. Recovering from IPV through Strengths and Empowerment (RISE) is an evidence-based brief counseling intervention for individuals experiencing IPV that is being implemented through a national training and implementation support program in VHA.

Methods: An IRB-approved program evaluation of VHA's national implementation of RISE is ongoing. The program employs a train-the-trainer model with implementation support via external facilitation. Training consists of a one-day virtual training and six months of clinical consultation with a regional trainer, during which time clinicians complete three cases with fidelity to become certified. Drop-in clinical consultation, one-on-one implementation support meetings, and advanced training workshops with CEUs are offered routinely by the training and implementation support program. During certification, clinicians complete a program evaluation summary following each

case to report patient outcomes. An annual survey is distributed to certified clinicians (89% response rate) to assess the use of RISE over time and barriers/facilitators to implementation. We used these program evaluation data sources to examine the adoption and sustainment of RISE (June 2021–September 2024), as well as health outcomes of paramount importance to patients experiencing IPV: self-efficacy (GSS) and depression (CES-D).

Findings: 415 clinicians have been certified in RISE, with 90% of VHA Medical Centers having at least 1 certified clinician onsite. The majority (93%) of clinicians report sustained delivery of RISE at least one year post-certification. Additionally, RISE improves patient outcomes in routine care, with significant and clinically meaningful improvements in self-efficacy [$t(704) = 19.44$, $p < 0.0001$, $d = 0.73$] and depressive symptoms [$t(582) = -15.02$, $p < 0.0001$, $d = 0.62$] from the first to the last RISE session. Program evaluation data indicate that witnessing the positive impacts of RISE on patient well-being, along with regular use of implementation supports (e.g., attending drop-in consultation), facilitates sustainment.

Implications for D&I Research: Developing a strong, multifaceted training program for an IPV-focused intervention with ongoing implementation support for clinicians can achieve adoption and sustainment of RISE, resulting in improved outcomes for individuals experiencing IPV.

Primary Funding Source: Department of Veterans Affairs

S8

Predictors of workforce turnover in wraparound care coordination: implications for implementation

Jonathan Olson¹, Tony Bonadio², Heidi Melz³, Jessica Lim², Grace Metz²

¹University of Washington, Seattle, WA, USA; ²University of Connecticut, Hartford, CT, USA; ³James Bell Associates, Arlington, VA, USA

Correspondence: Jonathan Olson (jro10@uw.edu)

Implementation Science 2026, 21(2):S8

Background

Current implementation science models underscore the importance of a trained and dedicated workforce to support the implementation of evidence-based practices within behavioral health systems. However, recent data suggest that turnover rates within this group are high, averaging over 30% across different segments of the workforce. Such turnover results in significant challenges within provider organizations that can lead to poor implementation outcomes and decreased service availability.

Methods

We examined workforce turnover within provider organizations that implement Wraparound Care Coordination, an intensive intervention for youth with complex behavioral health needs. We surveyed 210 care coordinators and 67 administrators from 65 provider agencies within 12 states in the United States. Questions focused on organization-level turnover rates, perceived predictors of turnover, organizational climate, and impact of turnover on practice outcomes.

Findings

Results suggest an average turnover rate of 30% among Wraparound providers, with about a third of current employees reporting that they are actively considering new employment. Approximately 30% of administrators reported that their agencies are chronically understaffed, and that this has had a negative impact on workload, administrative costs, morale, and quality of services. Direct service providers reported that commitment to the families they serve and dedication to Wraparound's mission were more important reasons for staying in their jobs than salary, even though average salaries for this group were low ($M = \$39,000$). Results of a hierarchical linear model were consistent with such reports. Statistically significant individual-level predictors of turnover behaviors included perceptions of an unsupportive organizational climate ($b = -0.324$, $t(108) = -3.66$, $p < 0.001$) and job-related stress ($b = 0.289$, $t(108) = 4.63$, $p < 0.001$); and a significant

organization-level predictor was being understaffed ($b = -0.340$, $t(36) = -2.93$, $p = 0.006$). Adjusted salary was unrelated to the outcome ($b = 0.00$, $t(40) = -1.04$, $p = 0.304$).

Implications for D&I Research

Together, these findings suggest that turnover rates among Wraparound providers are similar to those in other behavioral health fields. These high rates have a negative impact on staffing, morale, and service provision. Characteristics of the organization and job stress appear to be more important predictors of turnover than salary. Such findings underscore the importance of providing a supportive workplace to attract and retain a talented workforce.

S9

Implementation of a peer-support suicide prevention program in the United States Air Force

William Bevens^{1,2}, Nicole Stadnick^{1,2,3}, Gregory Aarons^{1,2,3}, Bryan Yates⁴, Anthony Pisani⁴, Peter Wyman⁴

¹University of California, San Diego, La Jolla, CA, USA; ²Child and Adolescent Services Research Center, San Diego, CA, USA; ³University of California, San Diego ACTRI Dissemination and Implementation Science Center, La Jolla, CA, USA; ⁴University of Rochester Medical Center, Rochester, NY, USA

Correspondence: William Bevens (wbevens@health.ucsd.edu)

Implementation Science 2026, 21(2):S9

Background: In response to increasing military suicides, the Wingman Guardian Connect (WGC) program was developed through collaboration between the US Department of the Air Force (DAF) and University of Rochester (UR). WGC is a group network enhancement suicide prevention model where facilitators teach Air Force personnel skills for managing career challenges while building group social bonds and cohesion. In preparation for an Air Force-wide scale-up, WGC is being evaluated in an implementation-effectiveness type 1 randomized controlled trial in nine US bases that involves each base transitioning to independence from direct UR oversight. This study explored barriers and facilitators to implementation using the Exploration, Preparation, Implementation, Sustainment (EPIS) framework, and describes the role of UR as a purveyor in supporting base-transitions.

Methods: Periodic reflections were conducted for each base at implementation launch with UR personnel who co-facilitated WGC delivery. Focus group interviews were completed across nine enrolled bases with two groups: 1) enlisted and civilian facilitators; 2) staff and leadership within Integrated Primary Prevention Workforce offices (IPPWs) responsible for managing WGC implementation and sustainment. Interview guides were developed based on EPIS and rapid qualitative analysis was used for analysis. Outer context was defined as departments within the DAF and the inner context were individual bases (base leadership and IPPWs).

Findings: Inner context leadership supported successful transitions through the recruitment and training of local facilitators and promoting schedule flexibility for Air Force personnel to participate in training. IPPWs who reported supportive inner context leadership also described greater independence to enact their mission and demonstrated proactive recruitment, delivery, and expansion of WGC. As purveyors, UR acted as a relational tie by building leadership support in the inner context, creating mission-alignment between outer and inner contexts, and supporting the de-implementation of prior resiliency programs.

Implications for D&I Research: This work described the functions, forms and importance of an academic bridging organization's activities to engage outer context systems and facilitate their interactions with inner context actors in this multilevel military context. Several factors that influenced implementation process through purveyor relationships were identified. Data collection for this work is expected to be completed by late September 2025.

Primary Funding Source: National Institutes of Health

S10

Seven randomized trials support the effectiveness of the Leadership and Organizational Change for Implementation (LOCI) strategy in diverse national and international contexts

Gregory Aarons^{1,2,3,4,5}, Karina Egeland⁶, Ane-Marthe Skar^{6,7}, Mark Ehrhart⁸, Marisa Sklar^{1,5,9,10}, Nathaniel Williams¹¹, Lauren Brookman-Frazee^{3,12}, Aubyn Stahmer^{13,14}

¹Implementation Science and Team Effectiveness in Practice (IN STEP) Children's Mental Health Research Center, San Diego, CA, USA; ²Implementation Science, London, United Kingdom; ³Child and Adolescent Services Research Center, San Diego, CA, USA; ⁴UC San Diego, La Jolla, CA, USA; ⁵UC San Diego Altman Clinical and Translational Research Institute Dissemination and Implementation Science Center, La Jolla, CA, USA; ⁶Norwegian Center for Violence and Traumatic Stress Studies, Oslo, Norway; ⁷Norwegian Institute of Public Health, Oslo, Norway; ⁸University of Central Florida, Orlando, FL, USA; ⁹UC San Diego Child and Adolescent Services Research Center, San Diego, CA, USA; ¹⁰University of California, San Diego, La Jolla, CA, USA; ¹¹Boise State University, Boise, ID, USA; ¹²University of California San Diego, La Jolla, CA, USA; ¹³University of California Davis, Sacramento, CA, USA; ¹⁴Mind Institute, Sacramento, CA, USA

Correspondence: Gregory Aarons (gaarons@health.ucsd.edu)

Implementation Science 2026, **21**(2):S10

Background

There is a need for building research evidence for implementation strategies. However, many implementation strategies have not been tested and most lack replications. This presentation will describe multiple replications and adaptations of the Leadership and Organizational Change for Implementation (LOCI) strategy in different contexts and countries. Studies took place in diverse contexts, including community mental health clinics, schools, substance use disorder treatment facilities (multiple US states including California, Arizona, Oregon, Idaho, Nevada), mental health clinics within health trusts (Norway).

Methods

Five randomized trials including cluster randomized and stepped-wedge designs were used in these studies. The LOCI strategy was adapted and tested and provided in some projects by the original LOCI development team (Aarons, Ehrhart, Sklar) and in other studies were translated and delivered by local teams in Norway (Egeland, Skar) that adapted the strategy for language and cultural relevance, and for autism intervention implementation in community mental health and schools (Brookman-Frazee, Stahmer) using a team-based approach with select core components of the strategy. The EBPs implemented included trauma focused treatment for posttraumatic stress disorder (PTSD; Cognitive Therapy for PTSD, Eye Movement Desensitization and Reprocessing (EMDR), Trauma-Focused Cognitive Behavioral Therapy (TFCBT), Motivational Interviewing, Measurement Based Care, Classroom Pivotal Response Training (CPRT), and Individualized Mental Health Intervention for Autism (AIM HI).

Findings

All five studies found positive effects of the LOCI strategy on implementation determinants, mechanisms, and outcomes including implementation leadership, implementation climate, and evidence-based practice reach. Three of the trials also tested clinical outcomes and demonstrated improvement, including reduced problem behaviors and/or improvement in symptoms among youths.

Implications for D&I Research

Assessment of implementation strategy adaptation and replication adds to the rigor of implementation science and advances understanding of core components and nuances of strategies. Effective implementation leadership is critical in creating a positive implementation climate and subsequent implementation outcomes, including reach, fidelity, and patient outcomes. Considering multiple studies in different national and international system and organizational contexts and ways of delivering LOCI advances the science of implementation. Even with these differences, five randomized trials in different service settings in the US and Norway all found positive outcomes.

Primary Funding Source: National Institutes of Health

S11

Readiness assessment as a D&I strategy: informing universal depression screening implementation in schools

Cynthia Blitz¹, David Amiel¹, Itzhak Yanovitzky²

¹Rutgers University, Somerset, NJ, USA; ²Rutgers University School of Communication & Information, New Brunswick, NJ, USA

Correspondence: David Amiel (david.amiel@rutgers.edu)

Implementation Science 2026, **21**(2):S11

Background: Adolescent depression rates have increased dramatically over the past decade, yet screening rates remain low even in schools, where near-universal screening rates can be achieved. In New Jersey, ten school districts were selected to pilot a program designed to implement universal depression screening for adolescents in grades 7–12, with parent consent. In line with D&I principles, we conducted a readiness assessment to understand the acceptability, feasibility, barriers, and facilitators for universal adolescent depression screening prior to the state's rollout.

Methods: This mixed-methods study surveyed school-based mental health professionals ($n = 70$) and interviewed key informants ($n = 15$). The survey assessed current practices, perceived feasibility, acceptability, and anticipated barriers; results were analyzed using descriptive statistics, chi-square tests to examine differences across practitioner sub-groups, and a logistic regression model to predict practitioner readiness. Interviews explored implementation requirements and necessary supports and were analyzed using a six-step thematic analysis.

Findings: While 62% of practitioners perceived adolescent depression as common in their schools, fewer than 5% reported routine screening practices. Practitioners strongly endorsed universal screening as feasible (87%), effective for identifying depression (86%), and research-based (82%). However, they anticipated significant barriers: limited capacity for follow-up services (63%), insufficient staffing and resources (61%), lack of standardized instruments (61%), and challenges providing real-time intervention (60%). Fewer than 40% expected parent support. Interviews revealed critical needs for explicit procedural guidance, dedicated funding, staff training, clarity on responsibilities following positive screens, and strategies for engaging parents. Practitioners emphasized that successful implementation would require comprehensive infrastructure support beyond screening mandates.

Implications for D&I Research: This study demonstrates how pre-implementation assessments can maximize the impact of evidence-based interventions by proactively identifying key implementation considerations. Findings provide actionable intelligence for policymakers to address challenges and enable implementers to benefit from practitioner-informed program design. Future D&I research examining if and how such assessments ultimately influence implementation success and sustainability is promising and is underway within this initiative. Such assessments represent a valuable D&I strategy for complex and system-level mental health interventions, enabling proactive resource planning and stakeholder engagement to realize the full public health impact of evidence-based practices.

Primary Funding Source: William T. Grant Foundation

S12

Characterizing outer contexts in behavioral health using policy mapping

Genevieve Graaf¹, Mimi Choy-Brown², Abigail Ross³

¹University of Texas at Arlington, Arlington, TX, USA; ²University of Minnesota, St. Paul, MN, USA; ³University of Pennsylvania, School of Social Policy and Practice, Philadelphia, PA, USA

Correspondence: Genevieve Graaf (Genevieve.graaf@uta.edu)

Implementation Science 2026, **21**(2):S12

Background: Outer context implementation drivers are underspecified in behavioral health settings, yet have significant impacts on uptake and sustainment for empirically-supported interventions (ESIs). The service context of home and community-based services (HCBS) for children and youth (CY) with behavioral health needs involves complex coordination and financing across multiple public sectors, professionals, and service settings—contributing to low uptake of ESIs. Leveraging existing policy and financial infrastructure may increase uptake and sustainability of ESIs. Policy characteristics are enumerated in public documents, but challenges arise in interpreting, codifying, and tracking these policies over time. This study describes the use of policy mapping to systematically identify and describe outer context features that shape behavioral HCBS for CY: 1) financed services 2) minimum provider qualifications, and 3) authorized service settings.

Methods: This longitudinal policy mapping study focused on HCBS Medicaid waiver programs for CY with behavioral health needs. Data was collected from approved state applications filed between 2006 and 2023 ($N=138$ documents; $K=19$ states). A data extraction tool was developed, piloted, and refined after a line-by-line review of each document. Multi-stage focused coding was guided by two established taxonomies. Matrices organized data by state and year to aid comparison.

Findings: Waiver-funded services and authorized settings varied across states ($n=19$) and over time while provider qualifications for each type of service did not. Services offered included case management ($n=19$), respite care ($n=19$), independent living training ($n=9$), wraparound care coordination ($n=5$), and crisis response ($n=5$). Provider qualifications: Most services had minimum education requirements. Only 6 services (11% of total services authorized), required a graduate degree. Providers without a graduate degree often required supervision by providers with Master's degrees. Settings: Most services (83% in 8 states) were authorized for delivery in home, community, or vocational settings.

Implications for D&I Research: Policy mapping can facilitate maximum task-technology fit by characterizing structural and financial features of service environments to assist in matching ESIs to real-world contexts. ESI adaptations can aid implementation where ESIs are incongruent with policy and service contexts. For example, findings highlight the potential for scaling EBIs through task-shifting to non-clinicians working in community settings.

Primary Funding Source: National Institutes of Health

S13

Mental health organizations' perceived needs for implementation science-based support to prepare for, implement, and sustain effective practices

Elizabeth Walker¹, Denise Williams², David Katz³, Robin McGee¹, Sean Karyczak⁴, Annie Lenoir⁵, Olga Vitruk⁶, Lydia Chwastiak⁶, Ricardo Canelo², Heather Gotham²

¹Rollins School of Public Health, Emory University, Atlanta, GA, USA;

²Stanford University School of Medicine, Palo Alto, CA, USA; ³Department of Global Health, University of Washington, Seattle, WA, USA; ⁴Department of Psychiatric Rehabilitation and Counseling Professions at Rutgers, The State University of New Jersey, Piscataway, NJ, USA; ⁵Texas Institute for Excellence in Mental Health at The University of Texas at Austin's Steve Hicks School of Social Work, Austin, TX, USA; ⁶Department of Psychiatry and Behavioral Sciences, University of Washington, Seattle, WA, USA

Correspondence: Heather Gotham (gothamh@stanford.edu)

Implementation Science 2026, 21(2):S13

Background: Although implementation science can accelerate mental health practice change, it has not historically been the basis for federally-funded technical assistance (TA) centers. Launched in October 2024, the federally-funded Center for Mental Health Implementation Support (CMHIS) provides TA to community-based organizations and agencies in all US states and territories. We provide implementation science-based support and resources structured around selection, implementation, and sustainment of effective mental health practices.

We conducted feedback sessions on the needs and priorities of our audiences related to implementing services to inform the design of CMHIS's support and resources.

Methods: We conducted 17 virtual qualitative discussion groups with participants from state and territory agencies, direct service organizations, and TA Centers. We designed the semi-structured discussion guides by mapping CMHIS's eight core topics (community engagement, contextual determinants, implementation strategies, continuous quality improvement, measurement-based care, dissemination and social marketing, implementation outcomes evaluation, sustaining service delivery) to the Exploration, Preparation, Implementation, and Sustainment (EPIS) framework. We used rapid qualitative analytic techniques to synthesize data and develop themes on the core topics and TA delivery. We identified themes on important elements, challenges and facilitators, opportunities to develop TA, and cross-topic connections.

Findings: The discussion groups included 107 participants from 36 states and territories and 75 unique organizations. All core topics resonated with participants' experiences. We identified broader themes related to the importance of partnerships, aligning leadership and staff, collecting and using meaningful data, and navigating workforce challenges. Each core topic was cross-linked with one to four other topics. For example, participants were interested in TA on how to develop sustainment plans, including how to evaluate effectiveness and implementation outcomes related to sustainment. Participants also valued TA that is participant-centered, in-depth (e.g., bundled implementation strategies), and engaging (e.g., peer sharing, problem-solving).

Implications for D&I Research: Our findings are shaping CMHIS's TA offerings and resources with the goal of making dissemination and implementation science more accessible to leaders and staff to support implementation of effective mental health practices. Lessons learned for designing and delivering TA include a focus on co-creation, peer learning, and bundled implementation strategies to enhance sustainable practice change that can improve mental health service delivery.

Primary Funding Source: Substance Abuse and Mental Health Services Administration

Building the future of D&I science: capacity building, infrastructure, and emerging research areas

S14

Achieving Maternal Engagement and Transforming Health through Implementation Science and Training (AMETHIST): an implementation science hub for the NIH IMPROVE initiative

LauraEllen Ashcraft

Veterans Health Administration, Philadelphia, PA, USA; University of Pennsylvania Perelman School of Medicine, Philadelphia, USA

Correspondence: LauraEllen Ashcraft (lauraellen.ashcraft@pennmedicine.upenn.edu)

Implementation Science 2026, 21(2):S14

Background: In 2023, the NIH funded Achieving Maternal Engagement and Transforming Health through Implementation Science and Training (AMETHIST@Penn), focused on implementation science (IS) consultation, community engagement, dissemination, training, and capacity building for the maternal health equity focused IMPROVE initiative.

Methods: The AMETHIST@Penn IS hub consists of five cores: (1) Administration, Leadership, and Evaluation; (2) Community, and Stakeholder Engagement; (3) Design, Methods, and Analysis; (4) Policy, Dissemination, and Communication; and (5) Training and Capacity Building. The AMETHIST@Penn "family" consists of investigators and consultants with expertise in IS, maternal health, health equity and justice, and related fields, along with program staff and a patient partner with expertise in training patients to "tell their story" to pair with project data. Activities with sites include yearly site visits, quarterly

Engagement Enhancement Collaborative (EEC) meetings meant to encourage joint learning around quality relationships with communities, regular consults on IS study design and measurement, workshops on dissemination (ie. social media utilization; op-ed writing), as well as a monthly topic seminars, pilot grant opportunities, and mentored training experiences.

Findings: AMETHIST@Penn is expected to continue through July 2030. All associated Centers of Excellence (CoEs) have completed in-depth intake and 1-year follow up surveys, as well as two virtual site visits with the AMETHIST team. AMETHIST held four EEC meetings, focused on engaging with Black and Indigenous communities. Consultation topics have included converting effectiveness trials to type 1 hybrid trials, selecting appropriate theories, models, and framework, specifying implementation outcomes, and using advanced qualitative methodology. AMETHIST held dozens of virtual seminars/workshops with over 200 attendees, including a Policy Day to equip IMPROVE investigators to interact with policymakers. AMETHIST@Penn has supported internal and external trainees, including three undergraduates, two medical students, two Maternal Health Implementation Science Physician Research Scholars, one Nurse Research Scholar, three pilot grant recipients, and three postdoctoral fellows. AMETHIST's research aim, to evaluate barriers and facilitators to high quality IS maternal health equity research, is underway.

Implications for D&I Research: A centralized hub facilitates consultation, technical assistance, and training in IS for maternal health equity research. This structure could be replicated in other networks to improve access to D&I expertise.

Primary Funding Source: National Institutes of Health

S15

Incorporating a tool for tracking implementation processes and outcomes into maternal health community implementation programs (CIPs)

Juan Villamar

Westat, Bethesda, MD, USA

Correspondence: Juan Villamar (juanvillamar@westat.com)

Implementation Science 2026, 21(2):S15

Background: The Maternal Health Community Implementation Program (MH-CIP) and IMPROVE-CIP seek to improve preventable maternal morbidity and mortality outcomes by funding community-academic teams to conduct community-engaged implementation research. As part of our administrative, scientific, and technical support, Westat developed a CIP IS log that integrates data from the Longitudinal Implementation Strategy Tracking System (LISTS), strategy tracking efforts, and community engagement activities. Here, we highlight how we used IS log data to understand the ways in which implementation science is being used in maternal health.

Methods: We implemented LISTS to capture: strategy specifics (Proctor specification, individual, multifaceted, blended); determinants targeted/leveraged by strategies (matched to an IS theory, model, framework), and the strategy outcome(s) selected for evaluation. CETAC held three to five, 1-h virtual meetings with 4 community-academic teams between August 2024 and January 2025 to collect this data. LISTS data for 3 IMPROVE-CIP teams and retrospective community engagement data for 4 MH-CIP and 4 IMPROVE-CIP teams will be collected in the Fall of 2025.

Findings: We documented 102 determinants (52 facilitators, 67 barriers) that were matched to the updated CFIR (CFIR), 54 determinants (27 facilitators, 27 barriers) matched to PRISM and CFIR, and 12 determinants (6 facilitators and 6 barriers) mapped to EPIS domains and factors. A total of 29 strategies (13 blended, 11 discrete, and 5 multifaceted) were implemented by MH-CIP teams. The most common implementation outcomes selected for evaluation were fidelity (20) and adoption (12), followed by cost (5), sustainability (4), reach (3), acceptability (3), penetration (2), maintenance (2), and feasibility

(1). Community engagement data linked the timing of strategy implementation (exploration, preparation, implementation, sustainment) with the role (govern, advise, implement, consult) and sector (e.g., health system, community-based, faith-based) of community partners. Trust, bidirectional learning, and financial distribution are to be collected in late 2025.

Implications for D&I Research: The CIP IS log is a new meta-science approach to organize LISTS data, strategy tracking, and community engagement activities conducted by MH-CIP and IMPROVE-CIP teams. By triangulating IS and community engagement data collected over time, the IS log supports systematic tracking and reporting, while also underscoring the central role that community partners have in implementation studies.

Primary Funding Source: National Institutes of Health

S16

Improving data standardization and strengthening implementation research capacity in maternal health through an NIH IMPROVE Data-Hub

Andreea Creanga

Johns Hopkins School of Medicine, Baltimore, MD, USA

Correspondence: Andreea Creanga (acreang3@jhu.edu)

Implementation Science 2026, 21(2):S16

Background: The NIH-funded Maternal Health Data Innovation and Coordination Hub (Data-Hub) works across 10 Centers of Excellence (CoE) in maternal health research to foster collaboration and support standardized data collection and analysis, thereby strengthening implementation research capacity. Between September 2023 and July 2025, we implemented key activities and tracked CoEs' engagement with the Data-Hub alongside changes in research capacity.

Methods: With our expert panel, we conducted a landscape analysis and two modified Delphi processes to develop common data elements (CDEs) for maternal health implementation research. We disseminated a CDE implementation toolkit including REDCap forms, a data dictionary, a checklist for ethical considerations, and data safety protocols. We offered a collaboration portal and training opportunities, including a "data mosaic activity" to answer common CoE research questions. We measured CoE engagement via CoE-specific administrative connections and activity attendance. We quantified CoE engagement using a score of 1 for each CoE-specific connection or activity, and 1 if CoE-affiliate attended a specific training; and a score of 3 for each data mosaic submission. Surveys administered in 2024 and 2025 used Holden's research capacity and culture (RCC) tool to rate CoE affiliates' skill levels from 1 (low) to 10 (high) on 17 research aspects.

Findings: Our expert panel recommended CDEs for 36 biomedical and 22 psychosocial maternal health constructs, and CoEs are in various stages of integration using our CDE implementation toolkit. Total engagement scores ranged between 15 and 45, with scores < 20 for two CoEs, 20–30 for five CoEs, and > 30 for five CoEs. We found improvements in RCC mean scores between 2024 ($N = 82$; score = 7.2) and 2025 ($N = 122$; score = 7.5), yet overall lower scores for early-stage and minority-group investigators.

Implications for D&I Research: Our Data-Hub supports data standardization and cross-CoE learnings in the field of maternal health implementation research. While the research is ongoing, process measures of engagement demonstrate practical ways to measure capacity development across a maternal research network. By working alongside CoEs in the development of CDEs and promoting ongoing engagement, we seek to strengthen quality and generalizability in maternal health implementation research, in order to have the most widespread impact on maternal outcomes at a national scale.

Primary Funding Source: National Institutes of Health

S17

"They know what they are doing": mechanism mapping to explicate how task shifting can improve HIV care in pluralistic settings

Misha Hooda¹, Gabriel Nuwagaba², William Bugeza², Constance Birungi², Maureen Tushabe², Sylvia Natukunda², Srijia Gogineni¹, Denis Nansera², Winnie Muyindike², Juliet Mwangi-Amumpaire², Radhika Sundararajan¹

¹Weill Cornell Medicine, New York, NY, USA; ²Mbarara University of Science and Technology, Mbarara, Uganda

Correspondence: Misha Hooda (mih4006@med.cornell.edu)

Implementation Science 2026, **21**(2):S17

Background: Task-shifting is an established implementation strategy used to expand access and uptake of evidence-based care. However, in global settings, where informal and biomedical providers often operate concurrently in medically pluralistic systems, task shifting efforts frequently confront tensions between care systems. The mechanisms through which task shifting generates collaborative change in such environments remain underexplored. We use qualitative data from an ongoing clinical trial to generate a mechanism map explicating causal pathways through which task shifting with traditional healers (TH) can advance care in pluralistic contexts.

Methods: In July 2023, 22 TH in Southwestern Uganda received training by HIV clinicians on HIV prevention, ART adherence, stigma reduction, and HIV self-testing. Between July 2023 and November 2024, we conducted longitudinal qualitative interviews with 22 TH and 20 healthcare workers at baseline and 9-months post training implementation. We employed reflexive thematic analysis to examine relationship transformation. Contextual determinants were organized using CFIR domains, and causal pathway diagramming informed the development of a mechanism map linking components of task shifting to proximal and distal implementation outcomes.

Findings: Harmonizing initially dissonant relationships between HIV clinicians and TH is foundational to effective task shifting in the study setting. We identified three distinct pathways that successfully transformed contentious relationships into bidirectional partnerships:

- (1) Legitimization pathway: integration strategies fostered professional identity and validation of TH ("we've been remembered")
- (2) Competency pathway: structured HIV training facilitated mutual respect through shared knowledge and capability recognition ("they know what they're doing")
- (3) Relational pathway: liaison systems fostered trust through consistent two-way communication ("we are like family")

These mechanisms catalyzed strengthened healer-clinician relationships, evidenced by increased professional trust, improved communication, and deeper social connectedness. This proximal implementation outcome is crucial to achieving successful system, clinical, and distal implementation outcomes.

Implications for D&I Research: This study demonstrates how task shifting activated interpersonal mechanisms—such as trust-building, role recognition, and communication—to achieve proximal relationship outcomes critical for intervention success in pluralistic systems. By mapping causal pathways, we offer a replicable method for analyzing and optimizing task shifting programs in other contexts. These findings highlight the value of targeting relational dynamics as core mechanisms in contexts where informal and biomedical providers intersect.

Primary Funding Source: National Institutes of Health

S18

The missing link? Exploring communication quality as a driver of sustainability capacity in D&I science

Sara Malone¹, Luke Zabotka¹, Veronica Chaitan¹, Samantha Hayes², Rebeca Caires², Miguel Aguilar Barragán², Maricela Robles-Murguía², Adolfo Cardenas-Aguirre², Bobbi Carothers³, Jocelyn Rivera⁴, Virginia McKay³, Dylan Graetz², Asya Agulnik²

¹Washington University in St. Louis, St. Louis, MO, USA; ²St. Jude Children's Research Hospital, Memphis, TN, USA; ³Center for Public Health Systems Science, Brown School, Washington University in Saint Louis, St. Louis, MO, USA; ⁴Hospital Infantil Teletón de Oncología (HITO), Querétaro, Mexico

Correspondence: Sara Malone (sara.malone@wustl.edu)

Implementation Science 2026, **21**(2):S18

Background

While implementation strategies have been widely explored to improve the integration of evidence-based practices (EBPs), less attention has been paid to how EBPs are sustained. One example of an understudied mechanism is communication quality. Communication has been theorized as important, however, its role as a driver of sustainability capacity has been underexplored. This study examines the relationship between communication quality and sustainability capacity in resource-variable hospitals using an EBP, pediatric early warning scores (PEWS).

Methods

As part of a longitudinal study on PEWS sustainability, we conducted a cross-sectional survey with clinicians in variably resourced hospitals. Communication quality was assessed using CritCom—a reliable, multilingual instrument with six domains: actionable, clarity, tone, empowerment, leadership, and teamwork. Sustainability capacity was measured using the Clinical Sustainability Assessment Tool, including seven domains: workflow integration, engaged staff, engaged partners, monitoring/evaluation, implementation/training, organizational readiness, and outcomes/effectiveness. Additionally, we collected data on individual demographics and site characteristics. Mixed effects modeling was used to examine the relationship between communication and overall sustainability capacity, adjusting for individual and site-level factors.

Findings

1,288 individuals from 14 countries and 47 hospitals completed the survey. Participants worked in all hospital areas (wards, ICU, emergency department); most were nurses (58.1%) and physicians (33.9%). The overall median CritCom score was 4.133/5 max, *IQR* [3.867–4.700], and the median CSAT score was 4.250/5 max, *IQR* [3.928–4.733]. Individuals in leadership positions perceived both communication quality and sustainability capacity as higher than bedside staff. Similarly, physicians scored both constructs higher than their nursing colleagues. There was a strong, positive correlation between overall communication quality and sustainability capacity (0.58, $p < 0.0002$). Mixed-effects modeling revealed that, after controlling for individual and setting-level variables, high perceptions of communication quality were significantly associated with overall clinical sustainability.

Implications for D&I Research

Communication quality is a crucial, yet underexplored determinant of EBP sustainability. Targeting communication can improve overall site capacities, thereby enhancing implementation efforts and ultimately leading to fully realized D&I benefits over time. These results highlight the importance of embedding a focus on team communication in implementation efforts and contribute to a systems-informed understanding of how structural dynamics influence implementation outcomes.

Primary Funding Source: National Institutes of Health

S19

Reimagining mechanisms and functions: reframing the purpose behind implementation strategies to embed equity and relationality

Julia Moore, Sobia Khan

The Center for Implementation, Toronto, ON, Canada

Correspondence: Julia Moore (julia.moore@thecenterforimplementation.com)

Implementation Science 2026, 21(2):S19

Background

There has been significant interest in exploring exactly how implementation strategies foster change. Much of this work has focused on advancing theory and empirical evidence linking strategies with mechanisms of change. While these efforts are essential and ongoing, there remains a pressing need to support professionals in understanding the underlying functions of the strategies they use. Additionally, existing conceptualizations of functions have been critiqued for: being top-down, overlooking relational aspects of change, lacking equity considerations, and reflecting Global North assumptions. To address these limitations, we reimagined implementation functions as “intentions”: the purpose and relational grounding of implementation strategies that more accurately represent the diverse ways strategies support change across people and contexts.

Methods

Using an iterative, mixed-methods approach, we developed the intentions framework through five key activities:

- literature can on functions and mechanisms;
- engagement with ten implementation research experts;
- integration of intentions described in practice; and
- back mapping intentions for 101 implementation strategies (ERIC list and other sources).

We built on Michie et al.’s nine intervention functions, implementation support/practitioner functions, and the literature on equity and relationships in implementation. The strategy-intention mappings were then linked to individual and contextual determinant frameworks.

Findings

The resulting framework includes 35 intentions. These intentions articulate the underlying purpose behind implementation strategies, including relational, equity-oriented, and systems-level intentions often underrepresented in existing taxonomies. Some intentions align with categories already described in the literature, such as those focused on *knowledge and skill-building* (educate, build skills) and *reinforcement and regulation* (remind, restrict). Other intentions emphasize *relational and equity-oriented processes* that foster connection, inclusion, and *psychological safety* (*cultivate trust, create belonging, diversify, be culturally/socially responsive*). Others center on *internal and reflective processes* that shape how individuals and teams approach implementation (*shift mindset, be reflexive*).

Implications for D&I Research

Expanding our understanding of the underlying intentions of implementation strategies can help us better elucidate how implementation strategies work. Adding intentions that emphasize the relational nature of most implementation strategies can help us assess mechanisms of change more accurately. To our knowledge, this is the first attempt informed by implementation science and practice to broaden our scope of knowledge and theory on strategy intentions.

S20

Evolving strategies through community-engaged implementation: mid-study findings from the Turtle Island Tales projectJames Merle¹, Emily Tomayko², Teresa Warne², Alexandra Adams², Paul Estabrooks¹¹University of Utah, Salt Lake City, UT, USA; ²Montana State University, Bozeman, USA**Correspondence:** James Merle (james.merle@utah.edu)

Implementation Science 2026, 21(2):S20

Background: Turtle Island Tales is a culturally grounded wellness promotion program implemented in partnership with Native American communities. The study prioritizes community engagement by embedding a Project Steering Committee (PSC) and Community Implementation Teams (CITs) as co-leaders in designing and executing implementation strategies.

Methods: Using the Longitudinal Implementation Strategy Tracking System (LISTS) method, we conducted bi-monthly meetings, which served as checkpoints for tracking strategy additions, modifications and discontinuations across seven communities. We provide a mid-study report on the dynamic strategy changes. During the study planning phase, strategies were pre-specified to promote reach, adoption, implementation, and maintenance, as documented in planning artifacts.

Findings: The initial proposal included three bundled strategies with 51 embedded strategies. Through the LISTS process, these three bundles were retained, but the total number of strategies was consolidated into 30 discrete, specified actions. To date, there are 22 active strategies, eight that have been discontinued or completed, four modifications that have been tracked, and eight strategies have been added. Key changes included material development (e.g., binders, bulk ordering), logistical and funding solutions (e.g., site visits, community-secured funding), integration with Head Start systems and educational standards, and emergent delivery adaptations. Although many strategies retained their original labels, such recruitment and sustainability planning and facilitation, their operationalizations diverged from initial specifications. For example, centralized recruitment strategies evolved into localized, CIT-led social media campaigns. These shifts reflect real-time responsiveness to community priorities, implementer capacity, and contextual constraints.

Implications for D&I Research: For implementation science, this study illustrates how LISTS can function as a participatory learning mechanism to support and document strategy evolution, particularly in culturally grounded, community-engaged D&I efforts. This mid-study report highlights the importance of flexible, community-driven implementation planning; although strategies were presumably well-specified during study planning, LISTS tracking revealed that community engagement often reshaped those strategies in unanticipated yet contextually valid ways. The LISTS process led to expanding and contracting the discrete actions within bundled strategies. These findings underscore the value of ongoing, structured community involvement to enhance fit and feasibility and to re-specify strategies over time in alignment with local needs and systems.

Primary Funding Source: National Institutes of Health

S21

Ongoing consultation and technical assistance can increase implementation research knowledge among teams conducting HIV-related implementation research

Nanette Benbow

Northwestern University Feinberg School of Medicine, Chicago, IL, USA

Correspondence: Nanette Benbow (nanette@northwestern.edu)

Implementation Science 2026, 21(2):S21

Background: As part of the evaluation of the CHESHIRE network, we assessed whether activities and support provided by Implementation Science (IS) Hubs to the NIH-funded Ending the HIV Epidemic (EHE) administrative supplements led to increases in implementation research knowledge among the funded EHE project teams.

Methods: We analyzed self-reported survey data collected by the NIH-funded Implementation Science Coordination Initiative (ISCI) from EHE projects during their supplement funding periods (between August 2021 and February 2025). To assess changes in IS knowledge and competencies, ISCI distributed the surveys before project initiation and after project completion (i.e., pre- and post-surveys), asking teams to rate their current level of expertise (beginner, intermediate, advanced)

on each of 10 implementation research competencies (103 of 107 projects completed both). We compared the proportions of projects by expertise level in the pre- and post-surveys and described the change in expertise level (increase [move from beginner to intermediate/advanced or from intermediate to advanced], no change, or decrease [moved from advanced to intermediate/beginner or from intermediate to beginner]).

Findings: In the pre-surveys, across the 10 competencies, the median percentage of EHE projects rating their knowledge as “beginner” was 26%, “intermediate” was 51%, and “advanced” was 21%. More of the projects had advanced knowledge at baseline of “describing the importance of stakeholder perspectives”; more of the projects had “beginner” knowledge of “describing relationships between organizational dimension and implementation research”. Among projects with beginner or intermediate knowledge in the pre-survey, there was an increase in knowledge from pre- to post: across the 10 competencies, a median 41% of projects (range by competency: 30%–58%) demonstrated an increase in implementation research knowledge. The competencies with the greatest proportionate increase in knowledge from pre- to post-survey were “determine when participatory research is appropriate for implementation” (57% of projects increased knowledge) and “identify and develop sustainable partnerships for implementation research” (58% of projects increased knowledge).

Implications for D&I Research: Being supported by an organized network of IS hubs providing technical assistance and consultation may lead to increased IS research knowledge among teams conducting HIV-related implementation research.

Primary Funding Source: National Institutes of Health

S22

The impact of a collaborative implementation science mentoring network: the mentee perspective

Mary Louise Millett

University of Texas Health Houston, Houston, TX, USA

Correspondence: Mary Louise Millett (mary-louise.e.millett@uth.tmc.edu)

Implementation Science 2026, **21**(2):S22

Background: CHESHIRE represents a novel model for HIV researchers across institutions in the United States, with hub IS experts providing technical assistance and other support for EHE-funded researchers. Little is known about how grantees interact with and benefit from a network delivering implementation science (IS) mentorship, collaboration, and educational opportunities. This qualitative study evaluated the impact of CHESHIRE hub technical assistance activities on mentee IS research capacity and the integration of IS methods into their EHE-funded projects.

Methods: We conducted 14 semi-structured interviews with investigators of EHE-funded projects. Four CHESHIRE-affiliated researchers interviewed grantees assigned to hubs. The interview guide explored engagement, the quality of support, factors influencing involvement, IS competency development, and hub impact on research execution. We also sought recommendations to improve the network's structure and support. Interviews were conducted via Zoom or Microsoft Teams, recorded, and transcribed. Analysis was undertaken in Atlas.ti. One researcher developed a preliminary codebook inductively using three transcripts. Two additional researchers independently coded two more transcripts using the draft codebook. The team compared interpretations, revised definitions, and reached consensus on a shared structure. All remaining transcripts were coded independently. The team met regularly to resolve discrepancies. We then developed higher-order themes by analyzing patterns and drawing on illustrative reflections that captured common experiences or offered distinct insights.

Findings: Our analysis identified three overarching themes: (1) factors shaping engagement, (2) forms of engagement, and (3) perceived

impact of the network. Participants described how institutional constraints and competing demands shaped their engagement. Interactions included tailored consultations with mentors, webinars, and communities of practice that fostered shared learning. Grantees viewed the network as helpful for IS learning and strengthening project design and execution. They recommended reexamining how grantees are assigned to hubs to facilitate engagement and establish compatible mentor–mentee pairings.

Implications for D&I Research: These findings provide guidance for structuring capacity-building models that are responsive to real-world research needs. Sustained investment in models like CHESHIRE is critical to supporting IS in HIV research and sustaining evidence-based HIV interventions.

Primary Funding Source: National Institutes of Health

S23

Fostering scientific relationships through the CHESHIRE network: a social network analysis

Alexandra Morshed

Rollins School of Public Health, Emory University, Atlanta, GA, USA

Correspondence: Alexandra Morshed (alexandra.morshed@emory.edu)

Implementation Science 2026, **21**(2):S23

Background: Targeted capacity building—through training, mentorship, and connected learning networks—is crucial for expanding the reach of HIV services. When capacity-building initiatives lead to increased interdisciplinary collaborations and knowledge sharing, there is potential to accelerate the integration of implementation science (IS) into HIV research, facilitating the faster rollout of evidence into practice. As part of an evaluation of CHESHIRE, this study describes and maps scientific collaboration across the network and evaluates changes following receipt of EHE funding.

Methods: We conducted a retrospective, one-group, pre-post design study to characterize and assess changes in scientific relationships among IS Hub teams and EHE supplement principal investigators (MPIs) using an online survey (administered January–May 2025) and social network analysis methods. We developed a contact list using publicly available NIH data for each supplement grant, information from IS Hubs and ISCI, and conducted internet searches to identify missing/undeliverable email addresses. We excluded MPIs if they were on leave, retired, or reported no involvement in the supplement project. Of 265 identified network participants, 152 people (57.4%) completed the survey. Respondents were presented with the full roster of invited individuals and asked to identify their professional contacts, whether they had a scientific relationship with them before and after receiving IS Hub or EHE supplement funding, the type of scientific relationship, and the frequency of contact.

Findings: The collaboration network (265 people) consisted of 205 (77.4%) EHE supplement grantees, 42 (15.8%) IS Hub team members, and 18 (6.8%) people who were both. Twenty-six (18.6%) of survey respondents rated their IS expertise as beginner, 73 (52.1%) as intermediate, and 41 (29.3%) as advanced. Among respondents, 26.7% sought IS advice outside of CHESHIRE. The overall number of scientific relationships was greater following receipt of EHE funding, with a greater average change in number of relationships in earlier CHESHIRE cohorts (years 2019 and 2020) than later cohorts (years 2021–2023).

Implications for D&I Research: Public investment in catalyzing scientific relationships is crucial for advancing HIV evidence translation into practice. CHESHIRE successfully facilitated new relationships that have the potential to accelerate research translation and evidence integration.

Primary Funding Source: National Institutes of Health

S24

The World Health Organization's global research agenda on Knowledge Translation (KT) and Evidence-informed Policy-making (EIP): development process, results, and next steps

Robert Terry¹, Jennifer Callaghan-Koru², Bastien Kolt³, Tanja Kuchenmüller³, Ahmed Mandil⁴

¹The Special Programme for Research and Training in Tropical Diseases (TDR), Geneva, Switzerland; ²University of Arkansas for Medical Sciences, Fayetteville, AR, USA; ³World Health Organization, Geneva, Switzerland;

⁴University of Alexandria, Alexandria, Egypt

Correspondence: Robert Terry (terryr@who.int)

Implementation Science 2026, **21**(2):S24

Background: The quantity and quality of global knowledge translation (KT) research has grown in recent decades but its findings are still underused in health policy-making. Challenges include: a mismatch between the KT products available and the needs of decision makers, fragmented and duplicative studies, and limited and uncoordinated funding. To address this, WHO developed a Global Research Agenda for KT (GRA) that was published in May 2025.

Methods: The agenda development process followed WHO's priority-setting guidance and included literature reviews, an online survey, a modified Delphi exercise, and two online consultations, overseen by an advisory committee (2023–2025). More than 130 global KT experts from 38 countries, identified from an open call, participated in the process. Two rounds of Delphi were completed to identify and then rank research priorities based on the outcome of the research having impact, improving inclusivity, and being feasible. In addition, experts allocated tokens to research areas based on perceived importance.

Findings: The Global Research Agenda on KT and EIP [1] identified 19 research priorities under three themes: 1) Interventions; 2) Barriers, facilitators, and opportunities; and 3) Methods, standards, measurements, theories, and frameworks. Highly-ranked research areas include: identify, develop, and assess strategies and approaches to institutionalize evidence production, translation, and use (Theme 1); analyze factors determining the engagement of decision-makers in evidence uptake (Theme 2); and, explore innovative and evolving technologies that support KT/EIP, such as Artificial Intelligence (Theme 3). The Agenda was launched during a global webinar in May 2025.

Implications for D&I Research: WHO will disseminate this agenda to funders and research partners to catalyze and guide investments in KT/EIP research. As part of the implementation process, TDR launched an open call for research proposals aligned with the GRA. The response was unprecedented, underscoring the strong demand for KT research: By the close of the call, 430 full submissions were received from 58 low- and middle-income countries. Further details on the funded research projects will be shared in this presentation.

Primary Funding Source: World Health Organization

Reference

1. A project summary, including the research agenda, can be accessed here: https://cdn.who.int/media/docs/default-source/campaigns-and-initiatives/epivnet/gra-project-summary-and-progress-report_may-2025.pdf?sfvrsn=31bc0e9d_3

S25

Realizing the benefits of D&I: a roadmap for integrating artificial intelligence with implementation science

Fernando Bruno¹, Karen Plevock Haase¹, Lilian Perez², Shahnaz Khan¹

¹National Institutes of Health, Bethesda, MD, USA; ²San Diego State University, San Diego, CA, USA

Correspondence: Fernando Bruno (fernando.bruno@nih.gov)

Implementation Science 2026, **21**(2):S25

Background: Integrating artificial intelligence (AI) and implementation science (IS) presents a critical opportunity to realize the full benefits of D&I by accelerating the delivery and uptake of evidence-based interventions and improving population health outcomes. Among the

benefits, AI could streamline data analysis, predict care needs, and automate tasks. However, without ethical safeguards, AI might perpetuate bias and widen health disparities. To explore optimal integration of AI and IS, the Center for Translation Research and Implementation Science (CTRIS) convened a multidisciplinary Think Tank to examine key challenges and identify future directions.

Methods: The Think Tank, held in December 2024, brought together clinicians, IS researchers, AI experts, and public health professionals divided into four groups:

1. Ethical and Equitable Use of AI in IS
2. Cutting-Edge Applications of AI in IS
3. Data Management and Privacy
4. Future Innovations in AI and IS

Each group identified gaps, challenges, and opportunities within their domain. Discussions were analyzed to identify cross-cutting prioritized themes and actionable strategies.

Findings: Key themes emerged across domains:

- **Ethical use:** Community-engaged co-design, plain-language explaining how AI models work, and continuous bias monitoring are essential for fostering trust and fairness in AI-enabled implementation.
- **Applications:** Tools such as chatbots and large language models show promise in enhancing care delivery and reducing clinician workload, but their use is limited to resource-rich settings.
- **Data and Privacy:** Federated learning enables secure model training across sites, yet challenges around data governance and authenticity remain unresolved.
- **Future Directions:** Participants proposed interdisciplinary "AI + IS studios," integration of AI training into medical and public health education, and incentives for equity-focused research and development.

Cross-cutting recommendations included standardizing data practices, investing in AI literacy, and developing shared evaluation frameworks (e.g., CONSORT-AI, RE-AIM) for responsible implementation.

Implications for D&I Research: AI can help close the evidence-to-practice gap, but only with intentional, community-centered design and infrastructure. The Think Tank identified critical areas for capacity-building and collaboration, including piloting federated-learning approaches, co-creating solutions with communities most impacted by health disparities, and establishing national guidance for unconventional data verification. These insights offer a roadmap for embedding AI into D&I research to maximize real-world impact.

Primary Funding Source: National Institutes of Health

S26

Mapping the landscape of dissemination and implementation science across the CTSA Consortium: a multi-domain environmental scan

Jing Li¹, Naomi Duffort¹, Larry Heard¹, Kristine Heard¹, Renee Heffron¹, Bertha Hidalgo¹, Katherine Mills², Michael Mugavero¹, Salisa Westrick³, Lisa Zubkoff¹

¹University of Alabama at Birmingham, Birmingham, AL, USA; ²Tulane University, New Orleans, LA, USA; ³Auburn University, Auburn, AL, USA

Correspondence: Jing Li (jingli2@uabmc.edu)

Implementation Science 2026, **21**(2):S26

Background

Dissemination and Implementation (D&I) science plays a central role in accelerating the translation of biomedical discoveries into health impact. The NIH Clinical and Translational Science Awards (CTSA) Program supports over 60 hubs to advance translational research

infrastructure and practice, with D&I Science prominently featured as a goal of the CTSA funding mechanism. Currently, the degree to which D&I science is integrated within CTSA hubs varies, warranting a comprehensive assessment, to guide growth and evolution.

Methods

We conducted a structured environmental scan of all 66 CTSA hubs using publicly available web content and published documents. Data were synthesized across six domains: (1) D&I core, (2) consultation services, (3) training grants and career development, (4) structured educational offerings, (5) pilot funding, and (6) inter-institutional partnerships. Descriptive summaries and qualitative synthesis identified trends, gaps, and models of excellence.

Findings

D&I infrastructure across the CTSA network is heterogeneous. Currently, 38% of hubs ($n=25$) maintain a dedicated D&I core, 20% ($n=13$) offer some services without a formal structure, while 42% of hubs show no visible D&I infrastructure. Consultation services are available at 53% of the hubs ($n=35$), typically covering study design, framework selection, and grant development; however, their structure varies widely, ranging from brief office hours to multi-step, team-based models. Training opportunities are limited: just 12% ($n=8$) of hubs provide formal D&I training grants (e.g., K12 or T32), and only 15% ($n=10$) offer D&I-focused coursework or graduate certificates. Most CSAs do not integrate D&I into broader translational career development programs. Ten hubs offer pilot funding for D&I (typically \$2,000–\$50,000). Engagement in D&I collaborative events (e.g., seminars, symposia, interest groups) is reported at 37% of CSAs. Although 48 CSAs have partner collaborators, with most maintaining 2–4 active partnerships, none provided information about D&I-specific collaborations.

Implications for D&I Research

This scan reveals an inconsistent yet a promising direction for D&I science across the CTSA network. Gaps in training, consultation, and coordination may limit equitable growth and impact. Given the CTSA's mission of accelerating the translation of discoveries into population health impact, the CTSA is then ideal program for building D&I by identifying best practices, supporting under-resourced hubs and strengthening implementation efforts nationwide.

S27

Building capacity among external facilitators in a longitudinal implementation initiative

Taylor Harris^{1,2}, Ron Calderon³, Sona Hovsepian³, Erin Finley³, Sonya Gabrielian⁴

¹VA Center for the Study of Healthcare Innovation, Implementation and Policy, VA Greater Los Angeles Healthcare System, Los Angeles, CA, USA; ²Veterans Health Administration, North Hills, CA, USA; ³Veterans Health Administration, Los Angeles, CA, USA; ⁴University of California, Los Angeles David Geffen School of Medicine, Los Angeles, CA, USA

Correspondence: Taylor Harris (taylor.harris2@va.gov)

Implementation Science 2026, **21**(2):S27

Background: External facilitation (EF) is widely recognized as an effective strategy for implementing complex, multi-faceted evidence-based practices (EBPs). Within longitudinal implementation initiatives, novice external facilitators often initially rely on didactic training, and gradually evolve towards highly partnered, creative and relationship-driven exchanges with implementing teams. However, this process of iteratively building capacity is not well understood. Drawing from a four-year national implementation initiative of Critical Time Intervention (CTI, a case management EBP) at 20 community-based agencies serving homeless-experienced Veterans (HEVs), we sought to identify key practices (e.g., activities, resources) to build external facilitators' capacity.

Methods: Three novice external facilitators supported CTI implementation across three consecutive cohorts. Over 7 months, facilitators conducted weekly 30-min EF sessions with each agency. Periodic reflections ($n=24$), a method of guided discussions for documenting contextual implementation information, conducted with facilitators, captured their learning experiences and capacity building practices.

Rapid qualitative analysis identified key processes that enhanced facilitators' capacity.

Findings: In periodic reflections, facilitators highlighted the importance of comprehensive training in CTI and EF which provided foundational skills and bolstered role clarification. Weekly debriefs (i.e., meetings to address implementation challenges and share best practices) with the EF team and reflective writing exercises (i.e., templated notes following each EF session) were salient supports that enhanced facilitators' capacities to respond to implementing teams' needs. Facilitators felt weekly debriefs served as a cohesion activity that reinforced implementation goals, prevented facilitator burnout, and encouraged innovative thinking. Reflective writing promoted facilitators' recognition of biases, critical thinking, and problem-solving skills. Periodic reflections also captured facilitators' shifts to collaborative roles, showcasing their more frequent use of reciprocal dialogue and integration of contextualized and relevant evidence as time progressed. With each cohort, facilitators gained exposure to diverse case management teams which enhanced their empathy and structured thinking, facilitating richer collaborative, co-creative interactions.

Implications for D&I Research: Our findings underscore the importance of identifying and using resources that promote facilitators' skills, adaptability and flexibility; building facilitator capacity ultimately can contribute to increased practice uptake and improved outcomes. These insights contribute to knowledge regarding successful and sustainable implementation supports, emphasizing the need for novice facilitators to be supported as they engage in longitudinal implementation efforts.

Primary Funding Source: Department of Veterans Affairs

S28

Building impactful community-engaged research translation partnerships: twenty-plus years of the cancer prevention and control research network

Stephanie Wheeler¹, Rebecca Lee², Mary Wangen², Amy Tran², Erin Laurie³, Meghan O'Leary³, Renée Ferrari³, Jingle Xu², Alison Brenner³, Rachel Hirschey²

¹Lineberger Comprehensive Cancer Center, UNC Chapel Hill, Chapel Hill, NC, USA; ²University of North Carolina, Chapel Hill, NC, USA; ³UNC Gillings School of Global Public Health, Chapel Hill, NC, USA

Correspondence: Stephanie Wheeler (stephanie_wheeler@unc.edu)

Implementation Science 2026, **21**(2):S28

Background: The Centers for Disease Control and Prevention-funded Cancer Prevention and Control Research Network (CPCRN) has existed since 2002 with additional support from the National Cancer Institute. The mission of CPCRN is to accelerate the adoption and implementation of evidence-based cancer prevention and control strategies through community engagement, enhance large-scale efforts to reach all populations to reduce the burden of cancer, and develop the dissemination and implementation workforce in cancer prevention and control. In 2002–2024, the Network included collaborating centers at a total of 19 academic institutions. The Network currently includes more than 300 members, representing 8 funded collaborating centers, as well as Affiliate Members and CPCRN Scholars from 45 additional community organizations and academic institutions. Here, we report on the scientific impact of CPCRN over its 20-plus year history.

Methods: The CPCRN coordinating center developed an online progress reporting system in 2004 to track network activities and accomplishments. Here, we summarize combined peer-reviewed publications, presentations, grant activity, and contributions to official reports, plans and policies within CPCRN through collaborating center core projects and cross-center workgroups in two time periods: 2019–2024 (corresponding to the CPCRN5 funding cycle), and 2004–2024 (since inception of the progress reporting system).

Findings: During the CPCRN5 funding cycle, members published a combined total of 969 CPCRN-related articles; since 2004, members published 2,818 CPCRN-related publications. In CPCRN5, members delivered 645 CPCRN-related presentations and 162 trainings; since

2004, members delivered 2,581 CPRN-related presentations. Trainings supported evidence translation in outside organizations, such as Federally Qualified Health Centers (129 presentations) and other community-based healthcare providers (247 presentations). During CPRN5, members submitted 240 CPRN-related grant applications, of which 186 applications (~78%) were funded, totaling > \$272 million in additional awards. Since 2004, CPRN members submitted a cumulative total of 1,513 grants, of which 809 (~53%) were funded, totaling > \$959 million in additional awards. In the CPRN5 funding cycle alone, members contributed to 39 official reports, plans, and policies to support local, state, and federal research translation in cancer.

Implications for D&I Research: Large, collaborative, thematic networks like CPRN enable impactful and productive partnerships at local, state, and federal levels.

Primary Funding Source: Centers for Disease Control and Prevention

S29

The power of a research network: CPRN's role in advancing pharmacy-based cancer prevention and control through innovation and evidence synthesis

Alison Brenner¹, Mary Wangen², Renée Ferrari³, Daniel Reuland⁴, Macary Marciniak⁵, Rachel Hirschey², Stephanie Wheeler⁶, Parth Shah⁷, The PharmCPC Workgroup⁸

¹Lineberger Comprehensive Cancer Center, UNC Chapel Hill, Chapel Hill, NC, USA; ²University of North Carolina at Chapel Hill, Chapel Hill, NC, USA; ³UNC Chapel Hill Gillings School of Global Public Health, Chapel Hill, NC, USA; ⁴UNC Chapel Hill, School of Medicine, Chapel Hill, NC, USA; ⁵UNC Eshelman School of Pharmacy, Chapel Hill, NC, USA; ⁶Lineberger Comprehensive Cancer Center, UNC Chapel Hill, Chapel Hill, NC, USA; ⁷University of Washington, Seattle, WA, USA; ⁸The Cancer Prevention and Control Research Network, Chapel Hill, NC, USA

Correspondence: Alison Brenner (alison.brenner@unc.edu)

Implementation Science 2026, 21(2):S29

Background

Pharmacies are among the most accessible healthcare settings in the U.S., particularly for populations with limited access to traditional care. As pharmacists' scope of practice has expanded over the past three decades, pharmacies are increasingly well-positioned to deliver preventive services. The Cancer Prevention and Control Research Network (CPRN) has served as a critical platform for conceptualizing, piloting, and scaling implementation strategies. Recognizing the broader potential for pharmacies as a healthcare setting, CPRN allowed for expanded exploration of pharmacy cancer prevention and control (CPC) activities.

Methods

Through CPRN's infrastructure, the University of North Carolina collaborating center explored an innovative model to deliver fecal immunochemical tests (FIT) for colorectal cancer screening through community pharmacies (PharmFIT™). We conducted formative interviews, surveys, and pilot tests and, following, expanded on the PharmFIT™ innovation to explore the potential of other CPC activities in pharmacies.

Findings

PharmFIT™ formative work demonstrated high acceptability among pharmacists, patients, and primary care providers. Pilot testing confirmed feasibility and potential for impact, with two pilot sites reporting near 100% FIT completion rates. Encouraged by the successful pilots, we successfully applied for an NCI R01 (1R01CA279010). Next, the CPRN PharmCPC Interest Group launched a multi-site scoping review to catalog the global landscape of pharmacy-based CPC interventions, currently underway with 13 reviewers across multiple CPRN sites. Insights from PharmFIT™ directly informed the development of a new intervention: PACTS (Pharmacist Assisted Cessation of Tobacco and Support for Screening). PACTS is a pharmacy-delivered lung cancer screening shared decision-making program, integrated

with existing tobacco cessation services. Currently, we are interviewing pharmacists and physicians to understand acceptability, potential for integration with tobacco cessation in pharmacies, and key aspects of the care coordination for accessing lung cancer screening services.

Implications for D&I Research

This progression, from innovation to evidence review to expansion of intervention design, demonstrates the unique capacity of a stable research network to accelerate the D&I research pipeline. CPRN's sustained infrastructure enables rapid cycle development, sustained community partnerships, and knowledge translation. This model offers a replicable framework for leveraging novel healthcare settings like pharmacies to expand reach and equity in cancer prevention.

Primary Funding Source: National Institutes of Health

S30

Scaling what works: advancing the reach and real-world impact of cancer prevention and control interventions

Rachel Hirschey¹, Mary Wangen¹, Amy Tran¹, Renée Ferrari², Rebecca Lee³, Meghan O'Leary², Jingle Xu¹, Alison Brenner^{4,5,6}, Cam Escoffery⁷, Prajakta Adsul⁸, Stephanie Wheeler⁹, The Scaling What Works Workgroup¹⁰

¹University of North Carolina at Chapel Hill, Chapel Hill, NC, USA; ²UNC Chapel Hill Gillings School of Global Public Health, Chapel Hill, NC, USA; ³University of North Carolina, Chapel Hill, NC, USA; ⁴Division of General Medicine and Clinical Epidemiology, University of North Carolina School of Medicine, Chapel Hill, NC, USA; ⁵UNC Chapel Hill, School of Medicine, Chapel Hill, NC, USA; ⁶Lineberger Comprehensive Cancer Center, UNC Chapel Hill, Chapel Hill, NC, USA; ⁷Rollins School of Public Health, Emory University, Atlanta, GA, USA; ⁸University of New Mexico, Albuquerque, NM, USA; ⁹Lineberger Comprehensive Cancer Center, UNC Chapel Hill, Chapel Hill, NC, USA; ¹⁰The Cancer Prevention and Control Research Network, Chapel Hill, NC, USA

Correspondence: Rachel Hirschey (hirschey@unc.edu)

Implementation Science 2026, 21(2):S30

Background

Investigators from the Cancer Prevention and Control Research Network (CPRN), in partnership with the Centers for Disease Control and Prevention (CDC), developed the *Scaling What Works* (SWW) program to support National Comprehensive Cancer Control Program (NCCCP) sites in implementing evidence-based interventions (EBIs). SWW focused on EBIs designed to address critical gaps in cancer prevention and care: (1) Project ECHO combined with patient navigation to improve outcomes for rural cancer survivors; (2) interventions targeting cancer risk factors during adolescence; and (3) strategies to expand access to gynecologic oncology services.

Methods

A multi-phase, multi-component approach was used to build capacity at participating NCCCP sites to adapt, implement, evaluate, and sustain EBIs. Activities included focus groups, information sessions with pilot implementers, a three-day interactive symposium incorporating the *Putting Public Health Evidence in Action* (PPHEIA) curriculum, and a virtual learning collaborative offering ongoing technical assistance, mentorship, and peer learning. Changes in implementation competencies were assessed using pre/post paired t-tests.

Findings

A total of 89 program staff and coalition members from 41 NCCCP sites (representing 32 states, 2 tribal organizations, and 7 U.S. jurisdictions) engaged in SWW activities. Baseline assessments revealed common capacity gaps in staffing, partnerships, funding, and infrastructure, particularly in rural areas. Participants also reported low confidence in planning for EBI implementation.

Following SWW, participants demonstrated statistically significant improvements in implementation competencies across all five core learning objectives: understanding (10.2 to 12.8), adapting (13.3 to 16.8), implementing (12.9 to 17.2), evaluating (13.7 to 17.4), and sustaining (15.8 to 20.4) EBIs.

Implications for D&I Research

The SWW program demonstrates how structured, network-supported capacity-building can enhance the dissemination and implementation of EBIs in cancer prevention and control. By leveraging scalable strategies such as training, mentorship, and peer exchange, SWW strengthened implementation readiness across NCCCP sites. This model provides a replicable approach for accelerating the public health impact of D&I science, particularly in medically underserved and rural communities.

Primary Funding Source: Centers for Disease Control and Prevention

S31

Training global D&I leaders: insights from the CPRN Scholars Program

Cam Escoffery¹, The CPRN Scholars Workgroup²

¹Rollins School of Public Health, Emory University, Atlanta, GA, USA; ²The Cancer Prevention and Control Research Network, Chapel Hill, NC, USA

Correspondence: Cam Escoffery (cescoff@emory.edu)

Implementation Science 2026, **21**(2):S31

Background

The CPRN Scholars Program was established in 2021 to prepare early-career professionals in dissemination and implementation (D&I) science to address challenges in cancer prevention and control across public health, clinical, and academic settings. Targeting graduate students, postdoctoral researchers, early-career faculty, and practitioners, the program emphasizes workforce development through applied learning, mentorship, and integration into the CPRN. The program's primary goal is to strengthen D&I capacity by combining knowledge-building with real-world experience and peer collaboration.

Methods

Developed by a national workgroup of CPRN investigators, the year-long CPRN Scholars program includes asynchronous training modules (i.e., NCI Implementation modules/Putting Public Health Evidence into Action curriculum), monthly webinars with national experts, and mentorship from experienced D&I scientists. Scholars also join CPRN workgroups and complete a mentored project that applies D&I methods to a priority issue in cancer prevention and control. The program encourages cross-institutional learning, professional networking, and active participation in collaborative research. A mixed-methods evaluation across three Scholar cohorts ($n=62$) included pre/post surveys, semi-structured exit interviews, and a nine-month alumni follow-up to assess changes in D&I knowledge, program satisfaction, and continued involvement in implementation science.

Findings

Participants reported a significant increase in the total D&I competency scores for all three cohorts for 4 overarching domains and 43 subdomains ($M_{Pre}=1.38$ $M_{Post}=1.89$), particularly in theory-informed planning and adaptation of evidence-based interventions. Most CPRN Scholars completed a mentored project and engaged in CPRN workgroups, which facilitated further skill development and collaboration. Scholars reported gains in confidence, conceptual clarity, and ability to apply D&I approaches. Alumni have completed dissertations, presented at national conferences, published peer-reviewed manuscripts, secured faculty roles, and remained active in D&I research networks, suggesting sustained engagement in the field. About 80% remain engaged with CPRN workgroups and investigators.

Implications for D&I Research

The CPRN Scholars Program offers a replicable model for training early-career professionals in cancer D&I science. Findings support the value of integrating formal instruction with mentored project work and network participation. The program may inform future efforts to expand D&I capacity across systems involved in cancer prevention and population health.

Primary Funding Source: Centers for Disease Control and Prevention

Clinical care settings: patient-level interventions

S32

Implementation facilitation of motivational interviewing to activate pain supports (MAPS) for veterans separating from military service

Steve Martino^{1,2}, Marc Rosen³

¹Veterans Health Administration, West Haven, CT, USA; ²Yale University School of Medicine, New Haven, CT, USA; ³Yale School of Medicine, Department of Psychiatry, West Haven, CT, USA

Correspondence: Steve Martino (steve.martino@yale.edu)

Implementation Science 2026, **21**(2):S32

Background: Over half of veterans at the Department of Veteran Affairs (VA) in the United States have musculoskeletal disorders (MSD) connected to military service. MAPS helps veterans seeking MSD VA disability compensation engage in pain care. In a 1,101 person, 8-site, 36-week, Type 1 hybrid trial, veterans receiving MAPS, compared to those in usual care, had significantly more motivation for pain care, more pain care encounters, and reduced pain intensity, pain interference, and impaired work ability. We sought to implement MAPS more widely and sustainably in VA.

Methods: We partnered with the VA's Post-9/11 Military2VA (M2VA) Case Management Program, where case managers serve recently separated veterans, including those with MSD VA disability claims. M2VA Case Managers screen every recently separated Veteran new to VA health care for case management needs, ensuring they can connect to necessary care, services and resources. MAPS, based on motivational interviewing, is compatible with this case management approach. We used Implementation Facilitation (IF), with a Relational Coordination framework, to support case managers' reach, adoption, and implementation of MAPS with veterans.

Findings: National M2VA leadership collaborated in the design and obtainment of an NIH-funded Type 2 hybrid trial testing the effectiveness of IF to support the use of MAPS in the program. Pre-implementation activities included presentations to key stakeholders, tailored IF activities and MAPS to fit M2VA workflow, and input from a formative evaluation. M2VA leadership recruited an employee to support MAPS implementation and helped secure 14 VA facilities to receive IF for MAPS. IF teams consisted of local pain experts as internal facilitators, M2VA program managers as champions, and pain and addiction treatment experts with IF experience as external facilitators. Eighty-eight case managers treating 30,000 new veterans annually were trained in MAPS, facilitating its use in Veteran engagement. National efforts to reduce the VA workforce have complicated MAPS implementation, though facility participation remains robust.

Implications for D&I Research: During a period of institutional change, invested national leadership, tailored IF and MAPS to fit M2VA practice, and continued organizational support through IF have been essential for sustaining MAPS and integrating M2VA into VA pain care.

Primary Funding Source: National Institutes of Health

S33

Barriers and facilitators to implementing a telehealth mindfulness-based intervention for pain within the VA healthcare system

Diana Burgess

Veterans Health Administration, Minneapolis, MN, USA; Minneapolis, MN, USA

Correspondence: Diana Burgess (diana.burgess@va.gov)

Implementation Science 2026, **21**(2):S33

Background: The objectives of the LAMP (Learning to Apply Mindfulness to Pain) trial was to test the effectiveness of two scalable, mindfulness-based interventions (MBIs) for Veterans with chronic pain and collect and describe implementation data.

Methods: Hybrid Type 1 effectiveness-implementation trial of two mindfulness-based, telehealth interventions, delivered via self-paced and group-based formats. We collected pre-implementation data from multi-level stakeholders, including leaders from Veterans Affairs (VA)

program offices who would be responsible for administering LAMP. Analysis focuses on key facilitators and barriers to implementation, organized using the Consolidated Framework for Implementation Research.

Findings: Trial results showed that both MBIs were effective at improving pain and pain-related outcomes among Veterans ($N=811$). We are now implementing the self-paced LAMP in VA within pain management teams, and there is a pending project to implement LAMP through virtually delivered academic detailing services in rural sites. This success in moving LAMP into clinical practice was informed by implementation science, including development of multi-level stakeholder relationships, who helped identify barriers and facilitators and provided input on adaptation of MBIs to fit the local context. The LAMP intervention was perceived as addressing an important need and aligned with VA priorities, with the self-paced MBI seen as low-resource and relatively easy to implement. LAMP was also seen as credible because it was studied with VA patients and published in a recognized journal. Outer setting facilitators were its alignment with Congressional mandates, the opioid epidemic, and ability to be delivered to rural Veterans via telehealth. The process of implementation was facilitated by ongoing engagement with operational partners, stakeholders, and champions, including through collaboration within the Pain Management Collaboratory (PMC). Outer setting barriers included federally driven plans to significantly restructure VA and reduce the VA workforce. These outer setting barriers negatively affected the inner setting, with project partners and champions facing competing priorities and uncertainties.

Implications for D&I Research: Results underscore the importance of close and ongoing collaboration with operational partners, and the benefits of a structure such as the PMC that facilitates collaboration, as well as potential outer setting barriers.

Primary Funding Source: Department of Veterans Affairs

S34

Whole health options and pain education: new findings from the wHOPE trial and experiences using dissemination and implementation strategies

Karen Seal

San Francisco VA Health Care System, San Francisco, USA

Correspondence: Karen Seal (Karen.Seal@va.gov)

Implementation Science 2026, **21**(2):S34

Background: Chronic pain is a significant problem within the Veterans Health Administration (VA). VA has promoted the “Whole Health” model, a patient-centered approach emphasizing multi-modal non-pharmacological, complementary integrative health (CIH) approaches (e.g., yoga, mindfulness) and interdisciplinary care for chronic pain. To date, only observational evidence supports the use of the Whole Health model in clinical care. In this 12-month pragmatic Type 1 hybrid implementation-effectiveness trial, 764 veterans with chronic pain were enrolled across 6 VA healthcare systems. Results showed that a Whole Health Team approach was superior to Cognitive Behavioral Therapy for Chronic Pain and Usual VA Pain Care in improving pain interference and other key secondary outcomes (e.g., pain severity, pain treatment satisfaction). With these positive trial results, we aimed for broader and sustainable implementation within the VA system.

Methods: The VA Office of Patient-Centered Care that developed the Whole Health model and the Office of Pain Management that funds Pain Management Teams have facilitated virtual meetings with VA clinical and operational study partners to disseminate results. Evidence-Based Quality Improvement (EBQI) methods have guided the presentation of trial results and solicitation of partner input regarding scaling of the Whole Health Team approach in VA clinical care settings. Emergent themes are analyzed using rapid qualitative methods.

Findings: To date, the research team has completed several EBQI meetings with the following operational partners: national-level VA

leadership, local VA facility leadership in Pain Management and Whole Health, and volunteer wHOPE clinicians at each study site. Meetings to obtain veteran input are planned, and results from a budget impact analysis are forthcoming. In presenting trial results favoring the Whole Health Team approach for chronic pain management that includes a Whole Health coach conducting Personalized Health Planning in collaboration with pain and CIH clinicians, trial partners have suggested strategies to make incremental budget-neutral changes to existing Pain Management Teams to incorporate the Whole Health model.

Implications for D&I Research: Ongoing VA and veteran partner input and buy-in using D & I strategies shows promise in further scaling the Whole Health Team approach to improve chronic pain management across the VA healthcare system.

Primary Funding Source: National Institutes of Health

S35

Building toward national implementation: findings and outcomes of the Cooperative Pain Education and Self-management (COPES) Expanding Treatment for Real-world Access (Extra) trial

Alicia Heapy

Veterans Health Administration, West Haven, CT, USA

Correspondence: Alicia Heapy (alicia.heapy@va.gov)

Implementation Science 2026, **21**(2):S35

Background: Cognitive behavioral therapy for chronic pain (CBT-CP) is a first line nonpharmacological treatment that improves pain and functional outcomes, but access barriers and treatment burden reduce uptake. A clinician-guided asynchronous version of CBT-CP called COPES is effective for improving pain related outcomes but the best way to implement it in the VA system is unknown. Over several trials we evaluated implementation strategies to address patient and system barriers to the use of COPES culminating in the COPES ExTRA Type 1 hybrid implementation-effectiveness trial of 764 veterans with chronic pain enrolled across 9 VA healthcare systems. COPES was superior to standard CBT-CP in improving pain interference, pain intensity, depressive symptoms, and sleep. COPES participants attended significantly more sessions, waited fewer days to begin treatment, and had lower healthcare costs in the year following treatment compared to standard CBT-CP. Based on these results, we pursued wider implementation of COPES in VA.

Methods: A series of prior trials implementation trials shaped the selection and refinement of implementation and treatment delivery strategies used in COPES ExTRA. Sustained partnership with the VA Pain Management, Opioid Safety and PDMP Program (PMOP) provided additional information about VA system needs. Collaboration with the VA Virtual Care QUERI resulted in a new VA-funded hybrid Type 3 implementation trial to evaluate implementation strategies tailored for deploying COPES in rural community-based clinics.

Findings: COPES ExTRA used a combination of implementation strategies identified in prior COPES trials. These included centralized coordination and treatment delivery, patient self-referral mechanisms, and low-burden electronic health record alerts that trigger during clinical visits to improve referral. Based on the strength of COPES ExTRA findings and with the support of PMOP, COPES is now available in VA sites across six Northeastern states with a plan for eventual national implementation. The ongoing QUERI trial will leverage advanced implementation strategy selection methods (e.g., concept/intervention mapping, causal pathway diagrams) to optimize implementation of COPES in rural settings.

Implications for D&I Research: Efforts are underway to scale COPES for systemwide implementation. Successive implementation studies, bundling implementation strategies, and sustained partnership with PMOP have critically informed successful implementation in the complex, nationwide VA system.

Primary Funding Source: National Institutes of Health

S36

Scaling an evidence-based electronic patient reported outcomes (ePRO) program to improve cancer care across a multi-state health system: a system-level approach

Ursula Wright, Jessica Engelbrecht, Jay Carlson, Dana Haynie, Kerry Bommarito, John Mohart

Mercy Health, St. Louis, MO, USA

Correspondence: Ursula Wright (Ursula.Wright@Mercy.Net)*Implementation Science* 2026, **21**(2):S36

Background: Electronic patient-reported outcomes (ePROs) have demonstrated improvement in symptom management and reducing ED visits and hospitalizations among oncology patients. In 2023, Mercy Health, a multi-state health system, launched its AI-driven, symptom-focused, SMS based ePRO program for adult patients with non-leukemic cancers receiving chemotherapy and identified as highest risk for hospitalization. As a participant in the Patient-Centered Outcomes Research Institute's Health Systems Implementation Initiative (HSII), Mercy is applying dissemination and implementation (D&I) science to expand ePRO monitoring across three domains: physical function, behavioral health, health-related social needs, while broadening patient reach and access.

Methods: Mercy's expanded program targets adult outpatients (18+) receiving chemotherapy alone or with adjuvant therapy across 57 oncology-related clinics across four states. Multi-modal patient engagement (SMS, MyChart, automated calls) enhances accessibility, while automated alerts integrated into Mercy's single-instance EHR enable real-time clinical monitoring and response. Implementation is guided by the RE-AIM and PARIHS frameworks. A quasi-experimental cohort design compares outcomes between ePRO responders and non-responders. Key system-level strategies include structured design and go-live process, clinician training, standardized workflows, and real-time outcome tracking.

Findings: Early findings (Nov.2023-Dec.2024) identified 6,627 unique patients completed 49,014 surveys with 7% triggering escalation, most commonly severe pain (67%). Responders demonstrated 0.8 percentage point reduction in ED visits within 30 days of chemotherapy (14.8% vs.15.6%). Engagement was highest among women, married patients, and patients in Oklahoma communities; lower response among patients aged 76+. Mercy's expanded program will measure improved symptom control and physical functioning and assuring equitable reach across demographic groups. Early implementation efforts focused on program co-design with clinicians and patients and workflow standardization.

Implications for D&I Research: Mercy's system-level implementation demonstrates how large, diverse health systems can scale evidence-based interventions. Mercy demonstrates how system-level strategies, including integrating patient-centered technology, standardizing workflows and leveraging a unified EHR infrastructure, can proactively improve symptom management through early communication with the clinical care team—reducing ED utilization. It contributes to the D&I science by showing how structured implementation processes such as early stakeholder engagement, change management, and real-time monitoring can be effective for operationalizing and sustaining evidence-based complex interventions in real-world clinical settings, and providing a replicable model for other health systems.

Primary Funding Source: Patient-Centered Outcomes Research Institute

S37

Cost-effectiveness analysis of digital medicine support models for alcohol use disorderXiang Li¹, Randall Brown², Alice Pulvermacher¹, Andrew Quanbeck³¹University of Wisconsin Madison, Madison, WI, USA; ²University of Wisconsin School of Medicine & Public Health, Madison, WI, USA; ³University of Wisconsin – Madison, Madison, WI, USA**Correspondence:** Xiang Li (xiang.li2@wisc.edu)*Implementation Science* 2026, **21**(2):S37**Background**

Alcohol use disorder (AUD) is a major public health issue, contributing to substantial morbidity, mortality, and economic burden. Despite its prevalence and impact on health, many patients with AUD (particularly those with mild-to-moderate AUD) go unrecognized or do not receive appropriate intervention. Digital health apps offer scalable solutions, but health systems require rigorous evaluations to inform adoption. This paper presents a cost-effectiveness analysis from a randomized clinical trial comparing three digital health support models for AUD: self-monitored (SM) use (no human support), peer-supported (PS) use (1:many support by peer coaches), and clinically integrated (CI) use (1:1 support by a certified health coach).

Methods

From a health system perspective, we estimated the delivery costs of each model and assessed effectiveness based on reduction in heavy drinking days (4+ drinks for women, 5+ drinks for men). A cost-effectiveness analysis was conducted to evaluate tradeoffs and inform decision-making around program adoption at varying scales of implementation.

Findings

Among 558 participants, all 3 models statistically significantly reduced heavy drinking days from baseline, with annualized reductions of 46, 54, and 75 days per patient for the SM, PS, and CI groups, respectively. Per-patient costs were \$103, \$953, and \$688, respectively. Average cost-effectiveness (cost per reduced heavy drinking day) was \$2.24 (SM), \$17.65 (PS), and \$9.17 (CI). Compared to SM, the incremental cost-effectiveness ratio for CI was \$20.17 per additional reduced drinking day; the PS model was dominated. Although between-group differences were not statistically significant, these results suggest potentially meaningful clinical and economic differences.

Implications for D&I Research

Economies of scale are important to consider for health systems implementing peer supported or clinically integrated use. Health coaching represents the most intensive and effective option, with relatively low startup costs but high marginal costs for each patient. High startup costs are required to implement monitoring and response systems for peer-supported use, making it only feasible to implement at scale (i.e., for patient populations above 500). Individuals may have different preferences regarding the level of support provided in using a digital health tool; thus, providing a range of support models may allow for digital health tools to better meet diverse needs of a population.

Primary Funding Source: National Institutes of Health

S38

Microcosting the implementation of extended-release buprenorphine in rural settingsAmelia Bennett¹, Philip Jeng¹, Danielle Ryan¹, Larissa Mooney², Yih-Ing Hser³, Sean Murphy¹¹Weill Cornell Medicine, New York, NY, USA; ²UCLA David Geffen School of Medicine, Los Angeles, CA, USA; ³University of California, Los Angeles, Los Angeles, CA, USA**Correspondence:** Amelia Bennett (agb4001@med.cornell.edu)*Implementation Science* 2026, **21**(2):S38

Background: Approximately 24% of the US population live in rural counties, 35% of which are classified as high-need regarding access to medications for opioid use disorder (MOUD). Extended-release buprenorphine (XR-BUP) may decrease barriers to rural MOUD, thanks to fewer required doses than sublingual buprenorphine (SL-BUP). The present study aims to estimate the “real world” resources/costs necessary to implement and sustain an XR-BUP intervention in rural settings.

Methods: The parent study is a randomized controlled pilot trial evaluating the effectiveness and feasibility of XR-BUP in rural settings. Participants were randomized 2:1 into XR-BUP or SL-BUP for an intervention period of 14 weeks. An activity-based microcosting analysis was conducted at 3 sites, 2 being primary care facilities with SUD providers and one specialty outpatient recovery clinic. The microcosting analysis entailed semi-structured interviews with relevant personnel to understand the resources required to implement and sustain each

intervention. Resources were divided into fixed start-up (supplies, initial staff trainings, etc.), time dependent (recurring meetings, certifications), and variable (participant treatment costs). Variable cost estimates assumed a census of 48 patients annually, per-site, assuming all patients moved from SL-BUP to XR-BUP in a real-world setting. Sensitivity analyses were run varying the percentage of those who transition from SL-BUP to XR-BUP. Nationally representative costs were assigned to each resource unit.

Findings: First-year implementation costs were estimated to be an average of \$54,032. Subsequent year sustainment costs were estimated to be \$52,413. On average, Fixed start-up costs were \$1,619 and time dependent costs were \$15,867 across sites. Average annual variable costs were \$36,545. The analysis utilized a “real-world”, clinic-level perspective, excluding research and medication costs and assessing the costs of treating patients in a real-world setting. Assuming 48 patients transitioned from SL-BUP to XR-BUP, we found the additional cost per participant on XR-BUP was \$71, an overall average difference of \$3,434 to maintain a patient population of 48 on XR-BUP versus SL-BUP.

Implications for D&I Research: A comprehensive understanding of the resources required to support rural MOUD will allow stakeholders to mitigate financial risk and uncertainty. Well-informed implementation will assist decision makers in promoting effective and sustainable integration of XR-BUP in rural settings.

Primary Funding Source: National Institutes of Health

S39

Implementation outcomes of nicotine replacement therapy sampling in community dental practices: a national dental practice-based research network cluster-randomized trial

Sarah Helseth¹, Christopher Dunne¹, Devyn Fernholz², Douglas Levy³, Brad Rindal⁴, Tamara Lischka⁵, Pearl Fang², Arthur Truong⁶, Denis Nyongesa⁵, Sarah Basile⁷, Amanda Gillesby⁸, Pat Ragusa⁹, Victoria Thomas¹⁰, Rita Cacciato¹⁰, Dorota Kopycka-Kedzierawski¹⁰, Emily Welle¹¹, Matthew Carpenter¹², Sandra Japuntich¹³, The National Dental PBRN Collaborative Group¹⁴

¹Northwestern University Feinberg School of Medicine, Chicago, IL, USA;

²Hennepin Healthcare Research Institute, Minneapolis, MN, USA; ³Mas-

sachusetts General Hospital, Boston, MA, USA; ⁴HealthPartners Institute,

Bloomington, MN, USA; ⁵Center for Health Research, Portland, OR, USA;

⁶Kaiser Permanente Center for Health Research, Portland, USA; ⁷Health-

Partners Institute for Education and Research, Bloomington, MN, USA;

⁸HealthPartners Institute, Bloomington, USA; ⁹University of Rochester

Medical Center, Rochester, NY, USA; ¹⁰University of Rochester Medical

Center, Rochester, USA; ¹¹Hennepin Healthcare Research Institute, Min-

neapolis, USA; ¹²Medical University of South Carolina, Charleston, USA;

¹³Hennepin Healthcare, Minneapolis, MN, USA; ¹⁴None, Birmingham, AL, USA

Correspondence: Sarah Helseth (sarah.helseth@northwestern.edu)

Implementation Science 2026, 21(2):S39

Background: Oral health provider (OHP) best practices recommend Assessing tobacco use; Advising smokers to quit; and Referral to state quitlines (AAR). AAR connects smokers to counseling but not medications. Giving free Nicotine Replacement Therapy samples (NRTS) has reduced smoking in primary care; it has not been examined in community dental contexts.

Methods: We present feasibility and acceptability outcomes from a hybrid type 1 cluster-randomized trial conducted in the National Dental Practice-Based Research Network (NationalDentalPBRN.org). Practices delivered AAR and were randomly assigned to provide either a 2-week supply of NRTS or an electric toothbrush (ET). NRTS participants included 36 OHPs ($n = 23$ dentists & dental therapists, $n = 13$ hygienists) and 486 patients who smoke. Providers rated Organizational Readiness to Implement Change (ORIC), Feasibility (FIM) and Acceptability (AIM) of Intervention Measures, and Intervention Appropriateness (IAM) at pre- and post-implementation. Patient acceptability was a post-intervention satisfaction questionnaire. A subsample of OHPs ($n = 15$) and patients ($n = 11$)

completed interviews about their experiences with tobacco cessation, NRTS, and future sustainability and scale-out; coding followed best practices.

Findings: Overall, 50% of NRTS patients used either nicotine patches (39%) or lozenges (32%); they reported liking the patch ($M = 5.6/10$, $SD = 2.7$) and lozenges ($M = 5.1/10$, $SD = 3.0$), indicating acceptability. OHP-reported readiness was high but decreased post-implementation (pre-post mean difference 3.2, $p = 0.009$); dentists (pre-post mean difference 4.3, $p = 0.025$) drove the effect, vs hygienists (mean difference 1.19, $p = 0.186$). OHPs rated NRTS feasible, acceptable and appropriate; scores did not change post-implementation ($ps > 0.377$). However, NRTS practices recruited fewer participants than ET practices ($p = 0.03$). Interviewees liked NRTS and found it appropriate for dental settings. Despite instructions to recruit all comers, OHPs noted challenges with recruitment of patients not ready to quit. Patient- and provider-level barriers to sustainability and ideas for scale out were discussed.

Implications for D&I Research: OHPs are ideal to support smoking cessation. Despite strong acceptability, dentists reported lower feasibility post-implementation; hygienist-reported feasibility was steady, indicating role-specific barriers among OHPs that may require additional supports. Patients also noted barriers to longer-term NRT use, indicating NRTS may benefit from enhancement to sustain NRT use.

Primary Funding Source: National Institutes of Health

S40

Insights from a rollout implementation trial of a multi-level strategy for improving depressive symptoms in cardiac patients: the iHeart DepCare hybrid II trial

Pooja Sonikar¹, Jennifer Mizhquiri Barbecho¹, Nohora Ospina¹, Samantha Simantiris², Meredith Diamond¹, Daniela Suarez-Rebling³, Siqin Ye¹, Joseph Schwartz¹, Nathalie Moise¹

¹Columbia University Irving Medical Center, New York, NY, USA; ²Care

Access, New York, USA; ³Columbia University, New York, NY, USA

Correspondence: Pooja Sonikar (ps3335@cumc.columbia.edu)

Implementation Science 2026, 21(2):S40

Background: Guidelines recommend preference-driven depression treatment to reduce the risk of recurrent cardiac events. We applied behavioral and implementation science methods to develop IHEART DEPCARE, a multi-level, theory-informed strategy centered on a digital patient activation tool (eSDM tool) to support patient decision-making. We aimed to assess the strategy's impact and offer insights into rollout implementation trial and hybrid II designs.

Methods: Due to COVID-19 and limitations in EHR depression screening data, we adapted our stepped-wedge design to a pre-post, single-arm, open-label rollout across four sites (eight primary care and cardiology clinic clusters) randomized to implementation timing. English- and Spanish-speaking adults (≥ 21 years) with CHD and PHQ-9 ≥ 10 were enrolled between 2019–2024 and completed baseline and 6-month follow-up surveys. As a cluster entered post-implementation, behavioral health providers received problem-solving/learning collaboratives (LCs), clinicians received education and preference alerts, and patients received the eSDM tool. Outcomes included Beck Depression Inventory (BDI, primary), decisional conflict, patient activation, treatment barriers, and 6-month self-reported treatment use. Multi-level logistic regression accounted for four clusters. Implementation determinants were assessed using CFIR 2.0 via LC transcripts, clinician interviews, and patient exit surveys.

Findings: Of 2,118 patients approached, 2,086 were screened and 336 enrolled (181 pre-, 155 post-implementation); mean age 67.8 (SD 12.1); 39% Non-Hispanic White, 13.7% Non-Hispanic Black, 17% Other, 11.9% Missing; 47% had a high school education or less. No demographic differences were observed pre- vs post-implementation. There were no significant changes in BDI (17.1 vs. 18.6), patient activation (58.9 vs. 61.7), or new treatment at 6 months (31 vs. 41). Decisional conflict was significantly lower post-implementation (25.1 vs. 36.9). Qualitative data showed the intervention prompted clinician follow-up (e.g.,

referrals, care adjustments) and facilitated communication between patients, providers, and care teams.

Implications for D&I Research: Our study demonstrates the limited effect of a theory-informed eSDM tool on depressive symptoms and treatment use in real world cardiology and primary care settings. It may be that key multi-level barriers were difficult to overcome, particularly in the context of COVID19, specialty settings, and complex cardiac patient populations. Nonetheless, we demonstrate novel statistical approaches to adapting and analyzing rollout trials and potential mechanistic targets for future research.

Primary Funding Source: National Institutes of Health

S41

Comparing two strategies for implementing evidence-based care for women Veterans: a hybrid type 3 trial using the Stages of Implementation Completion®

Erin Finley^{1,2}, Alison Hamilton^{2,3}, Ariel Lang^{4,5}, Tannaz Moin^{2,6}, Bevanne Bean-Mayberry^{2,6}, Lisa Saldana⁷, Rebecca Oberman², Ismelda Canelo², Karissa Fenwick², La Shawnta Jackson⁸, Rachel Lesser², Melissa Farmer²
¹Long School of Medicine, University of Texas Health Science Center, San Antonio, TX, USA; ²Veterans Health Administration, Los Angeles, CA, USA; ³David Geffen School of Medicine, University of California, Los Angeles, Los Angeles, CA, USA; ⁴Veterans Health Administration, San Diego, CA, USA; ⁵University of California San Diego, San Diego, CA, USA; ⁶UCLA David Geffen School of Medicine, Los Angeles, CA, USA; ⁷Chestnut Health Systems-Lighthouse Institute, Eugene, OR, USA; ⁸Veterans Health Administration, Los Angeles, USA

Correspondence: Erin Finley (erin.finley@va.gov)

Implementation Science 2026, 21(2):S41

Background: The Department of Veterans Affairs (VA) EMPOWER 2.0 Quality Enhancement Research Initiative (QUERI) conducted a cluster-randomized hybrid type 3 effectiveness-implementation trial comparing Replicating Effective Programs (REP), a lighter touch strategy involving quarterly technical assistance, and Evidence-Based Quality Improvement (EBQI), a more intensive strategy involving monthly facilitation. These strategies supported implementation of three evidence-based practices (EBPs) focused on virtual care for women Veterans: Telephone Lifestyle Coaching (TLC) and asynchronous Diabetes Prevention Program (DPP), to prevent cardiovascular disease and diabetes, respectively; and Reach Out, Stay Strong Essentials (ROSE), to prevent postpartum depression. We report trial findings comparing progression to implementation competence across 21 VA sites enrolled 2021–2024.

Methods: The Stages of Implementation Completion® (SIC) measures progress across diverse EBPs, settings, and strategies. We developed an adapted EMPOWER-SIC capturing implementation activities across each EBP (TLC, DPP, ROSE) and strategy (REP, EBQI). Sites were randomized to either REP or EBQI and, based on geographic cluster, offered the opportunity to implement either (1) TLC and/or ROSE; or (2) DPP and/or ROSE. On trial completion, we assessed sites' progression to "implementation competence," defined as achieving the pre-set patient enrollment goal for each EBP (TLC/DPP: 40 women; ROSE: 10 women).

Findings: Among 6 TLC sites, 67% of REP sites and 33% of EBQI sites progressed to competence. Among 14 DPP sites, 33% of REP sites and 50% of EBQI sites progressed to competence. Among 21 ROSE sites, 45% of REP sites and 50% of EBQI sites progressed to competence. Across EBPs, both REP and EBQI supported progression to competence for some sites, although site-level challenges related to staffing, varied size of patient population, role scope and coordination, and competing demands remained significant. TLC sites faced the additional challenge of a shortened timeline for enrollment, as the contract for TLC services was canceled unexpectedly.

Implications for D&I Research: While implementation trials remain the gold standard for assessing the effectiveness of distinct implementation strategies, these findings underscore the challenges of real-world implementation. Both REP and EBQI were successful in

achieving implementation competence in some sites. DPP and ROSE were more complex interventions and implementing sites may have derived additional benefit from the higher-intensity EBQI strategy.

Primary Funding Source: Department of Veterans Affairs

S42

Implementing structured interdisciplinary bedside rounds to improve team-based care for hospitalized oncology patients: a co-designed quality improvement initiative

Patricia Litkowski¹, Nino Naskidashvili¹, Mark Williams², Jing Li³

¹Washington University in St. Louis, Saint Louis, USA; ²Washington University in St. Louis, St. Louis, USA; ³University of Alabama at Birmingham, Birmingham, AL, USA

Correspondence: Jing Li (jingli2@uabmc.edu)

Implementation Science 2026, 21(2):S42

Background: Hospitalized oncology patients require highly coordinated, team-based care. Hospitalist-led interdisciplinary bedside rounds (IDRs) have the potential to improve communication, care planning, and patient-centered decision-making. Structured models on medical floors such as the Interprofessional Teamwork Innovation Model (ITIM) and the TEMP checklist have demonstrated positive outcomes. However, implementing IDRs in high-acuity oncology settings poses challenges, including workflow disruptions and inconsistent team engagement.

Methods

This initiative was conducted on the Oncology Hospital Medicine co-management units at an academic medical center using a structured, three-phase process:

1. Context Assessment: Conducted workflow observations, individual interviews with frontline clinicians, and engaged operational and clinical leaders to identify barriers and opportunities.
2. Model Design & Development: Co-designed the Oncology I-TEAM Care Model, a structured IDR approach tailored to the unit's clinical and operational needs. Developed standardized rounding protocols and conducted targeted training.
3. Pilot Implementation: Initiated a mini-pilot, followed by a three-month, weekly, iterative pilot. The project coordinator documented rounds and gathered real-time feedback from frontline to support continuous adaptation. The pilot included over 500 inpatients, 26 hospitalists, ~60 nurses, and additional interdisciplinary team members (pharmacists, case managers, social workers). Physician and nurse focus groups were held to assess feasibility, acceptability, and perceived impact.

Findings: Nurses, pharmacists, and case managers attended Oncology I-Team 68%, 73%, and 77% respectively, while hospitalist facilitating all rounds discussion. The average time in patient room was 6 min (SD 3 min) and the average total rounding time for 13 patients was 65 min (SD 19 min). Clinicians reported that I-TEAM improved communication, strengthened care coordination, and supported more patient-centered decision-making. Reported implementation barriers included scheduling constraints, variable team participation due to staffing and clinical interruptions, and time constraints. The co-design process and ongoing engagement were critical in adapting solutions to these challenges. Pilot data suggested acceptability and feasibility, with qualitative findings supporting its impact on team collaboration and care experience.

Implications for D&I Research: This project illustrates the value of structured, iterative implementation and frontline co-design in adapting complex care interventions. The model shows promise for scalability and provides a practical framework for advancing patient-centered, team-based oncology care in hospital settings.

Primary Funding Source: Barnes-Jewish Hospital Foundation Grant

S43

Implementation of medication adherence assessments in VA primary care clinics led to better chronic medication adherence for high-risk patients

Timothy Anderson^{1,2}, Stephen Frochen³, Franya Hutchins^{1,4}, Natasha Torrence¹, Susan Stockdale³, Elvira Jimenez^{3,5}, Michelle Wong³, Evelyn Chang^{3,5}, Ashok Reddy^{6,7}, Ann-Marie Rosland^{1,8}

¹Veterans Health Administration, Pittsburgh, PA, USA; ²University of Pittsburgh School of Medicine, Pittsburgh, USA; ³Veterans Health Administration, Los Angeles, CA, USA; ⁴University of Pennsylvania Perelman School of Medicine, Philadelphia, PA, USA; ⁵University of California Los Angeles, Los Angeles, CA, USA; ⁶Veterans Health Administration, Seattle, WA, USA; ⁷University of Washington, Seattle, WA, USA; ⁸Division of General Internal Medicine, University of Pittsburgh School of Medicine, Pittsburgh, PA, USA

Correspondence: Ann-Marie Rosland (Ann-Marie.Rosland@va.gov)

Implementation Science 2026, **21**(2):S43

Background: Medication regimen complexity among patients with multimorbidity can lead to inconsistent use of medications for chronic conditions. The VA High-Risk VETerans (RIVET) QUERI Program implemented the Medication Adherence Assessment (MAA) evidence-based practice for high-risk primary care patients. We assessed adherence barriers detected and effects on chronic medication adherence.

Methods: The MAA assessed adherence barriers in five domains (mental health/substances, health literacy, cognitive impairment, health beliefs, socioeconomic) and prompted clinicians with strategies to address barriers detected. Patients empaneled to participating primary care teams with > 90th-percentile hospitalization-risk score were targeted; some sites focused further on patients with recent ED visits/hospitalizations or uncontrolled chronic conditions. Assessments were delivered by primary care staff. All patients receiving MAA and using at least one chronic medication class (cardiovascular, diabetes, antidepressant, respiratory) at baseline were matched 1:3 to controls based on demographics, risk score, medication class, and site. Cohort entry date was the MAA receipt date, or closest primary care visit date within 30 days for controls. Continuous proportion of days covered (PDC) assessed adherence at the drug class-level in the 6 months pre- and 6 months post-entry date. Difference-in-differences (DID) analyses compared change in adherence between groups, accounting for covariates and within-patient clustering.

Findings: 68 MAA patients at 6 sites (mean age 72 years, 85% male, 82% white) were matched to 202 controls. MAA identified health belief concerns most often (91% patients) and mental health/substance concerns least often (16%). Adjusted mean baseline PDC was 71.1% (95%CI, 58.3% to 84.0%) for MAA and 72.3% (95%CI, 60.6% to 85.3%) for controls. Post-period PDC declined in MAA (-4.6%; 95%CI -8.8%, -0.4%) and controls (-11.2%; 95%CI -13.7%, -8.8%), but the decline was significantly smaller in the intervention group (DID 6.7%; 95%CI 1.8%, 11.5%). Similar trends were observed across medication categories.

Implications for D&I Research: Delivery of MAA to high-risk patients by primary care team members in real-world settings was associated with detection of barriers to adherence and slower declines in chronic medication adherence over time. Health systems could find implementation of complex chronic care management practices particularly beneficial when targeted to the highest-risk patients in their population.

Primary Funding Source: Department of Veterans Affairs

S44

Intervention fidelity as a driver of implementation success: lessons from the thrive transitional care program

Heather Brom^{1,2}, Jennifer Gil^{1,2}, J. Margo Brooks Carthon^{1,2}

¹University of Pennsylvania School of Nursing, Center for Health Outcomes and Policy Research, Philadelphia, PA, USA; ²The Leonard Davis Institute of Health Economics, Philadelphia, PA, USA

Correspondence: Heather Brom (hmbrom@upenn.edu)

Implementation Science 2026, **21**(2):S44

Background

Fidelity measures are often inconsistently reported in many implementation studies, despite their importance in evaluating the effectiveness of complex interventions. Proper fidelity assessment ensures interventions maintain their effectiveness in new contexts while advancing equitable care delivery. This study evaluates fidelity to Thrive, an equity-centered, evidence-based transitional care program providing 30-day post-discharge support to Medicaid-insured individuals. Thrive includes wraparound services, home health, interdisciplinary case management, and extended engagement with hospital-based providers. The objective of this study is to (1) evaluate adherence, dosage, and timeliness of Thrive, and (2) examine whether variations in fidelity explain differences in 30-day readmissions, emergency department (ED) visits, primary care (PC), and specialty care (SC) utilization.

Methods

We conducted an intervention fidelity evaluation during two stepped-wedge randomized control trials of Thrive (March 2023-May 2025) at three teaching hospitals in Philadelphia, Pennsylvania. Fidelity measures were developed using expert consensus, including adherence (8 core components), dosage (up to 4 weeks), and timeliness (home health within 2 days of discharge). Descriptive statistics characterized fidelity measures. Chi-square and t-tests assessed relationships between fidelity and 30-day readmissions, ED visits, PC and SC visits post-discharge.

Findings

Among 308 Thrive participants, intervention fidelity was high, mean adherence was 7.2 components per patient (SD = 0.6), 81.5% received all four weeks (dosage) and 43.2% received timely home health within two days of discharge. Higher adherence was associated with increased PC visits (7.4 vs. 7.2 components; $p < 0.05$). Patients without readmissions more often received a social needs screening (99.2% vs. 93.7%; $p < 0.01$). Patients receiving more weeks of Thrive had more PC and SC visits (3.8 vs. 3.6 weeks, $p < 0.05$). There were no differences in ED utilization by fidelity.

Implications for D&I Research

This study demonstrates the benefits of fidelity monitoring in complex interventions. High Thrive fidelity was achieved, with increased dosage linked to enhanced PC and SC engagement, an essential connection for vulnerable populations. Results suggest that variations in fidelity may drive outcome differences among participants receiving the same intervention. Incorporating fidelity monitoring during implementation of complex interventions can reinforce protocols and promote equitable outcomes. These findings provide methodological guidance for future implementation studies seeking to demonstrate and sustain intervention benefits across diverse healthcare settings.

Primary Funding Source: Rita and Alex Hillman Foundation

S45

Tailoring site-specific implementation strategy plans for a cesarean risk calculator trial: a mixed-methods approach

Rebecca Hamm^{1,2}, Rebecca Clark^{3,4}, Mucio Delgado^{1,5}, Elizabeth Howell⁵, Sarah Hulse⁵, Alicia Hussey⁵, Lisa Levine^{1,5}, Mary Steele⁵, Alexandra DeRita⁶, Cara Dolin⁷, Brett Einerson⁸, Michelle Moniz⁹, Jessica Page¹⁰, Abigail Ramseyer¹¹, Dr. Meghan Lane-Fall¹²

¹Leonard Davis Institute of Health Economics, Philadelphia, PA, USA;

²Perelman School of Medicine, University of Pennsylvania, Philadelphia, PA, USA;

³University of Pennsylvania School of Nursing, Center for Health Outcomes and Policy Research, Philadelphia, PA, USA;

⁴The Leonard Davis Institute of Health Economics, Philadelphia, PA, USA;

⁵University of Pennsylvania Perelman School of Medicine, Philadelphia, PA, USA;

⁶Henry Ford Macomb Hospital, Clinton Township, MI, USA;

⁷Cleveland Clinic, Cleveland, OH, USA;

⁸University of Utah, Salt Lake City, UT, USA;

⁹University of Michigan, Ann Arbor, MI, USA;

¹⁰Healthcare Delivery Institute, Office of Research, Intermountain Healthcare, Murray, UT, USA;

¹¹University of Michigan Health Sparrow, Lansing, MI, USA;

¹²Columbia University in the City of New York, New York, NY, USA

Correspondence: Rebecca Hamm (Rebecca.feldmanhamm@pennm.edu)

Implementation Science 2026, **21**(2):S45

Background: Our prior single-site study demonstrated that patient and clinician awareness of individualized patient cesarean risk, facilitated by a novel calculator, was associated with reductions in maternal morbidity and cesarean rates. To prepare for a 14-site stepped-wedge type I hybrid effectiveness-implementation trial of this calculator, we developed a multi-phase mixed-methods approach to create context-specific implementation strategy plans that center the opinions of front-line clinicians.

Methods: Guided by CFIR 2.0 and the Implementation Research Logic Model, our embedded sequential mixed-methods study (QUAN->QUAL; 8/2023–9/2024) systematically identified implementation determinants at 14 diverse labor and delivery sites to inform selection of ERIC strategies most likely to achieve successful local implementation. Strategies were categorized as “set” (required across sites), “optional” (potentially beneficial based on preliminary qualitative data), or “emergent” (responsive to newly identified site-specific barriers). First, we established site-based leadership teams (3–15 obstetricians, midwives, nurses, doulas, and patient/community partners). Next, pre-implementation clinician surveys across sites assessed barriers/facilitators, and perceived benefit/feasibility of four “optional” strategies: (1) preparing patients as active consumers (outpatient calculator discussion), (2) enhancing patient uptake (preliminary risk score outpatient), (3) electronic health record integration, and (4) visual aids for counseling. QUAN data were presented to site-based leadership teams to generate preliminary plans. These plans were refined, including the selection of emergent strategies, and finalized through clinician focus groups at each site.

Findings: Frontline clinicians participated meaningfully across the survey (QUAN, $n = 437$) and focus groups (QUAL, 21 groups, $n = 92$). Cross-cutting barriers reported by clinicians included time constraints, buy-in, and competing priorities. Site-specific barriers also emerged, e.g. minimal physician/midwife presence on nurse-driven labor units for risk counseling. Perceived benefit/feasibility of the “optional” strategies varied across sites; leadership teams’ preliminary plans combined these ratings with personal judgement. Focus groups were crucial for finalizing plans and identifying “emergent” strategies, such as development of a direct-to-patient educational video for use at nurse-driven sites.

Implications for D&I Research: This innovative methodology allowed development of 14 context-specific implementation strategy plans that center frontline perspectives prior to a national trial. Ongoing work will evaluate how these tailored plans impact calculator penetration and outcomes after trial completion, offering a scalable model for future multi-site implementation research.

Primary Funding Source: National Institutes of Health

S46

Improving care for older surgery patients at hospital-to-home transitions through participatory adaptation applying CFIR, ADAPT, and FRAME

Sevdnur Cizginer^{1,2,3}, Ferhat Yildiz⁴, Matthew Vrees⁵, Christy Cauley⁴, Rocco Ricciardi⁴, Donna Ron⁶, Steve Bartels⁴

¹Harvard Medical School, Boston, MA, USA; ²Division of Palliative Care and Geriatric Medicine, Massachusetts General Hospital, Boston, MA, USA;

³Center for Aging and Serious Illness, Massachusetts General Hospital, Boston, MA, USA; ⁴Massachusetts General Hospital, Boston, MA, USA;

⁵Brown University, Providence, RI, USA; ⁶Meir Medical Center and Tel Aviv University, Kfar Sava, Israel

Correspondence: Sevdnur Cizginer (scizginer1@mgm.harvard.edu)

Implementation Science 2026, 21(2):S46

Background: Hospital-to-home transitions pose significant risks for older surgical patients, yet evidence-based transitional care models remain largely unadapted for surgical populations. This implementation gap persists despite proven models for medical patients. Our study addresses this by systematically adapting and implementing a transitional care intervention for surgical patients using mixed methods in light of implementation science frameworks.

Methods: We employed a multi-step implementation science process. First, guided by a Logic Model approach, we used mixed methods to systematically integrate core components of an evidence-based medical care transitions intervention (CTI) with an inpatient geriatric surgery co-management program (OSCAR), creating the OSCAR-S model. Second, we conducted 42 in-depth interviews with national stakeholders representing 14 disciplines to achieve three objectives: (1) identify implementation determinants using the Consolidated Framework for Implementation Research (CFIR), (2) adapt OSCAR-S using the ADAPT framework to enhance intervention-context fit, preserving core intervention functions, and (3) identify and apply optimal implementation strategies. Throughout this process, we systematically documented all adaptations using the FRAME. Data analysis employed mixed methods and framework matrix approaches.

Findings: Key adaptations to the OSCAR-S included initiating the care transition planning from the pre-surgery phase, integrating telehealth, virtual mentoring from a geriatrician, surgical-specific red flags in coaching materials, and modified workflow integration for surgical patients. Major implementation determinants included model’s comprehensiveness and established connections between nurse coach and surgical team as facilitators. Barriers included competing clinical priorities, cross-system care coordination challenges, and time-intensive patient needs. Implementation strategies addressed these determinants through structural and process adaptations (e.g. embedding nurse coaches in surgical clinics, process redesign including post-implementation feedback loops). As the next step, we will conduct feasibility implementation effectiveness trial of OSCAR-S program guided by RE-AIM framework.

Implications for D&I Research: Implications for D&I Research: This study applied a systematic approach to adapt non-surgical interventions for surgical contexts while preserving their core components, utilizing established frameworks to identify implementation strategies during the adaptation phase. The findings demonstrate that implementation research methods are feasible and contribute to improving care for older surgical patients, particularly in complex interventions spanning multiple care transitions.

Primary Funding Source: National Institutes of Health

S47

Adaptation & testing of an integrated shared decision-making tool for combined family planning and HIV prevention counseling implementation

Katherine Anderson¹, Anandi Sheth², Whitney Rice¹, Alexandra Morshed³, Jessica Sales⁴

¹Emory University Rollins School of Public Health, Atlanta, GA, USA;

²Emory School of Medicine, Atlanta, GA, USA; ³Rollins School of Public Health, Emory University, Atlanta, GA, USA; ⁴Emory University, Rollins

School of Public Health, Atlanta, GA, USA

Correspondence: Katherine Anderson (kmande9@emory.edu)

Implementation Science 2026, 21(2):S47

Background

Approximately 20% of new HIV infections in the United States occur among women, primarily from heterosexual contact. These infections are preventable. Title X clinics provided sexual health (SH) care to >2 million sexually active women in 2023, but performed fewer than half as many HIV tests. Still, in 2023, Title X clinics diagnosed ~8% of new HIV cases in the US. While essential, HIV testing is suboptimally provided and remains a reactive approach to HIV prevention compared with proactive counseling. Title X clinicians may lack resources- such as Family Planning (FP)-HIV integrated counseling materials- to conduct recommended universal prevention counseling with or without testing. Responsively, we aimed to systematically adapt and assess perceived feasibility and acceptability of an evidence-based FP shared decision-making tool to integrate HIV prevention into FP counseling, to ultimately inform future implementation, effectiveness testing, and scaling.

Methods: Thirteen FP providers at Georgia FP clinics and 28 cisgender women in FP/SH care in Atlanta completed formative interviews. Key adaptations were identified via rapid qualitative analysis and underwent modified Delphi review by 17 FP/SH/HIV practitioners/experts. Adaptations were systematically documented using the FRAME framework; surveys assessing perceived feasibility/acceptability with 30 practitioners and 40 SH care-engaged women will inform final adaptations by 10/2025. Activities were exempt from IRB approval.

Findings: Providers endorsed perceived risk-based HIV prevention counseling with minimal counseling tool availability. Preferences included concise, accessible language, availability for pre-visit patient engagement, with information on HIV prevalence, prevention method comparison (cost/side effects/usage/maintenance) and guided questions. Women preferred universal counseling with minimal personalization that could indicate judgement. Expert review highlighted usefulness of comparison charts for STI/HIV and pregnancy prevention modalities, physical materials with QR codes, counseling checklists, and conversation prompts. Information on PrEP side effects, maintenance visits/testing, effectiveness, and comparison to condoms were prioritized. Responsive adaptation was conducted, followed by preliminary feasibility and acceptability testing.

Implications for D&I Research: Title X clinics represent a high-impact environment for reaching women with HIV prevention counseling, dependent on patient- and provider-sensitive implementation tools and acceptable processes. Formative research and systematic adaptation of resources for HIV prevention are vital to acceptable implementation to reduce HIV burden domestically and globally.

Primary Funding Source: National Institutes of Health

S48

Evaluating the MyPath by text intervention for women Veterans: a hybrid type 1 cluster randomized controlled trial

Lisa Callegari¹, Karin Nelson¹, David Arterburn², Christine Dehlendorf³, Alison Hamilton⁴, Samantha Benson¹, Siobhan Mahorter¹, Rachel Hunter-Merrill¹, Leslie Taylor¹, Aarthi Yogendran¹, Caroline Merkel¹, Na'imah Muhammad¹, Shannon Mitchell⁵, Sonya Borrero⁶

¹Veterans Health Administration, Seattle, WA, USA; ²Kaiser Permanente Washington Health Research Institute, Seattle, WA, USA; ³University of California, San Francisco, San Francisco, CA, USA; ⁴Veterans Health Administration, Los Angeles, CA, USA; ⁵Veterans Health Administration, Pittsburgh, PA, USA; ⁶University of Pittsburgh, Pittsburgh, PA, USA

Correspondence: Lisa Callegari (lisa.callegari@va.gov)

Implementation Science 2026, 21(2):S48

Background: Discussions about individuals' reproductive goals and decisions are a critical component of comprehensive primary care but frequently missing from primary care visits at the Veterans Administration (VA). *MyPath* is a patient-facing web-based decision support tool designed to help users identify and discuss their reproductive goals and healthcare needs with providers. We assessed the effectiveness of the *MyPath* tool and evaluated one-way text messaging as a patient-facing implementation strategy.

Methods: In a hybrid Type 1 cluster randomized controlled trial with primary care provider (PCP)-level randomization, women Veterans ages 18–44 with an upcoming primary care visit received *MyPath* by Text prior to their appointment (intervention arm) or usual care (control arm). Sixty-six PCPs were randomized and 349 Veterans enrolled across 12 sites. Texts with a link to the tool were sent using VA's appointment reminder system. Effectiveness was measured post-visit by self-reported occurrence of a reproductive health discussion (fertility/preconception health, contraception) at the visit. Mixed-effects logistic regression with multiple imputation was used for intent-to-treat analysis; complier average causal effect (CACE) analysis was used to account for tool non-use. Text messaging was evaluated among intervention participants by survey ($n=169$) and qualitative interviews ($n=23$).

Findings: No differences were found between groups in occurrence of reproductive health discussions in intent-to-treat analysis, however only 49% of intervention participants reported using *MyPath*. In CACE analysis, those intervention participants who used *MyPath* were more likely to report discussions than controls (74.4% vs. 65.5%, OR 2.81, 95%CI: 1.02–7.73). Intervention participants valued the ease of accessing *MyPath* on their phone, however some described missing or ignoring the text, not remembering to complete the tool, and being unsure if the text confirmed that their provider was willing to discuss reproductive goals.

Implications for D&I Research: Effectiveness findings in the *MyPath* by Text trial were limited by higher-than-expected non-use of the tool. The study, however, generated novel insights on one-way text messaging as a patient-facing implementation strategy for decision support tools. As texts alone generated only moderate use of the tool, adding higher-touch strategies (e.g., secure messages in addition to text messages, direct outreach or prompting from clinical staff) could increase use and thus impact.

Primary Funding Source: Department of Veterans Affairs

S49

Behaviorally-informed text message reminders to increase colorectal cancer screenings: a randomized quality improvement study at federally qualified health centers

Cathy Jian^{1,2}, Jeremy Lu^{1,2}

¹NYU Langone Health, New York, NY, USA; ²NYU Grossman School of Medicine, New York, NY, USA

Correspondence: Cathy Jian (yin.jian@nyulangone.org)

Implementation Science 2026, 21(2):S49

Background: Colorectal cancer (CRC) screening rates among patients at the NYU Family Health Centers (FHCs)—a network of Federally Qualified Health Centers in Brooklyn, NY—remain low. While guidelines recently lowered the recommended screening age to 45, the eligible population has grown, increasing the need for scalable, low-cost outreach strategies. At the FHCs, providers are prompted to order annual fecal immunochemical tests (FIT) for eligible patients, which are distributed in-person or mailed after virtual visits at no cost. Nurses provide education and follow-up phone calls on Day 7 for unreturned tests. Despite these efforts, in 2024 only 43% of FITs were completed within four weeks. Prior studies of mailed FIT campaigns suggest that reminders using behavioral insights, particularly deadlines to support planning, are more effective than standard reminders. This study evaluates whether a behavioral economics-informed, automated text messaging intervention could increase FIT completion rates and reduce outreach burden on clinic staff compared to standard phone-based outreach.

Methods: In a six-week randomized trial, 740 patients with new FIT orders were assigned to either a control group (one phone call on Day 7) or an intervention group (text messages on Days 2, 5, and 8). Messages included a deadline, and return-location details tailored to the patient's home clinic. Texts were sent in English, Spanish, or Chinese based on the patient's preferred language. FIT completion was measured at Days 7, 14, and 21.

Findings: Text messages significantly outperformed phone calls with higher FIT completion rates at each follow-up interval with significant differences at Day 14 (52.0% vs. 39.6%, $p < 0.001$) and Day 21 (57.3% vs. 50.4%, $p = 0.024$).

Implications for D&I Research: This study demonstrates that a behaviorally-informed, automated text message strategy can improve FIT completion in diverse primary care settings. As a simple and low-cost approach, the intervention yielded measurable improvements in preventive care uptake with minimal operational burden, including saving an estimated 6.8 staff hours per month and enabling approximately 63 additional FITs to be completed within 14 days each month if all patients received text messages instead of phone calls. Future studies will explore message variations to further increase screening uptake and reduce disparities.

S50

Comparing the effectiveness of two mailed outreach interventions to increase colorectal cancer screening in community health centers

Folasade May¹, Suzanne Brodney², Jessica Tuan³, Beth Glenn⁴, Adjoa Anyane-Yeboah⁵, Dr. Andrew Chan², Sue Regan⁵, Sapna Syngal⁶, Kelley LeBeaux⁷, Kimberly Schoolcraft⁸, Gina Johnson⁷, Chinedu Ukaegbu⁶, Anjee Davis⁹, Beverly Moy⁵, Dr. Erica Warner², David Drew⁵, Yuchiaio Chang², Jennifer Haas²

¹UCLA, Los Angeles, CA, USA; ²Massachusetts General Hospital, Boston, MA, USA; ³UCLA, Los Angeles, USA; ⁴UCLA Kaiser Permanente Center for Health Equity, Los Angeles, CA, USA; ⁵Massachusetts General Hospital, Harvard Med S, Boston, MA, USA; ⁶Dana Farber Cancer Institute, Boston, MA, USA; ⁷Great Plains Tribal Leaders' Heath Board, Rapid City, USA; ⁸Boston, USA; ⁹Fight CRC, Springfield, MO, USA

Correspondence: Jennifer Haas (jennifer.haas@mgh.harvard.edu)
Implementation Science 2026, **21**(2):S50

Background: Community health centers (CHCs) provide primary care to over 31 million low-income and uninsured individuals, yet colorectal cancer (CRC) screening rates in these settings are below the national average. Annual fecal immunochemical tests (FIT) are an accessible, inexpensive stool-based screening method. FIT-DNA, a newer option recommended every three years, offers an alternative, though not widely used in CHCs.

Methods: Two-arm pragmatic trial to compare CRC screening rates using FIT and FIT-DNA across eight CHCs, four in Boston and four in Los Angeles. Eligible participants were English or Spanish-speaking adults aged 45 to 75 due for screening. Clinics were matched by region and size, then randomized to receive mailed outreach with either FIT or FIT-DNA. A parallel site in Rapid City, South Dakota (SD) also participated in the FIT-DNA arm but lacked a comparator. In the FIT arm, patients received a mailed kit with instructions and three text message reminders over 35 days. In the FIT-DNA arm, patients received a mailed kit accompanied by the test manufacturer's standard outreach, including phone calls, texts, and letters. The primary outcome was CRC screening completion within 90 days of kit mailing; secondary outcomes included screening at 180 days and colonoscopy follow-up after an abnormal result.

Findings: Among 5,931 patients across the three regions, 58.4% were female, 64.4% identified as Hispanic, 13.7% American Indian, 56.7% preferred Spanish, and 42.8% Medicaid insured. Baseline screening rates ranged from 24%-67%. Overall, screening completion was 24.3% at 90 days and 27.9% at 180 days. At randomized sites, 90-day screening was significantly higher in the FIT-DNA vs. FIT arm (27.9% vs. 22.6%, $p=0.019$), with a meaningful uptake in both arms. In SD, FIT-DNA completion was 17.7%. Among the 126 patients with an abnormal stool test result, colonoscopy completion rate was 38.1% at 180 days; median time to colonoscopy was 69 days.

Implications for D&I Research: These findings demonstrate that mailed FIT and FIT-DNA outreach is feasible in CHCs and that FIT-DNA yields higher CRC screening participation than FIT alone, supporting broader implementation in these underserved settings. Systems are needed to address barriers to colonoscopy among those with positive stool test results.

Primary Funding Source: Stand Up 2 Cancer

S51

A multilevel randomized clinical trial evaluating clinician training and text message reminders to increase cancer prevention from HPV vaccination among adolescents

Stephanie Staras¹, Tuo Lin¹, Pengchen Wang¹, Marta Hansen¹, Lindsay Thompson², Matthew Gurka³, Gabriela Sanchez¹, Xiaofei Chi¹, Christopher Harle⁴, Elizabeth Shenkman¹, Georges Khalil¹, Carma Bylund¹

¹University of Florida, Gainesville, FL, USA; ²Wake Forest Baptist Medical Center, Winston-Salem, NC, USA; ³University of Virginia, Charlottesville, VA, USA; ⁴Regenstrief Institute, Inc., Indianapolis, IN, USA

Correspondence: Stephanie Staras (sstaras@ufl.edu)
Implementation Science 2026, **21**(2):S51

Background: Human papillomavirus (HPV) vaccination rates among adolescents remain lower than the Healthy People 2030 goals. We evaluated the effectiveness of clinician trainings and text message reminders in increasing HPV vaccination rates among 10- to 12-year-olds.

Methods: Florida primary care clinics ($n=30$) participated in a randomized trial between March 2022 and September 2023 with follow-up until October 2024. By clinic, clinicians were randomly assigned to one of three trainings: Bundled (recommended adolescent vaccines), Focused (HPV vaccine focus), and deferred. We randomized parents of patients who had not received adolescent vaccines to text message arms (Bundled, Focused, or none). Messages were sent initially (study start or 11th birthday) and with appointment reminders. We estimated Risk Ratios (RRs) for the effects of the interventions on two-year HPV vaccine initiation (1 dose) and up to date (UTD) (2 doses) with generalized estimating equations adjusting for clinic-level clustering.

Findings: At 2 years, 28 of 30 (93%) clinics provided vaccination records for 6,520 adolescents. Neither the training nor text message assignments were associated with vaccination rates (Table). Adolescents whose parents received text messages versus not had 17% higher initiation (95% CI 1.08–1.26) and 30% higher UTD rates (95% CI 1.09–1.56). Receiving 2 messages versus 0 increased initiation (RR=1.43, 95% CI=1.30–1.58) and UTD rates (RR=2.00, 95% CI=1.54–2.59). Among focused trained clinics, UTD rates were higher with bundled versus focused text messages (RR=1.49, 95% CI=1.10–2.33).

Implications for D&I Research: Evaluating interactions across intervention levels is important to understand interaction effectiveness.

Primary Funding Source: National Institutes of Health

Table 1 (abstract S51). See text for description

	Clinician Training			Text Messaging		
	Focused	Bundled	Deferred	Focused	Bundled	None
Initiation						
Rate (95% CI)	44.3% (38.9%, 50.0%)	45.4% (38.7%, 53.3%)	48.5% (40.3%, 58.5%)	46.5% (42.1%, 51.4%)	45.5% (41.2%, 50.2%)	45.9% (41.5%, 50.7%)
Risk Ratio (95% CI)	0.9 (0.7, 1.1)	0.9 (0.7, 1.2)	Reference	1.0 (0.9, 1.1)	1.0 (0.9, 1.1)	Reference
UTD						
Rate (95% CI)	8.0% (5.2%, 12.4%)	8.9% (5.8%, 13.8%)	13.1% (7.4%, 23.2%)	10.4% (7.7%, 14.1%)	10.1% (7.3, 14.1%)	9.2% (6.5%, 13.1%)
Risk Ratio (95% CI)	0.6 (0.3, 1.3)	0.7 (0.3, 1.4)	Reference	1.1 (0.9, 1.4)	1.1 (0.9, 1.3)	Reference

Clinical care settings: system-level interventions

S52

Recruitment and retention lessons learned: EvidenceNOW in Wisconsin primary care clinics

Edmond Ramly¹, Heidi Brown²

¹Indiana University, Bloomington, IN, USA; ²Kaiser Permanente Southern California, San Diego, CA, USA

Correspondence: Edmond Ramly (eramly@iu.edu)
Implementation Science 2026, **21**(2):S52

Background: EvidenceNOW – Managing Urinary Incontinence (MUI) funded implementation research to improve primary care screening and management of UI in adult women. We compared two implementation strategy bundles [streamlined practice facilitation (SPF) vs. SPF with supplemental partnership-building (SPF + PB)], on adoption and

maintenance of UI screening and management in primary care practices in Wisconsin (WI).

Methods: WI primary care practices that enrolled received \$600 to support project meetings during participation and \$3,000 upon completion of data collection. Consenting practices designated a change team of 2–5 employees to lead the UI screening and treatment intervention for their practice and participated in at least 4 virtual or in-person SPF meetings that followed a menu-based guide to facilitate tailoring and minimize burden. Practices allocated to SPF + PB had facilitated introductions to local UI specialists and community organizations and were enrolled in an online learning community. Change in UI screening rate (from baseline to post-implementation) among clinics allocated to SPF vs. SPF + PB, was ascertained by anonymous patient surveys collected from each practice to ascertain rates of UI screening and treatment before implementation. Participating practices also provided aggregate electronic health record (EHR) data to compare the proportion of adult woman patient visits at which a UI diagnosis was documented before, during, and following implementation.

Findings: Among 38 enrolled practices, 31 (83%) completed participation: 16 (52%) non-academic health system; 7 (23%) independent; 4 (13%) academic health system; 4 (13%) FQHC/tribal clinic. Disenrollment reasons included clinic closure ($n=2$), lack of bandwidth for the project ($n=2$), inability to collect patient surveys ($n=2$), and lack of alignment with practice goals ($n=1$). While 8 clinics closed during the study timeframe, only 2 (25%) disenrolled; EHR data were available for 6/8 (75%). Practices affiliated with health systems faced administrative delays; independent practices progressed rapidly through implementation and achieved higher sustainment. Rates of UI screening increased from pre- to post-implementation; ongoing analyses will be presented at the meeting.

Implications for D&I Research: Streamlined practice facilitation (SPF) allowed for tailoring without excessive burden, and effectively improved UI screening and treatment in diverse primary care practices. Minimizing burden facilitates ongoing sustainment, even in the face of clinic closures.

Primary Funding Source: Agency for Healthcare Research and Quality

S53

Implementation of urinary incontinence management in primary care – a sustainable approach

Sarah Koopman Gonzalez

Case Western Reserve University, Cleveland, OH, USA

Correspondence: Sarah Koopman Gonzalez (sjk98@case.edu)

Implementation Science 2026, **21**(2):S53

Background: EMPOWER partnered with 76 primary care practices (PCP) across an integrated health care system to improve diagnosis and care for Urinary Incontinence (UI) among women.

Methods: Participating PCPs were invited to participate in education about UI management and each site was asked to provide a paper UI screener (International Consultation on Incontinence Questionnaire – Urinary Incontinence Short Form (ICIQ-UISF)) and a pamphlet with a link to the screener to all women at every visit. At the end of the intervention, each site was offered a menu of options to sustain screening including the following: paper screeners, integration of the screener in the EMR, training of a staff member as a “Continence Advisor”, having a “Champion” provider, information resources for patients and providers, and a referral list for pelvic floor physical therapy and urology specialists. The implementation team engaged with each practice regularly and the frequency and content of these engagements were documented in a detailed log. Sustainability was prioritized in qualitative assessments of engagement.

Findings: Each site had an average of four contacts regarding sustainability (range 2–6). Additionally, the implementation team had six

meetings with leadership of the integrated health system primary care institute which included a discussion of sustainability. These contacts included a study introductory meeting with medical directors and onboarding visits with each site which included asking for opinions and suggestions on sustainability. Suggestions included integration of the screener into the EMR. At the end of the intervention period, each site in operation was contacted about sustainability options and in-person visits were conducted with 65 of the sites to offer the menu of sustainability options. Only 12 sites requested one or more of the options.

Implications for D&I Research: Despite an emphasis upon sustainability of management in contact with individual practices, few projects expressed a strong interest in available options to promote sustainability. Nevertheless, the health system has integrated the UI screener into the EMR, which notifies the primary provider when the patient has a score indicating greater than mild symptoms. Understanding the factors besides continued contact around sustainability for individual clinical sites is important for future D&I research.

Primary Funding Source: Agency for Healthcare Research and Quality

S54

Health system implementation of an automated screening and education tool in primary care for women's urinary incontinence

Stephen Persell

Northwestern University, Feinberg School of Medicine, Chicago, IL, USA

Correspondence: Stephen Persell (spersell@nm.org)

Implementation Science 2026, **21**(2):S54

Background: Urinary Incontinence (UI) is prevalent but under-identified despite its impact on quality of life, mental health and physical function.

Methods: Within a regional health system, a voluntary, clinic-level implementation strategy was offered to 74 primary care practices. Workflow included an EHR-delivered pre-visit questionnaire, in-room screening, interactive educational online module and additional components. Interrupted time series analysis was used to assess changes in management before and after implementation.

Findings: Implementation occurred between January 2023 and December 2024; 43 of 74 practices implemented the workflows. Among 102,041 women with eligible visits, 72,009 (71%) completed screening. Of those, 16.5% reported bothersome symptoms, and 9.1% expressed interest in treatment. Screening rates were high across practices with some variation. More screening was completed using the pre-visit questionnaire than during rooming. The regions of the health system used two different workflows for rooming—one used interruptive alerts, and the other used a passive EHR checklist. Rooming-based screening was more likely to occur with the interruptive alerts. Uptake of the patient-facing module was modest. The mean (SD) clinic-level rate of UI diagnosis increased from 4.2 (1.1) per 100 encounters to 5.9 (1.4), $p=0.011$ and annual rate of change in diagnosis was higher post implementation, increasing from 0.58 (95% CI 0.37–0.78) to 1.12 (95% CI 0.66–1.59) per 100 encounters ($p=0.033$). Rates of pelvic floor physical therapy orders increased on average (SD) from 0.4 (0.3) to 1.1 (0.7) per 100 encounters; the annual rate of change increased from 0.05 (95% CI -0.03–0.13) to 0.36 (95% CI 0.18–0.54) per 100 encounters ($p=0.001$). Referrals to subspecialists increased from 0.5 (0.2) to 0.8 (0.4) per 100 encounters. The rate of new UI medication prescribing was low before and during the program and did not change.

Implications for D&I Research: Implementation of standard EHR-supported workflows led to high rates of UI screening but the adoption was not mandated, and a substantial proportion of practices opted not to adopt. Patients at the adopting practices had high rates of screening and in turn increased their diagnosis of UI and corresponding referrals to pelvic floor physical therapy and subspecialists which appeared to be sustained during the first year after implementation.

Primary Funding Source: Agency for Healthcare Research and Quality

S55

Effectiveness of a stepped implementation strategy for improving suicide risk screening and evaluation in the Veterans Health Administration: results from a sequential multiple assignment randomized trial

Nazanin Bahraini¹, Bridget Matarazzo¹, Jeri Forster¹, Trisha Hostetter¹, Steve Dobscha², Catherine Barry³, Katherine Dollar⁴, Lisa Brenner¹, Edward Post⁵

¹Veterans Health Administration, Aurora, CO, USA; ²Veterans Health Administration, Portland, OR, USA; ³Veterans Health Administration, Palo Alto, CA, USA; ⁴Veterans Health Administration, Syracuse, NY, USA; ⁵Veterans Health Administration, Ann Arbor, MI, USA

Correspondence: Bridget Matarazzo (Bridget.matarazzo@va.gov)
Implementation Science 2026, **21(2)**:S55

Background

Suicide prevention remains a critical priority within the Veterans Health Administration (VHA). Despite establishing requirements for universal suicide risk screening and evaluation through the VA Risk ID Initiative, initial uptake of this program varied significantly across VHA facilities. This study evaluated the effectiveness of a stepped implementation strategy using a Sequential Multiple Assignment Randomized Trial (SMART) design to optimize implementation approaches based on facility performance.

Methods

We conducted a three-phase SMART trial involving 138 VHA facilities in partnership with the VA Office of Suicide Prevention. During Phase 1, Implementation as Usual (IAU) was provided to all facilities for three months, which included education & training, quality assurance measures, and technical assistance. During Phase 2, facilities not meeting 70% screening adherence were randomized to continue IAU ($n=69$) or receive augmented Audit & Feedback (A/F; $n=69$) for eight months. In Phase 3, non-responding A/F sites ($n=57$) were randomized to continue A/F only ($n=29$) or to receive additional External Facilitation (EF; $n=28$). Primary outcomes included measures of adherence for suicide screening and follow-up evaluation (rate and timeliness).

Findings

No facilities met adequate screening targets after Phase 1 (IAU alone). Following Phase 2, sites receiving A/F ($n=69$) showed significantly greater improvement in annual screening adherence ($p<0.0001$) compared to those receiving IAU alone ($n=69$). Among non-responding A/F sites in Phase 3 ($n=57$), those that accepted and received EF ($n=17$) demonstrated significantly greater improvement in timely screening (i.e., completed when due, $p=0.02$) compared to those who continued to receive A/F alone. Sites achieving adequate screening after A/F successfully maintained performance when stepped back to a less intensive A/F strategy, with no significant differences between facilities randomized to continue A/F ($n=6$) or switch to A/F Light ($n=6$).

Implications for D&I Research

This SMART trial demonstrates the value of adaptive implementation strategies for improving suicide screening adherence in VHA. Audit and feedback provided meaningful benefits over standard implementation approaches, while external facilitation offered additional value for persistently struggling sites. Furthermore, the stepped approach allows for efficient resource allocation while maintaining performance gains, providing an evidence-based framework for scaling suicide prevention initiatives across large healthcare systems.

Primary Funding Source: Department of Veterans Affairs

S56

Using co-designed Roll-out Implementation Optimization (ROIO) to enhance implementation in a hybrid type-2 comparative effectiveness RCT to prevent health care worker burn-out
Stephen Bartels¹, Giselle Perez-Lougee², Hao Trieu Weinmann², Aimee Huang², Julie Levison², Kelly Irwin², Nicholas Oreskovic³, David Krane², Anna Wilson²

¹Massachusetts General Hospital, Harvard Medical School, Boston, NH, USA; ²Massachusetts General Hospital, Harvard Medical School, Boston, MA, USA; ³Massachusetts General Hospital, Harvard Medical School, 02114, MA, USA

Correspondence: Stephen Bartels (sjbartels@mgh.harvard.edu)
Implementation Science 2026, **21(2)**:S56

Background: Front-line healthcare workers in high stress, understaffed, public sector settings serving individuals with complex needs are at high-risk of burn-out, anxiety, depression, missed workdays, and turnover. Evidence-based practices have been shown to be effective in improving resiliency and reducing workplace stress, however factors contributing to burnout are also systemic barriers to participation, including multiple competing demands, managing challenging clinical and social needs, and understaffing. We hypothesized that overcoming system-wide barriers to implementing health care worker burnout interventions will be facilitated by adaptations co-designed with community providers applying roll-out implementation optimization (ROIO).

Methods: We conducted a system-level, hybrid type-2 effectiveness-implementation RCT with $n=184$ group homes in two behavioral health provider organizations for persons with serious mental illness and developmental disabilities randomized to either 6-sessions of group-based integrated resiliency training and task sharing (IRTTS: $n=92$ group homes) or 9 months of workplace improvement learning collaboratives (WILC $n=92$ group homes). Participatory co-design and roll-out optimization (ROIO) was employed in phase two and three including weekly implementation dashboards monitoring staff uptake and participation reviewed with providers (audit and feedback) followed by problem-solving sessions identifying barriers to participation and co-designing and implementing adaptive solutions. Implementation outcomes include uptake and participation by eligible staff and group home managers.

Findings: Phase one IRTTS uptake: 28% of group home staff; mean participation 19%. WILC uptake: 74% of group home managers; mean participation 42%. Phase 2 and 3 (respectively) with audit and feedback and ROIO participatory co-designed adaptations: IRTTS uptake 38% and 53% of staff with mean participation 36% and 48%; WILC uptake 82% and 77% with mean participation 76% and 66% of managers. Co-designed adaptations included offering make-up, booster, and evening sessions; and combined WILC sessions across the two participating agencies. Staff reported increased feasibility, and acceptability of IRTTS and WILC following adaptations of IRTTS and WILC through ROIO.

Implications for D&I Research: ROIO co-designed adaptations contribute to overcoming systemic barriers to improve implementation uptake and participation in interventions to prevent burnout in health care providers serving individuals with complex needs in public sector behavioral health organizations.

Primary Funding Source: Patient-Centered Outcomes Research Institute

S57

Budget impact analysis of the balanced opioid initiative: a cluster randomized trial aimed at de-prescribing opioids for chronic pain in primary care settings

Andrew Quanbeck¹, Xiang Li², Alice Pulvermacher², Randall Brown³

¹University of Wisconsin – Madison, Madison, WI, USA; ²University of Wisconsin Madison, Madison, WI, USA; ³University of Wisconsin School of Medicine & Public Health, Madison, WI, USA.

Correspondence: Andrew Quanbeck (arquanbe@wisc.edu)
Implementation Science 2026, **21(2)**:S57

Background

This study compared the budget impact of four de-implementation strategies designed to promote guideline-concordant opioid prescribing per the Centers for Disease Control's opioid prescribing guidelines for chronic pain.

Methods

This cluster randomized trial (involving two health systems, 32 primary care clinics, and 268 clinicians) used a hybrid type 3 sequential multiple-assignment design. De-implementation strategies targeted the system, clinic, and prescriber levels. All clinics received system-level quarterly educational meetings with monthly audit/feedback reports. At month 3, half were randomized to practice facilitation, a clinic-level strategy (in addition to the system-level strategy). At month 9, half the clinics were again randomized to add prescriber peer consulting, a prescriber-level strategy, resulting in 4 groups: system, system/clinic, system/prescriber, and system/clinic/prescriber. The primary outcome was change in mean morphine milligram equivalent (MME) dose. Secondary outcomes included adherence to opioid risk mitigation metrics (urine drug testing, treatment agreements, pain/function screening, and mental health screening). We calculated the cost of delivering each implementation strategy and budget impacts associated with changes in healthcare utilization, including pairwise comparisons between groups.

Findings

Among 8978 patients (57.3% female; mean age 58.3 years), the primary contrast between the system/clinic/prescriber vs. system strategies showed statistically significant changes in mean MME (-2.4 mg/d, 6% reduction), increased pain/function screening (5.4%), decreased urine drug screening (-7.3%), and decreased treatment agreements (-6.7%). Implementation costs per clinic ranged from \$4,416 (system), \$5,610 (system/clinic), \$7164 (system/prescriber), and \$8,358 (system/clinic/prescriber). With healthcare utilization changes incorporated, the system strategy's adjusted per-clinic costs rose from \$4,416 to \$10,908, yielding the highest budget impact due to increased healthcare utilization; conversely, budget impact for clinics in the system/clinic/prescriber group were largely unchanged at \$8,703. The system/clinic strategy had the lowest budget impact at \$7,266; system/prescriber's budget impact was \$9,625.

Implications for D&I Research

Higher-intensity de-implementation strategies significantly reduced MME dose and increased pain/function screening, while decreasing treatment agreements and urine drug screening. Somewhat paradoxically, the lowest cost strategy (system) led to more costly downstream healthcare utilization, resulting in the highest budget impact. Adding clinic- and prescriber-level strategies may help health systems reduce reliance on opioids for chronic pain with lower budget impact than providing system-level strategies alone.

Primary Funding Source: National Institutes of Health

S58

Running the gauntlet to attain sustainable impact by embedding interventions into health systems: addressing feasibility constraints of systems and the priorities of many masters

Allison L'Hotta¹, Katy Trinkley², Russell Glasgow³, Amy Huebschmann⁴

¹University of Colorado Anschutz Medical Campus, Aurora, CO, USA;

²UCHealth, Aurora, CO, USA; ³Adult and Child Center for Outcomes

Research and Delivery Science (ACCORDS), Aurora, CO, USA; ⁴University of Colorado School of Medicine, General Internal Medicine, General Internal Medicine, Aurora, CO, USA

Correspondence: Allison L'Hotta (ALLISON.LHOTTA@CUANSCHUTZ.EDU)

Implementation Science 2026, 21(2):S58

Background

A cornerstone of implementation science is to align delivery of programs with the context of real-world settings and recipients. For system-level interventions, this requires balancing fidelity to core functions for research rigor with local adaptations. Digital health interventions integrated into the electronic health record (EHR) face additional

feasibility constraints and organizational approval chains that influence the decision to adopt. Based on a successful case example of EHR integration, our objectives were to: 1) align with key health system decision-maker priorities and characteristics and 2) define implementation strategies to overcome feasibility constraints to EHR integration.

Methods

We report our experience integrating a digital health patient intervention, My Own Health Report (MOHR), into the EHR of a large academic health system. Core functions of MOHR included: 1) flag health risks; 2) prompt patient to set goals for priority risks. Using Practical, Robust Implementation and Sustainability Model (PRISM), we examined how multi-level factors, and the contextual infrastructure influenced the decision to adopt and integrate in the EHR.

Findings

Four organizational teams and approval processes were critical to MOHR's adoption and EHR integration: 1) Clinician leaders in primary care, 2) EHR informatics, 3) Learning Health System, 4) EHR Patient Experience. While clinician leaders prioritized MOHR adoption due to alignment with value-based care metrics (e.g., blood pressure control), they could not ultimately approve its adoption due to the informatics team's limited bandwidth. A Learning Health System leader identified an alternate partner due to the alignment of MOHR with their priorities to increase Wellness goal setting for patients: the EHR Patient Experience team. To meet the EHR Patient Experience team's requirement for non-clinical, Wellness-focused domains, MOHR was adapted from its original 17 risks to four health risks while still maintaining the MOHR intervention core functions. Additionally, representation of the MOHR results in the EHR required tailoring to both system feasibility constraints and the preferences and needs of clinicians and patients.

Implications for D&I Research

Integration into the EHR has tremendous potential for impact by supporting system-wide implementation and sustainability. For successful implementation, researchers must align interventions with health system priorities, adapt to partner preferences and constraints, while maintaining fidelity to core functions.

Primary Funding Source: National Institutes of Health

S59

Enhancing the implementation of clinical practice and office systems changes in primary care: the impact of practice facilitation from the STop UNhealthy Alcohol Use Now (STUN) trial

Sean Riley¹, Sarah Birken², Leslie Brouwer¹, Marcella Boynton³, Colleen Barclay⁴, Debbie Grammer⁵, Chris Weathington⁵, Monique Mackey⁶, Mary McCaskill⁷, Adam Zolotor⁸, Kimberly Shoenbill⁴, Sam Cykert³, Darren DeWalt¹⁰, Daniel Jonas¹

¹The Ohio State University Wexner Medical Center, Columbus, OH, USA;

²Wake Forest University School of Medicine, Winston-Salem, NC, USA;

³University of North Carolina, Chapel Hill, NC, USA; ⁴University of North

Carolina at Chapel Hill, Chapel Hill, NC, USA; ⁵University of North Carolina,

Chapel Hill, USA; ⁶Area L AHEC, Rocky Mount, NC, USA; ⁷North Carolina

Area Health Education Centers, Chapel, USA; ⁸University of North Carolina,

Chapel Hill, Chapel Hill, NC, USA; ⁹University of North Carolina School

of Medicine, Chapel Hill, NC, USA; ¹⁰UNC Chapel Hill, Chapel Hill, NC, USA

Correspondence: Sean Riley (sean.riley@osumc.edu)

Implementation Science 2026, 21(2):S59

Background: Implementation of evidence-based interventions in primary care remains challenging, due to variability in implementation policies and practices. Practice facilitation (PF), which provides quality improvement coaching, electronic health record support, and structured implementation guidance, may address these barriers by standardizing processes and building organizational capacity. Nested within the STop UNhealthy Alcohol Use Now (STUN) trial, this analysis evaluates whether PF improves clinical practice and office systems changes to support the adoption of alcohol screening and counseling.

Methods: A cohort of small-to-medium primary care practices in North Carolina received 12 months of PF. Monthly changes were measured using the Key Driver Implementation Scale (KDIS), a validated

tool assessing the extent to which practices have implemented quality improvement (QI) activities aligned with the Chronic Care Model (range, 0–45). KDIS evaluates progress across six domains: Clinical Information Systems, Standardized Care Processes, Optimized Team Care, Patient Self-Management Support, Leadership Support, and Fiscal Health. Linear mixed-effects models were used to assess KDIS scores over 18 months, with baseline alcohol screening rate (high [$\geq 50\%$] vs. low [$< 50\%$] of adult patients screened at baseline) tested as a moderator.

Findings: Twenty-one practices serving 54,294 adult patients reported implementation data. There were statistically significant linear and quadratic effects for time (month; $p < 0.001$). Marginal effects indicated a predicted KDIS score of 18.7 at baseline, which incrementally increased over the study period by ~9 points, sustaining a predicted total score of ~28 points during the post-implementation period (months 12–18). We also observed a significant interaction between time and baseline alcohol screening rates, where practices with higher baseline rates showed faster and greater increases in KDIS scores compared to those with lower rates.

Implications for D&I Research

PF in the STUN trial led to statistically significant improvements in clinical practice and office system changes that support implementation. Notably, these effects were greatest in clinics with higher baseline screening rates, suggesting that existing infrastructure may amplify the impact of PF on implementation policies and practices. Future research should evaluate whether PF yields comparable improvements across other preventive services and settings, while clarifying the mechanisms that drive and sustain these changes over time.

Primary Funding Source: Agency for Healthcare Research and Quality

S60

Embedding UI care into PC: lessons from the evaluation of a multi-site implementation initiative

Shona Olalere Oluwatola¹, Nipher Malika², Gabriela Alvarado³, Rachel Burns⁴, Sangeeta Ahluwalia²

¹RAND, Santa Monica, CA, USA; ²RAND Corporation, Santa Monica, CA, USA; ³RAND Corporation, Washington, DC, USA; ⁴RAND, Santa Monica, CA, USA

Correspondence: Shona Olalere Oluwatola (oolalere@rand.org)

Implementation Science 2026, 21(2):S60

Background

Urinary incontinence (UI) is a prevalent yet underdiagnosed condition among adult women, often overlooked in primary care settings. To address this gap, five federally-funded grantees implemented system-level interventions aimed at integrating evidence-based, non-surgical UI care into routine primary care workflows. Conducted across 269 sites nationally, this initiative focused on improving screening, diagnosis, and management of UI by embedding changes in practice operations, staff roles, and patient engagement strategies. Here we present lessons learned from the multi-site evaluation on the system-level strategies and factors that supported adoption and early implementation of UI management in primary care.

Methods

Over three years, our evaluation team conducted 36 data collection sessions including semi-structured interviews, focus groups with grantee leaders, practice facilitators, and clinicians as well as practice site visits. All interviews were audio-recorded, transcribed verbatim, coded on Dedoose, and analyzed using a thematic analysis approach. All five grantees submitted quarterly quantitative data on participating practices, providers, and patients.

Findings

Using the RE-AIM framework to guide interpretation, we identified key themes. For Reach, practices varied in patient outreach, with higher engagement and higher likelihood of meeting target practice and patient recruitment goals in sites with leadership buy-in, greater staff capacity and prior experience with quality improvement initiatives. In terms of Effectiveness, the initiative improved provider awareness and shifted UI from a neglected issue to one

addressed through structured care. Adoption of UI screening and management revealed that one-size does not fit all and practices and healthcare systems need to find what works for them and their patients. Implementation varied across sites, with practice facilitators adapting tools to practice workflows. Under Maintenance, some practices sustained changes, while others struggled due to turnover, competing demands, or limited tool integration.

Implications for D&I Research

This study underscores the value of tailoring implementation supports—such as practice facilitation, training materials, and workflow tools—to the needs, capacity, and culture of each site. Practices that adapted tools to their local context and engaged motivated staff were more likely to sustain screening and care. In contrast, grantees facing leadership engagement gaps reported greater adoption challenges. D&I strategies should address variation in staffing, leadership, and infrastructure to enhance effectiveness.

Primary Funding Source: Agency for Healthcare Research and Quality

S61

Facilitation as an effective strategy to reduce excessive antibiotic prophylaxis in children's hospitals: a stepped-wedge cluster randomized controlled trial

Sara Malone¹, Emmanuel Tetteh², Jackie Saito³, Shawn Rangel⁴, Kelly Bono⁵, Jade Tao⁶, Jingxia Liu², Harry Obeng⁶, Andrew Atkinson⁶, Jason Newland¹, Virginia McKay⁷

¹Washington University in St. Louis, St. Louis, MO, USA; ²Washington University in St. Louis, Saint Louis, MO, USA; ³Children's National, Washington, USA; ⁴Harvard Medical School, Boston, MA, USA; ⁵Washington University in St. Louis, St. Louis, MO, USA; ⁶Washington University in St. Louis, Saint Louis, USA; ⁷Center for Public Health Systems Science, Brown School, Washington University in Saint Louis, St. Louis, MO, USA

Correspondence: Sara Malone (sara.malone@wustl.edu)

Implementation Science 2026, 21(2):S61

Published as a full article.

Primary Funding Source: Agency for Healthcare Research and Quality

S62

Co-development of an intrinsic capacity management programme and its implementation strategies in primary care for mildly frail older adults in Singapore: application of implementation science frameworks and tools

Ezra Ho¹, Siew Fong Goh¹, Qing Wang¹, Mimaika Luluina Ginting¹, Jonathan Gao¹, Woan Shin Tan¹, Yew Yoong Ding¹, David Ng Wei Liang², Jonathan Ng Choon Aik², Sing Cheer Kwek³, Richard Hui Jor Yeong³, Sinead Wang Zhen⁴, Chirk Jenn Ng^{4,5}, Grace Sum¹

¹Geriatric Education & Research Institute, Singapore, Singapore ²National Healthcare Group Polyclinics, Singapore, Singapore; ³National University Polyclinics, Singapore, Singapore; ⁴SingHealth Polyclinics, Singapore, Singapore; ⁵Duke-NUS Medical School, Singapore, Singapore

Correspondence: Yew Yoong Ding (ding.yew.yoong@geri.com.sg)

Implementation Science 2026, 21(2):S62

Background

To translate evidence-based complex healthcare innovations into practice, integrating programme co-development and tailoring of implementation strategies from the outset are critical to ensure contextual fit, sustainability, and scalability. This study serves two purposes: 1) To translate Singapore's 2023 National Frailty Strategy and the World Health Organization Integrated Care for Older People (WHO ICOPE) into practice by co-developing an intrinsic capacity management programme in five public primary care clinics; and 2) To co-select, co-specify, and co-develop tailored implementation strategies for each clinic.

Methods

Stakeholders (clinic leadership, healthcare workers, and older adults) from 5 primary care clinics participated in co-development meetings over a 6-month period to establish consensus on the programme's core functions and delivery. The Framework of Actions for Intervention Development guided development through problem definition, stakeholder consensus, function selection and programme prototyping. Concurrently, contextual determinants (barriers and enablers) were elicited from interviews and focus groups with the stakeholders was guided by the updated Consolidated Framework for Implementation Research (CFIR). The co-selection of implementation strategies was guided by the CFIR-Expert Recommendations for Implementing Change matching tool and literature-informed criteria (relevance, improvement opportunity, feasibility, difficulty, validity). Each strategy was specified using Proctor's framework (actor, action, action target, temporality, dose, justifications, implementation outcomes).

Findings

37 stakeholders (8 administrators; 10 clinicians; 10 nurses; 9 allied health practitioners) reached consensus on the core functions of the innovation across clinics, including six-domain IC screening (vision, hearing, mobility, vitality, cognition, psychological well-being) for older adults of Clinical Frailty Scale score 4–5, further clinical assessment, and referral pathways to health and social care services. Each clinic developed adaptable forms of delivery of core functions, including clinical assessment tools, protocols, workflows, and referral partners. Additionally, each clinic selected different combinations of 2 to 5 strategies to mitigate anticipated barriers and leverage anticipated enablers. These comprised educational meetings, mobilizing champions, implementation team meetings, patient nudging using text messages, and altering incentive structures. Strategies were specified to facilitate operationalization and fidelity tracking.

Implications for D&I Research

Integrating systematic stakeholder-driven co-development with tailored implementation strategies provides a comprehensive implementation science approach to accelerate the translation of national policies into clinical practice in ways that are sustainable and scalable.

Primary Funding Source: Geriatric Education and Research Institute

S63

Co-creation during crisis: harnessing implementation science to achieve impact

Christine Fahim¹, Jamie Boyd², Adrienne Chan³, Allison McGeer⁴, Sharmistha Mishra⁵, Stefan Baral⁶, Jennifer Gommerman⁷, Anne-Claude Gingras⁸, Rob Delatolla⁹, Claire Oswald¹⁰, Kim Gilbride¹¹, Mario Ostrowski¹², Keelia Quinn de Launay¹³, Alyson Takaoka¹³, Liana Sikharulidze¹⁴, Sharon Straus¹³

¹Li Ka Shing Knowledge Institute, Unity Health Toronto, Toronto, ON, Canada; ²Knowledge Translation Program, Li Ka Shing Knowledge Institute, St. Michael's Hospital, Unity Health Toronto, Toronto, ON, Canada; ³Sunnybrook Health Sciences Centre, Toronto, Canada; ⁴Sinai Health System, Toronto, Canada; ⁵Li Ka Shing Knowledge Institute, St. Michael's Hospital, University of Toronto, Toronto, Canada; ⁶Johns Hopkins Bloomberg School of Public Health, Baltimore, MD, USA; ⁷Dept of Immunology, University of Toronto, Toronto, Canada; ⁸Sinai Health System, Lunenburg-Tanenbaum Research Institute, Toronto, Canada; ⁹Faculty of Engineering, University of Ottawa, Ottawa, Canada; ¹⁰Dept. of Geography and Environmental studies, Toronto Metropolitan University, Toronto, Canada; ¹¹Dept. of Chemistry and Biology, Toronto Metropolitan University, Toronto, Canada; ¹²Dept. of Laboratory Medicine and Pathobiology, University of Toronto, Toronto, Canada; ¹³Li Ka Shing Knowledge Institute, St. Michael's Hospital, Toronto, ON, Canada; ¹⁴Toronto, Canada

Correspondence: Christine Fahim (christine.fahim@unityhealth.to)

Implementation Science 2026, 21(2):S63

Background: People living and working in long-term care and retirement homes were among the most affected by COVID-19, yet had limited ability to participate in research during the pandemic response. We describe the development and impact of Wellness Hub, which

evaluated the impact of tailored strategies to address COVID-19 challenges and facilitated SARS-CoV-2 research.

Methods: Using the Theoretical Domains Framework, we rapidly interviewed 91 leaders from 47 homes to identify their COVID-19 support needs, which spanned infection prevention and control (IPAC), vaccine uptake and confidence, and staff mental health. We assigned 48 homes to an intervention arm (tailored resources + guided facilitation) and 24 to a control arm (generic resources) and evaluated the impact on challenge resolution using a poisson regression (rate/10 challenges, rate-ratio, 95% CI). We formed a multidisciplinary network that spanned > 20 research and clinical disciplines and included front-line home staff and caregivers, to define immunity research questions pertaining to SARS-CoV-2. We conducted an immunity study that collected dried blood spots, demographic questionnaires, and other serosamples from residents, their caregivers, staff, and their household members. We also designed a wastewater study to track SARS-CoV-2 in congregate care settings to inform outbreak prevention measures.

Findings: We delivered 38 original educational resources, 50 education events, 22 community of practice meetings and funding supports to 72 LTCH/RH. Homes in the intervention arm were two times more likely to resolve COVID-19 challenges (2.02; 95% CI 1.50–2.70, $p < 0.0001$); IPAC challenges (1.85; CI 1.25–2.73, $p = 0.0002$) and four times more likely to resolve vaccine challenges (rate-ratio 3.78; CI 1.97–7.25, $p = < 0.0001$). Wellness Hub data advanced knowledge on SARS-CoV-2 seroprevalence and antibody response to mRNA vaccinations and were used to shape the national COVID-19 vaccine schedule in Canada. We implemented wastewater surveillance in 12 care settings and 20 neighborhoods where staff lived; data were fed back to homes and public health authorities to prompt implementation of outbreak prevention measures.

Implications for D&I Research: We used implementation research methods to rapidly co-design an adaptive support strategy that addressed homes' needs, while facilitating the conduct of multidisciplinary research among marginalized staff and residents at risk of infection and associated morbidity and mortality.

Primary Funding Source: COVID-19 Immunity Task Force

S64

Impact of the acute inpatient medicine – high reliability, learning environment, and workforce development initiative (AIM-HI) in Rural Veterans Health Administration Hospitals

Heather Gilmartin^{1,2}, Brigid Connelly², Marguerite Daus^{2,3}, Edward Hess², Amy Ladebue², John P. Nolan Jr.⁴, Paige Perry², Heidi Sjoberg², Soumya Subramaniam², Melver Anderson⁶, Chelsea Leonard^{2,7}

¹Colorado School of Public Health, Aurora, CO, USA; ²Veterans Health Administration, Aurora, CO, USA; ³University of Colorado Anschutz Medical Campus, Division of General Internal Medicine, Aurora, CO, USA; ⁴Independent Researcher, El Dorado, AR, USA; ⁵Veterans Health Administration, Seattle, WA, USA; ⁶Veterans Health Administration, Washington D.C., DC, USA; ⁷University of Colorado Anschutz Medical Campus, Aurora, CO, USA

Correspondence: Heather Gilmartin (heather.gilmartin@va.gov)

Implementation Science 2026, 21(2):S64

Background: Rural healthcare systems often struggle with limited resources, leading to provider shortages and higher patient mortality which necessitate innovative workforce development strategies. AIM-HI is an evidence-based workforce development program designed to foster supportive learning environments within Veterans Affairs (VA) rural hospital medicine teams to enhance culture and workforce well-being.

Methods: AIM-HI is a type II hybrid implementation study employing a convergent mixed methods approach to evaluate the Relational Playbook, a 6-month virtual team training program for clinical leaders (physician, nursing, social work, pharmacy). Guided by VA's learning healthcare system model, implementation strategies were tailored for rural settings and included behavioral nudges, learning collaboratives, and leadership coaching. In 2023, AIM-HI was implemented in 3

rural VA hospitals with hospital medicine programs and expanded to 7 more in 2024. The primary outcome was AIM-HI's acceptability, appropriateness, and feasibility with secondary outcomes on leader, staff, and learning environment impacts, all evaluated through surveys and interviews. Quantitative data were summarized, while qualitative data were thematically analyzed and triangulated with quantitative data.

Findings: From 2023 to 2024, AIM-HI enrolled 100 clinical leaders across 10 VA hospitals. Our first cohort ($n=24$) rated AIM-HI's implementation as acceptable, appropriate, and feasible (range 4.0–4.3; $n=11/24$; 46%). They reported that Playbook practices enhanced team dynamics, increased engagement, required minimal time, and delivered significant benefits. The second cohort ($n=76$) expressed a substantial need for the program and reported greater job satisfaction, reduced burnout, and stronger desire to stay in VA, suggesting effective leadership promotes psychologically safe, high-functioning teams. Both cohorts experienced engagement challenges from clinical demands. Facilitators included leadership coaching and learning collaboratives. Barriers were staffing shortages and increased workloads requiring a focus on patient care versus team development.

Implications for D&I Research: AIM-HI is a feasible and impactful rural hospital medicine workforce development program that supports clinical leaders' professional growth and well-being. Despite barriers, the program's positive impact on staff relationships and work environments highlights its potential to optimize the VA workforce to meet rural Veterans' needs. This work shows how D&I science can drive scalable, sustainable solutions to strengthen leadership and workforce resilience.

Primary Funding Source: Department of Veterans Affairs

S65

Health system-level implementation support strategies for an evidence-based cervical screening intervention in Ghana

Nadia A. Sam-Agudu¹, Emily Frueh², Obed Cudjoe¹, Alex Dankyi¹, Nancy Ebu Anyan¹, Sebastian Ken-Amoah¹, Patrick K. Akakpo¹, Dorcas Obiri-Yeboah³, Matthew Asare²

¹University of Cape Coast, Cape Coast, Ghana; ²Baylor University, Waco, TX, USA; ³University of Cape Coast School of Medical Sciences, Cape Coast, Ghana

Correspondence: Nadia A. Sam-Agudu (agudu@umn.edu)

Implementation Science 2026, **21**(2):S65

Background

The HOPE-inG study is a Type 2 hybrid implementation-effectiveness trial evaluating cervical cancer (CC) screening uptake among women living with HIV (WLWH) in Ghana. The HOPE multicomponent intervention consists of socio-behavioural communication messaging using the 3R Model (Reframing, Reprioritising, Reforming), HPV self-sampling, and patient navigation. We elucidate implementation strategies that addressed determinants of HOPE implementation at the health systems level.

Methods

This was the Preparation phase in our Exploration, Preparation, Implementation, and Sustainment (EPIS) framework-informed trial. We used a four-step nominal group technique (Generate ideas, Round-robin sharing, Discussion, and Voting) to rapidly collect data on barriers and facilitators from 4 nominal groups: WLWH (intervention recipients; $N=16$), healthcare workers (intervention deliverers; $N=16$), government officials ($N=17$), and health facility managers ($N=16$) (policy/decision-makers). Determinants were organized using the Consolidated Framework for Implementation Research (CFIR) and matched to implementation strategies in the Expert Recommendations for Implementing Change (ERIC). We focused on determinants and corresponding implementation strategies relevant to healthcare workers and health facility managers. We named these implementation strategies "implementation support strategies" (ISS), defined as implementation strategies most relevant to implementation support, which is concerned with moving research into practice through activities like training, providing tools and ongoing monitoring and evaluation.

Findings

Twenty-nine highly-ranked determinants (13 facilitators, 16 barriers) were identified across all CFIR domains: Inner Setting (12), Outer Setting (6), Implementation Process (6), Innovation (2), and Individuals (4). These determinants were matched to 73 non-unique implementation strategies. A total of 24 unique ISS specific to implementation support were identified. The top seven ISS were: Conduct ongoing training; Audit and provide feedback; Conduct educational meetings; Facilitation; Identify and prepare champions; Use data experts; and Use data warehousing techniques.

Implications for D&I Research

Implementation science promotes adoption and sustainability of EBIs by guiding the process of moving research to practice. We identified ISS as key implementation strategies to facilitate this process for the evidence-based HOPE intervention. These ISS included training, supervision and monitoring, and mentoring/technical assistance. The ISS categorization allows for identification and further evaluation of implementation strategies specifically addressing implementation support and bridging factors between the inner implementation and outer policy setting at the health systems level.

Primary Funding Source: National Institutes of Health

S66

Sustainability of evidence-based strategies for HPV vaccination in a large, urban Federally Qualified Health Center

Jennifer Tsui¹, Samantha Garcia¹, Michelle Shin², Emily Dang¹, Morgan Gill¹, Artur Galimov¹, Zehui He³, Sharon Hudson⁴, Benjamin Crabtree⁵, Lourdes Baezconde-Garbanati¹, Lawrence Palinkas⁶

¹University of Southern California, Los Angeles, CA, USA; ²University of Washington, Seattle, WA, USA; ³University of Southern California, Los Angeles, USA; ⁴AltaMed Health Services Corporation, Los Angeles, CA, USA; ⁵Rutgers Robert Wood Johnson Medical School, New Brunswick, NJ, USA; ⁶UC San Diego, La Jolla, CA, USA

Correspondence: Jennifer Tsui (Tsuijenn@usc.edu)

Implementation Science 2026, **21**(2):S66

Background: Numerous HPV vaccination evidence-based strategies (EBS) have been implemented in recent years, but a major challenge is integration and sustainment of EBS within dynamic clinical settings long-term. Our prior work evaluated implementation of HPV vaccination EBS in safety-net settings. We now evaluate longer-term sustainability outcomes and factors that impact sustainability for three EBS implemented in a large, urban Federally Qualified Health Center.

Methods: Using a mixed-methods design, we applied the Rapid Assessment Procedure-Informed Clinical Ethnography (RAPICE) approach and Shelton's RE-AIM extension to guide sustainability assessment April-June 2025 (18 months after implementation). The initial implementation study collected data at baseline (Oct-Dec 2022) and 12-months (Nov-Dec 2023). EBS implementation (Dec 2022-Nov 2023) included: provider communication training, clinic staff education/workflow training, and audit-and-feedback with leaders and clinic champions. We assessed continued use and impact of EBS across sites and clinical roles. We also used the Clinical Sustainability Assessment Tool (CSAT) to identify factors that lead to sustainability.

Findings: A total of 81 surveys (39 providers, 42 clinic staff) and 25 qualitative interviews (10 providers, 9 clinic staff, 5 system leaders) were completed. Surveys showed sustained EBS impact on discussion of HPV vaccination starting at age 9 (baseline: 63%, 12-month: 82%, sustainability: 88%) and high confidence in addressing parental vaccine concerns (baseline: 36%, 12-month: 48%, sustainability: 69%). Participants reported high CSAT factors for the provider communication training: easy to use (100%), integrated well into clinical workflow (85%) and can be used consistently (85%). CSAT factors for staff trainings were also high (>89% across similar measures). Qualitative data showed that while there was agreement all EBS were important, survey and interview findings showed mixed perspectives on interest in sustaining provider trainings. In contrast, there was strong support for

integrating staff trainings into onboarding or self-learning tools within nursing education workflows.

Implications for D&I Research: Our findings show engagement with providers, staff, and system leadership can inform how to sustain EBS for HPV vaccination beyond a research-funded intervention period. Adequate co-design from initial implementation and routine feedback with stakeholders within clinics can provide inputs that accelerate long-term system adoption and sustainability of interventions.

Primary Funding Source: National Institutes of Health

S67

Engaging practice champions to develop a system-wide implementation plan for hypertension management at primary care practices at UPMC

Elaine Kwok

UPMC Center for High-Value Health Care, Pittsburgh, PA, USA

Correspondence: Elaine Kwok (kwok@upmc.edu)

Implementation Science 2026, **21(2)**:S67

Background

UPMC created a standardized process to more efficiently plan for practice changes across its integrated payer-provider system as part of the PCORI Health System Implementation Initiative. This presentation outlines the application of this process, particularly the engagement with practice champions, to develop an implementation plan for hypertension evidence-based practice (EBP) adoption in primary care practice (PCP) settings.

Methods

First, a summary of the hypertension EBP was shared with health system leaders from primary care, EHR/IT infrastructure, data analytics, quality improvement, and payer teams to assess priority and feasibility for system-wide implementation. After the EBP was considered both important and feasible, independent outreach was conducted with each PCP group to assess interest in EBP adoption, and to discuss contextual factors and timelines. Lastly, interested practice groups were encouraged to identify a practice champion, a frontline provider from within each practice group, who was willing to work with the implementation team to design an implementation plan.

Findings

The process secured leadership support and built a strong business case for system-wide hypertension EBP adoption. Thirteen PCP groups and their practice champions informed a feasible, phased implementation approach involving 130 sites. Early adoption sites ($N = 30$) tended to be affiliated with academic or research hospitals (e.g., through residency training programs or previous research collaborations) and have strong leadership and administrative support, and agreed to help pilot and finalize new clinical workflow and EHR modifications. Mid-adoption sites ($N = 50$) tended to have existing quality improvement initiatives and/or dedicated quality improvement officers, and have established incentive programs with payers based on blood pressure control rates, and were willing to onboard once all EHR modifications are finalized. Sites preferring later participation ($N = 50$) cited staff shortages and burnout, limitations in data infrastructure, and concerns about workflow changes; therefore, they preferred to start implementation after outcomes data become available from early adoption sites.

Implications for D&I Research

System-wide implementation of EBP can be complex, but establishing a standardized process and engaging frontline practice champions helped the implementation team consider multi-level factors that determined the scope, pace, and milestones to facilitate EBP adoption in different contexts.

Primary Funding Source: Patient-Centered Outcomes Research Institute

S68

Applying a transformative healthcare cycle to aid in informing interventions and developing a stakeholder driven, economically sustainable implementation plan for uncontrolled hypertension at primary care practices, urgent care and emergency departments

Joyce Kerpchar

AdventHealth, Orlando, FL, USA

Correspondence: Joyce Kerpchar (joyce.kerpchar@adventhealth.com)

Implementation Science 2026, **21(2)**:S68

Background

AdventHealth applied the Transformative Healthcare Cycle—a standardized, enterprise-wide framework—to support evidence-based decision-making and implementation of hypertension management strategies. As part of the PCORI Health System Implementation Initiative, this model was used to guide intervention selection, implementation planning, and sustainability across primary care, urgent care, and emergency department settings.

Methods

The framework includes four phases: (1) Design – establishing urgency, engaging stakeholders, and aligning with strategic priorities; (2) Pilot – generating early wins and addressing barriers; (3) Implement – scaling successful practices; and (4) Sustain – embedding changes into organizational culture. During the Design phase, a comprehensive assessment evaluated the alignment of the PCORI Hypertension Initiative with AdventHealth's clinical priorities. This included analysis of population reach, supporting evidence, payor mix and overall economic feasibility. Findings were presented to leadership to inform intervention selection and build consensus.

Findings

Presenting clinical and economic impact data helped secure leadership support and informed a phased implementation plan. Analysis showed that patients presenting to emergency and urgent care settings with uncontrolled hypertension could benefit from a structured management program. A total of 385 clinical sites were identified—275 primary care practices and 128 urgent/emergency care locations. Based on feedback from a multidisciplinary steering group, implementation was staggered, beginning with primary care pilot sites. Dashboards are being developed to monitor clinical outcomes and economic feasibility, enabling real-time evaluation and resource planning.

Implications for D&I Research

This study illustrates how structured frameworks can guide leadership and clinical teams in the selection, implementation, and long-term sustainability of evidence-based interventions. The integration of timely economic evaluations into the implementation strategy, along with real-time tracking of both clinical and economic outcomes, supports adaptive decision-making and promotes sustainability across diverse care settings.

Primary Funding Source: Patient-Centered Outcomes Research Institute

S69

Development of hypertension clinical decision support tools as an implementation strategy at WellSpan Health

Nicole Hurst

WellSpan Health, York, PA, USA

Correspondence: Nicole Hurst (nhurst@wellspan.org)

Implementation Science 2026, **21(2)**:S69

Background

Through participation in the PCORI Health System Implementation Initiative, WellSpan Health developed a standardized, interdisciplinary team-based approach to guide large-scale implementation initiatives. This presentation describes the application of that approach to the design and development of a digital technology,

hypertension-focused clinical decision support (CDS) tools within Epic, WellSpan's electronic health record (EHR). These tools, including a SmartSet orderset and an Our Practice Alert, were selected as core implementation strategies to promote care standardization and support pragmatic evidence-based care delivery across primary care.

Methods

An interdisciplinary team composed of primary care clinicians, informatics, data analytics, pharmacy, nursing, quality improvement, and community health leaders collaboratively led CDS tool development. The team reviewed current workflows, regional variation in clinical practice, and national hypertension guidelines to shape the tools' structure and content. Key components included evidence-aligned lab and medication ordering, referral pathways, documentation templates, and embedded patient education. User-centered design principles ensured the tools were intuitive, clinically relevant, and integrated into varied workflows. Stakeholder perspectives were essential in uncovering workflow nuances, clinical preferences, and adoption barriers. Feedback gathered through usability testing, live demonstrations, and iterative refinement cycles informed both the design and implementation planning.

Findings

CDS tool development revealed variation in hypertension care across WellSpan's 63 primary care sites. Several unintended workflow disruptions surfaced, including role misalignment and friction caused by overlapping or nonintuitive tools. Interdisciplinary collaboration was key to surfacing and addressing these issues in real time. The process improved alignment with frontline operations and supported strategies to build provider trust. CDS tools were intentionally aligned with the expansion of Remote Patient Monitoring, embedding workflows and decision support to enhance hypertension management. Implementation planning emphasized in-person facilitation and relational engagement as key strategies to support adoption and build capacity across care teams.

Implications for D&I Research

Digital technologies, such as CDS tools, can support implementation at scale. Leveraging user-centered design through interdisciplinary collaboration strengthens stakeholder engagement, supports workflow integration, and enhances sensitivity to context and the nuances of care delivery. Iterative development, guided by diverse perspectives and responsive to variability across clinical settings, can produce more acceptable, feasible, and adaptable tools, better positioned for successful implementation and sustained use.

Primary Funding Source: Patient-Centered Outcomes Research Institute

S70

Implementing the National Hypertension Control Initiative in 350 Federally Qualified Health Centers from 2021 to 2023

Eduardo Sanchez

American Heart Association, Dallas, TX, USA

Correspondence: Eduardo Sanchez (eduardo.sanchez@heart.org)

Implementation Science 2026, 21(2):S70

Background

In December 2020, the Office of Minority Health (OMH) and Health Resources & Services Administration (HRSA) announced the National Hypertension Control Initiative (NHCI) a three-year implementation project to improve blood pressure (BP) control in 500 Federally Qualified Health Centers (FQHCs). The 500 FQHCs (roughly one-third of all FQHCs) were afforded funding from HRSA to provide self-measured blood pressure (SMBP) devices for patients with hypertension (HTN), and the American Heart Association (Heart) was awarded \$32 million through a 3-year cooperative agreement with OMH and HRSA to provide training and technical assistance (T&TA) for 350 of the 500 FQHCs to clinical care teams on implementation of evidence-based practices (EBP) to improve BP control. This abstract describes the implementation strategies to support adoption of EBP to improve BP control in 350 FQHCs in the lowest quartile for BP control.

Methods

A three-year T&TA curriculum based on the AMA MAP™ framework (measure accurately, act rapidly, partner with patients), ongoing assessment of clinical care practices and BP control, and feedback mechanisms were the key implementation strategies selected.

The clinical T&TA integrated a core medical educational curriculum, evidence-based tools and resources, and evaluation processes. T&TA was delivered through webinars, workshops, office hours, structured forums, and one-on-one coaching. T&TA was modified and tailored to address the evolving competencies and needs of the FQHCs. The webinar-based curriculum initially focused on hypertension screening, diagnosis, and management and the addition of technology webinars focused on SMBP devices, electronic health records, and remote patient monitoring. The activities spanned three years, with distinct goals and interventions for each year.

Findings

In year 3, 94% of FQHCs reported having a documented protocol or process for BP measurement, 79% using a BP treatment protocol, and 97% having an SMBP program. In the 350 FQHCs receiving T&TA, BP control rates increased from 51.1% in 2020 before NHCI to 60.9% in 2023 (19.2% relative improvement). By comparison, BP control improved 11.4% in the other FQHCs.

Implications for D&I Research

T&TA is a highly scalable implementation strategy. Further analysis will help delineate facilitating factors and barriers to BP control improvement in the participating FQHCs.

Primary Funding Source: Office of Minority Health

S71

Planning for sustainability: implementing a severe hypertension safety bundle in outpatient maternity care

Jennifer Leeman¹, Aparna Kachoria², Alasia Ledford², Shelby Smith-Janey², Maihan Vu², Narges Farahi², Johanna Quist-Nelson², M. Kathryn Menard³

¹UNC Chapel Hill, School of Nursing, Chapel Hill, NC, USA; ²University of North Carolina at Chapel Hill, Chapel Hill, NC, USA; ³The University of North Carolina at Chapel Hill, Chapel Hill, NC, USA

Correspondence: Jennifer Leeman (jleeman@email.unc.edu)

Implementation Science 2026, 21(2):S71

Background: Severe hypertension increases risk for serious pregnancy-related complications. The AIM Severe Hypertension During Pregnancy and Postpartum Period Safety Bundle (HTN Bundle) has demonstrated effectiveness at improving early detection and treatment of severe HTN in inpatient settings but has not been tested in the outpatient setting. To address this gap, we adapted the HTN Bundle for the outpatient setting (O-HTN), tailored strategies to support implementation (training, facilitation, simulation, and education materials), and are testing implementation and effectiveness in 19 clinics. We report findings on O-HTN Bundle sustainability for the study's first complete cohort of clinics ($n = 7$).

Methods: Using an explanatory sequential mixed-methods design, we administered the Clinical Sustainability Assessment Tool (CSAT) to clinic implementation team members ($n = 20$, 74% response rate) during month 10 of the 12-month implementation phase. We then engaged each implementation team ($n = 7$) in a semi-structured discussion of factors contributing to their CSAT results (month 11) and in meetings to plan for sustainability (month 12). Descriptive statistics were used to summarize data from the CSAT. Two team members used content analysis to code transcripts and meeting notes and met to reconcile findings.

Findings: Mean scores on each of six CSAT domains ranged from 5.48 to 6.42 on a 7-point scale. Themes from qualitative findings identified barriers and facilitators within each domain. For example, for the CSAT domain "organizational readiness," barriers included high staff turnover rates, competing priorities, and limited coordination across levels within a clinic network. For "monitoring & evaluation," barriers included difficulty accessing electronic health record (EHR) data and limited capacity for quality improvement. For the domain "engaged

staff and leadership”; facilitators included having a provider champion, leadership support, and a being a small clinic. Implementation teams identified a range of strategies they would use to sustain implementation (e.g., conduct team debriefs following a severe HTN event).

Implications for D&I Research: Sustaining interventions beyond active implementation is challenging. We applied mixed methods to identify barriers to sustainment and strategies clinics might use to overcome them. Findings will inform refinement of existing implementation strategies and the need for new strategies targeting factors beyond the clinic (e.g., network leadership and EHR systems).

Primary Funding Source: National Institutes of Health

S72

Understanding determinants influencing longitudinal pathways from implementation to sustainment of moral injury groups in the Veterans Health Administration: realizing the importance of “liminal” sustainment

Andrea L. Nevedal¹, Marilla Widerquist², Maria Arasim², Caitlin M. Reardon³, Jason Nieuwsma⁴, Melissa Smigelsky⁴, Summer Anderson⁴, Heather A. King^{5,6}, Brandolyn White⁷, Keith Meador⁴, George Jackson^{4,8}
¹Veterans Health Administration, Ann Arbor, MI, USA; ²VA Ann Arbor Healthcare System, Ann Arbor, MI, USA; ³VA Center for Healthcare Evaluation, Research, and Promotion, Pittsburgh, PA, USA; ⁴Durham Veterans Affairs (VA) Health Care System, Durham, NC, USA; ⁵Duke University School of Medicine, Durham, NC, USA; ⁶Veterans Health Administration, Durham, NC, USA; ⁷Durham VA Health Care System, Durham, NC, USA; ⁸University of Texas Southwestern Medical Center, Dallas, TX, USA

Correspondence: Andrea L. Nevedal (Andrea.Nevedal@va.gov)

Implementation Science 2026, 21(2):S72

Background: People experience moral injury (MI) when violations of deeply-held moral beliefs (e.g., killing someone in combat) lead to feelings of guilt, shame, or loss of meaning. The Veterans Health Administration (VA) developed MI-groups co-led by mental health providers and healthcare chaplains. We conducted a trial to compare an organizational network strategy called the Dynamic Diffusion Network (DDN) to technical assistance (TA) at sites implementing MI-groups. Our objective was to understand determinants influencing longitudinal pathways from implementation to sustainment of MI-groups.

Methods: Thirty-three semi-structured interviews were conducted among 18 eligible MI-group co-facilitators from nine VA sites (4 = DDN, 5 = TA) during pre-implementation and post-implementation of MI-groups. One-year post-implementation, 16 sustainment surveys were conducted. The updated Consolidated Framework for Implementation Research (CFIR) and the CFIR Outcomes Addendum informed rapid directed content analysis of qualitative interview and survey data (determinants indicated in parentheses). Outcomes included: anticipated implementation, actual implementation, anticipated sustainment, and actual sustainment.

Findings: Of the nine sites, six (2 = DDN, 4 = TA) had consistent successful pathways from anticipated implementation to actual sustainment to anticipated future sustainment. Three of the nine sites (2 = DDN, 1 = TA) had inconsistent pathways. Two DDN sites had inconsistent pathways because they successfully implemented MI-groups but had “liminal” sustainment (neither fully sustained as intended nor discontinued). One TA site had an inconsistent pathway because they anticipated successful implementation but did not complete implementation/discontinued MI-groups. Facilitators to implementation/sustainment included having DDN support (*Implementation Facilitator*); providers referring Veterans to MI-Groups (*Engaging*); co-facilitators to lead MI-Groups (*Opportunity*); and support for implementation (*High-Level Leaders*). Barriers to implementation/sustainment included losing a co-facilitator without having a replacement (*Opportunity*); insufficient eligible Veterans to refer (*Recipient Need*); and insufficient support for implementation (*High-Level Leaders*). Among DDN sites in “liminal” sustainment, one anticipated future sustainment as “likely” if replacing a co-facilitator (*Opportunity*) and the other anticipated future

sustainment as “neither likely/unlikely” because of insufficient eligible Veterans for MI-group referral (*Recipient Need*).

Implications for D&I Research: Most sites were consistently successful throughout implementation to sustainment of MI-groups. However, findings point to important future work defining and understanding sites in “liminal” sustainment, which may include enhancing earlier support and internal collaboration for those experiencing organizational change or needing adaptations.

Primary Funding Source: Department of Veterans Affairs

S73

Who needs to lead? Dimensions of leadership needed for system-wide, sustainable, integrated behavioral health implementation

Ann M. Nguyen¹, Alexandra Williams², Lisa Mikeseli³, Anushka Purkayastha¹, Stephanie Marcello⁴, Agota Szabo⁵, Ralph Gigliotti⁶, Benjamin F. Crabtree⁷

¹Rutgers Center for State Health Policy, New Brunswick, NJ, USA; ²Rutgers Robert Wood Johnson Medical School, New Brunswick, NJ, USA; ³Rutgers University, New Brunswick, NJ, USA; ⁴Rutgers University Behavioral Health Care, Piscataway, NJ, USA; ⁵Centre of Expertise on Global Governance, The Hague University of Applied Sciences, The Hague, Netherlands; ⁶Rutgers Center for Organizational Leadership, New Brunswick, NJ, USA; ⁷Department of Family Medicine and Community Health, Rutgers Robert Wood Johnson Medical School, New Brunswick, NJ, USA

Correspondence: Ann M. Nguyen (anguyen@ifh.rutgers.edu)

Implementation Science 2026, 21(2):S73

Background: Despite the availability of comprehensive integrated behavioral health (IBH) protocols, IBH has not been widely adopted across U.S. health systems. The literature has identified that a lack of effective leaders is a key barrier to successful IBH implementation. Powell et. al (2015) have recognized the critical role of leaders in intervention implementations in three discrete implementation strategies (i.e., inform local opinion leaders; have leaders mandate change; recruit/designate/train for leadership). Our study fills a gap in IBH implementation by examining who needs to lead IBH for successful system-wide, sustainable IBH implementation.

Methods: Qualitative in-depth interviews were conducted to identify the role, characteristics, and experiences of leaders in IBH implementation. We interviewed a national sample of prominent IBH leaders and experts ($n = 41$, 82% response rate), which included distinguished researchers, primary care providers with intensive IBH experience, behavioral health providers, IBH implementers, and senior policy influencers from a wide range of healthcare organization types. Rapid analysis during data collection and subsequent immersion-crystallization were conducted to ensure thematic saturation and identify themes.

Findings: Interviewees identified multiple dimensions of leadership that must be engaged to institute system-wide IBH adoption. Leaders must be engaged at the individual clinic level as well as the system level. Within each clinic, frontline staff and clinic leadership collectively serve as change advocates and gatekeepers. There must also be engagement from health system C-suite leaders, who can embed IBH into the system's strategic priority setting and allocation of needed resources and incentives. Systems where IBH has not been sustained lacked C-suite engagement. Additionally, leaders from both the primary care and behavioral health disciplines must work together to create a shared mental model.

Implications for D&I Research: As the field of D&I moves toward sustainable system-level interventions, it is critical that we hone implementation strategies that focus on a diversity of leaders at all levels. This study advances the knowledge on leadership dimensions that need to be engaged for IBH, which undoubtedly apply to other systems-level interventions. Our findings also reveal the critical role of C-suite leadership in fostering longevity in the changes. A focus on C-suite interventions merits further development.

Primary Funding Source: Agency for Healthcare Research and Quality

S74

Sustainability through financing and leadership: a mixed-methods study of tobacco treatment programs in NCI-designated cancer centers

Miranda Reid¹, Magda Montague¹, Sofia Anrecio¹, Jennifer LeLaurin¹, Carma Bylund², Mara Minion³, Gonghao Liu⁴, Ji-Hyun Lee², Edmond Ramly⁵, Donna Shelley⁶, Jamie Ostroff⁷, Graham Warren⁸, Ramzi Salloum¹

¹University of Florida College of Medicine, Gainesville, FL, USA; ²University of Florida, Gainesville, FL, USA; ³Carbone Cancer Center, University of Wisconsin-Madison, Madison, WI, USA; ⁴University of Florida Health Cancer Center, Gainesville, FL, USA; ⁵Indiana University, Bloomington, IN, USA; ⁶Department of Population Health, New York University School of Medicine, New York, NY, USA; ⁷Memorial Sloan Kettering Cancer Center, New York, NY, USA; ⁸University of Kentucky, Lexington, KY, USA

Correspondence: Miranda Reid (miranda.reid@ufl.edu)

Implementation Science 2026, **21**(2):S74

Background: Tobacco cessation markedly improves cancer treatment outcomes. As part of the National Cancer Institute (NCI) Cancer Moonshot program, the Cancer Center Cessation Initiative (C3I) funded 52 NCI-Designated Cancer Centers to promote the development of sustainable tobacco treatment programs (TTPs) from 2017–2021. This analysis characterized leadership and funding strategies used by centers to address TTP sustainability.

Methods: In 2023–2024, TTP leaders at 47 centers completed surveys on sustainability strategies and outcomes. TTPs at 20 of these centers also completed semi-structured interviews on TTP sustainability and adaptation. TTPs were selected for interviews to represent variation in program reach, geographic region, and perceived sustainability based on survey responses. For quantitative data, descriptive statistics were used to characterize use of implementation strategies, core program components, and funding sources. For qualitative data, three authors conducted a thematic analysis using the constant comparative method. Using a concurrent triangulation design (QUAL + quan), analytical integration occurred after initial parallel analysis.

Findings: Analytical integration revealed three core themes: 1) Right-sizing to ensure financial sustainability: in order to sustain funding, TTPs created a program whose scope was sustainable in their context by discontinuing or never starting core program components that were expensive to maintain (e.g. 34.8% maintained interactive voice response) and instead investing in lower-maintenance forms (e.g. 60.9% maintained electronic health record changes); 2) Reliance on institutional funding: given significant administrative hurdles, few programs (27.7%) were able to bill for services (6.4% sole source of funding, 6.4% majority source). Instead, most relied on institutional support after C3I funding ended (44.7% sole source of funding, 19.2% majority source); and 3) Wide use of leadership engagement strategies: TTPs engaged leadership and maintained their support after funding ended by continuing to meet regularly with leaders and champions (74.5%), aligning reported metrics with leadership priorities (55.3%), and marketing their program internally (66.0%).

Implications for D&I Research: TTP's ability to leverage leadership support and design programs of appropriate scope allowed them to access institutional funding when billing was not feasible. These strategies may be relevant in other implementation contexts, especially in areas where clinical revenue generation is limited.

Primary Funding Source: National Institutes of Health

S75

Designing and adapting a quality improvement project to enhance life-sustaining treatment documentation in VA home-based primary care

Jennifer Kononowech¹, Kate Magid², Leah Haverhals^{2,3}, Jazmin Beltran², Cari Levy^{2,3}

¹Veterans Health Administration, Ann Arbor, MI, USA; ²Veterans Health Administration, Aurora, CO, USA; ³University of Colorado School of Medicine, Aurora, CO, USA

Correspondence: Cari Levy (cari.levy@va.gov)

Implementation Science 2026, **21**(2):S75

Background: The Preferences Elicited and Respected for Seriously Ill Veterans through Enhanced Decision-Making (PERSIVED) intervention aimed to increase documentation of life-sustaining treatment (LST) preferences in the Department of Veterans Affairs (VA) electronic health record. PERSIVED partnered with VA Home Based Primary Care (HBPC) Programs and used audit with feedback and implementation facilitation to increase LST template completion for HBPC Veterans. We describe PERSIVED design and the impact of implementation activities tailored for sites.

Methods: HBPC programs were eligible for PERSIVED if < 50% of their patients had completed LST templates. The 6-month pre-implementation phase included interviews ($n = 56$) with site staff to identify champions and barriers and facilitators for completing LST templates. During the 15-month implementation phase, site champions participated in coaching calls and received feedback reports showing rates of LST template completion. Process evaluation interviews ($n = 67$) were conducted at months six and twelve during implementation and used to refine implementation activities. During the 12-month sustainability phase, feedback reports continued, but coaching calls ended, and sites worked independently to maintain improvements.

Findings: Eleven VA HBPC programs and 32 champions participated in PERSIVED, with 2–5 champions per site. Baseline interviews were used to design site-specific implementation plans. Feedback gathered during coaching calls and process evaluation interviews led to iterative tailored implementation activities such as adjusting the frequency of coaching calls, personalizing feedback reports, and providing education outside of regular calls. In interviews, participants mentioned the benefits of tailored feedback reports and satisfaction with the changes made to coaching calls. All 11 HBPC programs participating in PERSIVED increased LST template completion by the end of implementation (rates ranged from 64%–93%) and maintained these improvements during sustainability, suggesting that tailored audit with feedback and facilitation strategies contributed to program engagement and improvements in LST template documentation rates.

Implications for D&I Research: PERSIVED provides a case example of the design and evaluation of a quality improvement project and highlights the importance of collecting data at multiple stages to allow for continuous refinement of implementation strategies. Lessons learned can be used by implementation scientists to support design, implementation, evaluation, and sustainment of interventions conducted in complex healthcare settings.

Primary Funding Source: Department of Veterans Affairs

S76

Costs to implement an evidence-based practice intervention for seriously ill veterans in the Department of Veterans Affairs

Tiffany Radcliff^{1,2}, Matthew Griffith^{2,3}, Kate Magid², Jennifer Kononowech⁴, Anne Sales^{4,5}, Cari Levy^{2,3}

¹Texas A&M University School of Public Health, College Station, TX, USA;

²Veterans Health Administration, Aurora, CO, USA; ³University of Colorado School of Medicine, Aurora, CO, USA; ⁴Veterans Health Administration, Ann Arbor, MI, USA; ⁵University of Missouri, Columbia, MO, USA

Correspondence: Tiffany Radcliff (tiffany.radcliff@tamu.edu)

Implementation Science 2026, **21**(2):S76

Background: Evidence supports documenting life-sustaining treatment (LST) preferences in electronic medical records (EMRs) to align end-of-life care with a patient's goals and values. The Preferences Elicited and Respected for Seriously Ill Veterans (PERSIVED) intervention aimed to improve documentation of LST preferences for Veterans receiving VA Home Based Primary Care (HBPC). This analysis assessed PERSIVED implementation costs.

Methods: Descriptive assessment of costs to implement PERSIVED at each HBPC site and Central Program was conducted. A centralized database tracked program activities, staff time, and other resources by project phase (pre-implementation, implementation, and

sustainability). Personnel costs were estimated using national hourly wage data plus a standard allowance for fringe benefits and overhead costs. Costs estimates were compared across sites to assess sensitivity of the results to intervention site locations, site engagement, and team composition.

Findings: Site-level personnel costs ranged from under \$1,000 to more than \$3,500 per site, with another \$2,250 to \$4,200 in Central Program costs to support program delivery. Cost variation by site reflected differences in the types and number of staff involved and their level of participation, with clinicians having higher hourly costs than non-clinicians and some sites including more participants in intervention activities. Total personnel costs across all sites were estimated at \$52,734, with approximately \$22,234 for site-level personnel and \$30,500 for centralized staff. Central Program resources to implement PERSIVED relied heavily on core staff who delivered the intervention across sites; their efforts included maintaining systems for documenting intervention activities, recruiting site-level champions, coaching sites, and adjusting the intervention. HBPC site-level costs were primarily driven by VA staff whose time was allocated from other duties to serve as champions or participate in the intervention. Estimated total FTE for each HBPC site ranged from 0.02 to 0.07, and from 0.05 to 0.09 FTE for Central Program support per site.

Implications for D&I Research: PERSIVED represents a potentially low-cost, high-value strategy to improve LST documentation for Veterans in HBPC. Gaining a better understanding of the resources and related costs of interventions that improve documentation of patient preferences supports better patient care quality and more informed decisions regarding resources required for broader dissemination of LST coaching interventions.

Primary Funding Source: Department of Veterans Affairs

S77

Implementation outcomes of the Preferences Elicited and Respected for Seriously Ill Veterans through Enhanced Decision-Making intervention

Leah Haverhals^{1,2}, Jennifer Kononowech³, Kate Magid², Jazmin Beltran², Paula Langner², John Cashy⁴, Cari Levy^{1,2}

¹University of Colorado School of Medicine, Aurora, CO, USA; ²Veterans Health Administration, Aurora, CO, USA; ³Veterans Health Administration, Ann Arbor, MI, USA; ⁴Veterans Health Administration, Philadelphia, PA, USA

Correspondence: Leah Haverhals (Leah.haverhals@va.gov)

Implementation Science 2026, **21(2)**:S77

Background: The Department of Veterans Affairs (VA) quality improvement program, Preferences Elicited and Respected for Seriously Ill Veterans through Enhanced Decision-Making (PERSIVED), sought to improve documentation of life-sustaining treatment (LST) preferences for seriously ill Veterans. The PERSIVED team enrolled VA Home Based Primary Care (HBPC) teams in an audit with feedback and facilitation intervention using a six-month pre-implementation and 15-month implementation period. During implementation, HBPC site champions received monthly coaching calls and feedback reports showing LST template completion rates.

Methods: We conducted a difference-in-differences analysis to compare changes in monthly LST template completion rates at PERSIVED sites ($n = 11$ HBPC teams) versus non-intervention sites ($n = 123$ HBPC teams). We also conducted process evaluation interviews ($n = 67$) at 12- and 18-months post-enrollment with champions and site leadership and analyzed data through applying the Tailored Implementation in Chronic Diseases (TICD) framework to understand facilitators and barriers to improving LST rates.

Findings: PERSIVED HBPC teams demonstrated a significant increase in LST template completion with an overall average treatment effect of 0.212 (95% CI: 0.142–0.282), corresponding to a 21.2 percentage point increase over expected trends. Participants expressed that reviewing

PERSIVED feedback reports facilitated improvements in LST template completion and that PERSIVED coaching calls and resources created accountability and improved education and awareness about requirements for LST template completion. Participants stated PERSIVED proved compatible to HBPC workflows and motivated some sites to create multi-disciplinary team processes to ensure LST template completion. Participants were confident LST template completion would continue to improve, indicating high self-efficacy, intention and motivation, and positive attitudes toward completing LST templates. Staff turnover and competing priorities were barriers at some sites.

Implications for D&I Research: Our evaluation of PERSIVED suggests audit with feedback and facilitation were effective implementation strategies for a multi-site intervention and adds to the evidence on the efficacy of these strategies. Utilizing qualitative and quantitative approaches to understand the impact of PERSIVED proved critical and provided a comprehensive picture of intervention outcomes. Evaluators and researchers should consider incorporating multiple outcome measures in large implementation projects like this to better prepare efforts to expand and encourage sustainability.

Primary Funding Source: Department of Veterans Affairs

S78

Barriers and facilitators to sustaining improvements in documenting life sustaining treatment preferences for Veterans in home based primary care

Kate Magid¹, Leah Haverhals¹, Jennifer Kononowech², Jazmin Beltran¹, Cari Levy^{1,3}

¹Veterans Health Administration, Aurora, CO, USA; ²Veterans Health Administration, Ann Arbor, MI, USA; ³University of Colorado School of Medicine, Aurora, CO, USA

Correspondence: Kate Magid (kate.magid@va.gov)

Implementation Science 2026, **21(2)**:S78

Background: The Preferences Elicited and Respected for Seriously Ill Veterans through Enhanced Decision-Making (PERSIVED) intervention was developed to improve documentation of life-sustaining treatment (LST) preferences for Veterans receiving Department of Veterans Affairs (VA) Home Based Primary Care (HBPC). During the 15-month intervention, VA HBPC clinicians received monthly feedback reports and coaching from the PERSIVED team. Following implementation, programs entered a 12-month sustainability period during which feedback report data continued but coaching ceased. This abstract describes LST documentation rates of PERSIVED sites during the sustainability period and facilitators and barriers impacting sustainment.

Methods: We recruited HBPC programs with LST preference documentation rates of < 50% to participate. In total, PERSIVED included three cohorts with 11 HBPC programs and 32 champions. We conducted 10 semi-structured group interviews at months 4 ($n = 26$ champions) and 10 ($n = 25$ champions) of the sustainability period. One site declined to participate. Interviews were transcribed and analyzed via content analysis. Additionally, champions ($n = 25$) completed a survey about sustainability practices during month 8 of the sustainability period. We calculated the percentage of Veterans in each HBPC program with completed LST templates at the end of the sustainability period and determined which sites maintained or exceeded their highest LST template completion rate achieved during implementation.

Findings: At the end of the sustainability period, all sites maintained or exceeded the LST preferences documentation rate they achieved during implementation. Rates ranged from 75% to 96%. Champions continued to review feedback reports and document LST preferences, though use of materials provided by the PERSIVED team proved minimal. Facilitators to sustainment included leadership support, integrating LST documentation into existing processes and meetings, and training newly hired staff on LST documentation processes. Barriers included lacking ways to monitor LST documentation rates after PERSIVED ended, staffing shortages, and competing priorities.

Implications for D&I Research: Results highlight benefits of longitudinal assessments of sustainability. Given personnel and time investment of monthly coaching calls, future projects should explore coaching dosage required to sustain improvements and whether this can be tailored to each site. Barriers and facilitators to sustainability can inform sustainability planning for future interventions within health care settings.

Primary Funding Source: Department of Veterans Affairs

Closing health gaps through implementation science

S79

Fidelity measures for low threshold care as predictors of implementation outcomes

Wayne Kepner¹, Hannah Cheng², Berkeley Franz³, Andrea Jakubowski⁴, Margaret Lowenstein⁵, Mark McGovern⁶, Elena Rosenberg-Carlson⁶
¹Stanford Medicine, San Francisco, CA, USA; ²Stanford University School of Medicine, Stanford, CA, USA; ³Ohio University Heritage College of Osteopathic Medicine, Athens, OH, USA; ⁴Montefiore Medical Center/Albert Einstein College of Medicine, Bronx, NY, USA; ⁵Perelman School of Medicine, University of Pennsylvania, Philadelphia, PA, USA; ⁶Stanford University School of Medicine, Palo Alto, CA, USA

Correspondence: Wayne Kepner (wkepner@gmail.com)

Implementation Science 2026, 21(2):S79

Background: Measures of intervention fidelity are critical to assessing implementation outcomes, but such measures are often neither standardized nor validated. Low threshold care (LTC) approaches for medications for opioid use disorder (MOUD) aim to systematically remove treatment barriers and improve access. Yet no standardized implementation measures exist for LTC. We aimed to develop and test four tiered LTC indices as fidelity measures—representing the “Implementation” of RE-AIM—and examined their association with key implementation outcomes across 20 California primary care clinics initiating and expanding MOUD services.

Methods: Data come from a prospective implementation study within the Addiction Treatment Starts Here practice change collaborative (Feb 2023–June 2024). Implementation strategies included audit and feedback, learning collaborative workshops, external facilitation, and educational webinars. We developed four LTC indices (12, 5, 3, and 2 items) from the validated Integrating Medications for Addiction Treatment in Primary Care–Equity Version (IMAT-PCE) tool based on expert recommendations and federal guidelines. Implementation outcomes, assessed monthly, were informed by RE-AIM: Reach (new MOUD patients), Effectiveness (34-day retention), and Adoption (active prescribers). We used repeated-measures ANOVA for LTC change and Poisson GEE models for associations between LTC scores and implementation outcomes.

Findings: Clinics showed significant improvements in LTC scores over time. The LTC12 scale demonstrated the largest effect size ($d = 1.18$, $p = 0.003$), followed by LTC5 ($d = 1.1$, $p = 0.009$), and LTC2 ($d = 0.89$, $p = 0.029$). In terms of predictive validity, a one-point increase on the LTC3 index was associated with a 37% increase in Reach (incidence rate ratio [IRR] = 1.37, 95% CI [1.01, 1.86], $p = 0.047$). A one-point increase on LTC2 was associated with a 24% increase in Reach (IRR = 1.24, 95% CI [1.01, 1.53], $p = 0.049$).

Implications for D&I Research: Our findings provide preliminary empirical support for a replicable measure of LTC in primary care settings. Longer scales had greater internal consistency and were more sensitive to change over time, while brief scales showed utility for predicting patient reach. Future research should validate these measures in diverse settings and test causal relationships between fidelity of LTC and implementation outcomes.

Primary Funding Source: National Institutes of Health

S80

Use of a comprehensive assessment practice for high-risk primary care patients: fidelity to complex practice implementation

Natasha Torrence¹, Caroline Yoo², Franya Hutchins^{1,3}, Evelyn Chang^{2,4}, Susan Stockdale², Ashok Reddy^{5,6}, Elvira Jimenez², Michelle Wong², Kelsey Cummings², Emily Wong⁷, Katharine Borthwick², Ann-Marie Rosland^{1,8}

¹Veterans Health Administration, Pittsburgh, PA, USA; ²Veterans Health Administration, Los Angeles, CA, USA; ³University of Pennsylvania Perelman School of Medicine, Philadelphia, PA, USA; ⁴University of California Los Angeles, Los Angeles, CA, USA; ⁵Veterans Health Administration, Seattle, WA, USA; ⁶University of Washington, Seattle, WA, USA; ⁷Veterans Health Administration, Menlo Park, CA, USA; ⁸Division of General Internal Medicine, University of Pittsburgh School of Medicine, Pittsburgh, PA, USA

Correspondence: Natasha Torrence (Natasha.Torrence@va.gov)

Implementation Science 2026, 21(2):S80

Background: Fidelity to new practices is important to maximize effectiveness but can be difficult to enact with complex practices in real-world settings. The VA High-Risk VETERans (RIVET) QUERI Program implemented Comprehensive Assessment and Care Planning (CACP) to identify and address needs of high-risk patients in primary care (PC) clinics. The CACP guides clinicians in assessing seven domains in a synchronous patient visit: advance care planning, behavioral health, health literacy, cognitive impairment, functional limitations, acute care use, and social needs. We assessed fidelity to CACP domains, needs identified, and care delivered in response.

Methods: 13 clinics delivered CACP to 991 patients. While PC teams often adapted CACP delivery, full-fidelity was defined as completing all domains. We used EHR data generated by CACP templates to assess clinic-level average and type of domains completed and needs identified, and EHR encounters to assess need-related care received within 90-days of CACP completion.

Findings: 7 clinics had “high” CACP domain fidelity ($> 6/7$ domains/patient on average, $< 1SD$ variation in domains/patient), 6 clinics had “low” fidelity ($< 5/7$ domains or $> 1SD$ /patient). The behavioral health domain was most often incomplete (most skipped in 5/13 clinics), while functional limitations was most often completed (12/13 clinics). 674/991 patients had ≥ 1 need identified, with functional limitations most frequently identified (53% patients assessed), and behavioral health needs least often identified (14%). The most common care received related to a CACP-identified need was a social work encounter (40% of those with any need). Over 50% of those with an identified behavioral health need had an encounter with a mental health provider in the following 90 days.

Implications for D&I Research: While many high-risk patient needs were identified and addressed, fidelity to this multi-component practice varied, with key domains frequently skipped in some clinics. Clinicians possibly perceived that certain domains were not needed by patients, would be hard to address if detected, or were repetitive with other clinic initiatives. The least assessed domain was associated with the highest amount of care received when needs were detected, suggesting low-fidelity sites may have missed opportunities to provide needed care. Challenges of defining, communicating, and adhering to full fidelity should be addressed when implementing complex practices.

Primary Funding Source: Department of Veterans Affairs

S81

Fidelity of HIV pre-exposure prophylaxis implementation in family planning clinics in Kenya

Irene Mukui¹, Sandra Urusaro², Kenneth Mugwanya³

¹University of Washington, Seattle, USA; ²Hospital of the University of Pennsylvania, Philadelphia, PA, USA; ³University of Washington, Seattle, WA, USA

Correspondence: Irene Mukui (imukui@uw.edu)

Implementation Science 2026, 21(2):S81

Background

Integrating HIV pre-exposure prophylaxis (PrEP) into family planning (FP) services offers a strategic opportunity to reach women at risk of HIV, who may benefit from PrEP for HIV prevention, particularly in Africa, where their HIV risk is high. We assessed fidelity to core components of oral PrEP delivery, in a large programmatic project integrating PrEP delivery in real-world FP clinics.

Methods

Between March 21 and December 22, we conducted a stepped-wedge cluster randomized trial, integrating oral PrEP into 12 FP clinics in Kisumu, Kenya, delivered by existing FP healthcare providers. Standardized client (SC) actors conducted unannounced visits to determine whether core components of integrated PrEP services were delivered as intended and evaluate service quality. SC actors were trained on case scripts reflecting key elements: HIV risk screening, eligibility assessment, and PrEP initiation. After each visit, SCs completed checklists documenting services received: behavioral risk assessment, HIV prevention counseling, PrEP counseling, safety assessment, PrEP dispensing, quality-of-care and service experience. Service quality was rated on a 5-point Likert scale. We used descriptive analysis to summarize the proportion of visits where each component was completed.

Findings

Overall, 985 women aged ≥ 15 years-initiated PrEP. Twenty-four (12 initiation, 12 follow up) SC visits were conducted, two per clinic. Behavioral risk assessment occurred in 83% of visits; HIV prevention counseling (condom use, knowing partner HIV status, post-exposure prophylaxis, male circumcision) at all visits (100%); and PrEP counseling (daily use, repeat HIV testing, side effects, PrEP not protective against STIs) in 58% of visits. Clinical safety assessments (acute HIV symptoms, comorbidities) were conducted in 42% of visits, PrEP dispensed in 83% of visits, and 79% of clients given a return date. For service quality, providers used clear language (mean = 4.0), listened attentively (3.8), and were respectful (3.7), with no reports of judgmental behavior. Scores were lower for privacy (2.6) and providing comprehensive information (2.5).

Implications for D&I Research

In this project integrating PrEP provision into FP services, fidelity to key components of PrEP delivery and quality of services was high, with some gaps noted for improvement. PrEP delivery is feasible with high fidelity, using existing staff in African FP clinics.

Primary Funding Source: National Institutes of Health

S82

Acción PrEP: testing the feasibility of community co-designed implementation strategies to advance PrEP uptake among Latinos in the U.S. south

Sarah Wilson¹, Judith Montenegro², Lisvel Matos³, Carrie Dombeck¹, Carolina Mejia⁴, Jamilah Taylor¹, Aleida Espinal², Megan Reaves¹, Cristofer Navas⁵, Samantha Fortier⁵, Amy Corneli⁵

¹Duke University School of Medicine, Durham, NC, USA; ²Latino Commission on AIDS, Durham, NC, USA; ³Duke University School of Nursing, Durham, NC, USA; ⁴Department of Population Health Sciences, Duke University, School of Medicine, Durham, NC, USA; ⁵Duke University, Durham, NC, USA

Correspondence: Sarah Wilson (sarah.wilson@duke.edu)

Implementation Science 2026, 21(2):S82

Background: Despite systemic inequities in pre-exposure prophylaxis (PrEP) uptake among Latinos, clinic-level implementation strategies that address health equity-related PrEP barriers are underutilized. This study aimed to test the feasibility and acceptability of community-co-developed, equity-focused strategies to improve PrEP uptake among Latinos in the southern U.S.

Methods: This study was a community/academic partnership and was conducted in Mecklenburg County, North Carolina, a jurisdiction with high HIV incidence and a rapidly growing Latino population. The

Acción PrEP workgroup, a collective of local community and public health constituents with HIV/PrEP experience, co-developed PrEP implementation strategies over six months (March-August 2023), using implementation mapping, human-centered design, and participatory methods. This process yielded Acción PrEP, a package of implementation strategies that were subsequently pilot-tested at a federally qualified health center and a non-profit community clinic from March-August 2024. Data on strategy fidelity and modifications were collected. Outcomes included: acceptability of the workgroup process (via exit surveys) and feasibility and acceptability of the implementation strategies (assessed qualitatively October 2024-February 2025 via key informant interviews with 7 pilot implementers and 16 community partners).

Findings: Acción PrEP workgroup baseline and exit data indicated inclusiveness of varying identities (e.g., Latino ethnicity and lived experience of HIV/PrEP) and high levels of collaboration/satisfaction. The workgroup developed three health equity-focused strategies: a needs and assets assessment, blended implementation facilitation (combining external facilitation from a community-based organization with internal clinic facilitation), and a bilingual, culturally tailored PrEP awareness mass media campaign. Pilot-testing process data indicated: 1) 100% fidelity to the needs and assets assessment, 2) 85% average completion of weekly facilitation meetings, and 3) delayed campaign dissemination. Adaptations included fewer facilitation meetings, increased leadership engagement during implementation, and adding a sustainability planning workshop. Key informant interviews supported acceptability and feasibility. Strengths of Acción PrEP included observation-based feedback, concrete change goals, and the overall focus on Latino health equity. Barriers included insufficient time, staffing, clinic space, and challenges in identifying internal facilitators.

Implications for D&I Research: The community co-design process and Acción PrEP strategies were acceptable and feasible. Study findings underscore the need for longer implementation timelines and stronger support and protected time for internal facilitator roles.

Primary Funding Source: National Institutes of Health

S83

How do policymakers prioritize interventions for health benefits packages? Insights from Ethiopia

Sali Ahmed

University of Washington, Seattle, WA, USA

Correspondence: Sali Ahmed (salia@uw.edu)

Implementation Science 2026, 21(2):S83

Background

When deciding which interventions to prioritize for health benefits packages, policymakers often need to balance multiple criteria and competing priorities. These include maximizing health outcomes, improving the distribution of health (i.e., equity), and providing financial risk protection. Little is known about how policymakers weigh these criteria, so we conducted a discrete choice experiment in Ethiopia to better understand local preferences.

Methods

Our experiment was conducted among 49 policymakers involved in developing Ethiopia's health benefits package. The experiment used a four-attribute, forced-choice design. Vignettes presented two hypothetical alternative interventions, totaling 18 vignettes. The attributes specified for the two alternatives were (1) absolute reduction in mortality, (2) severity of disease targeted, (3) age group targeted, and (4) absolute reduction in medical impoverishment (assuming full public finance). We estimated the average preference weights for each attribute using a random-parameter mixed logit model and calculated the conditional relative importance of each attribute based on our estimates.

Findings

Most respondents worked in the Ministry of Health or a local insurance agency and were men. Based on 864 total observations, all four criteria were statistically significant and varied monotonically across different attribute levels, as expected. The conditional relative importance of each of the four attributes was 30% for the targeted age group and absolute reduction in mortality, 22% for the reduction in medical impoverishment, and 19% for the severity of the disease targeted.

Implications for D&I Research

We found that Ethiopian policymakers involved in EHBP development tend to prioritize interventions that target younger age groups, especially <5 years, and achieve substantial mortality reduction. Financial risk protection and disease severity were less critical determinants of decision-making. These findings provide empirical preference weights that can be incorporated into multicriteria decision analysis (MCDA), thereby enhancing the systematic and transparent development of EHBP. This study presents a novel stakeholder-informed method to guide priority setting. Aligning health policy decisions with local values can support the implementation of impactful policies that can close the health gaps in resource-limited settings.

S84

Implementation of NOURISH: a clinic-community linkage model to address the social, nutrition, and health needs of pregnant persons

Robin Ortiz¹, Shilpi Mehta-Lee², Stephanie Quintero Chacon³, Kelly Naranjo⁴, Eric Rochman⁴, Priscilla Upshaw⁴, Jessica Gonzalez², Joanna Madigan⁵, Kathleen Hopkins⁵, Meleen Chuang², Rebecca Gallagher⁵, Brita Roy⁵, Antoinette Schoenthaler⁶, Gbenga Ogedegbe⁷, Christina Kim³, Rachel Gross³

¹NYU Grossman School of Medicine, New York City, NY, USA; ²NYU Grossman School of Medicine, Brooklyn, NY, USA; ³NYU Grossman School of Medicine, New York, NY, USA; ⁴God's Love We Deliver, New York, NY, USA; ⁵NYU Langone, Brooklyn, NY, USA; ⁶NYU Grossman School of Medicine, New York, USA; ⁷Institute for Excellence in Health Equity, NYU Grossman School of Medicine, New York, NY, USA

Correspondence: Robin Ortiz (robin.ortiz@nyulangone.org)

Implementation Science 2026, **21**(2):S84

Background: Pre- and gestational diabetes in pregnancy (GDM) poses risk for cardiovascular disease and disparities across generations. Medically-tailored meals (MTM) are an evidence-based intervention that also addresses food access, but requires clinic-community linkage (CCL) to overcome implementation barriers. God's Love We Deliver (GLWD), a community-based organization, has historically provided registered dietitian-guided MTM throughout New York for numerous chronic conditions. In 2023, a new program designed for pregnant persons with GDM launched. However, implementation was challenged by lack of direct communication with referring clinical providers, overlaid with the time-sensitive nature of GDM diagnosis and treatment within the third trimester.

Methods: To enhance the adoption of this CCL, we established the NOURISH (Novel Outreach Using caregiver-child Relationships to Impact Social and Health) initiative to identify implementation barriers and develop strategies to increase CCL referrals. A mixed method evaluation assessed pre-implementation contextual factors, harnessing the Consolidated Framework for Implementation Research (CFIR) framework, and evaluated the initial intervention implementation by quantifying referrals, enrollments (meals delivered), and dis-enrollments, and capturing provider and client feedback.

Findings: Pre-implementation work identified two referral process barriers (inner setting): 1) inefficiency (excessive clinical resources and time), and 2) bottleneck workflow (physician sign-off). Two implementation strategies were employed to address these barriers including the addition of an EPIC Electronic Health Record (EHR) SmartSet and

community health worker support. Over an 18-month CCL period, referrals grew from 13 (baseline) to 208 with a sustained pattern of 10+ /month referrals, ~80%/month client enrollments, and consistent dependent/child enrollments (from 0 prior) after launch of the implementation strategies. Patients referred were racial/ethnically diverse (7 categories including 62% Hispanic/Latino), spoke 7 languages, and were 83% publicly insured. Positive spillover consequences included improved overall healthcare system GDM care delivery, reduction in caregiver stress for clients with children, and increases in addressed social needs (48% of CCL patients received additional social service referrals). Identified challenges inform continued work to improve data infrastructure and support program fidelity.

Implications for D&I Research: Our findings highlight that CCL partnerships can effectively expand reach and engagement in clinical pregnant populations by strengthening the referral pathways, while also generating positive spillover effects, including increased support for social needs.

Primary Funding Source: Philanthropic & Academic

S85

Recovery incentives: examining California's statewide implementation of contingency management for the treatment of stimulant use disorder

Tyler Erath¹, Darren Urada², Valerie Antonini², Avery Nork², Brittany Bass², Howard Padwa², Yiyu Xing², Edward Zakher², Liliana Gregorio², Anne Lee², Savannah Gourley², Richard Rawson^{1,2}

¹Vermont Center on Behavior and Health, Department of Psychiatry, University of Vermont, Burlington, VT, USA; ²Integrated Substance Use and Addiction Programs, Division of Addiction Psychiatry, University of California Los Angeles, Los Angeles, CA, USA

Correspondence: Tyler Erath (tyler.erath@uvm.edu)

Implementation Science 2026, **21**(2):S85

Background

Expanding the implementation of effective treatment for stimulant use disorder (StimUD) has become increasingly urgent as fatal drug poisonings involving stimulants have rapidly increased in the United States (US). Despite three decades of evidence supporting Contingency Management (CM) as the most effective treatment for StimUD, its availability in community settings has been limited to non-existent until recent. In 2023, California initiated the Recovery Incentives program, the first statewide implementation of CM for StimUD. This presentation presents implementation results from the first two years of the program.

Methods

The Recovery Incentives program is being systematically implemented using the Science-to-Service Laboratory approach. This implementation strategy includes three core training components (e.g., didactic training, performance feedback, and external facilitation) along with site readiness assessments and ongoing fidelity monitoring. The evaluation approach is organized around the RE-AIM framework. Here we report on findings to date for three evaluation components: (1) adoption (number of provider agencies delivering CM), (2) reach (number of participants in CM treatment), and (3) effectiveness (% stimulant-negative urine drug test results).

Findings

Adoption: as of June 2025, 155 provider agencies have opted to participate in the Recovery Incentives program, 100 of which are currently approved and have enrolled at least one participant for an average adoption rate of 4.4 programs per month. **Reach:** Between April 2023 to April 2025, 7,462 unique participants participated in the program. **Effectiveness:** Among 5,020 participants enrolled long enough to complete the 24-week treatment, the average retention was approximately 18 weeks; 36.3% completed all 24 weeks,

29.8% completed 12–23.5 weeks, and 33.9% completed < 12 weeks. The overall rate of stimulant-negative urine drug test results was 95.5% for all completed test and 74.9% when including unexcused absences as an assumed positive test result.

Implications for D&I Research

Findings represent outcomes from the first statewide and largest known implementation of CM for StimUD to date. Evaluation outcomes indicate strong uptake and adoption of CM into organizational service delivery, positive outcomes for the Recovery Incentive program's reach, and strong evidence of CM effectiveness to reduce stimulant use among individuals with StimUD. Together, findings provide a potential implementation model for future use in statewide CM efforts.

Primary Funding Source: Centers for Medicare and Medicaid Services

S86

Scaling evidence-based a1c monitoring through automation and asynchronous messaging: a system-level implementation across a multi-state health system

Ursula Wright, John Mohart, Kalli Castille, Hillary Barron, Jessica Engelbrecht, Gavin Helton
Mercy Health, St. Louis, MO, USA

Correspondence: Ursula Wright (Ursula.Wright@Mercy.Net)

Implementation Science 2026, **21**(2):S83

Background: Despite clear guidelines, many patients with diabetes experience delays in A1c testing, contributing to poor glycemic control. Mercy Health, a multi-state health system, implemented evidence-based automated processing of A1c orders and digital messaging to close this care gap. The program uses automated order entry and outreach via secure SMS to notify patients of overdue or uncontrolled A1c tests. The system-level intervention aligns with dissemination and implementation (D&I) science by integrating evidence-based practice into routine care through health IT infrastructure.

Methods: Guided by the RE-AIM and PARIHS frameworks, the program leverages Mercy's unified EHR and centralized diabetes registry to identify eligible patients (ages 18–85 with A1c of ≥ 8.0 and last A1c test greater than 90 days or no A1c test in the last 365 days) across 272 primary care clinics in Missouri, Arkansas, Oklahoma, and Kansas. Providers review monthly reports before automated A1c orders are placed using standardized approved orders. Patients receive HIPAA-compliant SMS based messages with educational content and opt-out options. Implementation is supported by structured go-live process, clinician training, a communications plan, and real-time outcome tracking.

Findings: The automated process reduced manual order entry and standardized outreach while preserving provider oversight. Early implementation (Jan-Dec 2022), 24% of patients receiving automated A1c labs had the test drawn. In the most recent 12-month period (Jan-Dec 2024) 19% of patients receiving automated A1c labs had the test drawn. The program expanded by 400% over 2 years going from 14,000 patients limited to specific payor in 2022 to 60,000 patients (all payor) in 2024. Clinicians report reduced administrative burden and improved workflow. Patient messaging supports engagement, self-management, and reinforces completion of chronic disease monitoring care interventions.

Implications for D&I Research: This system-level intervention demonstrates how large, diverse health systems can operationalize evidence-based practices using automation and digital outreach to close chronic disease care gaps. Mercy's approach—anchored in standardized workflows and a unified EHR—demonstrates how system-level strategies overcome barriers like clinical burden and patient access. This program contributes to D&I science by showing how automation and digital infrastructure supports scalable, low-burden, high-reach model supported by real-time monitoring, structured implementation and continuous feedback and audit between stakeholders.

S87

Perspectives towards implementing an evidence-based protocol to provide timely home health and outpatient visits for Heart Failure (HF) patients after hospital discharge: early findings from the I-TRANSFER-HF trial

Madeline Sterling¹, Michael Dicipinigitis¹, Daniel Spertus², Michelle Shum¹, Joselyne Aucapina², Joanna Bryan¹, Jonathan Tobin^{3,4}, Mangala Rajan⁵, Alicia Arbaje^{6,7}, Christine Jones⁸, Karen Ream⁹, Tanya Budnikova¹⁰, Kathrine Eckstadt¹⁰, Pamela Manning⁹, Jessica Nensel⁹, Brittany Cyriack⁹, Hemali Patel⁹, Joy Prabhakar⁹, Zuzanna Czernik⁹, Margaret McDonald¹¹, Kathryn Bowles^{12,13}

¹Weill Cornell Medicine, New York, NY, USA; ²Weill Cornell Medicine, New York, USA; ³The Rockefeller University Center for Clinical and Translational Science, New York, NY, USA; ⁴Clinical Directors Network, Inc. (CDN), New York, NY, USA; ⁵Weill Cornell Medical College, New York, NY, USA; ⁶Johns Hopkins University, Baltimore, USA; ⁷Johns Hopkins University, Baltimore, MD, USA; ⁸University of Colorado Denver—Anschutz Medical Campus, Aurora, CO, USA; ⁹University of Colorado Denver—Anschutz Medical Campus, Aurora, USA; ¹⁰Berkley Home Health Care, Aurora, USA; ¹¹VNS Health, New York, NY, USA; ¹²University of Pennsylvania, Philadelphia, USA; ¹³VNS Health, Center for Home Care Policy and Research, New York, NY, USA

Correspondence: Madeline Sterling (mrs9012@med.cornell.edu)

Implementation Science 2026, **21**(2):S87

Background: Timely receipt of home health care and outpatient follow-up after hospitalization improves outcomes in HF, yet only 12% of Medicare beneficiaries receive these evidence-based practices. We are conducting a Hybrid Type 1 stepped-wedge cluster RCT study to test the effectiveness and identify contextual factors for implementation across 4 hospital and home health agency (HHA) dyads. We assessed the participants' pre-implementation perspectives and readiness for change from 2 sites.

Methods: We conducted semi-structured interviews informed by the Consolidated Framework for Implementation Research 2.0 (CFIR2.0) to determine facilitators and barriers. Organizational Readiness for Implementing Change (ORIC) scores, a 12-item survey assessing change efficacy and confidence (score range 12–60), were summarized with descriptive statistics.

Findings: We conducted 42 pre-implementation interviews across 2 hospital-HHA dyads. Site 1 is a Mid-Atlantic, private academic medical center with an integrated HHA; Site 2 is a Mountain West, public university hospital associated with a small HHA. Across both sites, HHA-based participants had greater readiness than hospital-based participants (Site 1 ORIC median 55.0 [IQR: 53.0, 59.0] vs. 41.0 [38.0, 49.5], $p=0.03$; Site 2 ORIC median 57.5 [55.0, 60.0] vs. 48.5 [44.0, 52.0], $p=0.05$). Unique implementation facilitators included an integrated hospital-HHA with shared EHR, a HF bridge clinic, and adaptability for home care physical-therapy visits versus nursing (Site 1), along with prior experience with a similar study, a central telecommand center, and HHA workforce capacity (Site 2). Shared facilitators included mission alignment and compatibility of the intervention with existing workflows. Unique barriers were patient characteristics, limited inpatient staff time, and access to follow-up appointments (Site 1), as well as limited inpatient education on home care and insurance denials (Site 2). Shared barriers included eligible patient identification, timely outpatient provider access, and patient refusal.

Implications for D&I Research: This is the first known implementation of a pragmatic, evidence-based intervention to improve post-discharge outcomes for HF patients across hospital-HHA dyads. Findings highlight the importance of identifying common and site-specific facilitators and barriers to tailor strategies. Sustained practice change may depend on leveraging local resources and institutional knowledge between the hospital-HHA dyads, engaging stakeholders beyond formal meetings, and promoting cross-site learning.

Primary Funding Source: National Institutes of Health.

S88

Implementation of a multi-site financial navigation intervention in rural and non-rural oncology clinics: a hybrid II implementation-effectiveness clinical trial

Stephanie Wheeler¹, Austin Waters², Michelle Manning³, Arrianna Planey³, Bridgette Thom³, Lisa Spees³, Sarah Birken⁴, Katherine Reeder-Hayes³, Camille Murray³, Kevin Pignone³, Allison Deal³, Donald Rosenstein³
¹Lineberger Comprehensive Cancer Center, UNC Chapel Hill, Chapel Hill, NC, USA; ²Dana-Farber Cancer Institute, Boston, MA, USA; ³University of North Carolina at Chapel Hill, Chapel Hill, NC, USA; ⁴Wake Forest University School of Medicine, Winston-Salem, NC, USA

Correspondence: Stephanie Wheeler (stephanie_wheeler@unc.edu)
Implementation Science 2026, **21**(2):S88

Background: Financial toxicity (FT) among cancer survivors is prevalent and associated with worse quality of life, caregiver distress, and higher mortality. Although financial navigation (FN) has reduced FT in urban settings, evaluation of FN within geographically diverse practices is needed to reach broader populations suffering from FT.

Methods: We conducted a pragmatic hybrid II implementation-effectiveness clinical trial (Lessening the Impact of Financial Toxicity, LIFT) in 8 academic and non-academic oncology practice sites in North Carolina, adapting FN for 4 rural sites and 4 non-rural sites serving rural patients, with rurality defined using rural-urban community area (RUCA) codes greater than 3. All cancer patients within five years of diagnosis with a FACIT-COST score ≤ 22 (indicating moderate-to-severe FT) who understood English were eligible. Participants were recruited through clinic appointments, provider referral, or self-referral. FN, which consisted of comprehensive intake, 2–8 follow-up FN sessions, case management and personalized support for resource applications, was provided by local staff champions, mostly social workers, who received ~17 h training in FN, participated in monthly technical assistance calls, and joined monthly learning collaborative calls (i.e., implementation strategies). The primary effectiveness outcome was change in FACIT-COST score, pre- versus post-LIFT implementation. Implementation outcomes included uptake, fidelity, acceptability, costs, and sustainability. Here, we report on the FACIT-COST score change and uptake (# served by FN/# screened positive for FT).

Findings: Of 258 cancer survivors consented, 233 screened FT-positive. 171 cancer survivors received FN, and 125 completed all follow-up surveys. Survivors with complete data were, on average, 51.7 years (SD:14.2), more often female (65%), and non-Hispanic White (56%) or Black (36%), with 8% identifying as Hispanic; 42.4% lived in a rural area. After FN, FACIT-COST scores significantly increased by an average of 8.5 points ($p < 0.0001$; pre-mean score: 6.8, SD:5.1; post-mean score: 15.3, SD:10.1). Changes in FACIT-COST scores were similar for rural and non-rural survivors. Uptake was high, with 73.4% of those offered FN receiving it.

Implications for D&I Research: Social needs interventions like FN can be adapted to benefit diverse rural communities.

Primary Funding Source: National Institutes of Health

S89

Evaluating an implementation intervention for increasing mental health providers' adoption of best practices with transgender youth: a nonrandomized hybrid type 2 pilot trial

Maggi Price¹, Patrick Mulkern¹, Nathan Hollinsaid², Madelaine Condon¹
¹Boston College, Chestnut Hill, MA, USA; ²Harvard University, Cambridge, MA, USA

Correspondence: Maggi Price (maggi.price@bc.edu)
Implementation Science 2026, **21**(2):S89

Background

Transgender youth face significant mental health inequities, and standard treatments fail to meet their needs. Gender-Affirming Psychotherapy (GAP) is a community-co-designed clinical intervention comprising evidence-based practices (e.g., using affirmed names and pronouns) designed to close this gap. To facilitate GAP adoption, we

co-developed and tested an implementation intervention (bundle of implementation strategies) with key stakeholders: transgender youth, their parents, and mental health providers.

Methods

$N=78$ mental health providers, $N=22$ transgender youth, and $N=19$ parents completed a nonrandomized hybrid type 2 trial (NCT05626231) of the implementation intervention. Data from 18 months of Electronic Health Record (EHR) records from $N=125$ transgender youth were analyzed. The intervention comprised 12 implementation strategies: 8 individual-level strategies (e.g., shadow, simulate change) were embedded in an asynchronous training, and 4 inner-setting strategies (e.g., change data systems) were implemented clinic-wide. Providers were evaluated at pre-, post-, and monthly follow-up assessments for one year post-intervention. Youth and parents were evaluated weekly for 4 months post-intervention. The implementation outcomes were GAP adoption (reported by providers, transgender youth clients, and their parents) and provider knowledge of, attitudes towards, and self-efficacy using GAP. Effectiveness outcomes were transgender youth functioning, internalizing, and externalizing symptoms.

Findings

Implementation: Providers self-reported a significant increase in GAP adoption at 1-month ($\beta=0.80$) and 1-year follow-up ($\beta=0.38$). Transgender youth ($\beta=0.45$) and their parents ($\beta=0.80$) reported a significant increase in their providers' GAP adoption after intervention implementation. Provider knowledge ($\beta=1.28$), attitudes ($\beta=0.38$), and self-efficacy ($\beta=1.17$) showed significant improvement post-intervention; outcomes remained significant at 1-year follow-up (knowledge $\beta=1.32$; attitudes $\beta=0.26$; self-efficacy $\beta=1.14$). **Effectiveness:** Transgender youth experienced a significant increase in functioning, controlling for time and covariates ($\beta=0.46$), as reported by their parents; internalizing and externalizing symptoms results were nonsignificant.

Implications for D&I Research

This hybrid pilot demonstrated that a multilevel intervention is preliminarily effective in improving provider- and patient-level outcomes for a critical health disparities population. Researchers should consider community co-designed individual- and system-level strategies to close health disparity gaps. Finally, leveraging EHR data may be an efficient and low-burden method for analyzing patient-level outcomes.

Primary Funding Source: National Institutes of Health.

S90

A change in STRIDE: Site-reported adaptations of an evidence-based inpatient mobility program

Leah Zullig^{1,2}, Connor Drake^{1,2}, Jaime Hughes^{1,3}, Karen Stechuchak¹, Caitlin Kappler¹, Ashley Choate¹, Cassie Meyer¹, Amy Webster¹, Kelli Allen^{1,4}, Courtney Van Houtven², Susan Hastings^{1,2}

¹Veterans Health Administration, Durham, NC, USA; ²Duke University School of Medicine, Durham, NC, USA; ³Wake Forest University School of Medicine, Winston-Salem, NC, USA; ⁴University of North Carolina Chapel Hill School of Medicine, Chapel Hill, NC, USA

Correspondence: Leah Zullig (leah.zullig@duke.edu)
Implementation Science 2026, **21**(2):S90

Background: Hospital-based inactivity among older adults often leads to functional decline. While mobility programs like STRIDE (assisted early mobility for hospitalized older veterans) have proven effective, their implementation in clinical settings remains understudied. Program adaptation may improve implementation but requires study of how facilities modify intervention components to preserve intervention effectiveness and improve fit within the clinical context. Our objectives were to: 1) report adaptations made to the STRIDE program across Veterans Affairs Medical Centers (VAMCs) during implementation, using the Framework for Reporting Adaptations and Modifications-Enhanced (FRAME); and 2) assess the relationship of adaptations with adoption and sustainment.

Methods: This was part of the Function QUERI (Quality Enhancement Research Initiative), a type III effectiveness-implementation hybrid study. Thirty-five VAMCs were randomized 1:1 to receive foundational or enhanced implementation support. Site-level contacts completed forms at pre-implementation, six, and ten months, documenting adaptations to program eligibility, staffing, marketing, and documentation. Adaptations were analyzed using collaborative coding to determine alignment with core STRIDE components and fidelity. We defined programmatic benchmarks for adoption or sustainment at 6 or 8 months (defined as having 5 or more patients receiving STRIDE in a general medicine bed section in months 5–6 and 10 or more patients receiving STRIDE in months 7–8, respectively).

Findings: Of the 35 participating sites, 29% (10 sites) met initial adoption benchmarks at 6-months, increasing to 51% (18 sites) by eight months. Sites implemented various adaptations across four categories: patient eligibility, program enhancements and staffing model, program documentation, and program marketing and education. The adaptations were classified according to FRAME criteria including timing, planning status, decision-makers, modification type, delivery level, and fidelity consistency. All sites reported at least one adaptation. The largest number of adaptations per site was seven. We found that adaptations were positively associated with the likelihood of adopting an inpatient mobility program.

Implications for D&I Research: Our study demonstrates that sites can successfully implement fidelity-consistent adaptations, which may lead to improved implementation outcomes. Our study compliments existing research which shows that adaptations can enhance the local fit and increase uptake of interventions.

Primary Funding Source: Department of Veterans Affairs

S91

A 3-Step implementation model for family-based HIV prevention for Black girls through academic-community partnerships

Natasha Crooks

University of Illinois Chicago, Chicago, IL, USA

Correspondence: Natasha Crooks (ncrooks@uic.edu)

Implementation Science 2026, **21**(2):S91

Background: Black girls are disproportionately impacted by HIV and STIs in the United States. Engaging Black men in family-based HIV prevention programming provides a unique opportunity to protect Black girls. Community engagement through academic partnerships can support efforts to deliver family-based programming. To support sustainability, we employed a 3-Step Implementation Model (Prepare, Rollout, and Sustain) to engage community-based organizations (CBOs) in a randomized control trial ($n=300$ dyads) of a family-based program in Chicago.

Methods: CBOs partnered with an academic institution to implement IMAGE, a family-based program for Black girls and men. Guided by the determinants of EPIS, we assessed implementation factors through a mixed-methods evaluation using interviews, observations/notes, and surveys. We conducted an adapted framework analysis, organizing coded data in a matrix categorizing barriers and facilitators by low, medium, or high impact through data triangulation.

Findings: Structured, evidence-based implementation strategies were applied consistently across all CBOs ($n=8$). CBO engagement levels varied and were driven by inner context characteristics. During *Prepare*, the most significant factors were mission alignment and fit, funding constraints, and facility availability. Four CBOs advanced to *Rollout*, collectively delivering 10 IMAGE groups with an average fidelity score of 83%. Successful *Rollout* was associated with high levels of organizational leadership, team cohesion, mission alignment, and program fit. Outer-context factors, including service environment/policies limitations and challenges with participant access, emerged as major barriers. To transition to *Sustain*, local political will and capacity, and organizational mission alignment and fit are necessary. Two CBOs demonstrated potential to sustain IMAGE independently, while the

others required additional partnerships to address limited access to the target population.

Implications for D&I Research: This study demonstrates that applying a structured framework like EPIS with a 3-Step implementation plan can effectively facilitate the delivery of evidence-based interventions through community-academic partnerships. Key organizational and contextual factors, including mission and fit alignment, leadership, and the local environment, influence program success. Identifying and understanding these factors enables implementation scientists and community partners to develop tailored strategies to overcome barriers and maximize the impact of interventions to improve sexual and reproductive health outcomes among marginalized communities.

Confirmation: Ethics approval UIC IRB: 2023–1317

Primary Funding Source: National Institutes of Health

S92

Facilitators and barriers to implementing H-HOPE (Hospital-to-Home: Optimizing the Preterm Infant Environment) in six NICUs in the USA

Rosemary White-Traut

Children's Hospital of Wisconsin, Milwaukee, WI, USA

Correspondence: Rosemary White-Traut (rwhite-traut@childrenswi.org)

Implementation Science 2026, **21**(2):S92

Abstract Text

Background

H-HOPE is an example of a major implementation science and health care gap, the failure of an evidence-based behavioral intervention to become the standard of care. H-HOPE includes parent education and infant massage, delivered by parents. Our implementation using a modified 3-step implementation model research focused on assisting nursing administration and clinical staff adopt H-HOPE for preterm infants and parents in the neonatal intensive care unit (NICU) and the transition to home. This presentation identifies key facilitators and barriers that distinguished between sustaining and non-sustaining NICUs.

Methods

Our Hybrid Design examined H-HOPE implementation effectiveness and process across six NICUs that varied in size, acuity, and location. Two pairs of NICUs were in the same hospital system. Using the Consolidated Framework for Implementation Research, we described complex implementation factors across five domains: innovation, outer setting, inner setting, individuals, and implementation process. The final analysis examined facilitators and barriers affecting the decision to sustain.

Findings

Two NICUs from the same health system made a clear decision to sustain. Powerful barriers across all NICUs, including the two sustaining sites, were innovation-related complexity and insufficient hands-on training, and the inner setting work infrastructure, which was compromised by understaffing, insufficiently oriented new hires, high patient workloads, and leadership disruption, undermining leader engagement. Barriers were linked to non-compatibility of H-HOPE within the NICU and an individual barrier of low opportunity to provide H-HOPE. In the two sustaining NICUs, these barriers were offset by: parent-centered and developmentally focused unit culture, which supported greater endorsement of H-HOPE and greater individual capability to provide H-HOPE. These units also worked well as a team and adapted two aspects of the protocol to better fit their NICU.

Implications for D&I Research

An innovation with a strong evidence base may still present formidable implementation challenges. Interacting structural barriers reduced implementation feasibility. However, unit culture and process teamwork and adaptations allowed some NICUs to overcome these barriers and sustain H-HOPE, highlighting the importance of strong facilitators to overcome barriers. Future H-HOPE implementation should be modified by initially strengthening unit culture and infant behavior knowledge prior to more intensive and hands-on training.

Confirmation: IRBPRO00035600, Medical College of Wisconsin

Primary Funding Source: National Institutes of Health

S93

Community sustainment of an effective HIV prevention program in Malawi after funding

Kathleen F. Norr

Children's Wisconsin, Milwaukee, WI, USA

Correspondence: Kathleen F. Norr (norr.kathleen@gmail.com)

Implementation Science 2026, 21(2):S93

Background: A persistent implementation science gap is the failure of effective interventions to continue after funding ends. This success story shows that capacity-building enables post-funding community sustainment. Our six-session peer group HIV prevention intervention, *Mzake ndi Mzake* (Friend-to-Friend), initially implemented with NIH funding, is being sustained by communities. Program efficacy when implemented by health workers was established in prior studies. However, health worker shortages meant that the health system could not take ownership. Transferring *Mzake* to communities with health system support was a promising strategy that we tested. To guide implementation, we developed a simple 3-Step Implementation Model (prepare, rollout, sustain) for community use.

Methods: This effectiveness-implementation study was conducted in three communities in Phalombe District, which had high poverty and HIV rates. Using a stepped-wedge design, four repeated surveys ($n = 1008$), and a process evaluation were completed.

Findings: Community volunteers in all three communities successfully followed the 3-Step Model to deliver *Mzake* with fidelity and effectiveness. During Prepare, communities formed Coordinating Committees. During Rollout, communities offered *Mzake* to 1139 individuals; all but four individuals completed the 6-session program. Volunteer-led *Mzake* remained effective. HIV Knowledge increased and was significantly higher in the intervention group (log-odds estimate = 0.17, SE = 0.06, p -value < 0.01). Condom use at last sex increased in intervention communities compared to the control community [Adjusted Odds Ratio: 1.81 (1.13, 2.90)]. During Sustain, communities wrote proposals funded from the project. Through their projects, trained facilitators increased from 48 to 85, and *Mzake* reached 7,196 new people. After funding ended, communities chose to continue the program. Retired researchers and local leaders established the Ekodda Foundation to support *Mzake* and other initiatives. In the past three years, *Mzake* expanded to 6 new communities, reaching 1521 more people.

Implications for D&I Research: Filling an implementation gap, the 3-step model enabled communities to successfully implement and sustain *Mzake* beyond initial funding. Turning implementation over to communities has high potential to expand HIV prevention without burdening the health system, supporting long-term sustainment. Sustaining requires mutual trust and limiting researchers' control while building capacities to support the successful transfer of ownership to communities.

Confirmation: Ethics: COMREC P.10/15/1815 and UIC #2015 – 0691

Primary Funding Source: National Institutes of Health

S94

Scaling up midwifery-led and evidence-based group prenatal care in Malawi

Crystal Patil

University of Michigan, Ann Arbor, MI, USA

Correspondence: Crystal Patil (clpatil@umich.edu)

Implementation Science 2026, 21(2):S94

Background: Improving perinatal outcomes globally demands scalable implementation of evidence-based, person-centered models that retain women in care. To address this gap, we completed a rigorous hybrid effectiveness-implementation trial of Centering-based Group prenatal care (PNC) in seven clinics in Blantyre District.

Methods: We followed a 3-Step Implementation Model (Prepare, Rollout, and Sustain), and our process evaluation was guided by CFIR and

RE-AIM, triangulating data from fidelity checklists, training records, clinical records, surveys, and study notes.

Findings: During *Prepare*, we provided capacity-building training for 299 midwives. During *Rollout*, 84 groups were completed with high fidelity. Compared to individual PNC, women in Group PNC reported greater satisfaction with care (Estimate = 1.21, 99% CI = 0.07, 2.35), more visits (Estimate = 0.74, 99% CI = 0.50, 0.98), greater dietary diversity (Estimate = 0.35, 99% CI = 0.17, 0.53), birth preparedness (Estimate = 0.32, 99% CI = 0.16, 0.48), and consistent condom use (OR = 1.07, 99% CI = 1.00, 1.14), and less mental distress (Estimate = -0.61, 99% CI = -1.20, -0.02). More than 80% of women preferred Group PNC for a future pregnancy. During *Sustain* we provided ongoing mentorship for clinic teams and monitored new and existing site progress. Implementation strategies, including capacity-building training and mentor development, provided a replicable framework for establishing new perinatal care models. Midwife mentors trained an additional 103 providers and expanded Group PNC to six new clinics. Seven of the original clinics continue to offer Group PNC and have completed 147 additional groups, reaching over 1000 women. Fidelity, feasibility, and acceptability remained high. Four of six expansion clinics continue to offer Group PNC and have completed 23 groups. In 2024, the Malawi Ministry of Health hired our project Mentors to introduce Group PNC into district-level training as part of initiating national scale-up. In at least four districts sensitized to Group PNC several clinics are now offering Group PNC.

Implications for D&I Research: This study demonstrates the value of the 3-Step Implementation Model for scaling complex, midwifery-led interventions. Results provide evidence for advancing perinatal services redesign using a replicable framework that promotes positive perinatal outcomes, equity, and sustainability through effective, context-responsive implementation.

Confirmation: Ethics approval COMREC: P.10/18/2498 and UIC IRB: 2018–0845

Primary Funding Source: National Institutes of Health

S95

Bridging knowledge and collaboration: empowering public health professionals in dementia risk reduction

Rachel Goldberger

Alzheimer's Association, Chicago, IL, USA

Correspondence: Rachel Goldberger (rbgoldberger@alz.org)

Implementation Science 2026, 21(2):S95

Background: The Alzheimer's and Dementia Care ECHO® Program, hosted by the Alzheimer's Association, is a free, case-based telementoring program that uses videoconferencing to connect dementia experts with community-based professionals to increase access to dementia risk reduction, detection, diagnosis, and best-practice care management. In 2023, the Public Health Center of Excellence for Dementia Risk Reduction (Center) and the Alzheimer's Association Project ECHO Program collaborated to develop an ECHO program for state and local health officials to increase knowledge and confidence around dementia risk factors and integrate brain health messaging into chronic disease prevention programs. As evidence around dementia risk reduction continues to grow, public health professionals must understand the science and strategies to implement brain health messaging in their chronic disease prevention efforts and community health improvement plans.

Methods: To address this need, the Alzheimer's and Dementia Care ECHO® Program for Public Health Professionals was developed. This process involved designing and writing the curriculum, identifying subject matter experts and a host state to pilot the initiative. The resulting curriculum explores evidence-based dementia risk factors associated with cognitive decline, integrates the social determinants of health framework, and demonstrates how to embed these elements into chronic disease prevention programs and community health strategies. Designed to be both informative and adaptable, it encourages collaboration among local health departments. Each session

includes a 10–15-min didactic by a subject matter expert, followed by a case presentation led by one of the local health department attendees, and a group discussion. The curriculum is intentionally flexible, allowing states to incorporate unique data and experiences while bolstering participants' understanding and confidence regarding dementia and cognitive decline risk factors.

Findings: In February 2024, the Alzheimer's and Dementia Care ECHO® Program for Public Health Professionals pilot launched in partnership with the Tennessee Department of Health. Building on this momentum, Washington State Department of Health and Oklahoma State Department of Health launched their pilot programs in spring 2025.

Implications for D&I Research: This panel presentation will spotlight the development, implementation, and impact of these three state-led pilot series.

Primary Funding Source: Centers for Disease Control and Prevention.

S96

Tailoring the ECHO model for state-level public health infrastructure: lessons from Tennessee, Washington, and Oklahoma

Mickal Lewis

Alzheimer's Association, Chicago, IL, USA

Correspondence: Mickal Lewis (midlewis@alz.org)

Implementation Science 2026, **21**(2):S96

Background: Through the Alzheimer's and Dementia Care ECHO® Program for Public Health Professionals, three states, Tennessee, Washington, and Oklahoma, participated in the ECHO series to build public health capacity around dementia risk reduction. Each state adapted the core curriculum to align with its unique public health infrastructure, workforce needs and strategic priorities.

Methods: The Tennessee Department of Health (TDH) launched its pilot in spring 2024, and used it to further bolster their strong commitment to elevating dementia as a statewide public health priority. With support from the Tennessee Commissioner of Health, and leadership across TDH's Community Health Services and Central Office divisions, TDH was able to ensure alignment with existing chronic disease prevention programs and increase recruitment.

Findings: As a result, 98 individuals, representing 34 counties, one metro, and seven individuals representing Central Office, inclusive of 3 divisions participated in the pilot. The series showcased effective communication, strategic alignment, and integration of brain health messaging into public health planning. The Washington State Department of Health launched in spring 2025, focusing on cross-jurisdictional collaboration and peer learning. Designed to integrate dementia messaging into existing public health programs, the series covered topics ranging from social determinants of health to action planning and community engagement. As a result, 31 participants from 15 health departments engaged in the series, enriching both the learning experience and practical application. The Oklahoma State Department of Health prioritized rural and underserved communities by partnering with aging services and community-based organizations. The pilot supported 14 participants and 10 health departments, bringing valuable insights from local contexts and highlighting opportunities for community-level integration.

Implications for D&I Research: Across all three states, adaptation processes were guided by implementation of science principles, including community engagement, iterative feedback, and alignment with existing chronic disease prevention programs. Strong leadership support and strategic alignment were key drivers of successful implementation. This multi-state pilot illustrates how the ECHO model can be flexibly adapted while maintaining core fidelity, offering a replicable approach for other jurisdictions and contributing to the growing field of dissemination and implementation science.

Primary Funding Source: Centers for Disease Control and Prevention

S97

Evaluating the impact of the Alzheimer's and Dementia Care ECHO® Program for Public Health Professionals: a multi-state assessment

Olivia Woodward

Emory Centers for Public Health Training and Technical Assistance, Atlanta, GA, USA

Correspondence: Olivia Woodward (olivia.woodward@emory.edu)

Implementation Science 2026, **21**(2):S97

Background: The Alzheimer's and Dementia Care ECHO® Program for Public Health Professionals, was piloted to assess its capability to address dementia risk reduction in public health settings. To evaluate the program's impact, the Public Health Center of Excellence on Dementia Risk Reduction partnered with the Emory Centers to evaluate changes in knowledge, confidence in taking action, and collaboration around dementia risk reduction among participants from public health agencies in Tennessee, Washington, and Oklahoma. The evaluation of this multi-state pilot also captured participants' intentions to implement learnings from the series in their work and suggestions for improvement.

Methods: After the conclusion of the multi-state pilot, evaluation data was aggregated for comprehensive analysis. Across the three pilot states, 143 individuals from 62 public health agencies participated in the 6-week series. Participants represented a wide range of roles within their agencies; nurse (26%, $n = 37$) and health educator (22%, $n = 32$) were the top two roles among the sample. Almost all respondents (92%, $n = 99$) were affiliated with a local/regional or state health department. Ninety-five percent of participants ($n = 85$) attended at least four of the six ECHO sessions.

Findings: Evaluation surveys were administered before the first session and after the conclusion of the six-weeks. Survey respondents demonstrated a statistically significant increase in knowledge (+45%) and confidence (+29%) among dementia risk reduction topics. Additionally, almost all respondents (85%) reported more frequent discussions, and over half (62%) reported more frequent collaboration on dementia risk reduction activities after the series. Most respondents (72%) planned to implement learnings from the series immediately or within 30 days. Respondents reported integrating brain health into existing programs and strengthening new or existing interdepartmental collaboration, and increasing outreach to raise awareness of modifiable risk factors as a result of the series. Participants also provided recommendations for improving the series, such as expanding topics and increasing accessibility.

Implications for D&I Research: Overall, 93% of respondents were satisfied with the ECHO series, finding it an effective way to develop their capacity to implement dementia risk reduction activities.

Primary Funding Source: Centers for Disease Control and Prevention

S98

Task shifting evidence-based care to lower cadre health care workers: the case for working with traditional healers to deliver HIV testing in South Africa

Carolyn Audet¹, Praises Mnisi², Amanda Mwelase², Ryan Wagner², Wonderful Mabuza²

¹Vanderbilt University Medical Center, Nashville, TN, USA; ²University of the Witwatersrand, Johannesburg, South Africa

Correspondence: Carolyn Audet (carolyn.m.audet@vanderbilt.edu)

Implementation Science 2026, **21**(2):S98

Background: Task shifting of care delivery from professionals with higher levels of training to those with less formal training is a commonly employed implementation science strategy. In low- and middle-income countries, task shifting strategies can increase the reach of evidence-based practices, including HIV testing.

Methods: We recruited and trained traditional healers from ten clinical catchment areas in rural South Africa to conduct HIV counseling and testing services with their clients. Training followed national guidelines. Healers who successfully passed their coursework and exams completed surveys about implementation feasibility, acceptability,

and appropriateness of healer-initiated HIV testing. All were provided with materials (HIV test kits, symptom screening tools), linked with their local facility to ensure smooth referral of patients and contacted weekly by a study team to facilitate collection of data.

Findings: One hundred and forty-seven healers successfully completed each of the training components (94%). All healers agreed with the statements, “I welcome healer led HIV counseling and testing in my community” and “Healer led HIV counseling and testing seems like a good idea given the culture of this area.” When looking to the potential to address barriers to testing, 98% agreed or strongly agreed with the statement, “Healer led HIV counseling and testing will help address barriers to HIV counseling and testing.” The majority (87%) agreed or strongly agreed with the statement “I trust that the health system will respect the work I do to identify people living with HIV.” Over four months, healers consulted 1,297 clients; 918 (71%) agreed to receive allopathic testing and screening services. Of those tested, 43 (5%) were newly diagnosed with HIV.

Implications for D&I Research: Task shifting HIV counseling and testing services to lower cadre workers, even those not formally acknowledged by the health system, can increase access to vital testing and treatment services for those who otherwise avoid seeking care at allopathic health services.

Primary Funding Source: National Institutes of Health

S99

Bridging the gap: a pilot training and referral program for traditional healers to early breast cancer detection in Tanzania

Autumn Beavers¹, Elizabeth Msoka², Theresia Edward Mwakyembe³, Irene Masue³, Ayesiga Herman⁴, Blandina T. Mmbaga², Lily Gutnik⁵

¹University of Alabama—Birmingham, Birmingham, AL, USA; ²Kilimanjaro Clinical Research Institute, Moshi, Tanzania, United Republic of; ³Kilimanjaro Christian Medical College, Moshi, Tanzania, United Republic of; ⁴Kilimanjaro Christian Medical College, Moshi, Tanzania, United Republic of; ⁵The University of Alabama at Birmingham, Birmingham, AL, USA

Correspondence: Lily Gutnik (lgutnik@uabmc.edu)

Implementation Science 2026, **21**(2):S99

Background: Breast cancer mortality remains disproportionately high in low- and middle-income countries (LMICs) due to delayed diagnosis. In Tanzania, where many individuals seek care from traditional healers (THs), these practitioners may be uniquely positioned to support early detection. This study evaluated the feasibility and outcomes of a breast health training and referral pathway for traditional healers in rural Tanzania.

Methods: Guided by the Adoption domain of the RE-AIM framework, this study assessed real-world uptake and integration of breast health knowledge and referral practices among THs. Six registered THs (3 male, 3 female; mean age 53) completed a five-day in-person training on breast health, clinical breast exams, and referral protocols. Pre- and post-training knowledge assessments were administered. Following training, traditional healers referred patients to Kilimanjaro Christian Medical Centre (KCMC), a tertiary hospital. Referrals were made based on their clinical judgment and identification of suspicious breast findings, informed by the knowledge and skills acquired during the breast health training program. Monthly focus group discussions were conducted to evaluate implementation and identify barriers.

Findings: Before training, 67% of THs had no formal breast cancer education, though 83% reported some awareness. Median knowledge scores increased from 41% to 74% post-training ($p=0.01$). Over six months, 48 patients were referred, and 13 (27%) presented to KCMC (median age: 48). Of these, nine had a prior breast cancer diagnosis, including six with metastatic disease. Themes from focus group discussions included patient-level referral barriers, use of training content, referral decision-making, and collaboration with the biomedical system. Barriers included cost of care, fear, stigma, and spousal influence. Healers reported applying their knowledge to educate communities, screen asymptomatic individuals, and expand practice. They valued their engagement with the biomedical system, felt more integrated

into formal health networks, and expressed interest in sustaining referrals and expanding outreach.

Implications for D&I Research: Traditional healers, as trusted community health providers, represent a promising resource for early breast cancer detection in Tanzania. Strengthening their knowledge and referral skills may offer a scalable, community-based strategy for improving early detection and reducing diagnostic delays in LMICs.

Primary Funding Source: National Institutes of Health

S100

Ugandan traditional healers can help achieve the ‘last mile’ in HIV epidemic control

Radhika Sundararajan¹, Misha Hooda¹, Srijia Gogineni¹, Denis Nansera², Carolyn Audet³, Winnie Muyindike², Juliet Mwanga-Amumpaire²

¹Weill Cornell Medicine, New York, NY, USA; ²Mbarara University of Science and Technology, Mbarara, Uganda; ³Vanderbilt University Medical Center, Nashville, TN, USA

Correspondence: Radhika Sundararajan (ras9199@med.cornell.edu)

Implementation Science 2026, **21**(2):S100

Background: Only 75% of people living with HIV (PLWH) in rural Uganda are virally suppressed – far below the 95% target set by UNAIDS. This gap illustrates the need for tailored implementation strategies to identify PLWH and support their linkage and retention in care. Traditional healers (TH) are frequently the initial source of health-care in rural communities. Using a task-shifting strategy, Ugandan TH were trained to identify PLWH with unsuppressed HIV viral load at their practices and (re)link them to HIV services.

Methods: We trained 66 TH in Mbarara, Uganda to screen for adult PLWH clients through 1) facilitating HIV self-testing for those without testing in the prior 12 months and no prior HIV diagnosis; and 2) evaluating ART adherence using the CASE Adherence Index Questionnaire (CAIQ). Fidelity of these procedures was monitored via structured observations. Clients with a reactive self-test or CAIQ score < 10 underwent HIV-1 viral load (VL) testing. All virally unsuppressed PLWH (VL > 200 copies/mL) were referred to a local HIV clinic to (re)initiate care. A sample of 20 PLWH were interviewed post-enrollment to assess their experiences.

Findings: Between September 2023–July 2025, 277 PLWH were enrolled across 66 healer practices. Among our participants, about half were male, median age was 34 years, and median viral load at enrollment was 97,910 copies/mL. Most were newly diagnosed; of these, 48% ($n=81$) the healer facilitated test was their first lifetime HIV test. Most participants (re)linked to care within 30 days – more than half within 7 days. Qualitative interviews illustrate that task shifting HIV testing and linkage to care with TH was successful due to ease of access, perceived confidentiality, and trusting interpersonal relationships. These qualities were noted in contrast to clinic-based services.

Implications for D&I Research: A task shifting strategy with informal health providers like traditional healers has the capacity to fill important service delivery gaps in HIV care. About half of newly diagnosed individuals received their first HIV test through a healer, and those previously disengaged were successfully relinked to HIV care. Traditional healers can complement formal health services and play a crucial role in achieving the ‘last mile’ to end the HIV epidemic.

Primary Funding Source: National Institutes of Health

S101

Transforming access to mental health care through collaboration with traditional and faith healers in Nigeria

Akinyinka Omigbodun¹, Jibril Abdulmalik², Olayinka Egbokhare², Obafemi Jegede², Kafayat Aminu², Swaran Singh³, Olayinka Omigbodun⁴

¹College of Medicine, University of Ibadan, Ibadan, Nigeria; ²University of Ibadan, Ibadan, Nigeria; ³University of Warwick, Warwick, United Kingdom; ⁴Department of Child and Adolescent Psychiatry, and Centre for Child and Adolescent Mental Health, College of Medicine, University of Ibadan, Ibadan, Nigeria

Correspondence: Akinyinka Omigbodun (omigbodun@yahoo.com)

Implementation Science 2026, **21**(2):S101

Background: Help-seeking for mental illness in Nigeria is pluralistic, with patients frequently seeking care from traditional and faith healers (TFH) instead of formal health services. While TFHs can be important sources of support, help-seeking through them can lead to delays in accessing the necessary biomedical and psychological treatment. Therefore, collaborative approaches are widely advocated to improve access to formal mental health care and reduce delays and other disadvantageous practices of TFHs, whilst retaining the valued aspects of their activities.

Methods: The TRANSFORM project aims to improve access to care and outcomes of serious mental illness among people living in disadvantaged communities by developing an innovative collaborative care model with TFHs in Nigeria. Following in-depth studies into the local population's understanding of the explanatory models of illness and care, two training packages were developed and implemented to allow TFHs to identify serious mental illnesses, such as psychosis and suicidality, and refer patients to formal health and social services in the community. Training workshops were conducted in May 2024 and a pilot study was launched in July 2024.

Findings: Twenty TFHs were trained to screen their clients for mental illnesses and urgently refer those with signs of serious mental illness for care at local health facilities. Since launch of the screening program, four patients were identified as having signs of severe mental illness and were referred for urgent evaluation. Of these, three engaged with care at the health facility: two were diagnosed with Schizophrenia while one was diagnosed with psychosis with comorbid substance abuse. All three initiated treatment. One concern brought up by TFHs following the training was that was that the patients would not receive prompt attention in the health facility; however, among those urgently referred, all received care and medical stabilization immediately.

Implications for D&I Research: TFHs can be trained to recognize both common and serious mental health problems. Task-shifting mental health screening to TFHs can increase the reach and acceptability of evidence-based mental health treatments among communities who may otherwise avoid formal biomedical care. This approach is currently being scaled up across other communities in Nigeria.

Primary Funding Source: National Institute for Healthcare and Research (UK)

Global dissemination and implementation science

S102

Sustained mental health outcomes from the youth readiness intervention: a four-year follow-up study in Sierra Leone

Wijnand Van Den Boom, Kathryn Noon, Abdulai Jawo Bah, Matias Placencio-Castro, Theresa Betancourt
Research Program on Children and Adversity, Boston College, Chestnut Hill, MA, USA

Correspondence: Theresa Betancourt (theresa.betancourt@bc.edu)

Implementation Science 2026, **21**(2):S102

Background: Sustaining evidence-based mental health interventions (EBIs) in conflict-affected settings requires innovative implementation strategies. The Youth Readiness Intervention (YRI), a group-based EBI integrating cognitive behavioral/interpersonal/mindfulness approaches, was embedded within a youth entrepreneurship program in Sierra Leone as part of a 2019 Hybrid Type II implementation-effectiveness cluster-randomized trial. Guided by the EPIS (Exploration, Preparation, Implementation, Sustainment) framework, the intervention was implemented using the Collaborative Team Approach (CTA), a quality improvement strategy that supports non-specialist providers through coaching, supervision, fidelity monitoring, and iterative Plan-Do-Study-Act (PDSA) cycles. PDSA cycles were used throughout implementation to iteratively identify and address barriers (e.g., poor attendance) and tailor delivery to rural contexts. The original trial ($N=1,151$ youth, 528 third-party reporters) found improvements in depression and anxiety among YRI+ENTR participants, along with better work/training performance reported by community

leaders. This study presents findings from a four-year follow-up assessing the sustainability of those mental health outcomes.

Methods: A stratified random subsample of 584 youth was assessed across three study arms: control, entrepreneurship training only (ENTR), and entrepreneurship plus YRI (ENTR+YRI). Outcomes included emotion regulation, psychological distress, and (interpersonal) functioning. Linear mixed-effects models were used to assess the long-term effects of the intervention, and structural equation modeling examined whether COVID-19-related economic and relationship stressors mediated these outcomes.

Findings: At four-year follow-up, ENTR+YRI youth sustained modest but significant reductions in depressive symptoms ($\beta = -0.057$) and combined depression/anxiety symptoms ($\beta = -0.047$), compared to controls. No sustained effects were found for daily functioning or emotion regulation. ENTR-only participants did not differ from control and ENTR+YRI participants. COVID-19-related economic stressors partially mediated the relationship between intervention group and psychological distress.

Implications for D&I Research: We demonstrate that structured, team-based implementation strategies, such as CTA, guided by the EPIS framework, can support sustained mental health benefits over time. The durability of effects during the COVID-19 pandemic suggests that the YRI helped promote coping mechanisms that enhance psychological resilience. Findings contribute evidence toward the scalable use of D&I strategies (e.g., EPIS) in low-resource settings such as Sierra Leone.

Primary Funding Source: Global Development Services International

S103

Evaluating sustainment of an evidence-based intervention, Pediatric Early Warning Systems (PEWS), to identify deteriorating pediatric oncology patients in Latin America

Asya Agulnik¹, Sara Malone², Samantha Hayes¹, Rebeca Caires¹, Miguel Aguilar Barragán³, Maricela Robles-Murguía¹, Sayeda Islam⁴, Adolfo Cardenas-Aguirre¹, Maria Puerto-Torres¹, Kim Prewitt⁵, Bobbi Carothers⁵, Douglas Luke⁶, Dylan Graetz¹, Virginia McKay⁵

¹St. Jude Children's Research Hospital, Memphis, TN, USA; ²Washington University in St. Louis, St. Louis, MO, USA; ³Casa de la Amistad para Niños con Cáncer, Mexico City, Mexico; ⁴Washington University in Saint Louis, St. Louis, MO, USA; ⁵Center for Public Health Systems Science, Brown School, Washington University in Saint Louis, St. Louis, MO, USA; ⁶Center for Public Health Systems Science, Washington University in St. Louis, St. Louis, MO, USA

Correspondence: Asya Agulnik (Asya.Agulnik@STJUDE.ORG)

Implementation Science 2026, **21**(2):S103

Background: Pediatric early warning systems (PEWS) are evidence-based interventions that aid in the early identification of clinical deterioration in hospitalized children. Sustaining PEWS is crucial for improving patient outcomes. However, few studies successfully measure long-term use of evidence-based interventions, particularly in resource-variable settings. In this study we evaluate PEWS sustainment across diverse hospitals providing childhood cancer care in Latin America and Spain.

Methods: We employed a longitudinal mixed-method design with hospitals participating in Proyecto Escala de Valoración de Alerta Temprana (EVAT), a quality improvement collaborative focusing on PEWS implementation. Hospitals were included if they successfully implemented PEWS between 2018 and 2024. Data on PEWS sustainment were collected for 2 months after PEWS implementation completion and every 6 months from July 2021 to December 2024 (1–6 time points/center). PEWS sustainment was operationalized as > 85% fidelity on three core components: 1) using PEWS with every vital sign assessment, 2) correct PEWS scoring, and 3) adhering to the PEWS algorithm. Local teams collected data via chart review. Hospital organization type (oncology, general, etc.), public/private funding type, nurse:patient ratio, and World Bank country income levels were reported by site leads. Sustainment distributions

and descriptive comparisons across hospital characteristics over time were examined.

Findings: As of December, 2024, 61 Proyecto EVAT centers successfully implemented PEWS. Of these, 49 (80.3%) centers participated which completed PEWS implementation 0–6 years previously, contributing a total of 136 evaluation time points. PEWS sustainment was generally high. Only 6 centers were non-sustaining by at least one measure of fidelity during 9 individual timepoints (9/136 or 6.6%); only 1 center was non-sustaining across all 3 indicators at a single timepoint. Sustainment was high across centers regardless of organization and funding types, nurse:patient ratios, and country income-levels over time.

Implications for D&I Research: This study operationalizes and longitudinally assesses sustainment of an evidence-based intervention across resource-diverse hospitals, providing a template for a low-burden methodology to study intervention use over time. Findings demonstrate that PEWS is largely sustainable over time across resource-diverse centers of varying country, hospital, and unit characteristics. Future work will identify components of clinical capacity for sustainability related to challenges sustaining PEWS.

Primary Funding Source: National Institutes of Health

S104

A framework driven in-depth assessment of SAIA implementation fidelity at scale

Carmen Hazim¹, Mery Agostinho², Christopher Akiba³, Kristjana Ásbjörnsdóttir⁴, Fernando Amaral², Joana Coutinho⁵, Jonny Crocker¹, Maria Cruz², Fatima Cuembelo⁶, Aneth Dinis¹, Sarah Gimbel⁷, David Katz⁸, Isaias Ramiro⁹, Kenneth Sherr¹

¹University of Washington, Seattle, WA, USA; ²Comité para Saúde de Moçambique, Chimoio, Mozambique; ³RTI International, Seattle, USA; ⁴University of Iceland, Reykjavik, Iceland; ⁵Comité para Saúde de Moçambique, Beira, Mozambique; ⁶Eduardo Mondlane University, Maputo, WA, Mozambique; ⁷University of Washington School of Nursing, Seattle, WA, USA; ⁸Department of Global Health, University of Washington, Seattle, WA, USA; ⁹Comité para Saúde de Moçambique, Maputo, Mozambique

Correspondence: Carmen Hazim (chazim@uw.edu)

Implementation Science 2026, **21**(2):S104

Background

The Systems Analysis and Improvement Approach (SAIA) uses systems engineering tools of cascade analysis, process mapping, and quality improvement in monthly cycles to optimize prevention of mother-to-child HIV transmission (PMTCT) services. To better understand SAIA implementation at scale, we developed and applied an in-depth assessment of SAIA fidelity from a recent three-wave stepped wedge cluster randomized trial (SAIA-SCALE) which tested a district-led model of SAIA delivery in 36 facilities in Manica Province, Mozambique.

Methods

The conceptual framework for implementation fidelity by Carroll et al. (2007) was used to develop a multi-component, SAIA-specific fidelity measure that captures adherence to content, coverage, frequency, and duration of an ideal SAIA cycle. Using implementation tracking data captured during SAIA-SCALE, we assessed each individual fidelity component, quantified fidelity per facility-month as a composite classification (“high” or “low”) and determined each facility’s overall “fidelity experience” (“high” defined as >85% of facility-months with high fidelity). Finally, we explored associations between SAIA fidelity and trial wave, implementation phase, and facility characteristics such as urbanicity, staffing, and previous quality improvement training using logistic regression.

Findings

High fidelity was observed in 73% of 864 facility-months of SAIA implementation with high levels of adherence to SAIA cycle frequency (99%) and coverage (91%). Duration could not be assessed due to lack of data. Among the eight content sub-components assessed, process mapping (84%) and clear workplan tasks (83%)

were the least frequently fulfilled. Wave 1 facilities had on average lower proportions (63%) of facility-months with high fidelity compared to wave 2 (81%) and wave 3 (88%) facilities. Furthermore, compared to wave 1, odds of high-fidelity experience were 2.46 times greater [95% CI = 1.05–6.44] for each subsequent wave. High-fidelity experience was also more common among facilities with previous quality improvement training [OR: 5.68; 95%CI: 0.85–38.08].

Implications for D&I Research

Evaluations of fidelity to implementation strategies are important to contextualize program effectiveness but are rare and often lack standardization. As the first in-depth analysis of an expanded definition of fidelity to SAIA, these findings may be used further to investigate predictors of high fidelity and assess whether high fidelity moderates the effect of SAIA on service-level and patient health outcomes of PMTCT.

Primary Funding Source: National Institutes of Health

S105

Stakeholder-prioritized implementation strategies for scale-up and sustainability of a stepped-care tool for HIV care among youth

Sarah Hicks¹, Nok Chhun¹, James Kibugi², Patricia Odhiambo³, Tobias Odwar³, Alphonse Oguok³, Bryan Weiner¹, Kawango Agot², Mark Ayallo³, Kristin Beima-Sofie¹

¹University of Washington, Seattle, WA, USA; ²Impact Research and Development Organization, Kisumu, Kenya; ³IMPACT Research & Development Organization, Kisumu, Kenya

Correspondence: Sarah Hicks (smd722@uw.edu)

Implementation Science 2026, **21**(2):S105

Background

Interventions are more successful when relevant stakeholders are engaged in decision-making. The nominal group technique (NGT) is a prioritization method designed to equitably distribute decision-making power and enable stakeholders to drive the research process.

Methods

We conducted a modified NGT workshop to obtain diverse stakeholder perspectives and prioritize implementation strategies to support scale-up and sustainability of a stepped-care intervention for adolescents and young adults living with HIV (AYLHIV) in Kenya. Using a sequential explanatory mixed methods design, we conducted surveys with healthcare providers at 12 facilities participating in the Data-informed Stepped-care (DiSC; NCT05007717) study and hosted a multi-day stakeholder workshop to prioritize barrier-matched implementation strategies pre-identified by providers using the Consolidated Framework for Implementation Research (CFIR). Workshop participants ranked strategies on feasibility and effectiveness using a stepwise elimination process to prioritize a final set of strategies that aligned with the Expert Recommendations for Implementing Change categories. Facilitator debrief reports and participant evaluations were used to assess participants’ decision-making and workshop effectiveness.

Findings

The workshop was highly acceptable to participants and enabled efficient identification of 10 implementation strategies. In the pre-workshop survey, providers rated 18 pre-identified strategies aligned with CFIR across measures of feasibility, impact, acceptability, and appropriateness. Workshop participants ranked and rated these same 18 strategies and an additional 7 strategies generated during the workshop ($n=25$). Highly prioritized strategies included prioritization of AYLHIV with mental health and viral suppression challenges, improving provider training and resources, and continuous quality improvement meetings throughout stepped-care scale-up. Facilitator debrief reports highlighted the importance of cost-effectiveness, strategy alignment with current programs/systems, and technology access as key influencers. The majority of participants (82.5%) felt that appropriate strategies were identified. Workshop design elements (two-day format; implementation science education session) were viewed favorably in qualitative evaluations.

Implications for D&I Research

Given the diversity of contexts in a national scale-up campaign, there is a need for prioritization approaches that are less prescriptive and allow for identification of diverse implementation strategies that can be selected and optimized for local contexts. Future work is needed to evaluate the impact of using stakeholder-informed strategies over traditional standard-of-care strategies to support national scale-up progresses.

Primary Funding Source: Adolescent HIV Prevention and Treatment Implementation Science Alliance (AHISA)

S106

Harnessing implementation research methods to strengthen neonatal care improvement: lessons from NEST360

Kylie Dougherty¹, Nebiyou Hailemariam², Hannah Mwaniki³, Robert Tillya⁴, Salome Okunade⁵, Aynalem Hailemichael Frew⁶, Christine Bohne⁷, Lisa Hirschhorn⁸

¹Northwestern University Feinberg School of Medicine, Chicago, USA;

²Rice University, Addis Ababa, Ethiopia; ³Aga Khan University, Nairobi, Kenya; ⁴Ifakara Health Institute, Dar Es Salaam, Tanzania, United Republic of;

⁵NEST360 Nigeria, Abuja, Nigeria; ⁶Emory University, Saving Little Lives, Addis Ababa, Ethiopia; ⁷Rice University, Houston, USA; ⁸Center for Dissemination and Implementation Science, Northwestern University, Chicago, USA

Correspondence: Kylie Dougherty (kylie.dougherty@northwestern.edu)
Implementation Science 2026, **21(2)**:S106

Background

NEST360 is a multi-country alliance in Kenya, Malawi, Nigeria, Tanzania, and Ethiopia working to reduce neonatal mortality in sub-Saharan Africa by improving small and sick newborn care (SSNC). During Phase 1 (2019–2023), NEST360 introduced facility-level quality improvement (QI) and cross-facility data use to increase coverage of key evidence-based interventions (EBIs) such as kangaroo mother care, phototherapy, CPAP, and hypothermia prevention. In Phase 2 (2024–2029), NEST360 integrated implementation research (IR) methods to understand implementation barriers, strategy selection, and synthesize cross-country lessons to improve care coverage and strengthen EBI quality and scale. This abstract describes how IR was used to adapt and strengthen NEST360 implementation.

Methods

Using a multi-country, mixed-methods approach, NEST360 developed implementation research logic models (IRLMs) for specific program activities (e.g., device qualification) and broader program-wide implementation. Barriers and facilitators were identified across the SSNC Implementation Toolkit, and facility-level change ideas were mapped to ERIC (Expert recommendations for implementation change). Implementation outcomes were measured using Proctor's framework and RE-AIM. Elements of these same frameworks were embedded into QI resources such as change packages. Data sources included facility assessments, routine programmatic data, and stakeholder interviews across sites and global stakeholders.

Findings

IR methods provided a structured lens to identify cross-cutting and context-specific implementation barriers, including workforce, family involvement, and infrastructure, and link them to targeted strategies including integrating QI, biomed, data and clinical mentorship, increasing spread of cross-country lessons, and adaptation to device maintenance activities. Three IRLMs were developed to illustrate overall program implementation, QI activities, and device functionality initiatives. A comprehensive change package covering five EBIs and aligned with IR frameworks was shared with all NEST-supported hospitals. Implementation outcomes helped to uncover factors contributing to early reductions in neonatal mortality in supported hospitals.

Implications for D&I Research

Integrating IR methods into program implementation offers a context-sensitive model for spreading program learning to improve quality-of-care delivery in SSNC. The NEST360 experience shows how aligning global health efforts with IR frameworks can amplify locally generated solutions and inform scale-up. This work also adds explanatory depth to standard program outcomes, such as changes in mortality, increasing program replicability.

Primary Funding Source: Bill and Melinda Gates Foundation

S107

Describing the expansion of tele-dementia services to Veterans in Guam and American Samoa through applying the Consolidated Framework for Implementation Research 2.0

Leah Haverhals¹, Marisa-Francesca Brodrick², Marika Blair Humber², Christine Gould²

¹Veterans Health Administration, Aurora, CO, USA; ²Veterans Health Administration, Palo Alto, CA, USA

Correspondence: Leah Haverhals (Leah.haverhals@va.gov)

Implementation Science 2026, **21(2)**:S107

Background: Guam and American Samoa have some of the highest proportions of United States Veterans per capita, and many who served during wartime are rapidly aging. The Veterans Health Administration (VA) has clinics on these Pacific Islands for primary and mental health services; however, specialty care is delivered via telehealth, by VA providers traveling to the islands, through beneficiary travel (Veterans traveling), or via community care. In 2023, the VA regional telehealth hub (Clinical Resource Hub [CRH]) in California developed dementia evaluation and management services through their virtual geriatrics team. This abstract describes factors influencing implementation of these services to Veterans in Guam and American Samoa.

Methods: From February-June 2025, we interviewed VA leadership ($n=3$), staff and providers at Guam and American Samoa clinics ($n=10$), and the VA CRH team ($n=6$) by video conferencing technology or in-person during a site visit to Guam in March 2025. Interviews were recorded and transcribed. Two implementation frameworks guided analyses: the Consolidated Framework for Implementation Research (CFIR) 2.0 and the He Pikinga Waiori (HPW) Implementation Framework. Analysts coded data to CFIR and HPW constructs and created role-level matrices summarizing barriers and facilitators to implementing virtual services.

Findings: Barriers included unreliable internet connectivity affected by weather and navigating the substantial time difference. Translation of health recommendations and adjustment of neuropsychological tests administered were also needed. Initial planning to identify staff at clinics facilitated strong communication, as did collaborations with clinic and CRH leadership to create clinics and grids to streamline processes. Partnering with the clinic telehealth-care technician during visits requiring timed assessments proved essential to connect the CRH neuropsychologist by video. Such across-clinic partnerships led to more referrals, improved abilities to address Veterans' behavioral needs and diagnoses, and increased caregiver support. Veterans and caregivers in the Pacific Islands valued consistent relationships, and staff hoped services would continue.

Implications for D&I Research: Applying the CFIR 2.0 and HPW frameworks proved effective in describing barriers and facilitators to implementing tele-dementia and geriatrics services to Veterans in Guam and American Samoa. As there are similar care needs in Saipan and surrounding islands, identifying these factors so they can be addressed and care improved is essential.

Primary Funding Source: Department of Veterans Affairs

S108

Acceptability and feasibility of UNICEF's Service Delivery Framework for improving pediatric HIV service delivery in Mozambique, Nigeria, and Uganda

Susan Nakubulwa¹, Geoffrey Basalirwa², Chinye Osa-Afiana³, Herminio Cossa⁴, Christopher Lubega², Patience Kukundakwe¹, Gertrude Nanyonjo², Eleanor Moffatt⁵, Evaezi Okpokoro⁶, Nanlesta Pilgrim⁵, Brenda Okech², Nadia A. Sam-Agudu^{7,8}

¹Mildmay Research Centre Uganda, Kampala, Uganda; ²Uganda Virus Research Institute (UVRI) and the International AIDS Vaccine Initiative (IAVI), Entebbe, Uganda; ³International Research Center of Excellence, Institute of Human Virology, Nigeria, Abuja, Nigeria; ⁴Manhiça Health Research Centre (Centro de Investigação em Saúde de Manhiça, Manhica, Mozambique; ⁵ViiV Healthcare, Durham, NC, USA; ⁶International Research Center of Excellence, Institute of Human Virology Nigeria, Abuja Federal Capital Territory, Nigeria, Abuja, Nigeria; ⁷Institute of Human Virology Nigeria, Abuja, Nigeria; ⁸University of Minnesota Medical School, Minneapolis, MN, USA

Correspondence: Nadia A. Sam-Agudu (agudu@umn.edu)

Implementation Science 2026, 21(2):S108

Background: The Pediatric Breakthrough Partnership (PBP) was formed to implement UNICEF's 2019 Service Delivery Framework's (SDF's) implementation strategy package for improving pediatric HIV service delivery. The SDF incorporates strategies for system strengthening, demand creation, service delivery models, monitoring and evaluation, and adaptation and learning. We investigated the SDF's acceptability and feasibility in Mozambique, Nigeria, and Uganda.

Methods: We used a mixed methods approach to collect data in six districts across the three countries from January 2021-December 2023. Quantitative data was collected via survey questionnaires based on the Acceptability and Feasibility of Intervention Measures (AIM/FIM). The AIM and FIM each have 4 questions graded on a five-point Likert scale (max score = 5). An overall mean score was determined for each question and averaged for all 4 questions; the mean (standard deviation) for the group was then calculated. Qualitative data was collected via focus group discussions and in-depth/key informant interviews. Quantitative and qualitative evaluations were guided by the REAIM and CFIR frameworks, respectively. We used purposive sampling to identify participants based on their involvement in pediatric HIV services.

Findings: We recruited 191 participants delivering and receiving pediatric HIV services: 56 (29.3%) from Mozambique, 62 (33.0%) from Nigeria and 73 (38.2%) from Uganda. Survey findings showed an overall mean score (SD) of 4.5 (0.7) for acceptability and 4.4 (0.7) for feasibility of SDF strategies implemented by the PBP. Qualitative findings indicated high acceptability; particularly for HIV testing and ART dispensing in mobile brigades. Feasibility was noted through innovative strategies of peer lay/professional provider education and training. However, challenges including stigma, limited community HIV/ART knowledge, limited community leadership involvement and facility overcrowding were highlighted.

Implications for D&I Research: UNICEF's SDF package of evidence-based implementation strategies provides an opportunity for high burden, low resourced countries to consolidate efforts and solve problems in pediatric HIV program implementation. We found a high level of acceptability and feasibility for the SDF in Mozambique, Nigeria and Uganda. However, qualitative findings revealed community and system-level barriers. Further scale-up to similar settings should incorporate studies or solutions that evaluate and/or address these challenges to optimize adoption and sustainability of SDF innovations in pediatric HIV service delivery.

Primary Funding Source: ViiV Healthcare

S109

Challenges and adaptations in community mobilization for clean cookstove implementation in Lagos, Nigeria: FRAME-IS guided approach

Oghenebrozie Enakeno¹, Shivani Mishra², Deborah Onakomaiya³, Grace Apapa¹, Precious Tettehfiio¹, Oluwadamilola Opadotun⁴, Ayodeji Omoyeni⁵, Donald Ezinwanne¹, Temitope Akindeire¹, Weixi Chen⁶, Temiloluwa Fagbemi⁵, Nafesa Kanneh², Angela Aifah³, Gbenga Ogedegbe², Erin Hade⁷, Christopher Olopade⁸, Kikelomo O. Wright¹

¹Lagos State University, Lagos, Nigeria; ²Institute for Excellence in Health Equity, NYU Grossman School of Medicine, New York, NY, USA; ³Section for Global Health, Department of Population Health, NYU Grossman School of Medicine, New York, NY, USA; ⁴Lagos State University, Nigeria; ⁵Lagos State University College of Medicine, Lagos, Nigeria; ⁶Department of Biostatistics, NYU Grossman School of Medicine, New York, NY, USA; ⁷Department of Population Health, New York University Grossman School of Medicine, New York, NY, USA; ⁸University of Chicago School of Medicine, Chicago, IL, USA

Correspondence: Deborah Onakomaiya (deborah.onakomaiya@nyu.edu)

Implementation Science 2026, 21(2):S109

Background: Household air pollution (HAP) from traditional biomass fuels in peri-urban Lagos, Nigeria, increases the risk of poor cardiovascular health. Implementing Clean Fuel–Clean Stoves (CF-CS) to reduce HAP and help prevent hypertension, the leading risk factor for cardiovascular disease, faces barriers including cultural resistance, high cost, and logistical constraints. Community mobilization (CM) fosters ownership, empowers individuals, and builds collective capacity, increasing accessibility and adoption. We document challenges and adaptations in CM for a CF-CS intervention, using the Framework for Reporting Adaptations and Modifications to Evidence-based Implementation Strategies (FRAME-IS).

Methods: In a cluster-randomized trial, we enrolled 1,248 adults from 624 households across 32 peri-urban communities in Lagos to promote CF-CS with liquefied petroleum gas (LPG) or ethanol stoves. Implementation challenges and modifications were recorded through team notes, engagement logs, and input from community health extension workers, health education officers, and mobilizers. Trained staff organized data into FRAME-IS modules, systematically categorizing adaptations across pre-implementation, implementation, and post-implementation phases.

Findings: We identified 15 key implementation challenges for the CF-CS intervention, including inaccurate mapping of households, participant unavailability, skepticism due to prior research experience, cultural resistance to adopting new stoves, logistical barriers in riverine areas, and participant displacement. In response, 15 adaptations were implemented: two in the pre-implementation, 11 during the implementation, and two in the post-implementation. Adaptations included community reassessment, revised eligibility criteria, sensitization sessions, safety workshops, extended field operations, and re-engagement strategies for displaced participants. These adaptations involved structural changes to implementation strategies and the addition of new supportive activities.

Implications for D&I Research: Systematic, culturally sensitive modifications enhanced community mobilization for the CF-CS intervention in peri-urban Lagos, Nigeria. FRAME-IS enables structured documentation of adaptations, addressing cultural, logistical, and economic barriers, and improving intervention feasibility. Overall, these adaptations improved enrollment, retention, and timely data collection. Despite these efforts, economic barriers and stove stacking persisted as challenges. These findings highlight the value of FRAME-IS in guiding adaptive implementation strategies and present a transferable model for advancing implementation science in global health and environmental interventions.

Primary Funding Source: National Institutes of Health

S110

An implementation toolkit for integrating hypertension management into HIV care in Nigeria using the knowledge to action framework

Nafesa Kanneh¹, Shivani Mishra¹, Afua Siaw¹, Nina Uzoigwe², Deborah Onakomaiya³, Weixi Chen⁴, Oluwatosin Odubela⁵, Daniel Henry⁶, Angela Aifah³

¹Institute for Excellence in Health Equity, NYU Grossman School of Medicine, New York, NY, USA; ²NYU Grossman School of Medicine, New York, NY, USA; ³Section for Global Health, Department of Population Health, NYU Grossman School of Medicine, New York, NY, USA; ⁴Department of Biostatistics, NYU Grossman School of Medicine, New York, NY, USA; ⁵The Nigerian Institute of Medical Research, Lagos, Nigeria, Lagos, Nigeria; ⁶Cardiovascular Research Unit, University of Abuja, Abuja, Nigeria

Correspondence: Nafesa Kanneh (Nafesa.Kanneh@nyulangone.org)

Implementation Science 2026, **21(2)**:S110

Background

In Nigeria, the dual burden of HIV and hypertension demands integrated care approaches. However, implementation strategies are rarely packaged in ways that support adaptation or scale-up, resulting in fragmented efforts and limited cross-site learning. Drawing on two National Institute of Health (NIH)-sponsored implementation studies in Akwa Ibom and Lagos (UH3HL154498 and R01HL147811), we developed a practical implementation toolkit to address this gap.

Methods

Using the Knowledge-to-Action (KTA) framework—a model outlining how evidence is created and applied in practice—we mapped implementation strategies across knowledge creation and action phases. Inputs included training materials, fidelity tools, stakeholder engagement structures, communication approaches, and policy translation mechanisms. The toolkit was informed by discussions with two research staff members from the implementation teams of both studies to ensure comprehensive representation of all strategies.

Findings

The final toolkit includes 17 tools across five domains—training, workflow integration, community engagement, quality monitoring, and policy alignment—mapped to the KTA framework. For practical guidance, it features standardized training modules, screening cards, and fidelity checklists to support routine hypertension-HIV integration. Contextual adaptations include role adjustments for community health workers, locally tailored training content, and site-specific retention strategies such as radio jingles in Akwa Ibom and SMS reminders in Lagos. Communication tools include WhatsApp peer learning groups, policy briefing templates, and community materials to support engagement across clinical, community, and policy levels.

Implications for D&I Research

This implementation toolkit provides a replicable, flexible model for integrating noncommunicable disease services into HIV care in resource-limited settings. Guided by the Knowledge-to-Action (KTA) framework, the toolkit supports the integration of hypertension management into HIV care by providing practical guidance, contextual adaptations, and communication strategies to inform researchers, community-based organizations, and policymakers working on similar health challenges. By documenting and packaging locally adapted strategies across the full KTA cycle, the toolkit supports cross-context learning and accelerates the translation of implementation research into practice. This work highlights the role of toolkits as both implementation and dissemination mechanisms—bridging research and real-world delivery to advance the goals of Dissemination and Implementation (D&I) science.

Primary Funding Source: National Institutes of Health

S111

Adapting the HealthMpowerment (HMP) mobile application using the ADAPT-ITT model for HIV prevention and treatment services among youth in Africa

Marie-Claude Lavoie

University of Maryland School of Medicine, Baltimore, MD, USA

Correspondence: Marie-Claude Lavoie (mlavoie@ihv.umaryland.edu)

Implementation Science 2026, **21(2)**:S111

Background: Adaptation plays a critical role in implementation science (IS) by optimizing the fit of evidence-based interventions to local contexts and populations. Adaptation is especially relevant in digital health, where technologies, user preferences, and needs evolve. Yet, methodological approaches for guiding and documenting adaptations are seldomly included. We describe a multi-phase process, grounded in an IS framework, and results for the adaptation of a digital intervention, the HealthMpowerment (HMP) app, for HIV prevention and treatment for sexual and gender minority (SGM) youth in Africa.

Methods: Guided by the Assessment, Decision, Administration, Production, Topical Experts, Integration, Training, and Testing (ADAPT-ITT) IS model, we adapted HMP for SGM youth in Zambia, Malawi, Kenya, and Nigeria. We conducted eight focus group discussions (FGDs) with SGM youth ($N=40$) living with or at risk for HIV and four with other collaborators (clinicians, peers, policymakers) ($N=29$) to inform app adaptations. Subsequently, we conducted beta testing with 40 SGM youth to assess the app's usability using a validated scale. Data were collected from June 2024 to April 2025. Rapid qualitative and descriptive analyses were conducted. We used the Mobile App Rating Scale (MARS) to summarize the adaptations.

Findings: HMP adaptations were mainly within the MARS's Functionality and Information Quality domains. Functionality enhancements included adding content in local languages and integrating WhatsApp. Information Quality adaptations focused on expanding resources (e.g., mental health, economic strengthening) and ensuring that healthcare referrals made via the app were to SGM-competent providers. Engagement adaptations centered on increasing support and reducing response times from providers through the app, along with added gamification. Safety was prioritized and addressed by creating an emergency lockout code. For Subjective Quality, participants reviewed the app favorably for its PrEP/ART adherence support. The results from the beta-testing indicated that >86% of participants agreed/strongly agreed that the HMP app was easy to use, and 89% felt confident using it. Overall, 61% agreed/strongly agreed that they used the app frequently.

Implications for D&I Research: Guided by the ADAPT-ITT model, the HMP app was collaboratively and iteratively adapted with SGM youth and others. This methodological approach enhances HMP's functionality, quality, and safety features, responding to users' needs.

Primary Funding Source: National Institutes of Health

S112

Using FRAME to characterize provider-identified adaptations to integration of a stepped care intervention for perinatal mood and anxiety disorders in maternal-child-health clinics in Kenya

Agnes Karingo Karume¹, Asterico Neema¹, Yuwei Wang², Anne Kaggiah¹, Nancy Ngumbau¹, Alexandra Rose³, Lincoln Pothan², Anna Larsen³, Linnet Onger⁴, Amritha Bhat³, Dorothy Mangale⁵, John Kinuthia¹, Anjuli Wagner², Keshet Ronen²

¹Kenyatta National Hospital, Nairobi, Kenya; ²University of Washington, Seattle, WA, USA; ³UW Medicine, Seattle, WA, USA; ⁴Kenya Medical Research Institute, Nairobi, Kenya; ⁵Washington University, St Louis, MO, USA

Correspondence: Agnes Karingo Karume (karumeagnes@gmail.com)

Implementation Science 2026, **21(2)**:S112

Background

In Kenya, perinatal depression and anxiety are common yet often untreated due to limited access to mental health specialists and inadequate routine screening. The Integrated Perinatal Mental Health

(IPMH) Project evaluates a stepped-care package: universal screening, lay-delivered Problem Management Plus (PM+), and telepsychiatry. Successful implementation requires contextually appropriate adaptations to ensure feasibility, acceptability, and fidelity in maternal-child-health clinic settings. Our objective was to elicit and describe adaptations to the IPMH interventions.

Methods

Between February–June, 2025 we monitored provider-driven adaptations through structured group problem-solving meetings facilitated by the study team at 10 maternal-child health clinics in western Kenya. Health providers participated in five sessions per site to identify implementation challenges and propose solutions. Adaptations to the intervention package were coded using the Framework for Reporting Adaptations and Modifications-Expanded (FRAME) and to Evidence-based Implementation Strategies (FRAME-IS).

Findings

Thirty-one unique adaptations were identified (intervention = 26, strategies = 5) with 30 (97%) being context-related, primarily involving format changes to align with clinical workflows. One adaptation focused on provider training to enhance competency in delivering the intervention. Primary adaptation goals included improving feasibility ($n=11$, 35%), reach ($n=7$, 23%), fit with recipients ($n=6$, 19%) and retention ($n=5$, 16%). Most adaptations were driven by organizational barriers ($n=16$, 52%), followed by recipient-related constraints ($n=14$, 45%). Most targeted the intervention recipients—perinatal women ($n=21$, 68%) versus facility staff ($n=10$, 32%). The study team co-developed four cross-site adaptations in response to provider concerns: training to support women with pregnancy/infant loss, streamlining PM+ for missed sessions, appointment cards, and provider diaries. Screening adaptations ($n=12$, 40%) like reorganizing clinic space and adding privacy curtains aimed to enhance privacy. PM+ and telepsychiatry adaptations ($n=14$, 45%) focused on appointment coordination, using reminder calls and revised provider schedules to reduce missed sessions and improve retention. Most adaptations were adopted (97%).

Implications for D&I Research

Provider-identified adaptations targeted improved integration into routine clinic practices primarily aiming to reduce barriers to intervention feasibility and reach. These findings underscore the importance of responsive, context-driven adaptations to enhance adoption of integrated mental health services. Systematically capturing frontline worker feedback and supporting iterative adaptations can enhance fit, equity, and long-term sustainability.

Primary Funding Source: National Institutes of Health

S113

Effects of the 4 youth by youth HIV prevention intervention on sexually transmitted infection testing uptake among adolescents and young adults in Nigeria: findings from a stepped-wedge cluster-randomized trial

Peter Kalulu¹, Oluwakorede J Adediji², Olufunto A. Olusanya¹, Chiyere Arinze¹, Temitope Ojo¹, Ashley Bardon¹, Hong Xian³, Titilola Gbaja-Biamila⁴, Chisom Obiezu-Umeh⁵, David Oladele⁵, Ucheoma Nwaozuru⁶, Adesola Z. Musa⁴, Collins O. Airhihenbuwa⁷, Nora Rosenberg⁸, Rhonda BeLue⁹, Donaldson F. Conserve¹⁰, Jason J Ong¹¹, Lei Zhang¹¹, Susan Nkengasong¹², Weiming Tang¹³, Olugbenga G. Ogedegbe¹⁴, Joseph D. Tucker¹⁵, Oliver C. Ezechi⁴, Juliet Iwelunmor¹

¹WashU Medicine, St. Louis, MO, USA; ²University of Ilorin, Kwara, Nigeria;

³Saint Louis University, Saint Louis, MO, USA; ⁴Nigerian Institute of Medical Research, Lagos, Nigeria; ⁵Nigerian Institute of Medical Research, Yaba, Lagos, Nigeria; ⁶Wake Forest School of Medicine, Winston-Salem, NC, USA;

⁷Georgia State University, Atlanta, GA, USA; ⁸University of North Carolina at Chapel Hill, Chapel Hill, NC, USA; ⁹The University of Texas at San Antonio, San Antonio, TX, USA; ¹⁰George Washington University, Washington, DC, USA; ¹¹Melbourne Sexual Health Centre, Carlton, Australia; ¹²London School of Hygiene & Tropical Medicine, London, United Kingdom;

¹³University of North Carolina at Chapel Hill, Chapel Hill, NC, USA; ¹⁴New York University School of Medicine, New York, NY, USA; ¹⁵London School of Hygiene and Tropical Medicine, London, United Kingdom

Correspondence: Peter Kalulu (kalulu@wustl.edu)

Implementation Science 2026, 21(2):S113

Background: Sexually transmitted infections (STIs) pose a major public health issue globally, particularly among adolescents and young adults (AYAs). In Nigeria, high STI rates combined with low testing are exacerbated by fear, social issues, and limited access to services. This study evaluated the effectiveness of youth-led, participatory interventions known as 4 youth by youth (4YBY) on increasing STI testing among AYAs in Nigeria.

Methods: We used participatory action research, which included a crowdsourced open call, designathon, apprenticeship bootcamp, and participatory learning communities with trained facilitators, to co-develop a youth-friendly 4YBY intervention aimed at increasing the adoption of 1) 4YBY HIV self-testing bundles, containing STI testing instructions, and 2) peer support and technical assistance. The youth-friendly 4YBY intervention was implemented using a stepped-wedge cluster randomized trial across 32 local government areas in 14 states, covering four of Nigeria's geopolitical zones. The 4YBY intervention was sequentially rolled out across clusters over a 24-month period. Data were collected through surveys and clinical records, analyzed using generalized linear models, and reported with the RE-AIM framework to assess the effectiveness of the 4YBY intervention in increasing STI testing, including for syphilis, gonorrhea, and chlamydia.

Findings: A total of 1,500 AYAs (50.20% males and 49.80% females), aged 14–24, were reached across 32 LGAs, with most having completed secondary education (68.6%), primarily from Igbo (36%) and Yoruba (30%) ethnic groups. Baseline STI testing uptake was low, with syphilis at 3.16%, gonorrhea at 3.29%, and chlamydia at 2.08%. After 24 months, there were significant increases: syphilis testing increased to 43.25%, gonorrhea to 42.94%, and chlamydia to 43.01% ($p < 0.0001$ for all outcomes). Adjusted risk ratios indicated a strong effect of the youth-friendly 4YBY HIV prevention intervention: syphilis (RR 17.83), gonorrhea (RR 14.99), and chlamydia (RR 29.24).

Implications for D&I Research: Co-designed with youth through peer support and participatory learning, the 4YBY intervention achieved high adoption and reach of STI testing, demonstrating potential for long-term maintenance of improved health behaviors among AYAs. These findings highlight the importance of youth involvement in creating effective health interventions and support scaling youth-led, participatory strategies to enhance STI testing in similar settings.

Primary Funding Source: Eunice Kennedy Shriver National Institute of Child Health & Human Development of the National Institutes of Health.

S114

Quantifying healthcare worker preferences for HIV service delivery features using best–worst scaling in KwaZulu-Natal, South Africa

Noelle Le Tourneau¹, Aaloke Mody¹, Ellen Mukwekwerere², Buyisile Chibe³, Franco Musiello⁴, Kombatende Sikombe⁵, Zaynab Essack⁶, Anjali Sharma⁵, Elvin Geng¹, Alastair van Heerden⁶, Lindsey Filiatreau¹

¹Washington University in St. Louis, St. Louis, MO, USA; ²Washington University in St. Louis, St. Louis, MO, USA; ³Human Sciences Research Council, Pietermaritzburg, South Africa; ⁴Wits Health Consortium, Witwatersrand, South Africa; ⁵Centre for Infectious Disease Research in Zambia, Lusaka, Zambia; ⁶Human Sciences Research Council, Sweetwaters, South Africa

Correspondence: Noelle Le Tourneau (lnoelle@wustl.edu)

Implementation Science 2026, 21(2):S114

Background: As ART coverage has rapidly expanded across sub-Saharan Africa, preventing treatment interruptions after initiation and ensuring continued engagement in care are key to achieving sustained viral suppression. Understanding which HIV care service delivery features matter most to healthcare workers (HCWs) can inform program operations to improve retention and support clients upon re-engagement in care.

Methods: We used a best–worst scaling (BWS) survey, a preference methodology, to understand how HCWs rated and prioritized different care delivery features for supporting client engagement in HIV care at 36 clinics in KwaZulu-Natal Province, South Africa between May

13, 2024, and October 28, 2024. BWS was used to quantify the relative importance of 13 service delivery features (attributes) in HIV care (e.g., flexible refills, drug pick-up coordination, friendly providers, privacy) selected from the literature and reviewed by community members. Mean preference weights (MPWs), calculated from participants' most and least selected relevant barriers among 4 options across 13 choice sets, were rescaled using Hierarchical-Bayes estimation (out of 100) and compared across healthcare worker cadre.

Findings: Among 261 HCWs surveyed (81.2% female, median age of 42 years (IQR 35–50), 64.8% professional HCW, 16.5% lay HCW, and 18.0% administrative staff, with 74.7% working in their position for 6 or more years), the highest-rated service delivery feature for clients in HIV care was 1) patient privacy during visits (MPW: 15.13, 95% CI 14.53–15.75), followed by 2) timely viral load monitoring and sharing results with clients (12.76, 95% CI 12.11–13.42), and 3) providers that are kind when clients return to clinic after time away from care (9.59, 95% CI 9.11–10.06). HCWs ranked features such as hotline support (5.00, 95% CI 4.60–5.41), community delivery of ART (4.91, 95% CI 4.54–5.30), and flexible appointment scheduling (4.22, 95% CI 3.92–4.53) lowest. Preferences were consistent across healthcare worker cadre.

Implications for D&I Research: This best–worst scaling study highlights the importance of strategies to sustain engagement that should strengthen confidentiality, lab result timing and discussion with clients, while providing kind reception to returning clients and tailoring additional support for populations facing structural barriers. Preference heterogeneity underscores the need for differentiated care models responsive to both client and HCW priorities.

Primary Funding Source: Gates Foundation

S115

Decolonizing and innovating implementation of suicide prevention: results from a mixed-methods hybrid type II pilot cluster randomized controlled trial by mothers for mothers in rural Pakistan.

Ashley Hagaman¹, Gul Saeed¹, Sidra Mumtaz², Javeria Tanveer², Erum Hamid², Sundus Rehman², Noor Ul Huda², Atif Rahman³, Siham Sikander³
¹Yale School of Public Health, New Haven, CT, USA; ²Global Institute of Human Development, Islamabad, Pakistan; ³University of Liverpool, Liverpool, United Kingdom

Correspondence: Ashley Hagaman (ashley.hagaman@yale.edu)

Implementation Science 2026, **21**(2):S115

Background: Low- and middle-income countries account for 75% of global suicide deaths but produce less than 10% of research. South Asia has the highest rate of female suicide fatalities worldwide. In Pakistan, peer-led maternal mental health interventions have proven effective; however, structural and social stigma impede suicide disclosure and care-seeking and no community-level suicide prevention trials exist. Leveraging Pakistan's existing maternal mental health infrastructure, this study evaluated the feasibility, acceptability, and fidelity of a co-designed, decolonized, peer-led maternal suicide prevention intervention.

Methods: Following extensive co-design with a mother-led community advisory board of individuals with lived experience of suicide, we conducted a mixed-methods, hybrid type II cluster randomized controlled trial (cRCT) in rural Pakistan. Enhanced usual care included primary care integrating WHO mhGAP depression and suicide prevention protocols. The intervention, delivered by 11 Peer Mothers, included decolonized safety planning and six months of home visits. Implementation strategies tested included (1) peer-delivered brief interventions and (2) image-based, storytelling-driven safety planning, diverging from Western, literacy-dependent models. Implementation outcomes were measured through longitudinal periodic reflections, CFIR2.0 qualitative interviews, and culturally adapted quantitative tools assessing feasibility, acceptability, appropriateness, and fidelity. Clinical outcomes included suicidal ideation/behaviors, depression, help-seeking, and novel culturally-derived suicide risk measures.

Findings: The cRCT met all pre-determined milestones for feasibility, acceptability, and fidelity, attaining high recruitment (88%), retention (88% for mothers, 91% for peers), and strong peer-delivered intervention fidelity and competency. Intervention participants demonstrated improved clinical and help-seeking outcomes. Facilitators included collaborative learning with health workers and family engagement. Barriers included structural constraints (e.g., high turnover of medical officers) and complex family and community negotiations. Our triangulated mixed-methods data and decolonized approach enhance understanding of complex implementation and innovate on standard implementation measures.

Implications for D&I Research: D&I approaches are critical to co-designing implementation models for resource-strained, culturally diverse settings. Findings indicate that Peer-led intervention delivery is a feasible and acceptable model for suicide prevention among mothers in Pakistan. We contribute to the evidence base for culturally embedded, community-led suicide prevention implementation strategies in LMICs, reinforcing the potential of peer-led models to scale high quality access to mental health services.

Primary Funding Source: National Institutes of Health

S116

How to listen to multiple voices: moving from ethnography to implementation mapping

Ana Baumann¹, Sarah Burack², Comfort Asante³, Aliness Mwilaba-Dombola³, Sam Miti³, Christian Musirikare³, Lusungu Musimuko³, Diana Mwanza³, Tasha Ngalande³, Kelvin Kapungu³, Lindsey Kaufman¹, Thembekele Shato⁴, Petronella Singogo³, Michelle Silver⁵, Jean Hunleth⁶

¹Washington University in St. Louis, St. Louis, MO, USA; ²Washington University in St. Louis, Saint Louis, USA; ³National Health Research & Training Institute, Ndola, Zambia; ⁴Washington University in St. Louis, St. Louis, USA; ⁵Washington University in St. Louis, St. Louis, USA; ⁶Department of Surgery (Division of Public Health Sciences) and Alvin J. Siteman Cancer Center, Washington University School of Medicine, St. Louis, MO, USA

Correspondence: Ana Baumann (abaumannwalker@wustl.edu)

Implementation Science 2026, **21**(2):S116

Background: Scholars have long advocated for the engagement of different voices from the community in the selection and development of implementation strategies. Several methodologies exist to support the incorporation of multiple voices, including ethnography. In the Reaching for Equity in Adolescent Care through HPV vaccination (REACH) study, we drew on ethnographic methods to explore the contextual factors that shape implementation of HPV vaccination and engage different community member (adolescents, healthcare providers, household members, and community leaders) in three healthcare settings (rural, urban, and peri-urban) in Zambia. This study aims to show how we moved from ethnography to implementation mapping with a constant question in our mind: who is benefiting from this intervention?

Methods: We focused on understanding the experience of HPV vaccination at multiple levels, as well as from a range of perspectives. We did so through participant observation, key informant interviews, focus group discussions, and participatory workshops, including mapping and roleplays. Findings were organized and analyzed in multiple ways to honor the different methods used, including but not limited to inductive and deductive coding of interview and focus group transcripts and fieldnotes, drawing on theory from anthropology and childhood studies, as well as the different versions of the Consolidated Framework of Implementation Research. Data were triangulated, member checked with the community, and the team used reflexivity to reflect on the process of designing performance objectives and outcomes. Causal pathway diagrams for each strategy were designed to further clarify mechanisms of change. We held meetings with our Research Advisory Board and partners to gather feedback on the feasibility and impact of the proposed strategies.

Findings: The ethnographic methods have provided rich understanding of the contextual factors, including the discrepancies and commonalities across the different voices. Reflexivity helped examine

team process and bias. Through implementation mapping we selected three bundled strategies: education, monitoring of HPV vaccine, and monitoring of resources.

Implications for D&I Research: While scholars have advocated for listening to community members when selecting and designing strategies, the field of D&I could benefit from further guidance on decision making processes when information from different voices is discrepant.

Primary Funding Source: National Institutes of Health

S117

The full circle: global health reciprocal innovation of task-sharing strategies to address uncontrolled hypertension in the United States

Makeda Williams

National Institutes of Health, Bethesda, MD, USA

Correspondence: Makeda Williams (willimak@mail.nih.gov)

Implementation Science 2026, **21**(2):S117

Background: NHLBI supports rigorous domestic and global implementation research that advances implementation of evidence-based interventions (EBI) that address heart, lung, blood, and sleep (HLBS) disorders in the United States and worldwide. NHLBI stimulates research to generate knowledge regarding optimal implementation and sustainability of EBIs, strategies, and guidelines to reduce the domestic and global burden of HLBS disorders that creates unparalleled opportunities for reciprocal innovation. This talk will overview NHLBI's global health research strategy, which supports implementation research to build capacity in low- and middle-income countries, spur data science innovations appropriate to local settings, and develop technologies that can be used at the community level to address public health needs.

Methods: This strategy includes five pillars: (1) leverage partnerships/collaborations; (2) innovate implementation science; (3) advance mutually-beneficial priorities; (4) support training and career development; and (5) advance emerging areas of science. Annually, NHLBI completes a portfolio analysis of its investment in global health research grants to characterize progress and opportunities. Recently, NHLBI surfaced exemplars that illustrated real and potential impacts of reciprocal innovation, notably moving innovations from global to domestic contexts.

Findings: NHLBI's strategic partnerships with entities such as Fogarty International Center, UnitedHealth, and Global Alliance for Chronic Diseases have resulted in unique opportunities to leverage implementation research through the lens of reciprocal innovation to address shared health priorities. Given an estimated one billion adults worldwide suffer from hypertension, including nearly half of all adults in the United States, with many living in low-resource settings, NHLBI has funded several hypertension-focused global implementation research trials. NHLBI's support of scaling up healthcare delivery innovations to address the global burden of hypertension has proved crucial. Two trials—one in Ghana and one in NYC—resulted in reduced blood pressure through task-shifting, practice facilitation, lifestyle counseling/health coaching, and medication adherence support.

Implications for D&I Research: Trials such as these underscore the return on investment in reciprocity, bidirectional learning, and innovation through strategic partnerships and community engagement. Global health research offers unique benefits of reciprocal innovation such as improved treatment and health care delivery options for Americans dealing with health issues that are a burden globally.

Primary Funding Source: National Institutes of Health

S118

Task-shifting for hypertension control: a cluster trial in Ghana informing reciprocal innovation in the U.S.

Olugbenga Ogedegbe

NYU Langone Health, Institute for Excellence in Health Equity, New York, NY, USA

Correspondence: Olugbenga Ogedegbe (Olugbenga.Ogedegbe@nyulangone.org)

Implementation Science 2026, **21**(2):S118

Background: Limited access to care and physician shortages remains a critical barrier to hypertension control in sub-Saharan Africa. The *Task Shifting Strategy for Hypertension Control* (TASSH) in Ghana demonstrated that training nurses to deliver cardiovascular care within community clinics can effectively reduce systolic blood pressure (SBP). This trial has since informed the *Advancing Long-term Improvements in Hypertension Outcomes through a Team-based Care Approach* (ALTA) trial in New York, which adapts a similar task-shifting model to address persistent hypertension disparities among underserved patients. To inform scalable, equity-focused solutions in both global and U.S. health systems, we evaluated the comparative effectiveness of health insurance coverage (HIC) alone versus TASSH plus HIC among patients with uncontrolled hypertension in Ghana.

Methods: In a pragmatic cluster randomized implementation trial, 32 community health centers in Ghana were randomized to HIC alone or TASSH + HIC. A total of 757 patients were enrolled and followed for 24 months. Both groups received health insurance and scheduled nurse visits, but only the TASSH group received structured nurse-led cardiovascular risk management, lifestyle counseling, and medication initiation/titration. The primary outcome was change in SBP at 12 months; secondary outcomes included BP control and lifestyle changes.

Findings: At 12 months, SBP reduced significantly in both groups, with a greater reduction in the TASSH + HIC group (−20.4 mmHg; 95CI −25.2 to −15) compared to HIC alone (−16.8 mmHg; 95CI −19.2 to −15.6), yielding a significant between-group difference (−3.6 mmHg; $p=0.021$; 95CI −6.1 to −0.5; $p=0.021$). BP control improved similarly in both groups; lifestyle behaviors showed minimal change.

Implications for D&I Research: The TASSH + HIC strategy led to greater SBP reduction than HIC alone, with comparable BP control and minimal lifestyle change across groups. Findings from this trial reinforce the effectiveness of nurse-led, task-shifting strategies when paired with structural enablers like insurance coverage to reduce SBP in resource-constrained settings. As the foundational model for the ALTA trial in New York, TASSH demonstrates how global implementation strategies can drive reciprocal innovation and inform equity-focused hypertension control efforts across both low- and middle-income countries and underserved U.S. populations.

Primary Funding Source: New York University Langone Health

S119

From global insights to local impact: using practice facilitation and task-shifting to improve hypertension control in a Federally Qualified Health Center

Antoinette Schoenthaler

NYU Grossman School of Medicine, New York, USA

Correspondence: Antoinette Schoenthaler (antoinette.schoenthaler@nyulangone.org)

Implementation Science 2026, **21**(2):S119

Background: Federally Qualified Health Centers (FQHC) provide essential care to medically vulnerable communities but face barriers to implementing complex hypertension interventions due to workforce and resource constraints. Adopting innovations from global settings, like task-shifting, supported by practice facilitation, can help FQHCs optimize workforce roles and workflows to implement evidence-based interventions. Drawing on the *Task Shifting Strategy for Hypertension Control* in Ghana trial, we evaluated the effectiveness of practice facilitation versus usual care in implementing a team-based intervention (ALTA) for improved blood pressure (BP) control among 652 patients with uncontrolled hypertension at a New York City FQHC.

Methods: We conducted a stepped-wedge cluster randomized trial in six urban primary care practices from 2019 to 2024. All practices began with usual care, followed by a 6–12-month pre-implementation phase before transitioning to the 12-month implementation phase. Practice facilitators supported ALTA implementation, which included medical assistants screening patients for medication nonadherence, clinicians

referring patients for home BP monitoring and nurse-led virtual health coaching, and structured documentation and monitoring of patients' progress. The primary outcome was implementation fidelity, assessed using mixed methods, guided by Proctor's Implementation Outcomes Framework. The clinical outcome was the proportion of patients achieving BP < 140/90mmHg at the end of the implementation phase.

Findings: Mean patient age was 61.9 (SD 13.6) years, 56.7% were female, 59.5% identified as Hispanic, and 29.2% as Black. Implementation adherence varied across roles: medical assistants screened 53.1% of eligible patients (1,649/3,103) for medication nonadherence; clinicians referred 72.8% (1,200/1,649) for home BP monitoring and health coaching, and 54.3% (652/1,200) enrolled. Of those enrolled, 91.4% (596) had ≥ 1 health coaching session, and 94.5% (563) had ≥ 1 follow-up session. Median implementation dose was six coaching sessions over 5.5 months, with 2.1 BP readings per week. Implementation quality was high, with comprehensive documentation of sessions. In the adjusted model, the implementation phase was associated with 60% higher odds of BP control compared to usual care (z -value = 2.29, p = 0.02).

Implications for D&I Research: This study demonstrates that task-shifting, an effective strategy in global settings, can be successfully adapted through reciprocal innovation and practice facilitation support to optimize workforce capacity and workflows to implement evidence-based interventions in urban safety-net settings.

Primary Funding Source: New York University Langone Health

Health policy dissemination and implementation science

S120

Jurisdictional profiles to plan for policymaker engagement, fit, and scalability of implementation efforts

Elle Pope¹, Sofia Morales², Jair Hurta², Maxwell Krieger³, Katherine Dunham³, Brandon Marshall⁴, Xiao Zang⁵, Erika Crable⁶

¹University of California Los Angeles, Los Angeles, CA, USA; ²University of California San Diego, La Jolla, CA, USA; ³Brown University, Providence, RI, USA; ⁴Brown University School of Public Health, Providence, RI, USA; ⁵University of Minnesota, Minneapolis, MN, USA; ⁶University of California, Los Angeles, Los Angeles, CA, USA

Correspondence: Elle Pope (ellefpop@gmail.com)

Implementation Science 2026, 21(2):S120

Background: Implementation strategies that respond to local health policy environments and address current needs of policymakers are hypothesized to enhance implementation success and sustainability goals. Barriers to policymakers' use of research are well documented; however, there is little guidance for identifying key policymakers or framing research to effectively engage them in implementation activities. This study developed an interdisciplinary methodological approach—jurisdictional profiles (JPs)—to identify policymakers in 6 jurisdictions/states (Massachusetts, Minnesota, Missouri, New York City, Nevada, Rhode Island) and engage them in the implementation of jurisdiction-tailored simulation models supporting evidence-informed decisions to prevent overdoses. Simulation models use local data to identify optimal combinations of harm reduction services (e.g., naloxone, drug checking, syringe services) but are only effective if policymakers trust and embed models in decision-making processes. We hypothesize that JPs will guide tailoring of the simulation model to enhance model fit and scalability into decision-making processes, and implementation of evidence-informed harm reduction services.

Methods: JPs are living documents updated throughout the 4-year study to include: 1) Political landscape and decision-making authority for overdose prevention (identified through document review of jurisdiction websites); 2) Political rhetoric about harm reduction services (identified through discourse analysis of news/press releases to understand evidence framing); 3) Sentinel policy surveillance of

the legal status harm reduction services to inform which to include/exclude in jurisdiction-specific models; 4) Jurisdiction data sources, dashboards, locally de/prioritized statistics to include in the model; 5) Concurrent harm reduction efforts (identified through policy document review). A structured ethnographic template is used to evaluate the utility/relevance of JPs and policymakers' trust in simulation models.

Findings: To date, JPs were created for 6 jurisdictions to guide EPIS Framework Planning activities. JPs yielded initial lists of policymakers to engage based on their decision-making authority and political support for harm reduction efforts, and informed how to tailor recruitment emails, presentation materials, and simulation models presented in co-design meetings. To date, one co-design meeting occurred (Missouri) which included 12 policy decision-makers. By December 2025, we plan to conduct 6 co-design meetings.

Implications for D&I Research: JPs provide an interdisciplinary approach to support implementation scientists in planning for policy-engaged, tailored implementation efforts to achieve fit and scalability outcomes.

Primary Funding Source: National Institutes of Health

S121

Strategic translators: using the interactive systems framework to map the role of intermediaries in tobacco policy implementation

Ana Herrera, David Timberlake, Denise D. Payan
University of California, Irvine, Irvine, CA, USA

Correspondence: Denise D. Payan (dpayan@hs.uci.edu)

Implementation Science 2026, 21(2):S121

Background: Intermediaries, which include public health officials, community advocates, and government staff, can serve a pivotal role in translating health policy to practice. Examining their contributions (i.e., interpretation, adaptation, and/or communication), the challenges they encounter, and the supports they require is critical to develop effective strategies to support policy implementation.

Methods: We conducted key informant interviews ($N=19$) with individuals who implemented state and local flavored tobacco sales restrictions (FTSRs) in California. Data collected focused on strategies used to implement FTSRs, barriers/challenges experienced, and resources needed. We applied the Interactive Systems Framework (ISF) as an organizing lens to categorize strategies and develop a typology that highlights the hybrid roles of intermediaries. This framework informed our interpretation of how these strategies were perceived to impact implementation.

Findings: Policy implementation relied on coordinated, system-level strategies that addressed distinct yet interdependent functions. Strategies within the *Synthesis and Translation System* included developing educational materials, translating surveillance data for public use, and conducting media campaigns for health and policy education to enhance public understanding, bolster community support, and improve stakeholder policy awareness. The *Support System* included community engagement, technical assistance, advocacy, convening, and legal interpretation. These were described as key actions to strengthen local capacity, align sectors, and encourage compliance. The *Delivery System* involved direct enforcement activities through compliance checks, surveillance of tobacco use, and legislative development. While participants affirmed the value of these strategies, they also identified persistent challenges (i.e., limited resources, lack of enforcement capacity, unclear policy language, fragmented agency coordination, community mistrust). Participants highlighted the need for sustained funding, stronger interagency coordination, targeted policy education, and enhanced accountability in enforcement and stakeholder engagement to bolster effective policy implementation.

Implications for D&I Research: This study advances dissemination and implementation science by empirically mapping intermediary-led strategies, revealing hybrid roles that span ISF systems, and emphasizing the importance of centering intermediary perspectives in policy implementation research. Findings underscore that intermediaries actively shape implementation outcomes and overall policy impact by bridging informational, procedural, and relational gaps.

Primary Funding Source: California Tobacco-Related Disease Research Program of the University of California, Grant Number [T32KT4713]

S122

Implementing increased Medicaid funding for Mobile Crisis Teams under the American Rescue Plan Act: a cross-state comparative analysis

Jacob Jorem¹, Ej Kelley²

¹Harvard Medical School, Boston, MA, USA; ²Columbia University, New York, NY, USA

Correspondence: Ej Kelley (ejk2204@cumc.columbia.edu)

Implementation Science 2026, 21(2):S122

Background: Background: Mobile Crisis Teams (MCTs) play a crucial role in providing community-based responses to behavioral health crises across the United States. To reduce this variation in access to and funding for MCTs, the American Rescue Plan Act (ARPA) of 2021 introduced increased Medicaid funding to strengthen and standardize MCTs for states opting in. Despite these efforts, little is known about how ARPA has impacted MCTs or the facilitators and barriers to its implementation across states opting in—among them Colorado, Massachusetts, New York, and North Carolina.

Methods: We conducted qualitative semi-structured interviews with 16 purposefully sampled stakeholders, including state- and city-level Medicaid officials and both urban and rural MCT providers. Interviews were conducted between February and May 2025 in Colorado, Massachusetts, New York, and North Carolina selected to represent variation in geography, politics, and Medicaid system. The transcribed interviews were analyzed thematically, and informed by the innovation and outer setting domains with relevant constructs of the Consolidated Framework for Implementation Research (CFIR).

Findings: The limited adaptability of ARPA's requirements of 24/7 availability and two-person multidisciplinary teams limited the uptake of the increased Medicaid funding in New York and North Carolina. In contrast, Massachusetts and Colorado successfully integrated ARPA funding by aligning it with existing or parallel Medicaid behavioral health crisis system reforms. The 24/7 availability requirement seemed particularly challenging in rural areas, where low call volumes made sustaining adequate response teams challenging. Staffing shortages emerged as the primary barrier to meeting ARPA requirements. Key facilitators included strong interagency collaboration, supported by enabling legislation and robust community engagement. Effective referral networks linking MCTs to various levels of the crisis system, including 988, emergency departments, inpatient and outpatient settings, and law enforcement, also facilitated implementation.

Implications for D&I Research: This study highlights how federal funding depends on state-level system readiness and infrastructure alignment for effective implementation. Findings underscore the importance of policy adaptability and robust infrastructure for effective implementation. They also highlight the value of using implementation frameworks to assess how states translate federal behavioral health policy into practice—and how funding streams can be leveraged to sustain services across diverse contexts.

Primary Funding Source: The Commonwealth Fund

S123

Using an environmental policy scan to assess population prioritization and equity in COVID-19 public health strategies in Ontario, Canada

Claire Gapare¹, Sierra Sweet¹, Lames Danok², Jamie Boyd², Stefan Baral³, Adrienne Chan⁴, Antoinette Handley⁵, Sulaiman Lakoh⁶, Sharmistha Mishra⁷, Bailah Molleh⁸, Beate Sander^{9,10}, Stephen Sevalie^{6,8}, Sharon Straus¹¹, Christine Fahim¹¹

¹Knowledge Translation Program, Li Ka Shing Knowledge Institute, St. Michael's Hospital, Unity Health Toronto, Toronto, Canada; ²Knowledge Translation Program, Li Ka Shing Knowledge Institute, St. Michael's Hospital, Unity Health Toronto, Toronto, ON, Canada; ³Johns Hopkins Bloomberg School of Public Health, Baltimore, MD, USA; ⁴Sunnybrook Health Sciences Centre, Toronto, Canada; ⁵Dalla Lana School of Public Health, Toronto General Hospital Research Institute, Toronto, ON, Canada; ⁶Department of Medicine, College of Medicine and Allied Health Sciences, University of Sierra Leone, New England, Freetown, Sierra Leone; ⁷Li Ka Shing Knowledge Institute, St. Michael's Hospital, University of Toronto, Toronto, Canada; ⁸Sustainable Health Systems Sierra Leone, Freetown, Sierra Leone; ⁹Institute of Health Policy, Management and Evaluation, University of Toronto, Toronto, Canada; ¹⁰Health Systems and Policy Research Collaborative Centre, University Health Network, Toronto, Canada; ¹¹Li Ka Shing Knowledge Institute, Unity Health Toronto, Toronto, ON, Canada

Correspondence: Christine Fahim (christine.fahim@unityhealth.to)

Implementation Science 2026, 21(2):S123

Background: Income, housing and occupation shaped the unequal risk of infection, transmission, and disease severity during the COVID-19 pandemic. We assessed whether Ontario, Canada's COVID-19 policies were implemented uniformly or prioritized specific populations, and whether health equity was explicitly referenced.

Methods: We conducted an environmental scan of provincial and federal public health and socio-economic COVID-19 strategies in Ontario (January 2020–May 2022) using the Canadian Institute for Health Information (CIHI) COVID-19 Intervention Timeline and a grey literature search. Strategies were screened independently by two researchers and grouped into 10 CIHI-defined categories (e.g., "Vaccines", "Closures/Openings"). Three researchers double coded the strategies as either "uniform" (applied universally across the populations or contexts without the intent to reduce disproportionate risks of acquisition, transmission, and/or severity if infected) or "prioritized" (intended to reduce disproportionate risks of acquisition, transmission, severity if infected, and/or socioeconomic impacts – such as those related to occupation, housing or income – of COVID-19 for specific populations or settings. Coding was verified by a full team review. We examined whether strategies explicitly referenced equity in their rationale.

Findings: Of 819 strategies identified, 615 (394 provincial, 221 federal) met inclusion criteria; 460 (74.8%) were uniform and 155 (25.2%) prioritized. Strategies related to "Distancing," "Travel," and "Public Information" were entirely uniform ($n = 134$, 100%), followed by "Closures/Openings" ($n = 106$, 94.6%) and "Financial/Economic" supports ($n = 67$, 62.0%). "Health Services" ($n = 18$, 54.5%) and "Vaccine" ($n = 45$, 48.4%) strategies were often prioritized. Prioritized populations included people in congregate care settings, Indigenous communities, people with disabilities, and others experiencing heightened risk. Of the prioritized strategies, 32.5% explicitly cited equity considerations; other strategies also used equity-oriented language but were implemented uniformly. Some uniform strategies were later replaced with prioritized strategies due to unintended negative impacts (e.g., school closures on mental health).

Implications for D&I Research: Most COVID-19 public health strategies in Ontario were uniformly implemented. Although more targeted strategies—such as vaccine rollout—emerged later, these were not consistently prioritized. These findings highlight the need to embed co-creation and community-informed approaches into pandemic preparedness, so equity is prioritized early and responsive strategies are in place for those most at risk.

Primary Funding Source: New Frontiers Research Fund—Special Call 2022 (NFRF)

S124

Implementation of the ban on the sale of ultra-processed foods to minors in Oaxaca

Carolina Pérez Ferrer¹, Mariana López Santiago¹, Raúl Ulises Hernández Ramírez², Carlos Manuel López García¹, Alma Lizeth Alvarado López¹, Anna K. Porter², Leith Soledad León Maldonado¹, Donna Spiegelman², Tonatiuh Barrientos Gutiérrez¹

¹Instituto Nacional de Salud Pública, Cuernavaca, MR, Mexico; ²Yale School of Public Health, New Haven, CT, USA

Correspondence: Carolina Pérez Ferrer (carolina.perez@insp.mx)

Implementation Science 2026, **21**(2):S124

Background

In 2020, the Mexican state of Oaxaca enacted a law to ban the sale of unhealthy foods and beverages to minors. The law was revised in 2024 to eliminate the prohibition in stores but maintained it in schools. We aimed to explore the law's adoption, acceptability, appropriateness, feasibility and implementation barriers and facilitators over time.

Methods

This multi-study project employed random sampling of schools in Oaxaca's Central Valleys and convenience sampling of stores nearby sampled schools. The project included three components: (1) mystery clients to assess law adoption (unsuccessful purchase attempts); (2) a survey on perceptions of acceptability, appropriateness, and feasibility among stakeholders using validated 5-point ordinal scales; and (3) semi-structured interviews with stakeholders to identify implementation barriers and facilitators, guided by the Consolidated Framework for Implementation Research. Data was collected in both schools and stores in 2023 (baseline) and again in schools in 2025.

Findings

At baseline, the law was adopted in 48.3% of the 58 selected schools and in 1.5% of the 65 nearby stores. Overall, 105 stakeholders completed the surveys at baseline: 55 in schools and 50 in stores. The mean acceptability scores were 4.4 in schools and 3.9 in stores. For appropriateness, they were 4.1 and 3.6, respectively. Feasibility received lower mean scores, with 3.8 in schools and 3.3 in stores. There was little change over a year in schools with adoption at 50%, and mean scores of 4.3, 4.1 and 3.7 for acceptability, appropriateness and feasibility respectively. Interviews with 20 stakeholders revealed implementation barriers including inadequate law communication, retailers' economic interests, lack of champions, and no enforcement.

Implications for D&I Research

The initial age-of-sale law may have been partially adopted by schools but not by stores in Oaxaca. The level of adoption, acceptability, appropriateness, and feasibility of the law was similar after its revision to focus on schools only. While the law seems to be generally acceptable among stakeholders, especially schools, there were several barriers identified that affected its implementation. Barriers should be addressed with implementation strategies to improve adoption and maintenance of the law and achieve its potential public health impact.

Primary Funding Source: Medical Research Council (MRC)

S125

Beyond the letter of the law: a CFIR-informed analysis of street-level enforcement of flavored tobacco sales restrictions

Ana Herrera¹, David Timberlake¹, Alec Chan-Golston², Denise D. Payan¹

¹University of California, Irvine, Irvine, CA, USA; ²University of California, Merced, Merced, CA, USA

Correspondence: Denise D. Payan (dpayan@hs.uci.edu)

Implementation Science 2026, **21**(2):S125

Background: Inconsistent or limited enforcement of flavored tobacco sales restriction (FTSR) policies can compromise policy effectiveness and unintentionally reinforce or exacerbate inequities. These policies are typically enforced by a range of actors, including law enforcement,

public health agencies, and other municipal officials. Enforcement plays a critical role in shaping implementation outcomes such as reach, fidelity, sustainability, and equity. However, limited attention has been paid to how health policy enforcement is operationalized and adapted by various actors and across diverse settings.

Methods: We conducted key informant interviews with 14 implementation and 5 enforcement agents, including representatives from law enforcement, public health, and municipal offices across California to investigate local enforcement of FTSRs. Guided by the Consolidated Framework for Implementation Research (CFIR), we used a thematic analysis approach to code transcript data, focusing on determinants across the Outer Setting, Inner Setting, Individuals Involved, and Process domains. We paid particular attention to equity-influencing factors (e.g., differential enforcement practices and approaches) to understand how enforcement is shaped and adapted in local contexts, and how these adaptations influence implementation outcomes.

Findings: Enforcement of FTSRs was shaped by multilevel factors that directly influenced equity and implementation outcomes. Outer setting conditions, including structural inequities, sociocultural norms, retail dynamics, and variability in tobacco control infrastructure, led to uneven enforcement across jurisdictions. Internally, enforcement agencies varied in their access to resources, knowledge, leadership engagement, and alignment with organizational goals, affecting the reach, consistency, and sustainability of efforts. Perceptions of agency authority, capacity, and institutional prioritization revealed role misalignments that constrained fidelity and equity. Finally, process-level factors such as retailer engagement strategies, interaction quality, and enforcement structures further shaped implementation outcomes and overall FSTR impact, underscoring the need for coordinated, equity-focused enforcement approaches.

Implications for D&I Research: This study extends CFIR by integrating an explicit equity lens into the analysis of FSTR enforcement. By identifying how multilevel and intersecting factors shape enforcement processes and outcomes, the findings offer insights into how procedural, structural, and relational dynamics influence the reach, fidelity, sustainability, equity, and broader policy impact. By making equity consequences of enforcement visible, this work advances a framework promoting equitable enforcement and effective public health policy implementation.

Primary Funding Source: California Tobacco-Related Disease Research Program of the University of California

S126

Doula care for Medicaid beneficiaries in Virginia

Desirae Leaphart¹, Alison Cuellar², Hong Xue², Panagiota Kitsantas²

¹NORC at the University of Chicago, District of Columbia, DC, USA;

²George Mason University, Fairfax, VA, USA

Correspondence: Desirae Leaphart (ddel5r@gmail.com)

Implementation Science 2026, **21**(2):S126

Background: To address rising maternal mortality, maternal morbidity, and racial inequities in maternal care, states are increasingly opting to include doulas as a licensed provider category in the state and, further, to include doula services as a benefit under their Medicaid programs. This study evaluates the implementation of Virginia's Medicaid doula policy and concludes with lessons learned.

Methods: This mixed-methods study used Virginia Medicaid claims, state-certified doula registry data, American Community Survey data, and qualitative interviews with doula Medicaid providers. Quantitative analyses included spatial mapping to assess geographic alignment between doula provider locations and Medicaid-enrolled pregnant individuals, as well as descriptive statistics summarizing service utilization patterns in Medicaid claims data from 2022 to 2025. The qualitative component explored implementation experiences from the perspective of nine practicing Medicaid doulas through semi-structured interviews. All study procedures were reviewed and approved by the George Mason University Institutional Review Board.

Findings: There were 130 state-certified doula Medicaid providers in Virginia at the time of the study. Spatial analysis identified regions of Virginia with high and low concentrations of doula Medicaid providers relative to where pregnant Medicaid beneficiaries are located. Claims analysis revealed that fewer than half of certified doulas billed for services between 2022 and 2025, with most claims being for postpartum services. The qualitative research identified eight key themes that highlight policy implementation lessons regarding administrative burdens, reimbursement rates, and awareness among Medicaid beneficiaries of their eligibility for doula care.

Implications for D&I Research: This study offers actionable insights that could be applied to other states implementing or scaling Medicaid doula policies. Implementation strategies should include working collaboratively with doulas to ensure targeted recruitment in regions with high need but less doulas available; more support for doulas regarding the Medicaid enrollment and reimbursement processes; enhanced collaboration with healthcare providers, case managers, and use of social media and community events to raise awareness among Medicaid beneficiaries about their doula benefits; and adjusting reimbursement rates and the amount and length of billable visits to reflect doulas' scope of work. These lessons can inform broader efforts to expand equitable access to doula care through Medicaid.

S127

Understanding factors influencing HPV vaccination policy implementation in Kenya

Priya Sarma, MS¹, Cynthia Kairu², Lynette Kamau², Ramatou Ouedraogo², Doris Njomo, PhD³, Peter Szilagyi⁴, Risa Hoffman⁵, Jonathan Purtle⁶, Corrina Moucheraud⁷

¹New York University School of Global Public Health, New York City, USA;

²African Population and Health Research Center, Nairobi, Kenya; ³Kenya Medical Research Institute, Nairobi, Kenya; ⁴University of California Los Angeles, Los Angeles, CA, USA; ⁵David Geffen School of Medicine at the University of California, Los Angeles, USA; ⁶Health Management & Policy, New York University School of Global Public Health, New York, NY, USA; ⁷New York University School of Public Health, New York, USA

Correspondence: Corrina Moucheraud (cm6512@nyu.edu)

Implementation Science 2026, 21(2):S127

Background: In 2019, Kenya launched a national human papillomavirus (HPV) vaccination program targeting girls aged 9–14 – but vaccine uptake remains low, with only 26% of eligible girls having received at least one dose. This study aimed to generate knowledge about how HPV vaccination policies are implemented by different stakeholder groups and determinants that influence policy implementation success in Kenya.

Methods

We conducted 21 key informant interviews (KIs) with policymakers from the Kenyan Ministry of Health; county-level health officials; and representatives from implementing agencies, multilateral organizations (e.g., UNICEF), and non-governmental organizations supporting HPV vaccination.

Researchers from the African Population and Health Research Center led the interviews, with a guide informed by the Consolidated Framework for Implementation Research (CFIR). Questions focused on facilitators and barriers to implementing HPV vaccination policies (the Kenya Cancer Policy 2019–2030, the Kenya National Cancer Control Strategy 2023–2027, and the Kenya National Immunization Policy Guidelines 2023) per domains of the CFIR.

Findings

In the outer setting, low public awareness of HPV vaccination was cited as a major barrier to implementation. Religious leaders were frequently mentioned as influential actors in both facilitating and hindering vaccine uptake. Respondents also perceived threats to long-term sustainability of HPV vaccination policies due to insufficient government financing, reliance on external donors for vaccine supply, and lack of sustained political commitment, including recent changes in the global landscape.

In the inner setting, limited collaboration between government agencies at the county-level (e.g., the Ministries of Health and Education)

hindered school-based vaccine delivery. This included lack of joint planning and weak ownership of the vaccination agenda by some schools. Irregular review meetings and multiple demands on health worker time further constrained consistent implementation, particularly in reaching out-of-school and nomadic girls.

In the process domain, health worker training about HPV vaccination and performance reviews tied to county-level immunization targets enabled adaptive planning to implement these policies.

Implications for D&I Research

This study highlights important enablers and barriers for HPV vaccine policy implementation. These data can be used to assist policymakers and implementers to better design strategies that fit Kenya's unique social and health system context to increase HPV vaccine uptake.

Primary Funding Source: National Institutes of Health

Models, measures, and methods

S128

The next generation of implementation strategy testing

Cara Lewis¹, Shahnaz Khan², George Mensah¹

¹Center for Translation Research and Implementation Science; National Heart, Lung, and Blood Institute, National Institutes of Health, Bethesda, MD, USA; ²National Institutes of Health, Bethesda, MD, USA

Correspondence: Cara Lewis (cara.lewis@nih.gov)

Implementation Science 2026, 21(2):S128

Background: The value of implementation science depends on promising findings of strategies, or the methods used to improve adoption, implementation, sustainment, and scale-up of evidence-based interventions. Implementation strategy taxonomies have proliferated, supporting selection and operationalization of strategies, and curation of their evidence. Yet, the majority of studies included in a 12-year systematic review did not sufficiently test strategies to draw conclusions about their impact. This presentation offers methodological insights to advance the field.

Methods: We completed a portfolio analysis of 181 competing implementation science awards issued by the National Heart, Lung, and Blood Institute from 2014–2024. We coded implementation strategies to characterize awards by reviewing their specific aims using a consensus coding approach. We categorized discrete strategies per the Expert Recommendations for Implementing Change (ERIC) taxonomy published in 2015. We used a content analysis approach to build upon the ERIC taxonomy while maintaining parsimony that allowed for new categories, new discrete strategy labels, and revised definitions. We documented challenges to using the taxonomy, missed opportunities for strategy testing, and exemplars.

Findings: Across 181 grants, 70% of the ERIC strategies were used; with 45 strategies tested in fewer than 20 studies, and 34 strategies tested in fewer than 10 studies. *Assess for readiness and identify barriers and facilitators* was coupled with *tailor strategies* in 34% of studies, with *adapt intervention* included in 65% of those studies. *Engage consumers, utilize financial strategies, and change infrastructure* were among the least used categories. To adequately reflect tested strategies, we developed one new category—*improve intervention fidelity and access*—and 15 new strategies across new and existing categories. We observed six opportunities for advancement: (1) Iterate taxonomies more frequently to evolve with the field; (2) Expand strategy characterizations to explicitly include function, and not simply form; (3) Provide granular abstraction of strategies for replication; (4) Select core strategies based on intervention and setting; (5) Distinguish between the intervention and strategy; and (6) Invest in underutilized strategies given promising putative mechanisms.

Implications for D&I Research: This portfolio analysis highlights methodological advances that could refine strategy testing to improve selection, replication, impact, transparency, and scalability.

Primary Funding Source: National Institutes of Health

S129

Navigating complexity in implementation science clinical trials: towards a structured framework for risk stratificationVeronica Tutse-Tonwe¹, Nicole Nyack¹, Michelle Culp², Debbie Vitalis¹¹Center for Translation Research and Implementation Science; National Heart, Lung, and Blood Institute, National Institutes of Health, Bethesda, MD, USA; ²National Institutes of Health, Bethesda, MD, USA**Correspondence:** Veronica Tutse-Tonwe (veronica.tonwe@nih.gov)*Implementation Science* 2026, **21**(2):S129

Background: The goal of clinical trial monitoring is to ensure appropriate oversight of participant safety and data integrity. The 1998 NIH Policy for Data and Safety Monitoring recognizes that, “all trials, even those that pose little likelihood of harm, should consider an external monitoring body”. Monitoring should align with the trial’s nature, size, and complexity. Implementation science (IS) clinical trials present unique challenges due to complexity of design, recruitment, and outcomes, yet limited guidance exists for tailoring oversight and risk assessment to IS. A systematic, tailored approach to IS trial oversight is needed.

Methods: We conducted a portfolio analysis of the NHLBI, Center for Translation Research and Implementation Science (CTRIS) funded IS clinical trials from 2013–2024 to (1) characterize grants, (2) identify proposed oversight processes (related to risk assessment and monitoring strategies), and factors that may impact risks; and (3) develop a structured IS clinical trial risk stratification framework. We analyzed a subset of 62 grants for our in-depth portfolio analysis on clinical trial oversight. Using a structured coding framework, coders extracted data on eight variables from the NIH grants database.

Findings: CTRIS-funded IS clinical trials revealed substantial variability across study designs, consent practices, oversight mechanisms and risk reporting. Most studies employed hybrid designs (52%) with cluster and stepped-wedge approaches (63%), underscoring implementation study design complexities that are frequently not considered when assessing risks. Oversight processes included an independent monitor, DSMB, or simply the IRB, but details of their execution were inconsistently documented. Most studies lacked clarity around adverse event reporting and potential risks descriptions, identifying participant study risk as “minimal”, primarily citing confidentiality. Consent approaches varied; most used a multi-level recruitment consenting process, written consent at the patient-level, and waiver-based consent for provider/system-level trials.

Implications for D&I Research: Traditional models of clinical trial risk assessment do not sufficiently account for the ethical, logistical, and operational dimensions unique to IS trials, yet these findings highlight critical gaps in oversight management and risk stratification. A structured framework that integrates trial complexity, recruitment levels and size, nature and real-world context is critically needed.

S130

Social network alteration as implementation strategies: a secondary analysis of empirical studiesLauraEllen Ashcraft¹, Christina Yuan², Katherine Pickard³, Alicia Bunger⁴, Sobia Khan⁵, Reza Yousefi Nooraie⁶¹Veterans Health Administration, Philadelphia, PA, USA; ²Johns Hopkins Bloomberg School of Public Health, Baltimore, MD, USA; ³Emory University School of Medicine, Atlanta, GA, USA; ⁴The Ohio State University, Columbus, OH, USA; ⁵The Center for Implementation, Toronto, ON, Canada; ⁶University of Rochester, Rochester, NY, USA**Correspondence:** LauraEllen Ashcraft (lauraellen.ashcraft@pennmedicine.upenn.edu)*Implementation Science* 2026, **21**(2):S130**Background**

There is a growing recognition that relationships are foundational to implementation. Yet, the mechanisms and packaging for integrating social networks as implementation strategies remain unclear. This study examines how experimentally tested implementation strategies incorporate network alteration activities (i.e., intentionally

changing the structures and functions of social networks) and identifies underexplored opportunities for advancing network interventions in implementation research.

Methods

We included 129 experimental implementation studies identified in a recently published systematic review (Ashcraft et al., 2024). Preliminarily, we included 66 of the 129 studies based on the highest frequency of ERIC strategies pertaining to network alteration. Reviewers assessed implementation strategy descriptions for network alteration strategies corresponding to Bunger et al.’s (2023) typology (creating groups, changing actors’ networking skills, changing actors’ awareness of social networks, changing actors’ motivation for networking, changing actors’ prominence, changing network composition, changing environments to facilitate networking, targeting specific ties). We met regularly to ensure consistency and consensus. We captured the frequency of each network alteration type and their combinations, and packaging with other implementation strategies.

Findings

Network alteration activities are frequently embedded within multi-component strategies but are rarely tracked as primary strategy components, inconsistently identified, and rarely assessed. The most common network alteration strategy was creating groups ($n=35$; e.g., group training and formation of implementation teams), followed by changing network composition ($n=25$; e.g., an external facilitator joining local teams), and changing actors’ prominence ($n=21$; e.g., the identification and promotion of local champions and internal leads). The least common strategies were changing actors’ motivation and awareness, and changing the environment. The forthcoming analysis will analyze the packaging of network strategies and their association with RE-AIM outcomes, extracted from the original systematic review.

Implications for D&I Research

This analysis generated a crosswalk linking network alteration activities to implementation strategies and identified gaps for future methodological work. By clarifying how social network alterations are operationalized within implementation activities, we aim to expand the “frontiers” for designing and evaluating socially and contextually responsive implementation strategies by better incorporating social structures and dynamics in the intervention design and implementation planning.

S131

The Dissemination Research Logic Model (DRLM) for planning and executing dissemination projects: application to the Adolescent Medicine Trials Network for HIV InterventionsKatherine Merrill¹, Kristi Gamarel², Lisa Hightow-Weidman³, Sybil Hosek⁴¹University of Illinois Chicago, Chicago, IL, USA; ²University of Michigan School of Public Health, Ann Arbor, MI, USA; ³Florida State University, Tallahassee, USA; ⁴University of Illinois Chicago, Chicago, USA**Correspondence:** Katherine Merrill (kmg@uic.edu)*Implementation Science* 2026, **21**(2):S131

Background: The implementation research logic model (IRLM) is a widely used tool for enhancing the clarity and rigor of implementation projects, but no such tool exists in dissemination science. We set out to create a dissemination research logic model (DRLM) and apply it to the 25-year-old Adolescent Medicine Trials Network for HIV Interventions (ATN).

Methods: We began by articulating the key components of the dissemination process: the “thing” (i.e., program/practice/guideline) being disseminated and its source, goal and audience, determinants, strategies, channels, mechanisms of change, and desired outcomes. We retrospectively applied the model to several completed ATN projects, generating variations of the model. Here, we illustrate its application to the ATN 113 Protocol (Project PrEPare), the first major adolescent-specific pre-exposure prophylaxis (PrEP-Truvada) demonstration project in the U.S. for adolescent men who have sex with men ages 15–17.

Findings: The “thing” disseminated was a guideline based on findings from the ATN 113 (the source)—i.e., that PrEP is safe,

acceptable, and feasible, and effective in preventing HIV for adolescents with high medication adherence. The ATN sought to make PrEP available for adolescents through policy change, targeting policymakers. To overcome dissemination strategies, the network used a set of strategies to disseminate through various channels (e.g., conference presentations, peer-reviewed publications). These led to improved knowledge and awareness among policymakers (i.e., mechanisms), which led to three dissemination outcomes: 1) a change in policy through the FDA's expansion of Truvada PrEP use for adolescents; 2) updates to CDC guidelines to recommend PrEP for adolescents; and 3) updates to insurance coverage policies to cover PrEP for adolescents. Dissemination outcomes precipitated increased PrEP prescription for adolescents (i.e., implementation outcome) and subsequently reduced HIV incidence among adolescents (i.e., individual outcome).

Implications for D&I Research: The DRLM's application to ATN 113 clarifies the pathways through which the guideline disseminated led to changes in dissemination outcomes (i.e., policy changes), changes in implementation outcomes, and ultimately improved health outcomes. Our presentation of this example and others from the ATN will inform future researchers' dissemination projects. The DRLM will help realize the benefits of dissemination science, which has to date received notably less attention than the field of implementation science.

Primary Funding Source: National Institutes of Health

S132

Demystifying theories, models and frameworks: when, why, and how to integrate more than one

Christine Kowalski

Veterans Health Administration, Ann Arbor, MI, USA

Correspondence: Christine Kowalski (christine.kowalski@va.gov)

Implementation Science 2026, 21(2):S132

Background: Theories, Models and Frameworks (TMF) are tools that, together, represent a common language, allow cross comparisons, and ensure we consider relevant variables that help explain implementation processes, successes or failures. However, selecting one or more TMF(s) remains challenging due to an overwhelming number of options and lack of salient points for selection. This presentation will describe the rationale for selecting two or more TMFs and some beneficial tools to support selection.

Methods: The 5 overarching TMF categories will be explained along with why to use a TMF, including to describe and/or guide the process of translating research into practice, to understand and/or explain what influences implementation outcomes –i.e., explain why an intervention works or doesn't work, to evaluate implementation, to improve generalizability, to identify barriers or understand context, and to select/identify implementation strategies. Multiple resources will be shared that will help participants select a TMF that is a good fit for a research or quality improvement study. How and when to select more than one TMF will be explored, including how to carefully use multiple TMFs temporarily throughout your QI project or study including for planning, implementation, and evaluation—and how to use TMFs to understand what happened and why.

Findings: When used together TMFs can enhance effectiveness by uncovering relationships between constructs and mechanisms that influence implementation success. For example, multiple TMFs can allow system-level process factors and individual-level process factors. When using two TMFs it is important to check for redundancies and eliminate. Combinations can also be used to make the leap from a determinants list only, to elucidating impacts on outcomes.

Implications for D&I Research: Resources for where to turn for help will be shared. Attendees will gain an understanding of how to pick a TMF that is a good fit for an operational project or research question and gain a fundamental understanding of when and why you might need to use more than one TMF in conjunction. By ensuring that more practitioners and researchers understand thoughtful integration of

more than one TMF we can maximize the benefits of dissemination and implementation science.

Primary Funding Source: Department of Veterans Affairs

S133

Evaluating legal services for housing-unstable veterans: a combined use of RE-AIM and i-PARIHS

Bo Kim^{1,2}, Sonia Ruppert³, Samantha Ryan¹, Angie Moussa⁴, Sophia Bartels⁴, Ida Griesemer⁵, Amanda Midboe^{6,7,8}, Jack Tsai^{9,10}, Beth Ann Petrakis⁴

¹Veterans Health Administration, Boston, MA, USA; ²Harvard Medical School, Boston, MA, USA; ³University of Pittsburgh, Pittsburgh, PA, USA;

⁴Veterans Health Administration, Bedford, MA, USA; ⁵Veterans Health Administration, White River Junction, VT, USA; ⁶Veterans Health Administration, Menlo Park, CA, USA; ⁷University of California Davis School of Medicine, Davis, CA, USA; ⁸Stanford University School of Medicine, Stanford, CA, USA; ⁹Veterans Health Administration, San Antonio, TX, USA;

¹⁰University of Texas Health Science Center at Houston, Houston, TX, USA

Correspondence: Bo Kim (bo.kim@va.gov)

Implementation Science 2026, 21(2):S133

Background: Legal issues are major but addressable social determinants of health. The Department of Veterans Affairs' Legal Services for Homeless Veterans and Veterans At-Risk for Homelessness (LSV-H) grant program funds legal aid to address barriers to housing and healthcare. To identify factors influencing LSV-H implementation from the perspectives of veterans receiving and organizations delivering LSV-H-funded services, we used the RE-AIM (Reach, Effectiveness, Adoption, Implementation, Maintenance) framework to guide participant selection for interviews (distinct from RE-AIM's common use for summative evaluation), which we subsequently analyzed using the i-PARIHS (Integrated Promoting Action on Research Implementation in Health Services) framework and RE-AIM's *Effectiveness* domain.

Methods: We conducted semi-structured interviews with 21 veterans, as well as with 18 administrators and 15 legal service providers across 15 LSV-H grantee organizations purposively selected to vary in geography, organization type, and RE-AIM-aligned characteristics – number of veterans served [*Reach*], representativeness of target communities (e.g., rural) [*Adoption*], delivery of intended services [*Implementation*], and continued pursuit of LSV-H funding [*Maintenance*]. Rapid qualitative analysis extracted themes aligned with i-PARIHS constructs (*Innovation*, *Recipients*, *Context*, *Facilitation*) and RE-AIM's *Effectiveness* domain.

Findings: Participants viewed LSV-H as making legal services more accessible to underserved veterans, including those previously ineligible due to income thresholds. LSV-H was perceived to prevent eviction, secure housing, reinstate benefits, and reduce anxiety and stress [*Effectiveness*]. LSV-H funds increased grantees' service delivery capacity, while some grantees with less familiarity with LSV-H's federal reporting requirements found them challenging [*Innovation*]. Veterans' engagement and reported improvements reflected LSV-H's high acceptability and alignment with their goals [*Recipients*]. Grantees having co-located services with VA settings fostered veteran trust [*Context*]. Grantees' expertise with legal needs of veterans and subpopulations (e.g., tribal) helped tailor service delivery [*Facilitation*].

Implications for D&I Research

Combining RE-AIM and i-PARIHS enabled comprehensive understanding of LSV-H implementation dynamics. RE-AIM-guided participant selection helped identify implementation-relevant grantee characteristics, while i-PARIHS' emphasis on evolving, interrelated constructs and RE-AIM-aligned perceived effectiveness illuminated how contextual factors shaped implementation. Findings suggest that pairing these frameworks can enhance explanatory power and provide actionable insights. This evaluation adds to the growing literature on legal services' contributions to health, and how combining complementary frameworks can inform implementation of complex programs.

Primary Funding Source: Department of Veterans Affairs

S134

Integrating RE-AIM and CFIR to evaluate the implementation of a national intimate partner violence screening expansion

Galina Portnoy^{1,2}, Mark Relyea^{1,2}, Melissa Dichter^{3,4}, Candice Presseau^{1,2}, Katherine Iverson⁵, Joshua Price-Crist¹, Melissa Skanderson^{1,2}, Cynthia Brandt^{1,2}, Steve Martino^{1,2}

¹Veterans Health Administration, West Haven, CT, USA; ²Yale School of Medicine, New Haven, CT, USA; ³Veterans Health Administration, Philadelphia, PA, USA; ⁴Temple University School of Social Work, Philadelphia, PA, USA; ⁵Veterans Health Administration, Boston, MA, USA

Correspondence: Galina Portnoy (galina.portnoy@va.gov)

Implementation Science 2026, 21(2):S134

Background: The healthcare system plays an integral role in the detection and response of intimate partner violence (IPV), a significant public health problem with far-reaching consequences. The majority of IPV screening initiatives have historically targeted women of reproductive age, with little known about IPV screening experiences among other patient populations, including men. The Veterans Health Administration (VHA) has called for an expansion of IPV screening, providing a unique opportunity for a large-scale evaluation of IPV screening and response across patient populations. This presentation describes mixed-methods findings from a highly-partnered evaluation with VHA IPV operations partners, guided by an integration of the RE-AIM and CFIR frameworks.

Methods: Using medical record data from 6,970,533 patients (89.8% male) across 135 VHA facilities, we conducted analyses across RE-AIM components to examine the *reach* of IPV screening, including differences and disparities between those who were and were not screened across patient characteristics (e.g., gender, age, race, ethnicity, sexual orientation, marital status); *effectiveness* of screening; *adoption* across VHA facilities, provider types, and clinical settings; and *implementation fidelity*. Using data from CFIR-grounded qualitative interviews with providers ($n=32$) and patients ($n=24$), we identified multilevel barriers and facilitators of the screening expansion.

Findings: Over 1.5 million patients are being screened annually for IPV across VHA, yet some patient subgroups are more likely to be missed than others, such as men and Asian American patients (*reach*). Rates of screening *adoption* varies significantly by facility and clinical setting, as does screening *implementation*, particularly related to fidelity of screening per protocol among men. CFIR components across domains enabled us to categorize multilevel factors critical to screening expansion, potential barriers and enablers of implementation, and impact/effectiveness of screening on patient outcomes.

Implications for D&I Research: Integrating RE-AIM and CFIR allowed for an in depth and rich evaluation of VHA's IPV screening and response expansion. Findings yield clinical, policy, and research implication including recommendations for screening implementation initiatives and/or adaptations, optimization of strategies for screening implementation across patient populations, and underscoring the value of integrating D&I frameworks for conducting rigorous evaluations.

Primary Funding Source: Department of Veterans Affairs

S135

Benefits of integrating a novel resilience framework with CFIR to better understand adaptations during stressful times

Abigail Baim-Lance^{1,2,3}, Diane Addison^{2,4}, Norman Archer⁵, Peter Gordon⁶, Sharen Duke⁷, Virginia Shubert⁸, Denis Nash², McKaylee Robertson⁹

¹Veterans Health Administration, New York, NY, USA; ²City University of New York, New York, NY, USA; ³Icahn School of Medicine at Mount Sinai, New York, NY, USA; ⁴Graduate School of Public Health and Health Policy, City University of New York, New York, NY, USA; ⁵University of California San Francisco, San Francisco, USA; ⁶Division of Infectious Disease, Department of Internal Medicine, Columbia University Irving Medical Center, New York, NY; and, New York, USA; ⁷Veterans Health Administration, New York, USA; ⁸Housing Works, New York, NY, New York, USA; ⁹Institute for Implementation Science in Population Health, City University of New York, New York, NY; New York, USA

Correspondence: Abigail Baim-Lance (abigail.baim-lance@va.gov)

Implementation Science 2026, 21(2):S135

Background: Between 2019–2022, the Bottom Up Pilot Project leveraged a health information exchange and the ability to send real-time alerts to mobilize community-based outreach teams to quickly find and re-engage HIV positive patients back into healthcare. A collaboration of clinical, community, and academic partners located in New York City, Bottom Up can be characterized as a complex intervention with numerous tasks and coordination steps. Bottom Up faced a range of implementation challenges including the COVID-19 pandemic, and yet was successfully operationalized during the 3 year period. We explored using a novel implementation science framework incorporating organizational resilience to describe the processes by which partners overcame challenges experienced during implementation.

Methods: We conducted semi-structured interviews with implementation staff ($n=6$) and meeting minutes and protocols document reviews ($N=35$). The Consolidated Framework for Implementation Research (CFIR) combined with a novel organizational resilience framework guided thematic and process analyses. CFIR included construct assessment within all 5 of the domains. The resilience framework consisted of two attributes associated with challenges (acute, chronic) and three resilience types: absorptive to cope with adversity, adaptive to adjust as short-term solutions, and transformative to pro-actively adopt system-wide changes.

Findings: Bottom Up experienced 20 major challenges, 2–5 challenges per step. Challenges were multi-level, and chronic and crisis in nature. Implementers overcame challenges by leveraging multilevel factors that were absorptive, adaptive (most common), and transformative. Adaptive strategies were characterized by the project's ability to maintain consistency to core operations while also employing flexibility by strategically simplifying or "downshifting" activities to fit their capacities in the face of stresses. The project leveraged the CFIR-defined constructs of leadership and flexibility to respond to arising challenges. Transformational responses to upgrade technology functionality suggests that initiatives can catalyze broader organizational change.

Implications for D&I Research

Combining a process-oriented framework like organizational resilience with CFIR can enhance understanding the active and dynamic processes promoting and impeding the growth and success of implementation. This analysis is particularly useful to understand implementation during volatile periods and with complex interventions.

Primary Funding Source: Funding for the Evaluation of the Bottom Up pilot is supported by the Einstein-Rockefeller-CUNY Center for AIDS Research (P30AI124414) and the New York City Council

S136

Designing flexible tools for budget impact analysis to support adoption of innovations

Douglas Levy¹, Natalie Smith²

¹Harvard Medical School, Boston, MA, USA; ²University of Pittsburgh, Pittsburgh, PA, USA

Correspondence: Douglas Levy (dlevy3@mgh.harvard.edu)

Implementation Science 2026, 21(2):S136

Background

Intervention and implementation costs are recognized in most major implementation science frameworks as key determinants of innovation adoption and success. Cost per quality-adjusted life year (QALY) saved is a common metric used in economic evaluations, but it fails implementers in the U.S. for at least two reasons. First, by taking a societal perspective, it encompasses broad costs and benefits that may not be relevant to individual institutions making adoption decisions. Second, the cost per QALY metric informs what innovations should be adopted, but not which innovations are affordable and thus feasible to implementers. Budget impact analysis (BIA) is a method for estimating context-specific implementation costs, aiding decision-makers in determining the affordability of evidence-based programs. Still, BIAs remain scarce in implementation science.

As a case study, we describe the development of a pilot BIA tool tailored to the Smokefree Support Study (SSS), an evidence-based smoking cessation program for cancer patients, as well as potential future refinements and adaptations. The tool was designed to ensure accessibility, flexibility, and contextual adaptability through the incorporation of user-centered design principles.

Methods

Using publicly accessible R Shiny software, the BIA tool was constructed based on data from the original SSS cost-effectiveness study. The prototype allowed users to input context-specific variables such as hourly wages, enrollment estimates, and staffing models. We conducted usability testing of the tool through interviews with participants from nine sites within the National Cancer Institute's Cancer Center Cessation Initiative.

Findings

Participant feedback provided information on potential refinements to enhance usability, functionality, and alignment with program adoption needs. The finalized tool provided dynamic cost estimates for training, staffing, recruitment, and material expenses, allowing users to customize inputs for their specific contexts. Usability testing revealed the need for a tool designed with flexibility to reflect local adaptation. Feedback highlighted the necessity of integrating contextualized cost estimates with other decision criteria to support adoption.

Implications for D&I Research

This study illustrates the value of BIA and the importance of user-centered design in creating tools that meaningfully facilitate evidence-based program adoption. Findings emphasize the need for adaptability in tool design/inputs and underscore the importance of combining cost estimates with broader decision-making frameworks.

Primary Funding Source: National Institutes of Health

S137

Applying Program Budgeting and Marginal Analysis (PBMA) for substance use prevention: a pragmatic approach to inform resource allocation decisions

Andria Eisman

Wayne State University College of Education, Detroit, MI, USA

Correspondence: Andria Eisman (aeisman@wayne.edu)

Implementation Science 2026, **21**(2):S137

Background: Vaping among youth poses a public health challenge, yet schools face resource constraints in prevention. Traditional cost-effectiveness analysis may not be well-suited due to difficulties in quantifying diverse benefits and limitations in youth-focused outcomes. We introduce Program Budgeting and Marginal Analysis (PBMA) as a pragmatic alternative, focusing on resource allocation trade-offs. PBMA is valuable because the entity investing in prevention may not directly realize the long-term cost savings.

Methods: We employed a PBMA framework to assess the economic implications of vaping prevention interventions. PBMA is well-suited for implementation science, as it facilitates economically grounded decision-making by quantifying the marginal benefits of alternative implementation strategies. Our application involved: 1) defining scope, 2) identifying data sources, 3) estimating marginal costs of strategy options, 4) identifying cost offsets, 5) conducting marginal analysis, and 6) sharing findings.

Findings: Our analysis included multi-level partner perspectives, using pilot study data as well as school and district administrative data and results from other published research. We found the total cost of \$24,860 for the 4-school RAPD implementation, or \$6,215 per school. The coordinating body bore \$9,656 of the cost, followed by the RESA, who bore 32.7% of the cost. We estimated that the community partners' time was \$4,305, and the schools bore 11.2% of the cost (\$2,776). We determined that the cost of vape detector initiation and installation to be \$6850 per set of male/female bathrooms, plus time for administration and school staff to be trained on response methods and procedures for responding to alerts. Potential cost offsets

for RAPD include reduced disciplinary incidents and need for reactive interventions, increased collaboration; for vape detectors, offsets may include deterring students from vaping, reducing immediate risk behaviors. We also consider potential longer term cost offsets.

Implications for D&I Research

PBMA offers a pragmatic framework for evaluating implementation strategies in school-based vaping prevention. By emphasizing value-for-money and opportunity cost considerations, and incorporate partner central partner considerations and competing demands in complex environments, PBMA facilitates transparent allocation decisions, highlighting relative costs, benefits, feasibility, and long-term system impact. PBMA helps align prevention investments with priorities of schools and communities.

Primary Funding Source: National Institutes of Health

S138

Decision Analysis for Intervention Value Efficiency (DAIVE): a tool for cost-informed decision-making in intervention optimization

Jillian Strayhorn¹, Jennifer Cantrell², Linda Collins³, David J. Vanness⁴, Michael Fiore⁵, Timothy Baker⁶, Megan Piper⁵

¹New York University, New York, USA; ²New York University School of Global Public Health, New York, USA; ³New York University School of Global Public Health, New York, NY, USA; ⁴Pennsylvania State University, University Park, PA, USA; ⁵University of Wisconsin, Madison, WI, USA; ⁶School of Medicine and Public Health, University of Wisconsin, Center for Tobacco Research and Intervention, Madison, WI, USA

Correspondence: Jillian Strayhorn (jillian.strayhorn@nyu.edu)

Implementation Science 2026, **21**(2):S138

Background

Intervention optimization is a research paradigm in which alternative versions of a multicomponent intervention are rigorously compared in an optimization phase of research that precedes subsequent evaluation. This optimization process is "implementation-forward" in that its motivating goal is to empirically inform the construction of an intervention that is not only effective but also readily implementable. To aid determinations as to which intervention version (e.g., comprising which type or dose of intervention content) is optimal, given finite resources, we have advanced Decision Analysis for Intervention Value Efficiency (DAIVE), a decision analytic framework for selecting optimized interventions based on data from factorial optimization trials. In this presentation, we illustrate DAIVE using the empirical example of a 2⁶⁻¹ fractional factorial trial to optimize a tobacco cessation intervention. We consider how DAIVE fits into a larger toolkit of economic evaluation approaches, including traditional CEA.

Methods: Using data from a 2⁶⁻¹ fractional factorial optimization trial, we estimate the individual and combined effects of six candidate intervention components, each with two levels: 1) preparation nicotine patch vs. none; 2) preparation nicotine gum vs. none; 3) preparation counseling vs. none; 4) intensive cessation in-person counseling vs. minimal; 5) intensive cessation phone counseling vs. minimal; and 6) 16 vs. 8 weeks of combination nicotine replacement therapy (nicotine patch + nicotine gum). We apply DAIVE to summarize the performance of the 64 resulting intervention versions, each comprising some combination of candidate component levels, and identify the intervention that accomplishes the best strategic balance of effectiveness, in terms of cessation, and cost-to-implement, in dollars.

Findings: In this factorial optimization trial, results indicate that not all components have to be set to their higher, more resource-intensive levels to achieve the best expected effectiveness. DAIVE facilitates the identification of which intervention versions remain effective while conserving implementation resources, as well as the selection among those, given opportunity costs.

Implications for D&I Research: Optimizing for value efficiency using DAIVE can support the construction of effective interventions that do not include needless intervention components (or

needlessly intense component doses), given opportunity costs, minimizing waste of implementation resources.

Primary Funding Source: National Institutes of Health

S139

Evaluation of implementation efficiency using stochastic frontier analysis: case example from an adaptive trial to implement medications for opioid use disorder

Hannah Cheng¹, Mark McGovern¹, Jay Ford², H                ¹, Todd Wagner³

¹Stanford University School of Medicine, Palo Alto, CA, USA; ²University of Wisconsin-Madison, Madison, WI, USA; ³Stanford-Surgery Policy Improvement Research and Education Center (S-SPIRE), Stanford, CA, USA

Correspondence: Hannah Cheng (chenghh@stanford.edu)

Implementation Science 2026, **21**(2):S139

Background: Effective and efficient implementation strategies are crucial for scaling evidence-based practices (EBPs). While studies increasingly have begun to document the cost of implementation strategies, few have examined how these costs translate to improvement in implementation outcomes. This study estimates total implementation costs and applies stochastic frontier analysis (SFA) to assess the relative efficiency of implementation strategies in improving access to and quality of medications for opioid use disorder (MOUD).

Methods: Sixty-nine addiction treatment programs and primary care clinics in Washington state participated in an adaptive trial offering sequential strategies: Audit and Feedback (A&F), Workshop, Internal Facilitation, and External Facilitation. Using activity-based microcosting, we estimated delivery and participation costs of implementation strategies prospectively. Total implementation costs were linked to monthly RE-AIM outcomes: Reach (patients prescribed MOUD), Effectiveness (MOUD initiation within 72 h of diagnosis), Adoption (active MOUD prescribers), and Implementation (implementation capability scale total). All cost and outcomes data have been collected; we will apply SFA to estimate cost-efficiency using A&F as a proof of concept by November 2025.

Findings: As part of the SFA model, inputs will include the delivery and participation costs of A&F, which incurred a setup cost of \$32,266 and an average annual recurring cost of \$4,231 per clinic. Outputs will include Reach (mean = 41.66%, range: 0–100%), Effectiveness (mean = 36.32%, range: 0–100%), Adoption (mean = 4, range: 0–16), and Implementation (mean = 3.57, range: 2.21–5.00). We will present the average cost-efficiency score across clinics and compare how the most efficient clinics differ in RE-AIM outcomes per dollar spent compared to the average clinics.

Implications for D&I Research: Applying SFA helps identify clinics that achieve the greatest improvement in implementation outcomes per dollar spent, which supports resource allocation decisions based on efficiency, rather than cost or effectiveness alone. This efficiency-driven approach can maximize sustainment by investing in efficient systems that do more with less while conserving resources that can be redirected to those who need it most.

Primary Funding Source: National Institutes of Health

S140

From context to strategy: Developing the Context Compass Framework and Tool to operationalize context and guide implementation strategy selection

Sobia Khan, Julia Moore

The Center for Implementation, Toronto, ON, Canada

Correspondence: Julia Moore (julia.moore@thecenterforimplementation.com)

Implementation Science 2026, **21**(2):S140

Background

Context is widely recognized as critical to implementation success, yet commonly used frameworks like the Consolidated Framework for Implementation Research (CFIR) have been critiqued due to its limited focus on system-level factors and challenges with applicability in other settings, like in low- and middle-income countries. In addition, while attempts have been made to align ERIC strategies with CFIR constructs (CFIR-ERIC matching tool), selecting strategies to address context remains challenging. To address these issues, we developed the Context Compass Framework and tool—designed to consolidate key contextual domains across existing contextual frameworks, center usability, and support selection of implementation strategies that address contextual determinants.

Methods

Drawing on a scoping review (Nilsen & Bernhardsson, 2019) of contextual frameworks, we identified factors from 13 established context frameworks commonly used in implementation science (e.g., CFIR, EPIS). We identified and grouped recurring contextual domains into a framework using an inductive coding approach and collaborative review. Key domains were refined through iterative consultation with implementation researchers and practitioners, ensuring theoretical alignment and usability. We developed an interactive tool to identify implementation strategies to address contextual barriers or facilitators. Strategy mapping was grounded in established taxonomies (e.g., ERIC, Behaviour Change Techniques) and experiential evidence.

Findings

The Context Compass Framework consists of a simplified structure comprising 15 factors and 39 sub-factors that can be assessed at the site or system level. By describing factors in ways that can address multiple levels of the system, we were able to reduce the total number of factors. Factors were also grouped based on relevance across the implementation stages (fit, readiness, implementation, sustainability).

Contextual factors and sub-factors were linked to implementation strategies through the underlying functions. The interactive online tool allows users to identify high-priority contextual challenges and explore implementation strategies that can address those challenges. Early user feedback indicates the framework improves clarity, accessibility, and actionability of context assessments.

Implications for D&I Research

This work offers a more practical and structured way to bridge context assessment with implementation strategy selection. The Context Compass Framework can support research design and implementation, offering a foundation for future empirical validation studies. The next steps include conducting usability testing with the tool.

S141

From a blank slate to contextual fit: iterative macroergonomic observations guiding pharmacy intervention design

Emily Hoffins¹, Katherine G. Moore², Maria E. Berbakov¹, Jay Ford II¹, Aaron M. Gilson¹, Jamie A. Stone², Stephanie M. Resendiz¹, Shiyang Mai¹, Rutuja Gadgil¹, Michelle A. Chui²

¹University of Wisconsin—Madison School of Pharmacy, Madison, WI, USA;

²University of Wisconsin—Madison School of Nursing, Madison, WI, USA

Correspondence: Michelle A. Chui (michelle.chui@wisc.edu)

Implementation Science 2026, **21**(2):S141

Background

Expanding services within community pharmacies is a national public health priority, yet most interventions are developed based on assumptions about how workflows function rather than a true understanding of daily practice. Researchers often believe they know what changes are needed to improve systems, but, without

direct, open-ended observation, these assumptions can miss critical realities. Community pharmacy workflows are dynamic with frequent interruptions and competing demands for staff time. As pharmacists offer more clinical services, these services must be integrated into the workflow without compromising medication safety or overwhelming staff. Adopting a “blank slate” approach through field observations allows for an unbiased, system-level depiction of pharmacy work, ensuring that future interventions fit naturally rather than being retrofitted within existing workflows.

Methods

The Macroergonomic Analysis and Design (MEAD) 10-Step Process, a systematic iterative method for analyzing and optimizing complex work systems, was adapted to a three-phase, observation protocol in six community pharmacies. Observation 1 captured a systems-level view of the entire pharmacy environment, including staff roles, tools and technology, and environmental conditions. Observation 2 detailed the prescription dispensing process by documenting task sequences, interruptions, and role delegation. Observation 3 examined non-dispensing services (such as medication management) to understand how these activities are performed concurrently with dispensing work. Between observation cycles, analytical pauses were used to refine data collection and identify disruptions in the workflow.

Findings

The iterative observation process produced a detailed depiction of the current pharmacy workflow and identified bottlenecks to inform future intervention design. Across all sites, prescription dispensing was a primary, constant workflow. Non-dispensing services were routinely layered onto the dispensing workflow without formal integration or task delegation, which created role ambiguity, documentation challenges, and cognitive burden for pharmacy staff. By systematically observing the sequence of tasks and interruptions, the observation cycles captured how staff navigated competing responsibilities in real time.

Implications for D&I Research

This study demonstrates how taking a “blank slate approach” and conducting iterative observations can guide the design of sustainable interventions. Generating a comprehensive understanding of the work system without preconceived notions promotes contextual fit, reduces cognitive burden, and improves the likelihood of adoption and long-term sustainment in healthcare settings.

Primary Funding Source: Agency for Healthcare Research and Quality

S142

Human-centered redesign of evidence-based interventions and implementation strategies to enhance usability: cross-project outcomes from the UW ALACRITY center

Aaron R. Lyon¹, Sean A. Munson², Kathryn Bruzios², Katie P. Osterhage², Michael D. Pullmann², Ryan Allred², Brittany Mosser³, Alejandra Lopez², John Fortney⁴, Brittany Blanchard², Tricia Aung², Doyanne Darnell², Patricia Areán³, UW Alacrity Researchers³

¹Psychiatry and Behavioral Sciences, University of Washington, Seattle, WA, USA; ²University of Washington, Seattle, WA, USA; ³University of Washington, Seattle, USA; ⁴Veterans Health Administration, Seattle, WA, USA

Correspondence: Aaron R. Lyon (lyona@uw.edu)

Implementation Science 2026, 21(2):S142

Background: Implementation strategies and patient-facing interventions both tend to be complex psychosocial processes. When these innovations are not readily usable or aligned with their destination contexts, the potential for communities and the field to benefit from advances in both intervention and implementation science is significantly reduced. The NIMH-funded University of Washington ALACRITY Center (UWAC) applies human-centered design (HCD) methods to adapt implementation strategies and mental health interventions to ensure their usability in community settings. UWAC leverages the three-phase Discover, Design/Build, Test (DDBT) process to drive iterative redesign based on information about innovation usability and

contextual constraints. Using data farate UWAC projects this presentation will describe the types of redesign solutions made to implementation strategies and interventions to improve their usability and adoption potential.

Methods: All 11 projects employed user testing and evaluation methods (e.g., usability testing, cognitive walkthroughs, user interviews) to identify usability issues, which served as modification targets for implementation strategy and intervention redesign. Redesign solutions were articulated by project teams using the Framework for Reporting Adaptations and Modifications – Expanded (FRAME) for evidence-based interventions and its extension for implementation strategies (FRAME-IS).

Findings: Of the projects, eight sought redesign solutions for a patient-facing intervention, two sought redesign solutions for an implementation strategy, and two projects sought redesign solutions for both an intervention and an implementation strategy. Projects reported usability issues across 12 categories which were linked to 15 types of content modifications. Redesign most often included the FRAME category of *tailoring or refining intervention/strategy content*. Of the 11 projects, four identified usability issues that were not able to be addressed in a redesign. These issues were highly variable but some of the most common issues left unaddressed were categorized as inadequate scaffolding for intervention/strategy recipients or that the intervention/strategy did not build adequate buy-in by demonstrating its value to its intended users.

Implications for D&I Research: Integration of data across 11 UWAC projects allowed for identification of common redesign solutions. These findings can inform future intervention and implementation strategy development and redesign to advance the science of adaptation.

Primary Funding Source: National Institutes of Health

S143

From partnership to practice: community-informed strategies for sustainable preconception counseling implementation

Angie Suarez¹, Calvin Foster², Oluyemi Farinu¹, Adenike McDonald¹, Madison Haiman³, Morehouse Medicine¹, Tiyaika Williams¹, Jaydon Taylor¹, Amy Huebschmann⁴, Meredith Fort⁵, Danette McLaurin-Glass⁶, Rasheeta Chandler⁷, Rasheeta Chandler⁷, Cheryl Franklin¹, Latrice Rollins¹, Natalie Hernandez-Green¹

¹Morehouse School of Medicine, Atlanta, GA, USA; ²Project IMPACT Implementer, Greensboro, NC, USA; ³University of Texas at Austin, Austin, TX, USA; ⁴University of Colorado School of Medicine, General Internal Medicine, General Internal Medicine, Aurora, CO, USA; ⁵Colorado School of Public Health, Aurora, CO, USA; ⁶First Team America, Charlotte, NC, USA; ⁷Emory University, Atlanta, GA, USA

Correspondence: Oluyemi Farinu (ofarinu@msm.edu)

Implementation Science 2026, 21(2):S143

Background: The underutilization of preconception counseling (PC) represents a missed opportunity to reduce high rates of maternal morbidity and mortality in the Southeastern U.S. Recent policy shifts have reduced funding for family planning and PC resources, disproportionately affecting communities. In this evolving political landscape, adapting evidence-based health interventions and developing implementation strategies through a community-engaged lens is essential. The objective of Project IMPACT (IMproving Preconception Actions and Choices for Tomorrow) is to utilize the Practical Robust Implementation and Sustainability Model (PRISM) and the Implementation Research Logic Model (IRLM) tool alongside partner community-based organizations (CBO) to guide the collaborative development of an implementation strategy package for PC delivery.

Methods: Project IMPACT is a joint effort between academic and partner CBOs testing the delivery of a PC intervention within community-based settings as a type II hybrid effectiveness trial. The study team developed a preliminary implementation strategy package in an IRLM developed from a formative mixed methods assessment of PRISM

determinants that also identified Screening, Brief Intervention and Referral to Treatment (SBIRT) as the community's preferred PC intervention to deliver. We developed a regional accountability board (RAB) of community stakeholders that met quarterly to further refine the implementation strategies.

Findings: The final IRLM co-created with the RAB and site implementer input included 3 overarching themes for implementation strategies: 1) to ensure the SBIRT intervention was reflective of the PRISM community perspectives and needs/characteristics (Engage participants), 2) include a strong standard of operations (SOP) and other implementation materials (Technical assistance), and 3) implementer capacity-building. Through collaborative engagement with partner CBOs on implementation strategies, the Project IMPACT SOP and other materials were further iteratively refined using insights gathered from annual strategy sessions, RAB meetings, and researcher-implementer meetings.

Implications for D&I Research: Our findings reflect the innovative use of both PRISM and the IRLM to guide our community-based approach to allow for novel and feasible strategies for delivering PC through Project IMPACT. This approach has yielded culturally relevant, sustainable solutions that we expect will lead to translational benefits to Southeastern communities with a high burden of maternal mortality: improvements in PC access, maternal health outcomes and strengthened community resilience.

Primary Funding Source: National Institutes of Health

S144

Utilizing Artificial Intelligence (AI) rapid qualitative methods for implementation of Veterans Health Administration National AI Institute initiatives

Cassie Cunningham Goedken^{1,2}, David Hook², Donna Hill², Jessica Holman^{2,3}, Daniel Morgan^{4,5}, Katlyn Brinkley², Evan Carey^{2,6}, Westyn Branch-Elliman^{2,7}

¹Veterans Health Administration, Iowa City, IA, USA; ²Veterans Health Administration, Washington DC, USA; ³Veterans Health Administration, Seattle, WA, USA; ⁴Veterans Health Administration, Baltimore, MD, USA; ⁵University of Maryland, Baltimore, MD, USA; ⁶Veterans Health Administration, Denver, CO, USA; ⁷Veterans Health Administration, Los Angeles, CA, USA

Correspondence: Cassie Cunningham Goedken (cassie.goedken@va.gov)
Implementation Science 2026, **21(2)**:S144

Background: Qualitative methods gather information to better understand contextual factors and perceptions influencing implementing interventions. While these methods are traditionally resource intensive, rapid qualitative methods have been successfully utilized to efficiently analyze qualitative data. Artificial intelligence (AI) has increasingly been used in health care settings for text summarization. Generative AI, which creates output based on user supplied inputs, can increase the efficiency of qualitative work. The United States Department of Veterans Affairs (VA) developed VA GPT(Beta); a generative AI tool housed behind the VA firewall to use with sensitive data. We used VA GPT(Beta) to assist in rapid qualitative analysis of data for two VA National AI Institute (NAII) initiatives.

Methods: NAII supports operationalizing AI technologies to improve Veteran health outcomes and alleviate workload burden. Rapid qualitative evaluation of initiatives is critical to inform leadership decisions. From April–May 2025, we implemented novel rapid AI-assisted qualitative analysis methods to evaluate two independent AI-focused pilots: Pilot1-VAGPT(Beta), Pilot2-CLABSI AI detection. Evaluation methods included structured interviews (pilot1) and surveys (pilot2). Interviews ($n=12$) were recorded and transcribed using MS Teams integrated functionality for pilot1. Surveys ($n=20$) were sent post-pilot via MS Forms, comprised of closed and open-ended questions for pilot2. AI-assisted rapid analysis for both pilots involved: 1) cleaning and formatting participant open-ended survey/interview responses into a raw dataset, 2) selecting and refining prompts, 3) inputting interview/MS form questions, tailored prompts, and participant raw data into VA

GPT (Beta), 4) human validation of AI-generated output against the raw dataset.

Findings: VA GPT(Beta) was successfully integrated into rapid qualitative analyses, identifying key themes, providing insights and producing effective summaries of open-ended survey responses and qualitative interviews. This novel method delivered timely outputs to enable data-informed decision making.

Implications for D&I Research: AI-assisted qualitative research methods may offer greater efficiency than traditional approaches. We found this method to be successful at organizing, categorizing, and summarizing qualitative responses. However, trained qualitative researchers and content experts are needed to provide context and identify nuances within analyses to ensure AI output accuracy. Future research should explore refining prompts and AI's use for in-depth thematic qualitative analysis.

Primary Funding Source: Department of Veterans Affairs

S145

Documenting and troubleshooting challenges to experimental trial designs in implementation science

Cathleen Willging¹, Daniel Shattuck¹, Marisa Sklar², Erika Crable³, Joella Friedman⁴, Mark Ehrhart⁵, Manon Ironside⁴, Alexandra Kandah⁶, Gregory Arons⁴

¹Pacific Institute for Research and Evaluation, Albuquerque, NM, USA; ²University of California, San Diego, La Jolla, CA, USA; ³University of California, Los Angeles, Los Angeles, CA, USA; ⁴University of California San Diego, La Jolla, CA, USA; ⁵Implementation Science and Team Effectiveness in Practice (IN STEP) Children's Mental Health Research Center, San Diego, USA; ⁶University of Central Florida, Orlando, FL, USA

Correspondence: Cathleen Willging (cwillging@pire.org)
Implementation Science 2026, **21(2)**:S145

Background

Experimental trial designs are favored in implementation science because they provide a rigorous structure for generating credible evidence about both the outcomes of interventions and the processes that influence their adoption, fidelity, and sustainment. This qualitative study explores the perspectives and responses of researchers to multilevel factors affecting the use of a complex stepped-wedge trial that tested the Leadership and Organizational Change for Implementation – System Level (LOCI-SL) strategy to facilitate the adoption of Motivational Interviewing (MI) and a natural language processing artificial intelligence (NLP/AI) fidelity feedback platform to promote positive implementation outcomes in substance use disorder treatment.

Methods

We conducted monthly periodic reflections and weekly ethnographic observations involving ten research team members who participated in the trial and implemented LOCI-SL over a three-year period across 33 clinics within a single state's behavioral healthcare system in the Western United States. Observational data included detailed field notes, and periodic reflections were audio-recorded and transcribed. We analyzed these qualitative data iteratively using open and focused coding techniques, along with NVivo software, to identify patterns and themes related to implementing and maintaining the integrity of the stepped-wedge design.

Findings

Recurring factors influencing the stepped-wedge trial comprised five main themes: (1) site recruitment challenges; (2) variable buy-in and adoption among implementers for the EBP and the NLP/AI platform; (3) turnover and internal transitions within sites and the research team; (4) inconsistent participation in quantitative and qualitative data collection throughout the multiyear study; and (5) the nuances of maintaining multilevel communication and coordination across sites and system partners. These themes highlight a range of interrelated factors that research teams may need to troubleshoot when initially engaging and then sustaining both site and implementer involvement in a large-scale trial.

Implications for D&I Research

Multisite experimental trials are regarded as the gold standard for evaluating implementation outcomes in real-world settings. However, they require flexibility and dedication from research teams to succeed. This trial's multilevel focus on testing the LOCI-SL strategy to implement both MI and an NLP/AI technology platform elucidates the struggles that researchers encounter and their potential solutions when applying these demanding implementation trial designs.

Primary Funding Source: National Institutes of Health

S146

Focusing qualitative inquiries through a PRISM lens to sharpen the rigor of implementation research

Emily Dunston^{1,2,3}, Amy Huebschmann^{1,3,4,5}, Katy Trinkley⁶, Samantha Harden⁷, James Pittman^{8,9}, Rebecca Guerin¹⁰, Marcia Ory^{11,12}, Emily Treichler^{8,13}, Paul Estabrooks¹⁴, Russell Glasgow^{3,6}, Borsika Rabin^{8,15}
¹University of Colorado School of Medicine, Department of Medicine, Division of General Internal Medicine, Aurora, CO, USA; ²University of Colorado Cancer Center, Aurora, CO, USA; ³Adult and Child Center for Outcomes Research and Delivery Science (ACCORDS), Aurora, CO, USA; ⁴Ludeman Family Center for Women's Health Research, Aurora, CO, USA; ⁵University of Colorado School of Medicine, General Internal Medicine, General Internal Medicine, Aurora, CO, USA; ⁶University of Colorado Anschutz Medical Campus, Aurora, CO, USA; ⁷Virginia Tech, Blacksburg, VA, USA; ⁸University of California San Diego, San Diego, CA, USA; ⁹VA Center of Excellence for Stress and Mental Health, San Diego, CA, USA; ¹⁰Centers for Disease Control and Prevention, Cincinnati, OH, USA; ¹¹Department of Environmental and Occupational Health, School of Public Health, Texas A&M University, College Station, TX, USA; ¹²Center for Population Health and Aging, Texas A&M University, College Station, TX, USA; ¹³VA San Diego, San Diego, USA; ¹⁴University of Utah, Salt Lake City, UT, USA; ¹⁵University of California San Diego, Altman Clinical and Translational Research Center, San Diego, CA, USA

Correspondence: Emily Dunston (emily.dunston@cuanschutz.edu)
Implementation Science 2026, **21**(2):S146

Background: Qualitative data collection is an important component of dissemination and implementation (D&I) science, yet some view qualitative research as less rigorous than quantitative research. The application of a D&I theory, model, or framework (TMF) can enhance the rigor of qualitative methods for D&I research by providing systematic guidance. The Pragmatic, Robust Implementation and Sustainability Model (PRISM) is well-positioned for use in qualitative D&I research because it facilitates the assessment of multi-level contextual factors and dynamically connects outcomes with context. While some TMFs have qualitative guidance, there is a notable lack of formal guidance on how to apply PRISM for qualitative research.

Methods: We convened a group of D&I scientists ($n = 10$) with PRISM and qualitative research expertise to develop a set of general considerations for qualitative data collection and analysis. First, the expert group held a round-table discussion on the use of PRISM for qualitative research, informed by recent reviews of PRISM. Further, each expert shared qualitative interview and focus group guides using PRISM to develop a library of qualitative data collection instruments. From this library, we compiled each individual question and its attendant PRISM domains into a database.

Findings: The expert panel roundtable outlined four key ways PRISM can be used to inform qualitative D&I research: 1) defining research questions, 2) informing interview and focus group guides, 3) conducting qualitative data analysis, and 4) developing intervention and implementation strategies to address multi-level contextual determinants. Additionally, the panel emphasized provision of guidance for use of PRISM across study phases including exploration, development, planning, implementation, and sustainment. We collected 29 interview and focus group guides that utilized PRISM from 13 distinct research studies, cataloged example interview items by PRISM domain, and developed example open-access interview guides and coding templates to support implementers and researchers.

Implications for D&I Research: Robust guidance for utilizing PRISM in qualitative research will significantly enhance the rigor of contextual inquiry within D&I science. The example interview guides and coding templates developed to date will support a more systematic approach to understanding contextual factors influencing PRISM outcomes such as reach, adoption and maintenance, thereby accelerating the translation of evidence-based practices into real-world settings.

S147

Developing analytic profiles for multi-site trials: layering multi-method qualitative data to identify contextual factors impacting longitudinal implementation

Lauren Hoffmann¹, Eleni Skaperdas¹, Maharshi Rawal¹, Talia Panadero¹, Erica Fletcher¹, Matthew McCoy², Agatha Palma¹, Taylor Harris³, Jenny Barnard¹, Sonya Gabrielian¹, Kristina Cordasco⁴, Erin Finley⁵
¹Veterans Health Administration, Los Angeles, CA, USA; ²Veterans Health Administration, Los Angeles, USA; ³Veterans Health Administration, North Hills, CA, USA; ⁴David Geffen School of Medicine, University of California, Los Angeles, CA, USA; ⁵Long School of Medicine, University of Texas Health Science Center, San Antonio, TX, USA

Correspondence: Lauren Hoffmann (Lauren.Hoffmann@va.gov)
Implementation Science 2026, **21**(2):S147

Background

Although qualitative data is often used to assess contextual factors impacting implementation of evidence-based practices (EBPs), single-method study designs may be inadequate to capture shifting conditions over time, particularly in under-resourced settings with high staff turnover. In a hybrid type 3 implementation effectiveness study of Critical Time Intervention (CTI)—a time-limited, case management EBP—we developed site profiles to layer multi-method data across multiple time points to understand how shifting contextual factors at 32 community-based homeless service agencies affected CTI implementation.

Methods

Over three years, we conducted 161 interviews with case managers and supervisors across 32 agencies, and VA social workers ("liaisons") who coordinate activities between the agencies and their local VA facilities. Interviews were conducted at baseline (pre-implementation), 12 months, and 18 months after CTI implementation. Additionally, we gathered periodic reflections (brief, ethnographic snapshots of implementation activities) from VA liaisons at 3-, 6-, 9-, 12-, and 18-months ($n = 92$) post-implementation. Drawing on rapid qualitative analytic methods (i.e., structured interview and reflection summaries used to conduct matrix analyses) and informed by the updated Consolidated Framework for Implementation Research (CFIR), we integrated interview and reflections data across sources and timepoints to develop a longitudinal "site profile" of each agency.

Findings

Site profiles were templated as tables, with rows ascending chronologically from pre-implementation through 18 months, capturing data from all respondents summarized into five columns reflecting updated CFIR domains: Innovation, Outer Setting, Inner Setting, Characteristics of Individuals, and Implementation Process. Interview and reflection data were viewable by domain sequentially from pre- to post-implementation, highlighting changes in staff, infrastructure, and policies that reflect the dynamic nature of site-level implementation. Each profile distilled ≥ 15 data sources collected over 18 months into an accessible resource facilitating cross-site analysis and comparison.

Implications for D&I Research

Understanding contextual factors that impact implementation in multi-site trials can enhance the scalability and sustainability of evidence-based practices in under-resourced settings. We leveraged multi-method qualitative data across multiple time points, sources, and perspectives to develop pragmatic and analytically concentrated site profiles. Future implementation efforts may consider similar approaches to capture variability in context over time, particularly in large longitudinal trials in dynamic environments.

Primary Funding Source: Department of Veterans Affairs

Prevention and public health

S148

Strategies to improve the reach of evidence-based lifestyle interventions: Expanded insights from a scoping review

Paul Estabrooks

University of Utah, Salt Lake City, UT, USA

Correspondence: Paul Estabrooks (paul.estabrooks@health.utah.edu)

Implementation Science 2026, 21(2):S148

Background: Improving the reach of evidence-based interventions is foundational to dissemination science and achieving population level impact. Indeed, evidence-based lifestyle interventions that often struggle with reach when implemented in typical community or clinical settings. This presentation will examine the utility of reach strategies to increase the individual-level dissemination of lifestyle interventions.

Methods: Using JBI and PRISMA-ScR guidance we identified 98 studies reporting a strategy designed to improve reach and at least one reach outcome measure (i.e., number, proportion, or representativeness). Strategies were inductively grouped into 15 categories by the research team and participation rate, representativeness, and information on the actor, action, channel, dose, timing, theory, mechanism, and cost of strategies were extracted. Exploratory cross-tabulations examined reach of evidence-based interventions that differed in characteristics that could influence participation.

Findings: Only 8/98 studies (8%) used randomized comparisons, and one-third (32%) bundled multiple strategies without reporting unique reach outcomes. Major reporting gaps included timing/dose (50% unreported), theory (93%), mechanism (95%), and cost (87%). Denominators for eligible and exposed groups were inconsistently defined, limiting comparability. Participation rates varied widely by strategy type (median 3%–41%), with population health management, place-based, and opt-out approaches often highest when denominators were clear. Exploratory cross tabulations suggest that when the reach strategy is tied to shorter, less intensive lifestyle interventions, participation rates were higher based on hours of contact (≤ 10 h = 35% vs > 20 h = 13%), length of the program (≤ 3 months = 30% vs > 12 months = 12%), number of contacts, (0–4 contacts = 37% vs > 12 contacts = 17%), and length of sessions (≤ 30 min = 39% vs 31–60 min = 19%). Differences by setting, format, and delivery channel were modest but variable. Finally, representativeness for subgroups (e.g., rurality, language, insurance status) was reported in fewer than one-third of studies.

Implications for D&I Research: Our results suggest that advancing individual-level dissemination science depends on three priorities. First, researchers should systematically report denominators for who is eligible and exposed to each reach strategy. Second, strategies must be clearly specified (actor, action, dose, timing, channel, mechanism) to enable replication and comparison. Third, more experimental and hybrid studies are needed to directly test strategies and establish causal effects.

Primary Funding Source: Margolis Foundation

S149

Multi-level strategies for increasing reach of evidence-based tobacco cessation at FQHCs

Chelsey Schlechter

University of Utah School of Medicine, Salt Lake City, UT, USA

Correspondence: Chelsey Schlechter (chelsey.schlechter@hci.utah.edu)

Implementation Science 2026, 21(2):S149

Background: Tobacco use remains the leading cause of death in the USA and is disproportionately concentrated among low socioeconomic status (SES) populations. Community Health Centers (CHCs) are a key venue for reaching low SES populations with evidence-based tobacco cessation treatment such as Quitlines. Electronic health record

(EHR)-based strategies at the point-of-care, text messaging (TM), and phone counseling have the potential to increase Quitline reach.

Methods: QuitSMART Utah was a multilevel, two-phase SMART trial conducted with CHCs ($n = 10$ systems; 32 clinics) and CHC patients ($n = 11,605$). All clinics implemented Ask – Advise – Connect (AAC) to assess patient tobacco use, advise patients to quit, and directly and electronically connect patients to the Quitline. Patients who enrolled in the Quitline in response to AAC received no further strategies. Among individuals who did not enroll in the Quitline in response to AAC, phases one and two were patient-level randomizations. In phase one, patients who were non-responders were randomized to receive either text messaging (TM) or the clinic only (CO) AAC strategy. In phase two, patients in the TM condition who were non-responders in Phase one (i.e., did not enroll in Quitline) were randomized receive either continued-TM (TM-Cont) or TM + Motivation And Problem Solving (MAPS) phone coaching (TM + MAPS).

Findings: Patients were 54% male, 28% Latino, 18% spoke Spanish as preferred language, and 35% were uninsured. Sixty-three of the 11,605 QuitSMART Utah patients enrolled in the Quitline following AAC. 11,542 patients were randomized in Phase 1 to TM or CO. At 6 months, patients in TM were significantly more likely to enroll in the Quitline compared to CO (2.0% in TM vs 0.7% in CO; OR = 3.09; 97.5% CI = 1.77, 5.98; $P < 0.001$). In Phase 2, 6,604 patients were randomized to TM + MAPS or TM-Cont. At 12 months, patients in TM + MAPS were significantly more likely to enroll in the Quitline compared to TM-Cont (2.2% TM + MAPS vs 0.9% TM-Cont; OR = 2.54; 97.5% CI = 1.57, 4.26; $P < 0.001$).

Implications for D&I Research: Strategies such as proactive text messaging and phone coaching are effective at increasing reach of evidence-based tobacco cessation treatment among diverse and low SES patients seen at CHCs.

Primary Funding Source: National Institutes of Health

S150

SCALE-UP II: pragmatic randomized trial examining population health management interventions to increase reach of at-home COVID-19 testing among Community Health Center patients

David Wetter

University of Utah, Salt Lake City, UT, USA

Correspondence: David Wetter (david.wetter@hci.utah.edu)

Implementation Science 2026, 21(2):S150

Background: Racial/ethnic minorities, low socioeconomic status (SES), and rural populations experienced lower COVID-19 vaccination rates, higher hospitalization rates, and higher mortality. While the availability of at-home tests provides a convenient and rapid option for self-testing, significant disparities persist in the reach of at-home testing among populations such as those served by Community Health Centers (CHCs). Scalable strategies are needed to increase the reach of at-home COVID-19 testing among CHC patients.

Methods: SCALE-UP II was a multi-site, randomized pragmatic clinical trial. Eligible patients were ≥ 18 years old, with a primary care visit in the last three years, and valid cellphone in the CHC electronic health record. Participants who self-reported that they had a smartphone were randomized in a $2 \times 2 \times 2$ factorial (Smartphone study) to receive (i) Chatbot or Text Messaging; (ii) option to request PN (yes or no); and (iii) intervention frequency every 10 or 30 days. All other participants were randomized in a 2×2 factorial (Non-Smartphone study) to receive (i) option to request PN (yes or no); and (ii) intervention frequency every 10 or 30 days. The primary outcome was the proportion of participants requesting at-home COVID-19 tests by 90-days.

Findings: The trial enrolled 2,117 in the Smartphone study and 31,439 in the Non-Smartphone study. Overall, 2,341 out of 33,556 (7.0%) participants requested a test kit. In the Smartphone study, the proportion of participants who requested test kits in the Chatbot arm was lower than in TM (16.6% vs. 52.1%, aRR = 0.317 [98.33% CI 0.27–0.38], $P < 0.0001$). In the Non-Smartphone study, the proportion of participants who requested test kits was higher if messaged every 10 days

rather than every 30 days (5.5% vs 4.8%, aRR = 1.144 [97.5% CI 1.03–1.28], $P = 0.0055$) and lower for participants who could request access to PN compared to those in No PN (4.3% vs 5.9%, aRR = 0.729 [97.5% CI 0.65–0.81], $P < 0.0001$).

Implications for D&I Research: Simple bidirectional text messaging every 10 days was effective at increasing the reach of COVID-19 testing. Text messaging is a low cost, scalable strategy that is effective at increasing reach of at-home COVID-19 testing.

Primary Funding Source: National Institutes of Health

S151

Evaluation of reach strategies in real world settings: impact of agency-selected reach strategies Jennie Hill

University of Utah, Salt Lake City, UT, USA

Correspondence: Jennie Hill (jennie.hill@hsc.utah.edu)

Implementation Science 2026, 21(2):S151

Background: The Supplemental Nutrition Program for Women, Infants, and Children (WIC) provides nutritious supplemental food, nutrition education, breastfeeding support, and referrals to health and community resources for pregnant, postpartum, and breastfeeding women, infants, and children up to age five. Despite a robust body of evidence that demonstrates WIC improves participant outcomes, WIC remains underutilized, with only half of eligible individuals participating. To increase reach—defined as the number, proportion, and representativeness of those who enroll and are retained—community-driven strategies are needed. Projects funded within a WIC innovation-initiative presents a unique opportunity to evaluate strategies developed by state and local WIC agencies to increase reach and retention.

Methods: REAIM and implementation strategy taxonomies (i.e., ERIC) were used to specify, track, and evaluate reach strategies within a type III hybrid effectiveness-implementation (HEI) evaluation. Data sources included a retrospective review of application materials submitted by 36 innovation projects and prospective reporting of reach outcomes during implementation. Each workplan was analyzed using framework-aligned protocols, codebooks, and tools to identify 1) reach to individuals—including current WIC participants and eligible non-participants—and 2) penetration to organizations through new or strengthened outreach partnerships.

Findings: A total of 1,015 non-unique implementation strategies were identified across work plans. Most focused on developing/distributing educational materials, hosting outreach events, and establishing referral pathways. Reach strategy categories engaging priority populations ($n = 347$), training/educating collaborators ($n = 180$), changing infrastructure ($n = 144$), developing partner relationships ($n = 130$), supporting delivery agents ($n = 83$), tailoring to context ($n = 25$), evaluation ($n = 28$), and providing financial incentives ($n = 5$). Engaging priority populations and building partner relationships were foundational strategies as demonstrated by active referral pathways and personalized follow-up yielding > 15% enrollment versus 2–8% from mass media. All subgrantees reported at least one reach or penetration outcome (median = 4), yet denominators were inconsistently defined making some comparisons challenging.

Implications for D&I Research: Given the scarcity of pragmatic, non-randomized evaluations suited to existing community evidence-based interventions such as WIC, our findings demonstrate how multilevel, partner-co-designed approaches can identify “what works” when conventional randomized trials are not feasible. Tailoring strategies to local context and engaging system partners therefore shows particular promise for improving WIC’s reach.

Primary Funding Source: USDA Food & Nutrition Services

S152

Building trust through community-informed dissemination: a roadmap for connecting science and community

Kristi Lambert¹, Sharon Sanders^{2,3}, Anna Huff Davis^{2,3}, Taren Massey-Swindle^{2,3}

¹Community Member, Little Rock, AR, USA; ²University of Arkansas for Medical Sciences, Little Rock, AR, USA; ³Arkansas Children’s Research Institute, Little Rock, AR, USA

Correspondence: Taren Massey-Swindle (tswindle@uams.edu)

Implementation Science 2026, 21(2):S152

Background: While implementation strategies have been developed and tested extensively, dissemination strategies, particularly those grounded in community perspectives, remain underexplored. This gap limits research impact and weakens trust between communities and researchers. This project sought to understand community preferences for receiving research-based information and to develop a roadmap to guide community-centered dissemination.

Methods: The LINK (Leveraging Implementation and eNgagement for Knowledge impact) is a service core that connects science with communities, aiming to support community-engaged implementation and enhance dissemination as a means of building trust and engagement. The LINK Community Advisory Board (CAB) co-designed and piloted a guide for community listening sessions. A CAB member of The LINK led 7 listening sessions with 60 community members across a Southern U.S. state. Participants responded to questions regarding perceived barriers to engagement in research and preferred formats for receiving research information. Using directed content analysis informed by Bauman et al. (2022), we coded transcripts across the domains of dissemination: (1) Processes, (2) Determinants, (3) Strategies, and (4) desired Outcomes. Results informed the development of a structured Dissemination Roadmap for The LINK and its partnering researchers, designed to support planning and reflection.

Findings: Based on community input, the Roadmap Process begins with relationship-building and continues through co-creation, feedback, and ongoing evaluation. Key Determinants included trust, relevance, clear language, and structural supports (e.g., transportation, childcare). Preferred Strategies involved in-person updates, research briefs, digital tools (e.g., newsletters, resource hubs, social media), and capacity-building sessions to apply findings. Desired outcomes ranged from increased research awareness and use to long-term improvements in child health and community-led research. Planned products include a podcast platform, educational events, plain-language materials (e.g., infographics), and an online hub for access and communication.

Implications for D&I Research: This study provides a replicable, community-informed model for improving dissemination based on community preferences. By operationalizing community perspectives into a roadmap, this work can support future empirical testing of dissemination strategies and the broader goal of building sustainable research-community partnerships. This model can be adapted by dissemination and implementation or community engagement cores to improve the relevance, reach, and use of research findings.

Primary Funding Source: Arkansas Biosciences Institute

S153

Engaging community-driven coalitions to improve uptake of prevention and public health interventions

Ruth Rechis¹, Katherine Oestman², Haley Gardiner¹, Travis Anthony¹, Jacqueline Dan-Jumbo¹, Stephanie Nutt¹

¹The University of Texas M.D. Anderson Cancer Center, Houston, TX, USA;

²Daybreak Consulting, Las Vegas, NV, USA

Correspondence: Katherine Oestman (katy@daybreakway.com)

Implementation Science 2026, 21(2):S153

Background

Multi-sector community-wide interventions are effective in addressing numerous health outcomes especially when guided by a community-driven coalition. Coalitions ensure community needs are integrated into evidence-based intervention (EBI) implementation. While many coalitions exist, they do not always sustain or result in improved uptake of prevention and public health interventions. Be Well Communities™ is MD Anderson's place-based strategy for cancer prevention and control, working with communities to reduce modifiable cancer risk factors. Be Well Communities works with community and clinical practice settings to fund, deliver and evaluate EBIs. Over 9 years, across three communities, we have developed a model for creating and evaluating coalitions that guide EBI implementation and improve health outcomes.

Methods: Annually, 50+ organizations participated across communities in a survey assessing the initiatives goals, effectiveness, outcomes, and sustainability. Survey responses were aggregated across communities, and descriptive statistics were calculated at baseline and endline. Coalition members also report annually on implementation and sustainability. Coalition effectiveness and EBI implementation are analyzed as part of the overall evaluation methodology which utilizes the RE-AIM framework.

Findings: Survey results related to coalition effectiveness include the following:

- 95% agreed that the MD Anderson team is trusted by all organizations at baseline and 96% agreed at endline
- 88% agreed that coalition members support each other to carry out programs effectively at baseline and 91% agreed at endline
- 84% indicated confidence in long term sustainability of the community collaborations and 95% indicated confidence at endline.

Since 2017, 45 EBIs have been implemented, reaching over 111,000 people improving health outcomes related to obesity, cancer screening, and vaccination. Notably, one year post funding, 95% of interventions are sustained.

Implications for D&I Research

Understanding how to develop, manage, and evaluate effective coalitions can inform integrating EBIs into a multitude of clinical and community settings. This approach and methodology can inform other D&I efforts seeking to sustainably implement EBIs aimed at improving population health outcomes. Further, the components of this coalition survey and the evaluation strategy can be utilized by others to inform dissemination and implementation processes focused on how the context of relationships impact results.

Primary Funding Source: MD Anderson Cancer Center

S154

Integrating preferences from primary caregivers and community partners for a multicomponent lifestyle intervention to address childhood obesity and cardiometabolic risk in Samoa

Courtney Choy¹, Rochelle Rosen², Christina Soti-Ulberg³, Maria Arorae⁴, Vaimoana Filipo⁴, Kima Savusa⁴, Nicola Hawley¹, Mona Sharifi⁵, Siufaga Simi³

¹Yale School of Public Health, New Haven, CT, USA; ²The Miriam Hospital, Providence, RI, USA; ³Ministry of Health, Apia, Samoa; ⁴Samoa Obesity, Lifestyle and Genetic Adaptations Study (OLaGA) Group, Apia, Samoa; ⁵Yale University School of Medicine, New Haven, CT, USA

Correspondence: Courtney Choy (courtney.choy@yale.edu)

Implementation Science 2026, 21(2):S154

Background: Preventative interventions are critical to reduce cardiometabolic diseases (CMD). In Samoa, childhood obesity prevalence has doubled over the past decade, accompanied by rising risk factors. Interventions have been slow to reach Pacific communities, with knowledge gaps about optimal design. We aimed to understand community preferences for the development of an intervention to address CMDs among Samoan children.

Methods: From 2024–25, we purposively sampled Samoan primary caregivers who were potential end users and key stakeholders involved with child programs in Samoa. Focus group discussions and interview agendas were guided by the Active Implementation Framework, using a Hexagon tool to explore options. Participants assessed six contextual fit and feasibility indices (need, evidence, fit, usability, capacity, and support for implementation) for common components of existing interventions identified in a scoping review. Using a Framework Matrix Analysis, we charted qualitative data and summarized preferences for preferences in intervention age group, setting, implementers, delivery mode, duration, components, and goals.

Findings: We conducted eight focus groups ($n = 74$ female primary caregivers, median age: 42 years, IQR: 36–49) and 19 interviews (84% female stakeholders, range: 5–40 years of work experience). Many participants endorsed the feasibility of implementing an intervention before the age of 9. They suggested leveraging existing community assets and collective mindset to protect the next generation. Participants preferred an intervention design and delivery that includes families, offers both group and individual in-person sessions bi-weekly for up to six months, and focuses on nutrition, physical activity, sleep, and mental wellness. A team approach and holistic programs that focus on multiple aspects of a child's life to support their physical, mental, spiritual, and emotional health were emphasized. These insights directly informed a conceptual model (key components and processes to support healthful behaviors with family involvement) for an intervention to be pilot tested in Samoa.

Implications for D&I Research: Engaging primary caregivers and key stakeholders to share feedback on existing components can help to refine intervention design within the context of communities and enhance the likelihood for high feasibility and acceptability. D&I science facilitates the translation of community feedback into action for testing interventions, which can lead to improved implementation and health outcomes.

Primary Funding Source: National Institutes of Health

S155

The EPICC implementation capacity survey: identifying challenges for implementing evidence-based health promotion programs in 30 African American congregations

Jemal Gishe¹, David Schlundt², Leah Alexander³, Sharon Jones², Rebecca Selove¹

¹Tennessee State University, Nashville, TN, USA; ²Vanderbilt University, Nashville, TN, USA; ³Meharry Medical College, Nashville, TN, USA

Correspondence: Jemal Gishe (jgishe@tnstate.edu)

Implementation Science 2026, 21(2):S155

Background

Implementing evidence-based health promotion programs (EBPs) in community settings is an effective approach to preventing chronic disease and reducing health disparities. Community organizations offer benefits of proximity and trust for engaging individuals in EBPs to change behavior. The Engaging Partners in Caring Communities (EPICC) Project was developed to provide technical assistance (TA) to churches serving predominantly Black communities to enhance their capacity to implement one of six EBPs identified by EPICC researchers. The EPICC survey was designed using the Consolidated Framework for Implementation Research (CFIR) as a tool for assessing implementation capacity and identifying congregations' strengths as well as potential challenges they might encounter in implementing the EBP they selected.

Methods:

Teams from 30 African American congregations completed the online EPICC survey. Automated scoring identified potential implementation challenges across CFIR constructs. Each team received an individualized report which was discussed in a recorded feedback interview. Descriptive statistics summarized the findings for the research team.

Findings

Congregation sizes ranged from 18 to 1,000 members (median 62.5, IQR 126.3; mean 136.7, SD 196.9). The number of challenges per congregation (out of 31 CFIR constructs) ranged from 1 to 19 (median 4.5, IQR 4; mean 5.6, SD 4). The most common challenges were cost ($n=28$), design quality and packaging ($n=24$), and readiness for implementation ($n=13$). Based on number of challenges identified in survey responses, congregations were categorized as beginner ($n=4$), intermediate ($n=16$), or advanced ($n=10$) in implementation capacity. Congregation size was not correlated with the number of challenges ($r=-0.04$). The Project's TA teams used the results to provide feedback to the church's implementation team and used the feedback interview to plan and tailor their approach to supporting each congregation.

Implications for D&I Research

We developed a process for identifying potential challenges in churches implementing EBPs, and providing theory-driven, evidence-based results to church partners. While challenges can be identified during implementation, our approach anticipates these challenges and suggests strategies for overcoming them during the planning stages of program implementation. This approach can be adapted to other community groups attempting to implement a wide range of strategic initiatives.

Primary Funding Source: National Institutes of Health

S156

Advancing equitable access to HIV/HCV testing among people who inject drugs: A mixed-method study of a multicomponent implementation strategy to increase adoption and implementation of opt-out testing at syringe services programs

Tyler Bartholomew¹, Marina Plesons¹, Hansel Tookes²

¹University of Miami Miller School of Medicine, Miami, FL, USA; ²University of Miami, Miami, FL, USA

Correspondence: Tyler Bartholomew (tsb61@miami.edu)

Implementation Science 2026, **21**(2):S156

Background

Improving HIV and Hepatitis C (HCV) testing among people who inject drugs (PWID) remains fundamental in attaining the UNAIDS and WHO goals for 2030. Syringe services programs (SSPs) are ideal venues for reaching PWID with comprehensive testing services. In 2023, The US CDC endorsed opt-out testing for HIV/HCV in the SSP setting. This study examined the impact of a multicomponent implementation strategy – ACCESS – on adoption and implementation of opt-out HIV/HCV testing at two newly sanctioned SSPs in Florida.

Methods

A community-driven implementation mapping approach was used to describe the most significant barriers to HIV/HCV testing within SSPs (financial resources, organizational capacity, staff training, data systems, and technical support) and identify discrete implementation strategies (low-threshold funding and practice facilitation) for the ACCESS strategy. Two SSPs in Florida then participated in a 24-month pilot of ACCESS. Quantitative data on participant-level demographics, HIV/HCV testing uptake, and testing outcomes were obtained from SSP administrative records at 12 and 24-months post-baseline. Qualitative interviews with SSP directors/staff were conducted to ascertain ACCESS's acceptability and feasibility.

Findings

Mean funding amounts provided for each 12-month period was \$75,962 per site. Mean HIV/HCV testing uptake was 70.1% and 69.3%, respectively, among SSP participants ($n=475$). There was significant variation between sites, with Site 1 testing 99.5% and 99.1% of participants for HIV/HCV, respectively, compared to 44.3% and 43.1% at Site 2. Interviews elucidated implementation fidelity issues, with Site 2 highlighting significant staff turnover during the initial 12-month period. Strategy adaptations (monthly check-in meetings and real-time data reports) were applied, after which HIV/HCV testing uptake

at Site 2 significantly increased to 74.8% and 64.8%, respectively, at 24 months.

Implications for D&I Research

Our ACCESS strategy (low threshold funding and practice facilitation) was acceptable, feasible, and showed preliminary efficacy within the SSP context to influence adoption and sustainment of opt-out HIV/HCV testing.

Primary Funding Source: GILEAD Sciences

S157

Implementation strategies and adjunctive interventions for HIV testing and linkage-to-care in the United States: a systematic review

James Merle¹, Amelia Van Pelt², Olutobi Sanuade³, Timothy Levensgood⁴, McKenzie Swan⁵, Ana Michaela Pachicano⁶, Alithia Zamantakis², J.D.

Smith¹, Virginia McKay⁷, Dennis Li², Brian Mustanski⁶, Nanette Benbow²

¹University of Utah, Salt Lake City, UT, USA; ²Northwestern University Fein-

berg School of Medicine, Chicago, IL, USA; ³University of Utah Spencer

Fox Eccles School of Medicine, Salt Lake City, UT, USA; ⁴Boston Univer-

sity School of Public Health, Boston, MA, USA; ⁵Washington University

in St. Louis, Saint Louis, USA; ⁶Northwestern University, Chicago, IL, USA;

⁷Center for Public Health Systems Science, Brown School, Washington

University in Saint Louis, St. Louis, MO, USA

Correspondence: James Merle (james.merle@utah.edu)

Implementation Science 2026, **21**(2):S157

Background: Despite advances in HIV prevention, over 30,000 new HIV diagnoses occur annually in the U.S., with many individuals unaware of their status. The Ending the HIV Epidemic initiative aims to reduce new infections to 3,000 per year by 2030. Achieving this goal requires effective implementation of HIV testing and linkage-to-care (LTC) strategies. However, the field lacks a comprehensive synthesis of implementation strategies and adjunctive interventions that support these efforts.

Methods: We conducted a systematic review following PRISMA guidelines to identify U.S.-based studies evaluating implementation strategies (targeting system- or provider-level change) or adjunctive interventions (targeting individual-level behavior change) for HIV testing/LTC. Searches were conducted across MEDLINE, PsycINFO, and Web of Science, with updates through June 2024. Studies were screened and coded using established frameworks (ERIC taxonomy for implementation strategies and the Theoretical Domains Framework (TDF) and COM-B model for adjunctive interventions). Descriptive statistics and post-hoc analyses were used to assess strategy use and effectiveness.

Findings: From 190 included studies, we identified 756 discrete implementation strategies. The most common ERIC domains were "Train and Educate Stakeholders" and "Change Infrastructure." Reach was the most frequently measured outcome (88%), followed by adoption (35%). Few studies conducted an experimental design (8%). Among 93 studies eligible for effectiveness coding, 74% reported positive effects. Moreover, 39 adjunctive interventions were mapped to TDF and COM-B domains, with most targeting motivation and capability; however, few included implementation strategies to support adjunctive intervention delivery.

Implications for D&I Research: This review catalogs the breadth of strategies used to promote HIV testing/LTC. Effective strategies included integrating HIV testing/LTC programs into new settings and utilizing peer and social network strategies. A persistent gap in rigorous strategy evaluation emphasizes the need for more rigorous study designs and use of D&I frameworks, such as the Implementation Research Logic Model to ensure that change methods match their target level and the Longitudinal Implementation Strategy Tracking System to improve reporting, transparency, and reproducibility. The integration of behavioral frameworks like COM-B can enhance the design of adjunctive interventions, however, separate strategies are needed to implement adjunctive interventions. These insights guide future implementation research and practice, supporting more effective and equitable HIV prevention efforts.

Primary Funding Source: Centers for Disease Control and Prevention

S158

Utility of PrEP training in Title X-funded safety-net family planning clinics for increasing HIV testing and PrEP reach among women in Atlanta: lessons learned from provider focus groups

Jessica Sales¹, Katherine Anderson², Deja Er³, Peyton Williams⁴, Sara Sullivan⁴, Michael Brooks⁴, Melissa Kottke⁵, Anandi Sheth⁶

¹Emory University, Rollins School of Public Health, Atlanta, GA, USA;

²Emory University Rollins School of Public Health, Atlanta, GA, USA;

³Emory University, Atlanta, GA, USA; ⁴The Family Health Centers of Georgia, Inc., Georgia Family Planning System, Atlanta, GA, USA; ⁵Emory University School of Medicine, Atlanta, GA, USA; ⁶Emory School of Medicine, Atlanta, GA, USA

Correspondence: Jessica Sales (jmcdern@emory.edu)

Implementation Science 2026, **21**(2):S158

Background: PrEP remains underutilized by women in the US, in part, because PrEP is not widely available in women's health care settings. Based on formative findings, we developed a bundle of tailored implementation strategies (trainings, technical assistance, resources) to support PrEP delivery in Title X-funded family planning clinics across Atlanta, Georgia (GA) – a high priority jurisdiction for the Ending the HIV Epidemic initiative. Guided by the Interactive Systems Framework, we partnered with GAs Title X grant holder to provide these strategies to this clinical network. We conducted a comprehensive evaluation of Title X clinic strategy use and describe pre- and post-strategy engagement PrEP practices.

Methods: Between January 2022–November 2023, 47 staff, providers and clinic leadership from 11 clinical groups representing 22 of 31 Atlanta Title X clinics (71%) participated in baseline (pre-training) focus groups (FGs). 21 trainings were provided to the 11 clinical groups ($n=415$ participants). Ten groups participated in FGs 1-year post-training(s). Rapid qualitative analysis of FGs was performed; themes describe PrEP delivery landscape at baseline and 1-year follow-up, utility of training and remaining barriers to PrEP care. We are currently analyzing clinic-level aggregate HIV testing and PrEP prescribing pre- and post-strategy engagement data; results will be added to the presentation.

Findings: At baseline, few clinics (< 5; 22%) were providing PrEP. After training, seven clinical groups were offering PrEP, while one offered referrals to a PrEP-specific clinic. Three clinics were able to provide injectable PrEP, though noted challenges navigating insurance barriers. Two clinical groups were not offering PrEP comprehensively/consistently, citing patient barriers and capacity limitations due to staff turnover. Overall, clinics reported trainings helped increase their readiness/capacity to provide PrEP. Particularly beneficial trainings included sexual history taking, creation of safe spaces/cultural sensitivity, and the overview of PrEP care. Five clinical groups noted “refresher” trainings would be beneficial because of staff turnover; four expressed a desire for additional trainings.

Implications for D&I Research: The implementation strategies were utilized by Atlanta Title X clinics. Brief, tailored trainings can improve PrEP provision in safety-net family planning settings and sustainable access to them via a centralized agency is necessary to overcome ongoing capacity-related barriers.

Primary Funding Source: National Institutes of Health

S159

Co-designing a model for home delivery of long-acting injectable pre-exposure prophylaxis in Chicago

Amelia Van Pelt^{1,2}, Elizabeth Casline^{1,2}, Byrd G. M. Cook¹, Jorge Cestou³, Brian Mustanski^{2,4}, Gregory Phillips^{1,4}, Rinad Beidas¹

¹Northwestern University, Chicago, USA; ²Northwestern University

Feinberg School of Medicine, Chicago, IL, USA; ³Chicago Department

of Public Health, Chicago, USA; ⁴Northwestern University, Chicago, IL, USA

Correspondence: Amelia Van Pelt (amelia.vanpelt@northwestern.edu)

Implementation Science 2026, **21**(2):S159

Background: Participatory design can center the voice of recipients and improve operationalization of implementation strategies. To maximize uptake of a new form of long-acting, injectable PrEP (CAB-LA), co-designing implementation strategies is critical. A recent innovation tournament in Chicago crowdsourced ideas from the community on CAB-LA implementation and selected delivery of CAB-LA in the home as the top strategy. Through a partnership with key constituents, this research aimed to operationalize this approach for sexual minority men (SMM) (≥ 13 years) through user-centered design (UCD).

Methods: The UCD process involved two steps: 1) workshop to draft the home-based model and 2) semi-structured interviews to iterate the model. First, transdisciplinary constituents (e.g., clinicians, public health leadership, community-based organizations, implementation scientists, SMM) convened in an in-person four-hour meeting to outline a blueprint for the home-based delivery model. Second, virtual semi-structured interviews were conducted to elicit additional feedback from workshop participants. Rapid qualitative analysis was completed to finalize the model.

Findings: Nineteen constituents participated in the workshop. Key phases of the model included: pre-implementation (e.g., attainment of buy-in, procurement of shots, financing), implementation (e.g., enrollment of patients, identification of staff, scheduling, administration in homes), and maintenance (e.g., post-administration/re-engagement). Patient actions focused on indicating preferences for visit, preparing home for visit, and scheduling follow-up care. Deliverer actions emphasized prescribing CAB-LA, sharing financing information, and administration and at-home procedures. Ten workshop participants completed follow-up interviews. Constituents suggested additional considerations for clarification (e.g., specification of lab requirements) and potential barriers to implementation given change in context (e.g., stigma toward SMM, concerns visiting homes).

Implications for D&I Research: This research produced a blueprint for a co-designed model of “low-barrier PrEP” which can guide plans for CAB-LA integration in Chicago. Future efforts will test the success of this model on increasing the uptake of CAB-LA among SMM. Further, the approach to utilize UCD to generate a community-informed strategy can provide a framework for participatory approaches in implementation science. Given that the co-design of implementation efforts often does not involve the participation of individuals with lived experience, this work centered the voices of those who will benefit most.

Primary Funding Source: National Institutes of Health

S160

Sustaining the positive impacts of Physically Active Children in Education (PACE): a cluster randomised controlled trial

Alix Hall¹, Adam Shoesmith¹, Luke Wolfenden², Edward Riley-Gibson¹, William Pascoe¹, Rebecca Jackson², Nicole McCarthy², Adrian Bauman³, Rachel Sutherland², James Boyer⁴, Bernadette Duggan⁵, Nicole Nathan²

¹University of Newcastle, Callaghan, NSW, Australia; ²Hunter New England

Population Health, Wallsend, NSW, Australia; ³University of Sydney, Syd-

ney, NSW, Australia; ⁴New South Wales Department of Education, Turrella,

NSW, Australia; ⁵Catholic Schools Office Maitland Newcastle, Newcastle

West, NSW, Australia

Correspondence: Alix Hall (Alix.Ivers@health.nsw.gov.au)

Implementation Science 2026, **21**(2):S160

Background: Despite policies that mandate schools deliver a minimum amount of physical activity (PA) per week, many schools find it difficult to meet such requirements. We have developed an effective, award winning, multicomponent implementation intervention, *Physically Active Children in Education (PACE)*, to help teacher's increase their delivery of PA across the school week. However, evidence suggests that following the withdrawal of active implementation support, less than 20% of health promotion programs continue to be delivered (i.e., sustained). To ensure the positive benefits of PACE continue, we co-designed and evaluated a multi-component sustainment strategy to support teachers to continue implementing a school PA policy 12 months after initial adoption.

Methods: We conducted a clustered randomised controlled trial with 25 primary schools across four health jurisdictions in New South Wales, Australia. Schools were randomised to receive a multi-component sustainability strategy or to usual care. The sustainability strategy included formal Principal commitment, in-school champions, prompts and reminders, resource packs, school recognition materials, and parent-facing communication tools. Minutes of PA scheduled by classroom teachers was collected via completion of a daily activity logbook over one school week. Linear mixed models were used to assess between group differences in the change of teacher's scheduling of PA from baseline to 12-month follow-up. Approval was obtained from Hunter New England and University of Newcastle Human Research Ethics Committees.

Findings: 100 teachers at baseline and 81 at 12-month follow-up, provided valid scheduling data. From baseline to 12-month follow-up, a significant difference in the change of PA scheduled by intervention and control teachers was observed ($p = 0.002$). Specifically, preliminary data indicates intervention teachers increased their scheduling by ~15 min per week while control teachers decreased their scheduling by ~21 min per week.

Implications for D&I Research: This is one of the first trials globally to test a strategy to sustain a school PA policy. Findings show that a co-designed, multi-component strategy, developed based on Implementation Science principles, can support schools to sustain their delivery of PA over time. Continued evaluation at 24 months will determine whether these effects are maintained long term.

Primary Funding Source: NHMRC (National Health & Medical Research Council)

S161

Enhancing fidelity in school-based prevention: results from a randomized controlled trial of getting to outcomes to support Project ALERT implementation by middle school teachers

Matthew Chinman^{1,2}, Andrea Phillips³, Jill Cannon⁴, Michelle Bongard⁴, Jacobo Pereira-Pacheco³, Courtney Kase²

¹Veterans Health Administration, Pittsburgh, PA, USA; ²RAND Corporation, Pittsburgh, PA, USA; ³RAND CORPORATION, Santa Monica, CA, USA; ⁴RAND, Santa Monica, CA, USA

Correspondence: Matthew Chinman (chinman@rand.org)

Implementation Science 2026, **21**(2):S161

Background: It is critical to conduct substance use prevention with middle school youth to prevent risky use of alcohol, marijuana, nicotine, and misuse of prescription medication in high school. Project ALERT (PA) is an 11-session evidence-based prevention program (EBPP) targeting all 7th graders that is widely adopted, cost-free, and accessible (online teacher training). Data show, like with many substance use EBPPs, PA is often poorly implemented (e.g., truncated or omitted lessons), undermining its effectiveness. Getting To Outcomes (GTO)—an implementation strategy bundle of coaching, training, and tools that stimulate quality improvement—improved EBPP fidelity in community-based organizations delivering programs to middle school youth. This is the first test of GTO with an EBPP in the school context.

Methods: As part of a larger RCT evaluating PA and GTO, we assessed the fidelity of PA implementation comparing middle school teachers randomly assigned to implement Project ALERT on their own ($N = 9$) to teachers assigned to receive GTO-based support ($N = 8$). In two successive school years for each teacher, research staff rated a subset of PA sessions (PA-alone: $N_{Y1} = 29$; $N_{Y2} = 22$; PA + GTO: $N_{Y1} = 24$; $N_{Y2} = 21$) for adherence to session activities (0 = none, 1 = partial, 2 = full). On a 1 = least to 5 = most scale, ratings were also made on each teacher's pace, engagement effort, class management, enthusiasm, empathy, and on students' engagement and interest. We conducted linear mixed-effects models, accounting for repeated teacher observations, to test for fidelity differences between groups and across years. We also interviewed the GTO teachers about their experience after their second year and extracted common themes.

Findings: In Year 1, GTO teachers' adherence ratings were significantly higher than PA-alone, and then significantly improved further in Year 2. In Year 2, teacher pace, engagement effort, management, empathy, and student interest were significantly higher for GTO teachers than PA-alone, and management and student interest also significantly improved for GTO teachers from Year 1. In interviews, teachers said GTO helped them plan and deliver lessons, set goals, and reflect on lesson delivery to improve future lessons.

Implications for D&I Research: This study demonstrates the value of implementation strategies like GTO for bridging the gap between EBPP availability and effective delivery in real-world educational contexts.

Primary Funding Source: National Institutes of Health

S162

Prevention of parental substance use and mental health disorders by intervening on social risk factors: implementation of a continuum of care prevention program in child welfare settings

Lisa Saldana¹, Gracelyn Cruden¹, Jeffrey Peterson², Ryan Singh¹, Maria Bybee³

¹Chestnut Health Systems-Lighthouse Institute, Eugene, OR, USA; ²Chestnut Health Systems-Lighthouse Institute, Eugene, OR, USA; ³Chestnut Health Systems—Lighthouse Institute, Eugene, OR, USA

Correspondence: Lisa Saldana (lsaldana@chestnut.org)

Implementation Science 2026, **21**(2):S162

Background

Parents with opioid and/or methamphetamine use (O/MU) and mental health (MH) disorders have a high likelihood of adverse social risk factors in all five areas identified by Healthy People 2030 as being essential for healthy development and quality of life. Just Care for Families (JCFF) is an emerging practice developed for parents along a continuum of preventive care need (primary to tertiary) involved with, or at-risk for involvement with, the child welfare system (CWS). In partnership with Oregon's CWS, JCFF is being implemented in five rural counties.

Methods

JCFF was developed out of an identified need, from a different hybrid trial, for a continuum of care prevention model. A multi-level implementation roadmap was followed and measured implementation process fidelity. Two of the counties self-initiated JCFF implementation, after their respective agencies successfully implemented in other counties in the first trial.

While parent data collection was designed for hybrid trial research, it now has been integrated into practice to inform clinical care. Parents are assessed weekly for 18 months, via a survey received by text, email, or phone about the social risk factors they experience as well as O/MU and MH outcomes. Survey questions span all five categories of social risk.

Findings

All counties have implemented JCFF and sustained for two years post-launch. To date, 192 of a targeted 250 parents have consented to participate. Most parents elect to receive their weekly survey via text (76%) versus email (24%) or phone ($n = 1$) and most (74%) have completed 85–100% of the weeks. Preliminary analyses show a significant change in social risk factor needs and clinical outcomes, in the first 6-months. In collaboration with CWS, expansion into four more counties, following the same implementation roadmap, is planned to commence this Fall.

Implications for D&I Research

Through collaboration with public systems, such as CW, implementation of preventive interventions can successfully reach a range of parents, along a continuum of care need, to address public health crises such as O/MU and MH disorders. Following a well-defined implementation roadmap, agencies can learn how to implement well and replicate in other regions, allowing the potential for system-wide scale-up and public health impact.

Primary Funding Source: National Institutes of Health

S163

Predicting implementation fidelity across two statewide school mental health initiatives: findings from REACH and RSSI in IllinoisDavid W. Johnson¹, Grace Daley², Beatriz Cabello¹, Danielle Hatchimonji¹, Mashana Smith¹, Colleen Cicchetti¹¹Lurie Children's Hospital of Chicago, Chicago, IL, USA; ²Center for Childhood Resilience, Ann & Robert H. Lurie Children's, Chicago, IL, USA**Correspondence:** David W. Johnson (dajohnson@luriechildrens.org)*Implementation Science* 2026, **21**(2):S163

Background: Previous research attests to the important role that schools play in addressing the unmet need for youth mental health services; however, more research is needed to understand school-level predictors of engagement in initiatives supporting the implementation of evidence-based mental health supports. The Resilience Education to Advance Community Healing (REACH) initiative and its successor, Resilience Supportive Schools Illinois (RSSI), represent statewide efforts to increase schools' capacity to implement trauma-responsive, equitable, and culturally relevant strategies for supporting student mental health and resilience. Both initiatives use structured processes and statewide data tools to guide needs assessment, action planning, and continuous improvement. To support more effective dissemination, implementation, and scale-up strategies, it is critical to identify what school-level characteristics are associated with participation and completion of key implementation milestones. This reflects a core question in hybrid implementation-effectiveness research: understanding organizational and contextual factors that influence implementation success alongside downstream outcomes.

Methods: We conducted multivariate regression analyses using two datasets: (1) REACH data from 2019–2024 ($n = 1,169$ schools) and (2) RSSI pilot data from the 2024–25 school year ($n = 400$ schools). For both initiatives, we examined school-level predictors of two outcomes: participation in REACH or RSSI and completion of key implementation milestones, including needs assessment, action plan development, and engagement with asynchronous online professional learning. Predictor variables include measures of school context and characteristics from publicly available administrative data.

Findings: School participation in REACH and completion of key milestones are associated with improved academic and workforce outcomes, including increased teacher retention, reductions in student chronic absenteeism, and reductions in out-of-school suspensions. Preliminary findings indicate that participation in the RSSI pilot is associated with reductions in chronic absenteeism. The current study tests hypotheses regarding school-level characteristics associated with both participation and engagement in REACH and RSSI, including school size, grade level composition, Title I status, student demographics, urbanicity, geographic region, and adversity index score.

Implications for D&I Research: This study advances dissemination and implementation science by identifying predictors of uptake and engagement with school mental health capacity-building initiatives. Findings contribute actionable insights for scaling systems-level, trauma-responsive, and equity-focused models across diverse school contexts.

Primary Funding Source: Blended funding

S164

Advancing preventive care through Medicaid policy innovation: Florida's cancer screening incentive initiativeJaclyn Hall¹, Rahma Mkuu¹, Peyton Lurk², Curtis Warren³, Belal Ghaffari³, Christopher Cogle⁴¹University of Florida, Gainesville, FL, USA; ²Florida State University, Tallahassee, FL, USA; ³Knowli Data Science, Tallahassee, FL, USA; ⁴Florida Medicaid, Gainesville, FL, USA**Correspondence:** Jaclyn Hall (Jaclynha@ufl.edu)*Implementation Science* 2026, **21**(2):S164

Background: The COVID-19 pandemic caused a nationwide disruption in preventive services, including an estimated 3.8 million missed colorectal cancer (CRC) screenings in 2020. To address this screening gap among vulnerable populations, Florida Medicaid implemented a financial incentive for managed care plans: *Alleviation of Liquidated Damages* if $\geq 50\%$ of eligible enrollees completed CRC screening between October 1, 2022, and June 30, 2023. This novel statewide initiative leveraged administrative claims data, health plan-specific screening metrics, public-private partnerships, and bi-weekly cross-sector meetings with state officials, health plans, and external stakeholders to increase screening utilization.

Methods: This quasi-experimental pre-post policy evaluation used Medicaid claims data to identify adults aged 45–75 (by October 1st in each period) continuously enrolled for 12 previous months and not up-to-date with colorectal cancer screening guidelines, excluding those with frailty or advanced illness. During bi-weekly meetings Plans shared practice-level screening rates, incentivized physicians to achieve screening targets, contacted enrollees, and hosted community awareness events. Medicaid claims data were analyzed for three cohorts: a pre-initiative year, pilot year, and post-initiative year. Secondary administrative data were used to compare colorectal cancer screening rates before and during the initiative, with descriptive statistics and two-sided chi-squared tests ($P < 0.05$) used to assess differences.

Findings: During the initiative, the statewide CRC screening rate among Medicaid enrollees increased to 51.3%, up from 46.9% in the prior year, equating to an estimated 6267 additional patients screened, with the largest gains seen in those aged 45–65. The total number of colonoscopies and stool-based screenings increased in the subsequent year, yet, due to an increase in the number of enrollees eligible, the rate of those screened dropped to 45.1%. Notably, the use of noninvasive stool-DNA testing (Cologuard) grew in all age groups from 1.3% of total tests pre-initiative to 7.4% post-initiative and contributed to an increase in follow-up screenings.

Implications for D&I Research: This case highlights how state-level financial incentives along with health plan level operational changes catalyzed large-scale improvements in preventive care. Financial incentives should be integrated with accessible screening modalities and upstream plan level drivers of access and utilization to sustain gains, increase follow-up screenings, and reduce costs beyond the intervention period.

S165

Comparing the cost of harm reduction vending machine implementation models in the HEALing Communities Study: the New York State experienceJazmine Li¹, Aurora Yuan², James David³, Nabila El-Bassel⁴, Bruce Schackman⁵, Laura Starbird⁶¹Weill Cornell Medicine, New York, NY, USA; ²University of Pennsylvania, Philadelphia, PA, USA; ³Columbia University, New York, NY, USA; ⁴Columbia University, New York, USA; ⁵Weill Cornell Medical College, New York, NY, USA; ⁶University of Pennsylvania School of Nursing, Philadelphia, PA, USA**Correspondence:** Laura Starbird (starbird@nursing.upenn.edu)*Implementation Science* 2026, **21**(2):S165**Background**

Harm reduction vending machines provide free and anonymous access to supplies such as naloxone, fentanyl and xylazine test strips, and safe sex supplies. They facilitate community access to products that lower the risk of opioid-related morbidity and mortality. We identified different approaches to implement harm reduction vending machines that were selected by communities in New York State (NYS) and estimated the cost of each approach.

Methods

We collected invoice, tracking, and key informant interview data from five communities in NYS that participated in Wave 2 of the HEALing Communities Study in NYS and installed harm reduction vending machines during the intervention period (2022–2023). We classified the planning, installation, and maintenance strategies employed by each community and conducted an activity-based micro-costing

analysis to estimate the one-time start-up and ongoing implementation cost of each harm reduction vending machine strategy.

Findings

Across the five communities, we identified three unique community-selected approaches to implementing harm reduction vending machines: 1) electronic vending machines with tracking software; 2) refurbished vending machines; and 3) naloxone housing unit-adjacent implementation (e.g., newspaper box or free-vend machine like a restroom tampon dispenser). The two communities that installed electronic vending machines with tracking software incurred a higher cost over a 1-year period per machine (\$3,816 and \$1,863) compared with the community that chose a refurbished vending machine (\$1,098). Electronic vending machines also incurred the highest one-time cost per machine (\$19,808 and \$16,620) compared to installing a refurbished vending machine (\$11,626). The two communities that selected a naloxone housing unit-adjacent approach incurred the lowest 1-year implementation cost per machine (\$498 and \$328). These two communities also had the lowest one-time costs per machine (\$2,452 and \$6,828).

Implications for D&I Research

Harm reduction vending machines provide an opportunity for communities to offer publicly available naloxone and other safe drug use equipment to reduce fatal overdoses. Depending on a community's resources, we offer multiple implementation options for harm reduction vending machines that range in costs, maintenance needs, and stock. The wide range of costs depending on the implementation approach allows stakeholders to choose the strategy that fits their budget, time, and personnel needs, with considerations for scalability and sustainability.

Primary Funding Source: National Institutes of Health

S166

Is flexibility in decision-making a catalyst for innovation adoption? Rethinking organizational drivers of implementation during public health crises

Sapna Mendon-Plasek¹, Gabriel Hassler², Carl Berdahl³, Christina Huber⁴, Catherine Cohen², Peter Mendel⁵, Shira H. Fischer¹

¹RAND Corporation, Boston, MA, USA; ²RAND, Santa Monica, CA, USA;

³Cedars-Sinai Medicine, Los Angeles, CA, USA; ⁴University of California, Los Angeles, Los Angeles, USA; ⁵RAND Corporation, Santa Monica, CA, USA

Correspondence: Sapna Mendon-Plasek (smendon@rand.org)

Implementation Science 2026, **21**(2):S166

Background

During 2020, over 100,000 patients were hospitalized for acute COVID-19, most through emergency departments (EDs). Rapidly evolving infection control challenges demanded adaptations to protocols to combat shortages in resources and to optimize treatments. Although positive leadership support, learning climate, and collaborations are known to promote adoption of interventions during steady state conditions, their role in facilitating implementation of innovations during public health crises is less clear. This study examines how organizational context and clinician attitudes influence the adoption to use innovations in ED settings.

Methods

Between February and September 2024, we conducted a nationally representative survey of 1,556 ED clinicians (MDs and RNs). The survey, based on focus groups with clinicians in 8 EDs across the US, assessed organizational characteristics and use of innovations. We examined adoption of three purposively selected innovations during the COVID-19 pandemic: personal protective equipment (PPE) reuse, high flow oxygen (HFO) to treat hypoxia, and ivermectin prescribing. For each innovation, we fit a mediation model measuring the effects of learning climate, policy dissemination, leadership support, and collaboration on innovation adoption, with flexibility in decision-making as a mediator.

Findings

PPE reuse had the most adoption (93%) compared to HFO (80%) and ivermectin (6.6%). The mediation model revealed significant

mediation effects for MDs: unit increases in learning climate and collaboration resulted in 1.7% and 1.2% increases in HFO use via their effects on flexibility. Unit increases in leadership support resulted in 0.6% decreases in PPE reuse mediated through flexibility. We observed no significant effects for nurses. Direct effects of flexibility on adoption were much greater, with unit increases in flexibility significantly associated with 13.8% (MDs) and 16.8% (RNs) increases in HFO, 4.3% (MDs) and 9.1% (RNs) decreases in PPE reuse, and 3.9% (RNs) increases in ivermectin use.

Implications for D&I Research

These unexpected findings challenge the established view that organizational factors like climate and leadership support drive intervention adoption, suggesting these constructs may be less relevant during public health emergencies. Rapidly evolving crises require real-time support and decision-making; implementation science methods and frameworks give us insights into what is relevant to focus on during public health emergencies and can inform strategies to improve adoption.

Primary Funding Source: National Institutes of Health

Publisher's Note

Springer Nature remains neutral with regard to jurisdictional claims in published maps and institutional affiliations.

